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**Child Care and Development Fund (CCDF) Plan
for
State/Territory Maine**

FFY 2025 – 2027

Version: Amendment 1

Plan Status: Approved as of 2025-05-27 21:04:29 GMT

This Plan describes the Child Care and Development Fund program to be administered by the State or Territory for the period from 10/01/2024 to 9/30/2027, as provided for in the applicable statutes and regulations. The Lead Agency has the flexibility to modify this program at any time, including amending the options selected or described.

For purposes of simplicity and clarity, the specific provisions of applicable laws printed herein are sometimes paraphrases of, or excerpts and incomplete quotations from, the full text. The Lead Agency acknowledges its responsibility to adhere to the applicable laws regardless of these modifications.

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Overview

Introduction

The Child Care and Development Block Grant Act (CCDBG) (42 U.S.C. 9857 *et seq.*), together with section 418 of the Social Security Act (42 U.S.C. 618), authorize the Child Care and Development Fund (CCDF), the primary federal funding source devoted to supporting families with low incomes afford child care and increasing the quality of child care for all children. The CCDF program is administered by the Office of Child Care (OCC) within the Administration for Children and Families (ACF) at the U.S. Department of Health and Human Services and provides resources to State, Territory, and Tribal governments via their designated CCDF Lead Agency.

CCDF plays a vital role in supporting family well-being and child development; facilitating parental employment, training, and education; improving the economic well-being of participating families; and promoting safe high-quality care and learning environments for children when out of their parents' care.

As required by CCDBG, this CCDF Plan serves as the State/Territory Lead Agency's application for a three-year cycle of CCDF funds and is the primary mechanism OCC uses to determine Lead Agency compliance with the requirements of the statute and regulations. CCDF Lead Agencies must comply with the rules set forth in CCDBG and corresponding ACF-issued rules and regulations. The CCDF Plan is a fundamental part of OCC's oversight of CCDF and is designed to align with and complement other oversight mechanisms including administrative and financial data reporting, the monitoring process, error rate reporting, audits, and the annual Quality Progress Report.

Organization of Plan

In their CCDF Plans, State/Territory Lead Agencies must describe how they implement the CCDF program. The Plan is organized into the following sections:

1. CCDF Program Administration
2. Child and Family Eligibility and Enrollment and Continuity of Care
3. Child Care Affordability
4. Parental Choice, Equal Access, Payment Rates, and Payment Practices
5. Health and Safety of Child Care Settings
6. Support for a Skilled, Qualified, and Compensated Child Care Workforce
7. Quality Improvement Activities
8. Lead Agency Coordination and Partnerships to Support Service Delivery
9. Family Outreach and Consumer Education
10. Program Integrity and Accountability

Completing the Plan

This revised Plan aims to capture the most accurate and up-to-date information about how a State/Territory is implementing its CCDF program in compliance with the requirements of CCDF. In responding to plan questions, Lead Agencies should provide concise and specific summaries and/or bullet points as appropriate to the question. Do not insert tables or charts, add attachments, or copy manuals into the Plan. A State/Territory's CCDF Plan is intended to stand on its own with sufficient information to describe how the Lead Agency is implementing its CCDF program without need for added attachments, tables, charts, or State manuals.

OCC recognizes that Lead Agencies use different mechanisms to establish CCDF policies, such as State statute, regulations, administrative rules, policy manuals, or policy issuances. Lead Agencies must submit their CCDF Plan no later than July 1, 2024.

Review and Amendment Process

OCC will review submitted CCDF Plans for completeness and compliance with federal policies. Each Lead Agency will receive a letter approximately 90 days after the Plan is due that includes all Plan non-compliances to be addressed. OCC recognizes that Lead Agencies continue to modify and adapt their programs to address evolving needs and priorities. Lead Agencies must submit amendments to their Plans as they make substantial policy and program changes during the three-year plan cycle, including when addressing non-compliances.

Appendix 1: Implementation Plan

As part of the Plan review process, if OCC identifies any CCDF requirements that are not fully implemented, OCC will communicate a preliminary notice of non-compliance for those requirements via an emailed letter. OCC has created a standardized template for Lead Agencies to submit as their 60-day response to that preliminary notice. This template is found at Appendix 1: Lead Agency Implementation Plan. This required response via the Appendix will help create a shared understanding between OCC and the Lead Agency on which elements of a requirement are unmet, how they are unmet, and the Lead Agency's steps and associated timelines needed to fully implement those unmet elements.

CCDF Plan Submission

CCDF Lead Agencies will submit their Plans electronically through the Child Care Automated Reporting System (CARS). CARS will include all language and questions included in the final CCDF Plan template approved by the Office of Management and Budget (OMB). Note that the format of the questions in CARS could be modified from the Word version of the document to ensure compliance with Section 508 policies regarding accessibility to electronic and information technology for individuals with disabilities.

1 CCDF Program Administration

Strong organizational structures, operational capacity, and partnerships position States and Territories to administer CCDF efficiently, effectively, and collaboratively.

This section identifies the CCDF Lead Agency, CCDF Lead Agency leadership, and the entities and individuals who will participate in the implementation of the program. It also identifies the partners who were consulted to develop the Plan.

1.1 CCDF Leadership

The governor of a State or Territory must designate an agency (which may be an appropriate collaborative agency) or establish a joint interagency office to represent the State or Territory as the Lead Agency. The Lead Agency agrees to administer the program in accordance with applicable federal laws and regulations and the provisions of this Plan, including the assurances and certifications.

1.1.1 Designated Lead Agency

Identify the Lead Agency or joint interagency office designated by the State or Territory. OCC will send official grant correspondence, such as grant awards, grant adjustments, Plan approvals, and disallowance notifications, to the designated contact identified here.

- a. Lead Agency or Joint Interagency Office Information:
 - i. Name of Lead Agency: **Department of Health and Human Services**
 - ii. Street Address: **11 State House Station**
 - iii. City: **Augusta**
 - iv. State: **Maine**
 - v. ZIP Code: **04333**
 - vi. Web Address for Lead Agency: **<https://www.maine.gov/dhhs/>**
- b. Lead Agency or Joint Interagency Official contact information:
 - i. Lead Agency Official First Name: **Sara**
 - ii. Lead Agency Official Last Name: **Gagne-Holmes**
 - iii. Title: **Commissioner**
 - iv. Phone Number: **(207) 287-4223**
 - v. Email Address: **sara.gagne-holmes@maine.gov**

1.1.2 CCDF Administrator

Identify the CCDF Administrator designated by the Lead Agency, the day-to-day contact, or the person with responsibility for administering the State's or Territory's CCDF program. The OCC will send programmatic communications, such as program announcements, program instructions, and data collection instructions, to the designated contact identified here. If there is more than one designated contact with equal or shared responsibility for administering the CCDF program, identify the Co-Administrator or the person with administrative responsibilities and include their contact information.

- a. CCDF Administrator contact information:
 - i. CCDF Administrator First Name: **Gina**

- ii. CCDF Administrator Last Name: **Forbes**
- iii. Title of the CCDF Administrator: **Child Care Services Manager**
- iv. Phone Number: **(207) 592-0865**
- v. Email Address: **gina.forbes@maine.gov**
- b. CCDF Co-Administrator contact information (if applicable):
 - i. CCDF Co-Administrator First Name: **Tara**
 - ii. CCDF Co-Administrator Last Name: **Williams**
 - iii. Title of the CCDF Co-Administrator: **Associate Director of Early Care and Education**
 - iv. Phone Number: **207-557-2342**
 - v. Email Address: **tara.williams@maine.gov**
 - vi. Description of the Role of the Co-Administrator: **The Co-Administrator oversees CCDF funds, supports equal access provisions of the CCDF state plan, and coordinates child care workforce and quality improvement activities. The Co-Administrator helps manage implementation of some CCDF services (workforce, quality improvement), collaborates with staff at Maine DOE (PreK, children with special needs), writes the Quality Progress Report in collaboration with Child Care Services Manager, supports oversight of the spending of funds by contractors, supports program and service monitoring, and supports the development of the CCDF Plan every three years.**

1.2 CCDF Policy Decision Authority

The Lead Agency has broad authority to administer (i.e., establish rules) and operate (i.e., implement activities) the CCDF program through other governmental, non-governmental, or public or private local agencies as long as the Lead Agency retains overall responsibility for the administration of the program. Administrative and implementation responsibilities undertaken by agencies other than the Lead Agency must be governed by written agreements that specify the mutual roles and responsibilities of the Lead Agency and other agencies in meeting the program requirements.

1.2.1 Entity establishing CCDF program rules

Which of the following CCDF program rules and policies are administered (i.e., set or established) at the State or Territory level or local level? Identify whether CCDF program rules and policies are established by the State or Territory (even if operated locally) or whether the CCDF policies or rules are established by local entities, such as counties or workforce boards.

Check one of the following:

- a. ☒ All program rules and policies are set or established by the State or Territory. (If checked, skip to question 1.2.2.)
- b. ☐ Some or all program rules and policies are set or established by local entities or agencies. If checked, indicate which entities establish the following policies. Check all that apply:

- i. Eligibility rules and policies (e.g., income limits) are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- ii. Sliding-fee scale is set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- iii. Payment rates and payment policies are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- iv. Licensing standards and processes are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- v. Standards and monitoring processes for license-exempt providers are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- vi. Quality improvement activities, including QIS, are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- vii. Other. List and describe any other program rules and policies that are set at a level other than the State or Territory level:

1.2.2 Entities implementing CCDF services

The Lead Agency has broad authority to operate (i.e., implement activities) through other agencies, as long as it retains overall responsibility for CCDF. Complete the table below to identify which entity(ies) implements or performs CCDF services.

Check the box(es) to indicate which entity(ies) implement or perform CCDF services.

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who conducts eligibility determinations?	☒	☐	☐	☐	☐ Describe:

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who assists parents in locating child care (consumer education)?	[x]	[]	[]	[]	[x] Describe: Maine Roads to Quality (MRTQ)/The Professional Development Network (PDN) is responsible for the Child Care Choices website, a resource for finding consumer education including child care by address, city, or zip code in Maine. The Maine Department of Health and Human Services (DHHS) is partnered with the Opportunity Alliance Contact Center to supply an online directory and resource center with the website 2-1-1 Maine at

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
					(https://211maine.org/) https://211maine.org/ or through text messaging capabilities. DHHS also implements the Help ME Grow (HMG) model as a system for improving access to existing resources and services by linking families those that they need at the community-based level.
Who issues payments?	☒	☐	☐	☐	☐ Describe:
Who monitors licensed providers?	☒	☐	☐	☐	☐ Describe:
Who monitors license-exempt providers?	☒	☐	☐	☐	☐ Describe:

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who operates the quality improvement activities?	[x]	[]	[]	[]	[x] Describe: MRTQ PDN, partners with the Lead Agency (DHHS) on early care and education providers professional development and the Quality Rating and Improvement System (QRIS) in Maine called Rising Stars for ME. MRTQ PDN manages the Rising Stars for ME database and DHHS verifies quality ratings through onsite visits. MRTQ PDN implements Maine's ECE workforce registry. MRTQ PDN also provides early care and education

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
					coaching and consultation virtually and onsite and implements on demand and facilitated trainings. DHHS provides early childhood mental health consultation.

1.2.3 Information systems availability

For any activities performed by agencies other than the Lead Agency as reported above in 1.2.1 and 1.2.2, identify the processes the Lead Agency uses to oversee and monitor CCDF administration and implementation activities to retain overall responsibility for the CCDF program.

Check and describe how the Lead Agency includes in its written agreements the required elements. Note: The contents of the written agreement may vary based on the role the agency is asked to assume or type of project but must include, at a minimum, the elements below.

a. Tasks to be performed.

☒ Yes. If yes, describe: **All DHHS contracts include deliverables to be met in the Rider A.**

☐ No. If no, describe:

b. Schedule for completing tasks.

☒ Yes. If yes, describe: **All DHHS contracts include timelines and due dates for meeting deliverables and reports.**

☐ No. If no, describe:

c. Budget which itemizes categorical expenditures in accordance with CCDF requirements.

☒ Yes. If yes, describe: **All DHHS contracts include a contract budget with itemized categorical expenditures.**

☐ No. If no, describe:

d. Indicators or measures to assess performance of those agencies.

☒ Yes. If yes, describe: **All DHHS contracts include quarterly performance measure reports (PMR)**

that are monitored by the program manager and/or CCDF Administrator.

☐ No. If no, describe:

- e. In addition to the written agreements identified above, describe any other monitoring and auditing processes used to oversee CCDF administration. **The CCDF administrator meets on a regular basis with each contract provider to review contract performance measures. Maine's DHHS Contract Management (DCM) has close oversight of the contract. Any contract over \$1 million goes through the SPRC review. When technology is included in the contract the Department's OIT does a review.**

1.2.4 Certification of shareable information systems.

Does the Lead Agency certify that to the extent practicable and appropriate, any code or software for child care information systems or information technology for which a Lead Agency or other agency expends CCDF funds to develop is made available to other public agencies? This includes public agencies in other States for their use in administering child care or related programs.

☒ Yes.

☐ No. If no, describe:

1.2.5 Confidential and personally identifiable information

Certification of policies to protect confidential and personally identifiable information

Does the Lead Agency certify that it has policies in place related to the use and disclosure of confidential and personally identifiable information about children and families receiving CCDF assistance and child care providers receiving CCDF funds?

☒ Yes.

☐ No. If no, describe:

1.3 Consultation in the Development of the CCDF Plan

The Lead Agency is responsible for developing the CCDF Plan, and consultation with and meaningful input and feedback from a wide range of representatives is critical for CCDF programs to continually adapt to the changing needs of families, child care programs, and the workforce. Consultation involves meeting with or otherwise obtaining input from an appropriate agency in the development of the State or Territory CCDF Plan. As part of the Plan development process, Lead Agencies must consult with the following:

- (1) Appropriate representatives of general-purpose local government. General purpose local governments are defined by the U.S. Census at https://www2.census.gov/govs/cog/g12_org.pdf.
- (2) The State Advisory Council (SAC) on Early Childhood Education and Care (pursuant to 642B(b)(1)(A)(i) of the Head Start Act) or similar coordinating body pursuant to

98.14(a)(1)(vii).

- (3) Tribe(s) or Tribal organization(s) within the State. This consultation should be done in a timely manner and at the option of the Tribe(s) or Tribal organization(s).

1.3.1 Consultation efforts in CCDF Plan development

Describe the Lead Agency's consultation efforts in the development of the CCDF Plan, including how and how often the consultation occurred.

- a. Describe how the Lead Agency consulted with appropriate representatives of general-purpose local government: **The Lead Agency consulted with representatives of general-purpose local government through staff at the Great Portland Council of Governments and Portland City Government. The CCDF administrator had meetings with staff to discuss elements of the state plan and the Department's child care goals and work to support children, families, and the early care and education workforce. Local government staff shared about their work to support local child care programs and families through federal COVID-relief funds and their goals and work ahead.**
- a. Describe how the Lead Agency consulted with the State Advisory Council or similar coordinating body: **The State Advisory Council is the Children's Cabinet Early Childhood Advisory Council (CCECAC). The Lead Agency has representation on the council. Coordination between the Lead Agency's CCDF Plan and the CCECAC's strategic plan, occurs through the ongoing monthly meeting to ensure alignment on increasing access to affordable quality early care and to recruit, prepare, and retain a diverse early childhood workforce. The details of the draft plan were presented to the CCECAC prior to the public comment period for feedback. In addition, input and feedback is coordinated regularly on the market rate survey and the cost modeling work.**
- b. Describe, if applicable, how the Lead Agency consulted with Indian Tribes(s) or Tribal organizations(s) within the State: **Each of the five Maine Tribal CCDF administrators are contacted on a regular basis by the State Administrator with updates to and invitations concerning child care policies and rules in Maine. The five CCDF tribes recognized in Maine are the Aroostook Band of Micmac Indians, the Houlton Band of Maliseet Indians, Penobscot Nation, Passamaquoddy Tribe at Indian Township and Passamaquoddy Tribe at Pleasant Point. Copies of the CCDF Plan Preprint were sent to each of the tribal administrators in March 2024 with an invitation for discussion and recommendations for the State Plan. The Lead Agency met with the CCDF Administrator for the Houlton Band of Maliseet's in late March and corresponded with CCDF administrators from the other tribes. The Lead Agency sent copies of the drafted CCDF State Plan and public hearing notice in May 2024 to each of the tribal administrators. Correspondence followed with the CCDF Administrator for Passamaquoddy Pleasant Point. The Lead Agency did not receive comments or recommendations for the plan from the other tribes at that time. Background on the relationship building between the Lead Agency and Tribes in Maine: In May of 2023, the CCDF Administrator was invited to meet with the five CCDF Tribal Administrators during the Onsite Visit for Tribal Nationals in Maine along with ACF representatives. The goal was to share updates and barriers/successes from all. Communication between individual administrators was strengthened at that meeting and has continued based on the Tribal goals. The CCDF Administrator meets individually each year with a Tribal CCDF Administrator to discuss any updates or concerns regarding their tribe's early care and education systems. Several topics this past year included health and**

safety standards, data management systems, and professional learning and training.

- c. Identify other entities, agencies, or organizations consulted on the development of the CCDF Plan (e.g., representatives from the child care workforce, or statewide afterschool networks) and describe those consultation efforts: **MRTQ PDN, partners with the Lead Agency on early care and education providers professional development and the Quality Rating and Improvement Scale (QRIS), also collaborated extensively with the Lead Agency on the development of the CCDF plan. Maine's ECE Business Collaboratory Team was consulted on a variety of efforts where business and workforce were the key elements. Items included the market rate survey, cost modeling, and Maine's Child Care Employer Toolkit.**

1.3.2 Public hearing process

Lead Agencies must hold at least one public hearing in the State or Territory, with sufficient Statewide or Territory-wide distribution of notice prior to such a hearing to enable the public to comment on the provision of child care services under the CCDF Plan.

Describe the Statewide or Territory-wide public hearing process held to provide the public with an opportunity to comment on the provision of child care services under this Plan.

- i. Date of the public hearing: **5/31/2024**

Reminder: Must be no earlier than January 1, 2024. If more than one public hearing was held, enter one date (e.g., the date of the first hearing, the most recent hearing date, or any hearing date that demonstrates this requirement).

- ii. Date of notice of public hearing: **5/10/2024**

- iii. Was the notice of public hearing posted publicly at least 20 calendar days prior to the date of the public hearing?

☒ Yes.

☐ No. If no, describe:

- iv. Describe how the public was notified about the public hearing, including outreach in other languages, information on interpretation services being available, etc. Include specific website links if used to provide notice **The Lead Agency posted the notice on their website and in the newspaper, the CCDF administrator sent the notice and draft plan to stakeholder groups, and paper copies were sent through regular USPS upon request. While we didn't conduct outreach in multiple languages, we did offer services/accommodations for anyone that requested support to access the public hearing, with a request to contact us ahead of the hearing. "The Department requests that any interested party requiring special arrangements to attend the hearing contact the agency person listed above before May 30, 2024 at 1:00 p.m."**

<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/CCDF%20State%20Plan%20Public%20Notice%20May%202024.pdf>

- v. Describe how the approach to the public hearing was inclusive of all geographic

regions of the State or Territory: **The Lead Agency held a statewide public hearing via Zoom. Members of the public could register.**

- vi. Describe how the content of the Plan was made available to the public in advance of the public hearing (e.g., the Plan was made available in other languages, in multiple formats, etc.): **The Lead Agency posted the notice on their website and in the newspaper, the CCDF administrator sent the notice and draft plan to stakeholder groups, and paper copies were sent through regular USPS upon request. The Plan was not made available in multiple languages. It was made available electronically and paper copies were available by request.**
- vii. Describe how the information provided by the public was taken into consideration regarding the provision of child care services under this Plan: **The Lead Agency reviewed each comment and responded to every individual or group who submitted comments. Edits were incorporated based on the validity and guidelines of both Federal and State policies. There were 25 commenters on the Plan, which included individual parents and providers and groups organized together. For example one commenter included combined comments from the Maine Head Start Directors Association, the Family Child Care Association of Maine, and the Maine Association for the Education of Young Children. Some commenters expressed gratitude for the multiple ways the Plan was shared and sent out to different stakeholder groups and for increasing eligibility for families to 125% of State Median Income with state funds. Some commenters requested increased engagement in Plan development and child care policy development and decisions. The Lead Agency responded, "The Department will work with the Children's Cabinet Early Childhood Advisory Council (CCECAC) and stakeholders to continue to refine and improve ongoing engagement strategies." We are working on a new communication and stakeholder engagement plan for the next two years to increase our collaboration and coordination. Several comments focused on requests for the Lead Agency to implement new federal rules, including paying by enrollment with prospective payments, lowering family copayments, paying the market rate, and having online documentation submission. The Lead Agency responded by sharing plans to implement specific rules and the current work to implement a new child care management system for the Child Care Affordability Program in Maine. We have created a document that outlines all the requests to change specific policies and will be using that to guide our stakeholder engagement over the next year, to follow up with specific groups and have discussions on each of their requests and comments.**

1.3.3 Public availability of final Plan, amendments, and waivers

Lead Agencies must make the submitted and approved final Plan, any approved Plan amendments, and any approved requests for temporary waivers publicly available on a website.

- a. Provide the website link to where the Plan, any Plan amendments, and waivers (if applicable) are available. Note: A Plan amendment is required if the website address where the Plan is posted changes. **This website is where Maine posts CCDF State Plans and has added the 5 amended versions of the FY2022-2024 Plan.**
<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>

- b. Describe any other strategies that the Lead Agency uses to make submitted and approved CCDF Plan and approved Plan amendments available to the public. Check all that apply and describe the strategies below, including any relevant website links as examples.
- i. ☒ Working with advisory committees. Describe: **The Lead Agency regularly updates the Children's Cabinet Early Childhood Advisory Council (CCECAC) on updates and changes to the CCDF Maine State Plan, sending the approved CCDF Plan and approved Plan amendments to the council.**
 - ii. ☐ Working with child care resource and referral agencies. Describe:
 - iii. ☐ Providing translation in other languages. Describe:
 - iv. ☐ Sharing through social media (e.g., Facebook, Instagram, email). Describe:
 - v. ☒ Providing notification to key constituents (e.g., parent and family groups, provider groups, advocacy groups, foundations, and businesses). Describe: **The Lead Agency regularly updates the Family Child Care Association of Maine, YMCA Alliance of Northern New England, Maine Association for the Education of Young Children, and Maine Head Start Directors Association, and Right from the Start Coalition on updates and changes to the CCDF Maine State Plan, sending the approved CCDF Plan and approved Plan amendments to key constituents.**
 - vi. ☒ Working with Statewide afterschool networks or similar coordinating entities for out-of-school time. Describe: **Through the partnership with MRTQ PDN, the Maine Afterschool Network is represented as a partner of MRTQ PDN and receives the approved CCDF Plan and approved Plan amendment directly.**
 - vii. ☒ Direct communication with the child care workforce. Describe: **The Lead Agency sends updates through the OCFS Listserv and encourages members of the public to sign up to receive communications through the Listserv. Child care providers and educators receive direct communication through the OCFS Listserv and the MRTQ PDN weekly newsletter, that included the approved Plan and approved Plan amendments.**
 - viii. ☐ Other. Describe:

2 Child and Family Eligibility and Enrollment and Continuity of Care

Stable and reliable child care arrangements facilitate job stability for parents and healthy development of children. CCDF eligibility and enrollment policies can contribute to these goals. Policies and procedures that create barriers to families accessing CCDF, like inaccessible subsidy applications and onerous reporting requirements, interrupt a parent's ability to work and may deter eligible families from participating in CCDF.

To address these concerns, Lead Agencies must provide children with a minimum of 12 months between eligibility determinations, limit reporting requirements during the 12-month period, and ensure eligibility determination and redetermination processes do not interrupt a parent's work or school.

In this section, Lead Agencies will identify how they define eligible children and families and how the Lead Agency's eligibility and enrollment policies support access for eligible children and families.

2.1 Reducing Barriers to Family Enrollment and Redetermination

Lead Agency enrollment and redetermination policies may not unduly disrupt parents' employment, education, or job training activities to comply with the Lead Agency's or designated local entity's requirements. Lead Agencies have broad flexibility to design and implement the eligibility practices that reduce barriers to enrollment and redetermination.

Examples include developing strategies to inform families and their providers of an upcoming redetermination and the information that will be required of the family, pre-populating subsidy renewal forms, having parents confirm that the information is accurate, and/or asking only for the information necessary to make an eligibility redetermination. In addition, Lead Agencies can offer a variety of family-friendly methods for submitting documentation for eligibility redetermination that considers the range of needs for families in accessing support (e.g., use of languages other than English, access to transportation, accommodation of parents working non-traditional hours).

2.1.1 Eligibility practices to reduce barriers to enrollment

- a. Does the Lead Agency implement any of the following eligibility practices to reduce barriers at the time of initial eligibility determination? Check all that apply and describe those elements checked.
 - i. ☐ Establishing presumptive eligibility while eligibility is being determined. Describe the policy, including the populations benefiting from the policy, and identify how long the period of presumptive eligibility is:
 - ii. ☐ Leveraging eligibility from other public assistance programs. Describe:
 - iii. ☒ Coordinating determinations for children in the same household (while still ensuring each child receives 12 months of eligibility). Describe: **The Child Care Affordability Program may add children to the household mid award cycle. The new child will be determined for 12-month eligibility and the children with current awards will be extended to have the end date align with the new child so that redetermination have the family unit will take place at one time.**
 - iv. ☐ Self-assessment screening tools for families. Describe:
 - v. ☐ Extended office hours (evenings and/or weekends).
 - vi. ☒ Consultation available via phone.
 - vii. ☒ Other. Describe the Lead Agency policies to process applications efficiently and make timely eligibility determinations: **The Lead Agency has 15 days to process at the time of a completed application.**
 - viii. ☐ None.
- b. Does the Lead Agency use an online subsidy application?
☐ Yes.
☒ No. If no, describe why an online application is impracticable. **The Lead Agency is developing a new child care data management system with an expected launch date of May 2025 that will include online capabilities. The implementation of a online application into our current system MACWIS would take the same build time as the development of the online application being developed for the data management system BAXTER.**

MACWIS will be obsolete with the role out of BAXTER. The current online website does not hold security rating qualifications for uploading for tax documentation for attachments for parent documentations.

c. Does the Lead Agency use different policies for families receiving TANF assistance?

☐ Yes. If yes, describe the policies:

☒ No.

2.1.2 Preventing disruption of eligibility activities

a. Identify, where applicable, the Lead Agency's procedures and policies to ensure that parents do not have their employment, education, or job training unduly disrupted to comply with the State's/Territory's or designated local entity's requirements for the redetermination of eligibility. Check all that apply.

i. ☒ Advance notice to parents of pending redetermination.

ii. ☒ Advance notice to providers of pending redetermination.

iii. ☐ Pre-populated subsidy renewal form.

iv. ☐ Online documentation submission.

v. ☐ Cross-program redeterminations.

vi. ☐ Extended office hours (evenings and/or weekends).

vii. ☒ Consultation available via phone.

viii. ☐ Leveraging eligibility from other public assistance programs.

ix. ☐ Other. Describe:

b. Does the Lead Agency use different policies for families receiving TANF assistance?

☒ Yes. If yes, describe the policies: **The Lead Agency coordinates between OCFS and the Office for Family Independence to have the family transition directly into FEDCAP/Transitional child care services.**

☐ No.

2.2 Eligible Children and Families

At eligibility determination or redetermination, children must (1) be younger than age 13; (2) reside with a family whose income does not exceed 85 percent of the State's median income (SMI) for a family of the same size and whose family assets do not exceed \$1,000,000; and (3)(a) reside with a parent or parents who are working or attending a job training or educational program (which can include job search) or (b) receive, or need to receive, protective services as defined by the Lead Agency.

2.2.1 Eligibility criteria: age of children served

Lead Agencies may provide child care assistance for children less than 13 years of age, including continuing to provide assistance to children if they turn 13 during the eligibility period. In addition,

Lead Agencies can choose to serve children up to age 19 if those children are unable to care for themselves.

- a. Does your Lead Agency serve the full federally allowable age range of children through age 12?

☒ Yes.

☐ No. If no, describe the age range of children served and the reason why you made that decision to serve less than the full range of allowable children.

Note: Do not include children incapable of self-care or under court supervision, who are reported below in 2.2.1b and 2.2.1c.

- b. Does the Lead Agency extend eligibility for CCDF-funded child care to children ages 13 and older but below age 19 who are physically and/or mentally incapable of self-care?

☐ No.

☒ Yes.

- i. If yes, the upper age is (may not equal or exceed age 19): **18.00**

- ii. If yes, provide the Lead Agency definition of physical and/or mental incapacity: **As defined by the Americans with Disabilities Act of 1990 (ADA) (42 U.S.C. § 12102): An individual with a disability is defined by the ADA as a person who has a physical or mental impairment that substantially limits one or more major life activities, a person who has a history or record of such an impairment, or a person who is perceived by others as having such an impairment.**

- c. Does the Lead Agency extend eligibility for CCDF-funded child care to children ages 13 and older but below age 19 who are under court supervision?

☐ No.

☒ Yes. If yes, and the upper age is (may not equal or exceed age 19): **18.00**

- d. How does the Lead Agency define the following eligibility terms?

- i. “residing with”: **A Child is living with the Parent who is maintaining a home or principal domicile for the Child.**

- ii. “in loco parentis”: **In-loco Parentis refers to any individual who assumes custody and responsibility for the care of a Child (whether or not court-ordered).**

2.2.2 Eligibility criteria: reason for care

Lead Agencies have broad flexibility on the work, training, and educational activities required to qualify for child care assistance. Lead Agencies do not have to set a minimum number of hours for families to qualify for work, training, or educational activities, and there is no requirement to limit authorized child care services strictly based on the work, training, or educational schedule/hours of the parent(s). For example, the Lead Agency can include travel or study time in calculating the amount of needed services.

How does the Lead Agency define the following terms for the purposes of determining CCDF eligibility?

- a. Identify which of the following activities are included in your definition of “working” by

checking the boxes below:

- i. ☒ An activity for which a wage or salary is paid.
- ii. ☒ Being self-employed.
- iii. ☐ During a time of emergency or disaster, partnering in essential services.
- iv. ☐ Participating in unpaid activities like student teaching, internships, or practicums.
- v. ☒ Time for meals or breaks.
- vi. ☒ Time for travel.
- vii. ☐ Seeking employment or job search.
- viii. ☒ Other. Describe: **In a two-parent household, one parent is working, and the other does not work due to a disability.**

A Child placed under the legal guardianship of an individual who has reached Retirement Age as defined by Social Security.

Transportation Time means the time it takes a Parent (a) to leave a Child Care Provider and arrive at work, an Educational Program, or Job Training Program and (b) to leave work, an Educational Program, or Job Training Program and arrive back at the Provider. Transportation can take place by car, bus, bike, walking, boat, or others means as indicated on the application

- b. Identify which of the following activities are included in your definition of “attending job training” by checking the boxes below:
 - i. ☒ Vocational/technical job skills training.
 - ii. ☒ Apprenticeship or internship program or other on-the-job training.
 - iii. ☐ English as a Second Language training.
 - iv. ☐ Adult Basic Education preparation.
 - v. ☒ Participation in employment service activities.
 - vi. ☐ Time for meals and breaks.
 - vii. ☒ Time for travel.
 - viii. ☒ Hours required for associated activities such as study groups, lab experiences.
 - ix. ☒ Time for outside class study or completion of homework.
 - x. ☒ Other. Describe: **For Parents who are attending an Educational Program or Job Training Program, enrollment hours per week will be determined based upon:**
 - a. Actual class hours;**
 - b. Related Transportation Time; and**
 - c. Reasonable Study Time, as determined by the Department.****Reasonable Study Time is determined based on the Parent’s enrollment in classes as a Student, the Parent Student’s special needs, the amount of time between such Student’s classes, and the ages of the Student’s Child(ren).**

Transportation Time means the time it takes a Parent (a) to leave a Child Care Provider and arrive at work, an Educational Program, or Job Training Program and (b) to leave work, an Educational Program, or Job Training Program and arrive back at the Provider. Transportation can take place by car, bus, bike, walking, boat, or others means as indicated on the application

- c. Identify which of the following diplomas, certificates, degrees, or activities are included in your definition of “attending an educational program” by checking the boxes below:

- i. ☒ Adult High School Diploma or GED.
- ii. ☒ Certificate programs (12-18 credit hours).
- iii. ☒ One-year diploma (36 credit hours).
- iv. ☒ Two-year degree.
- v. ☒ Four-year degree.
- vi. ☒ Travel to and from classrooms, labs, or study groups.
- vii. ☒ Study time.
- viii. ☒ Hours required for associated activities such as study groups, lab experiences.
- ix. ☒ Time for outside class study or completion of homework.
- x. ☐ Applicable meal and break times.
- xi. ☒ Other. Describe: **For Parents who are attending an Educational Program or Job Training Program, enrollment hours per week will be determined based upon:**
 - a. Actual class hours;
 - b. Related Transportation Time; and
 - c. Reasonable Study Time, as determined by the Department.

Reasonable Study Time is determined based on the Parent’s enrollment in classes as a Student, the Parent Student’s special needs, the amount of time between such Student’s classes, and the ages of the Student’s Child(ren).

Transportation Time means the time it takes a Parent (a) to leave a Child Care Provider and arrive at work, an Educational Program, or Job Training Program and (b) to leave work, an Educational Program, or Job Training Program and arrive back at the Provider. Transportation can take place by car, bus, bike, walking, boat, or others means as indicated on the application

- d. Does the Lead Agency impose a Lead Agency-defined minimum number of hours of activity for eligibility?

☒ No.

☐ Yes.

If yes, describe any Lead Agency-imposed minimum requirement for the following:

☐ Work. Describe:

☐ Job training. Describe:

☐ Education. Describe:

☐ Combination of allowable activities. Describe:

☐ Other. Describe:

- e. Does the Lead Agency allow parents to qualify for CCDF assistance based on education and training without additional work requirements?

☒ Yes.

☐ No. If no, describe the additional work requirements:

- f. Does the Lead Agency extend eligibility to specific populations of children otherwise not eligible by including them in its definition of “children who receive or need to receive protective services?”

Note: A Lead Agency may elect to provide CCDF-funded child care to children in foster care when foster care parents are *not* working or are *not* in education/training activities, but this provision should be included in the Lead Agency’s protective services definition.

☐ No. If no, skip to question 2.2.3.

☒ Yes. If yes, answer the questions below:

Provide the Lead Agency’s definition of “protective services” by checking below the sub-populations of children that are included:

☐ Children in foster care.

☒ Children in kinship care.

☒ Children who are in families under court supervision.

☒ Children who are in families receiving supports or otherwise engaged with a child welfare agency.

☐ Children participating in a Lead Agency’s Early Head Start - Child Care Partnerships program.

☐ Children whose family members are deemed essential workers under a governor-declared state of emergency.

☒ Children experiencing homelessness.

☐ Children whose family has been affected by a natural disaster.

☐ Other. Describe:

- g. Does the Lead Agency waive the income eligibility requirements for cases in which children receive, or need to receive, protective services on a case-by-case basis?

☒ No.

☐ Yes.

- h. Does the Lead Agency waive the eligible activity (e.g., work, job training, education, etc.) requirements for cases in which children receive, or need to receive, protective services on a case-by-case basis?

☐ No.

☒ Yes.

- i. Does the Lead Agency use CCDF funds to provide respite care to custodial parents of children in protective services?

☒ No.

☐ Yes.

2.2.3 Eligibility criteria: deciding entity on family income limits

How are income eligibility limits established?

☒ There is a statewide limit with no local variation.

☐ There is a statewide limit with local variation. Provide the number of income eligibility tables and describe who sets the limits:

☐ Eligibility limits are established locally only. Provide the number of income eligibility tables and describe who sets the limits:

☐ Other. Describe:

2.2.4 Initial eligibility: income limits

- a. Complete the appropriate table to describe family income limits.

- i. Complete the table below to provide the statewide maximum income eligibility percent and dollar limit or threshold:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1	4907.89	125.00	6134.86
2	6418.01	125.00	8022.51
3	7928.13	125.00	9910.16
4	9438.25	125.00	11797.81
5	10948.37	125.00	13685.46

- ii. Does the Lead Agency certify that they use other funds if the income eligibility limit percent exceeds 85% SMI?

☐ Not applicable. The Lead Agency does not allow income eligibility limits above 85% SMI.

☒ Yes, the Lead Agency certifies that they use other funds (non-CCDF funds) for families with income that exceeds 85% SMI.

☐ No. The Lead Agency establishes income eligibility limits above SMI and includes CCDF funds to pay for families with income that exceeds 85% SMI. If checked, describe:

- b. Complete the table below if the Lead Agency has local variation in the maximum income

eligibility limit. Complete the table for the region/locality with the highest eligibility limit, region/locality with the lowest eligibility limit, and the region/locality that is most populous:

i. Region/locality with the highest eligibility limit:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

ii. Region/locality with the lowest eligibility limit:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

iii. Region/locality that is most populous:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

iv. Does the Lead Agency certify that they use other funds if the income eligibility limit percent exceeds 85% SMI?

☐ Not applicable. The Lead Agency does not allow income eligibility limits above 85% SMI.

☐ Yes, the Lead Agency certifies that they use other funds (not CCDF funds) for families with income that exceeds 85% SMI.

☐ No. The Lead Agency establishes income eligibility limits above 85% SMI and includes CCDF funds to pay for families with income that exceeds 85% SMI. If checked, describe:

c. How does the Lead Agency define “income” for the purposes of eligibility at the point of initial determination? Check all that apply:

- i. ☒ Gross wages or salary.
- ii. ☒ Disability or unemployment compensation.
- iii. ☒ Workers’ compensation.
- iv. ☒ Spousal support, child support.
- v. ☒ Survivor and retirement benefits.
- vi. ☒ Rent for room within the family’s residence.
- vii. ☒ Pensions or annuities.
- viii. ☒ Inheritance.
- ix. ☒ Public assistance.
- x. ☐ Other. Describe:

d. What is the effective date for these income eligibility limits? **10/19/2024**

e. Income limits must be established and reported in terms of current SMI based on the most recent data published by the Bureau of the Census, even if the federal poverty level is used in implementing the program.

What federal data does the Lead Agency use when reporting the income eligibility limits?
☒ LIHEAP. If checked, provide the publication year of the LIHEAP guideline estimates used by the Lead Agency: **2025**

☐ Other. Describe:

f. Provide the direct URL/website link, if available, for the income eligibility limits. **Website:**
<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>

Direct URL to PDF:

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/10192024%20income%20guidelines.pdf>

2.2.5 Income eligibility: irregular fluctuations in earnings

Lead Agencies must take into account irregular fluctuations in earnings in initial eligibility determination and redetermination processes. The Lead Agency must ensure that temporary increases in income, including temporary increases that can result in a monthly income exceeding

85 percent of SMI from seasonal employment or other temporary work schedules, do not affect eligibility or family co-payments.

Check the processes that the Lead Agency uses to take into account irregular fluctuations in earnings.

- i. ☒ Average the family's earnings over a period of time (e.g., 12 months).
Identify the period of time **Average the family's earnings over a 12-month period of time.**
- ii. ☒ Request earning statements that are most representative of the family's monthly income.
- iii. ☒ Deduct temporary or irregular increases in wages from the family's standard income level.
- iv. ☐ Other. Describe the other ways the Lead Agency takes into account irregular fluctuations in earnings:

2.2.6 Family asset limit

- a. When calculating income eligibility, does the Lead Agency ensure each eligible family does not have assets that exceed \$1,000,000?
☒ Yes.
☐ No. If no, describe:
- b. Does the Lead Agency waive the asset limit on a case-by-case basis for families defined as receiving, or in need of, protective services?
☒ No.
☐ Yes. If yes, describe the policy or procedure:

2.2.7 Additional eligibility criteria

Aside from the eligibility conditions or rules which have been described in 2.2.1 – 2.2.6, is any additional eligibility criteria applied during:

- a. ☐ Eligibility determination? If checked, describe:
- b. ☐ Eligibility redetermination? If checked, describe:

2.2.8 Documentation of eligibility determination

Lead Agencies must document and verify that children receiving CCDF funds meet eligibility criteria at the time of eligibility determination and redetermination.

Check the information that the Lead Agency documents and verifies at initial determination and redetermination and describe what information is required and how often.

Required at Initial Determination	Required at Redetermination	Description
[x]	[]	Applicant identity. Describe how you verify: The parent provides a birth certificate, passport, driver's license, or immigration documentation at time of application.
[x]	[]	Applicant's relationship to the child. Describe how you verify: The parent provides a child's birth certificate, passport, or immigration documentation at time of application.
[x]	[]	Child's information for determining eligibility (e.g., identity, age, citizen/immigration status). Describe how you verify: The parent provides child's birth certificate, passport, or immigration documentation at time of application.
[x]	[x]	Work. Describe how you verify: The parent provides employment information sheet or 4-weeks of paystubs.
[x]	[x]	Job training or educational program. Describe how you verify: The parent provides school enrollment or job training documentation and information at time of application and redetermination.

Required at Initial Determination	Required at Redetermination	Description
[x]	[x]	Family income. Describe how you verify: The parent provides all earned and unearned income at time of application and redetermination. Acceptable verification of earned income includes one (1) or more of the following: • Tipped wages, piece work, commission, etc. (per diem work must provide paystubs); • Four (4) weeks of current (within sixty (60) calendar days of application) and complete paystubs; • W-2 Form (if representative of current and future earnings); • State and/or Federal Income Tax Return (if representative of current and future earnings); • Self-employment bookkeeping records, including but not limited to, profit and loss statements (only accepted if the business has not been opened for one (1) year and requires submission of EIN number); • Sales and expenditure records; • Employment information sheet provided by CCAP; • Statement of employment and expected gross earnings, signed and dated by the employer on company letterhead; • Employer's wage record; • Employment Security Office records; • Verbal verification from Caseworker for Care and Custody Department/Tribal referrals; and • A signed release of information from the applicant which authorizes the Department to pursue verification or further clarification. Acceptable verification of unearned income includes, but is not limited to, the following: • Benefit check (viewed and photocopied by the Department); • Benefit programs award letters; • Signed income tax records (interest income, dividends, royalties, estates, trusts, deferred compensation plans, capital gains, etc.); • Support and alimony payments evidenced by court order, divorce or separation papers, or check copies; • Social Security Query Card Response; • Social Security District Office verification; • Bank statement;

Required at Initial Determination	Required at Redetermination	Description
		• Worker's Compensation verification; • Insurance company verification; • Verbal verification from Caseworker for Care and Custody Department/Tribal referrals; or • A signed release of information from the applicant which authorizes the Department to pursue verification or further clarification.
[x]	[x]	Household composition. Describe how you verify: The parent provides household composition (lists household members and relations and attests by signature) on the application and at redetermination.
[x]	[x]	Applicant residence. Describe how you verify: Documentation of Maine residency includes a Maine home address where the Parent lives and one (1) or more of the following: a Maine individual income tax return for the immediate past tax year that was timely filed with the Maine Revenue Service and that indicates Maine Resident status, valid Maine driver's license, Maine State ID, current Maine motor vehicle registration, Maine hunting/fishing license, proof of undergraduate Student in-State tuition payment, or other Department-Approved verification, which may include a utility bill, lease agreement or mortgage bill at the time of application and redetermination.
[]	[]	Other. Describe how you verify: N/A

2.2.9 Exception to TANF work requirements

Lead Agencies must ensure that families with young children participating in TANF will be informed of their right not to be sanctioned under the TANF work requirement if the custodial parent has a demonstrated inability to obtain child care for a child under age six, in accordance with Section 407(e)(2) of the Social Security Act.

- a. Identify the TANF agency that established these criteria or definitions: **The State of Maine Department of Health and Human Services Office for Family Independence is the agency responsible for the administration of the TANF block grant.**
- b. Provide the following definitions established by the TANF agency:
 - i. "Appropriate child care": **Appropriate child care is affordable child care furnished by a child care provider, of the participant's choice, who has passed background checks as required by 22 M.R.S. §§ 8301-A and 8302-A0.**
 - ii. "Reasonable distance": **Reasonable distance means that the ASPIRE participant is required to commute no more than 15 additional miles between home and work**

activity in order to transport the child to the child care provider.

- iii. **“Unsuitability of informal child care”:** Unsuitable child care is that provided by an individual who cannot pass a required background check or is unaffordable or would require the ASPIRE participant to travel an unreasonable distance.
 - iv. **“Affordable child care arrangements”:** Affordable child care arrangements are those for which the participant incurs no cost or is reimbursed by another program such as ASPIRE or through a deduction for child care from income by the TANF program or by any combination of these methods.
- c. How are parents who receive TANF benefits informed about the exception to the individual penalties associated with the TANF work requirements?
- i. ☒ In writing
 - ii. ☒ Verbally
 - iii. ☐ Other. Describe:

2.3 Prioritizing Services for Vulnerable Children and Families

Lead Agencies must give priority for child care assistance to children with special needs, families with very low incomes (considering family size), and children experiencing homelessness. A Lead Agency has the flexibility to prioritize other populations of children.

Note: Statute defines children with disabilities, and CCDF rule gives flexibility to Lead Agencies to include vulnerable populations in their definition of children with special needs.

CCDF defines “child experiencing homelessness” as a child who is homeless, as defined in Section 725 of Subtitle VII-B of the McKinney-Vento Act (42 U.S.C. 11434a).

2.3.1 Lead Agency definition of priority groups

Describe how the Lead Agency defines:

- d. **“Children with special needs.”** Child up to thirteen (13) years of age who has been determined and documented by a qualified professional to be a ☒ child with a disability ☐ as defined in section 602 of the Individuals with Disabilities Education Act (20 U.S.C. § 1401); is eligible for early intervention services under Part C of the Individuals with Disabilities Education Act (20 U.S.C. §§ 1431, et seq.); is eligible for services under section 504 of the Rehabilitation Act of 1973 (29 U.S.C. § 701); meets the definition of disability under the Americans with Disabilities Act of 1990 (ADA) (42 U.S.C. § 12102); is considered at-risk for health and/or developmental problems as a result of established biological Risk Factors, and/or as a result of identified environmental Risk Factors including, but not limited to, Homelessness, abuse and/or neglect, lead poisoning, and prenatal drug or alcohol exposure; or is a Child who is between thirteen (13) years of age and eighteen (18) years of age, inclusive, who is physically or mentally incapable of caring for him or herself, or is under court supervision.
- e. **“Families with very low incomes.”** Very Low Income refers to when the Gross Income or Allowable Net Income, adjusted to Family size, does not exceed one hundred percent (100%) of the Federal Poverty Guidelines. Children with special needs, who are homeless or who are from a family with very low income must be given priority over all other

children on the CCAP waiting list. Among these three priority groups, children are selected for services on a first-come, first-served basis by county based on the date of application.

2.3.2 Prioritization of child care services

Identify how the Lead Agency will prioritize child care services for the following children and families.

a. Complete the table below to indicate how the identified populations are prioritized.

Population Prioritized	Prioritize for enrollment in child care services	Serve without placing on waiting list	Waive co-payments as described in 3.3.1	Pay higher rate for access to higher quality care	Use grants or contracts to reserve spots	Other
Children with special needs	☒	☐	☐	☐	☐	☒ Describe: The Lead Agency provides an additional 35% reimbursement to a provider providing care to a child with special needs.
Families with very low incomes	☒	☐	☐	☐	☐	☐ Describe:

Population Prioritized	Prioritize for enrollment in child care services	Serve without placing on waiting list	Waive co-payments as described in 3.3.1	Pay higher rate for access to higher quality care	Use grants or contracts to reserve spots	Other
Children experiencing homelessness, as defined by CCDF	☒	☐	☐	☐	☐	[x] Describe: If a Parent of an Eligible Child is Homeless at the time of initial application or at time of redetermination, such Parent will be approved for Child Care Affordability. A ninety (90) calendar day grace period will be provided to the Parent to allow time for submission of required eligibility documentation as required by federal law (45 C.F.R. § 98.41). During such grace period, the Child shall be deemed to be eligible for the Child Care Affordability Program.
(Optional) Families receiving TANF, those attempting to transition off TANF, and those at risk of becoming dependent on TANF	☒	☐	☐	☐	☐	[] Describe:

a. Does the Lead Agency define any other priority groups?

☒ No.

[] Yes. If yes, identify the populations prioritized and describe how the Lead Agency prioritizes services:

2.3.3 Enrollment and grace period for children experiencing homelessness

Lead Agencies must allow (after an initial eligibility determination) children experiencing homelessness to receive CCDF services while required eligibility documentation is obtained.

Lead Agencies must establish a grace period that allows children experiencing homelessness and children in foster care to receive CCDF assistance while providing their families with a reasonable time to take any necessary actions to comply with State, Territory, or local immunization and other health and safety requirements. The length of such a grace period must be established in consultation with the State, Territorial, or Tribal public health agency.

Note: Any payment for such a child during the grace period may not be considered an error or improper payment.

- a. Describe the strategies to allow CCDF enrollment of children experiencing homelessness while required eligibility documentation is obtained: **If a Parent of an Eligible Child is Homeless at the time of initial application or at time of redetermination, such Parent will be approved for Child Care Affordability. A ninety (90) calendar day grace period will be provided to the Parent to allow time for submission of required eligibility documentation as required by federal law (45 C.F.R. § 98.41). During such grace period, the Child shall be deemed to be eligible for the Child Care Affordability Program.**
- b. Describe the grace period for each population below and how it allows them to receive CCDF assistance while providing their families with a reasonable time to take any necessary actions to comply with immunization and other health and safety requirements.
 - i. Provide the policy for a grace period for:

Children experiencing homelessness: **If a Parent of an Eligible Child is Homeless at the time of initial application or at time of redetermination, and meets other eligibility requirements, such Parent will be approved for the Child Care Affordability Program.**

Licensed child care programs and License-exempt child care, upon request, provide documentation that the Children receiving Child Care Affordability Program Payments are age-appropriately immunized and meet the latest recommendation for Childhood immunizations in Maine, as recommended by the Department's Center for Disease Control (CDC).

A Licensed child care program must have a current record of immunization on file for each Child, which clearly documents each Child's present immunization status based on the Department's Day Care Immunization Standards (available at <https://www.maine.gov/dhhs/mecdc/infectious-disease/immunization/documents/Daycare-Immunization-standards-revised.pdf> and published August 8, 2021), within 30 days of the Child's first admission to the Child care and updated as needed thereafter.

For License-exempt providers, the Child Care Affordability Program allows

for a 90 calendar day grace period to be granted while Parents are taking the necessary actions to comply with the immunization requirements.

Children who are in foster care: **Licensed child care programs and License-exempt child care, upon request, provide documentation that the Children receiving Child Care Affordability Program Payments are age-appropriately immunized and meet the latest recommendation for Childhood immunizations in Maine, as recommended by the Department's Center for Disease Control (CDC).**

A Licensed child care program must have a current record of immunization on file for each Child, which clearly documents each Child's present immunization status based on the Department's Day Care Immunization Standards (available at <https://www.maine.gov/dhhs/mecdc/infectious-disease/immunization/documents/Daycare-Immunization-standards-revised.pdf> and published August 8, 2021), within 30 days of the Child's first admission to the Child care and updated as needed thereafter.

For License-exempt providers, the Child Care Affordability Program allows for a 90 calendar day grace period to be granted while Parents are taking the necessary actions to comply with the immunization requirements.

- ii. Does the Lead Agency certify that the length of the grace period was established in consultation with the State, Territorial, or Tribal public health agency?

☒ Yes.

☐ No. If no, describe:

- c. Describe how the Lead Agency coordinates with licensing agencies and other relevant State, Territorial, Tribal, and local agencies to provide referrals and support to help families with children receiving services during a grace period comply with immunization and other health and safety requirements: **Children's Licensing and Investigation Services provides technical assistance to the provider if needed regarding strategies to obtain necessary information or resources that will support the family in obtaining the documentation. Families are referred to Help Me Grow Maine for assistance in accessing resources and finding services needed, including those that support immunization and other health and safety requirements.**

2.4 Lead Agency Outreach to Families Experiencing Homelessness, Families with Limited English Proficiency, and Persons with Disabilities

The Lead Agency must conduct outreach and provide services to families with limited English proficiency, families experiencing homelessness, and persons with disabilities.

2.4.1 Families with limited English proficiency and persons with disabilities: outreach and services

- a. Check the strategies the Lead Agency or partners utilize to conduct outreach and provide services to eligible families with limited English proficiency. Check all that apply.

- i. ☒ Application in languages other than English (application and related documents, brochures, provider notices).
 - ii. ☒ Informational materials in languages other than English.
 - iii. ☐ Website in languages other than English.
 - iv. ☒ Lead Agency accepts applications at local community-based locations.
 - v. ☒ Bilingual caseworkers or translators available.
 - vi. ☐ Bilingual outreach workers.
 - vii. ☒ Partnerships with community-based organizations.
 - viii. ☒ Collaboration with Head Start, Early Head Start, or Migrant and Seasonal Head Start.
 - ix. ☐ Home visiting programs.
 - x. ☒ Other. Describe: **The Lead Agency provides all staff with Language Link, a telephone language line for assistance with translation. The State of Maine provides translators as needed. The Child Care Choices website's Google Translate allows translation for more than 90 languages available to select from. Help ME Grow has a position dedicated as a cultural broker to support the program in a variety of needs that include culturally appropriate practices.**
- b. Check the strategies the Lead Agency or partners utilize to conduct outreach and provide services to eligible families with a person(s) with a disability. Check all that apply.
- i. ☐ Applications and public informational materials available in braille and other communication formats for access by individuals with disabilities.
 - ii. ☒ Websites that are accessible (e.g., Section 508 of the Rehabilitation Act).
 - iii. ☐ Caseworkers with specialized training/experience in working with individuals with disabilities.
 - iv. ☒ Ensuring accessibility of environments and activities for all children.
 - v. ☒ Partnerships with State and local programs and associations focused on disability- related topics and issues.
 - vi. ☐ Partnerships with parent associations, support groups, and parent-to-parent support groups, including the Individuals with Disabilities Education Act (IDEA) federally funded Parent Training and Information Centers.
 - vii. ☒ Partnerships with State and local IDEA Part B, Section 619 and Part C providers and agencies.
 - viii. ☒ Availability and/or access to specialized services (e.g., mental health, behavioral specialists, therapists) to address the needs of all children.
 - ix. ☐ Other. Describe:

2.4.2 Families experiencing homelessness: Outreach and technical assistance efforts

- a. Check, where applicable, the procedures used to conduct outreach for children experiencing homelessness and their families.

- i. ☒ Lead Agency accepts applications at local community-based locations.
 - ii. ☒ Partnerships with community-based organizations.
 - iii. ☒ Partnering with homeless service providers, McKinney-Vento liaisons, and others who work with families experiencing homelessness to provide referrals to child care.
 - iv. ☐ Other. Describe:
- b. The Lead Agency must provide training and technical assistance (TA) to providers and appropriate Lead Agency (or designated entity) staff on identifying and serving children and families experiencing homelessness.
- i. Describe the Lead Agency's training and TA efforts for providers in identifying and serving children and their families experiencing homelessness. **MRTQ PDN provides online training, "Caring for Children and Families Experiencing Homelessness." This training is available to all for free and on demand. MRTQ PDN technical assistance staff are available upon request to offer TA to child care providers who request it. MRTQ PDN and the Lead Agency also work closely with Maine Head Start programs that have been working with families experiencing homelessness for decades to share best practices.**
 - ii. Describe the Lead Agency's training and TA efforts for Lead Agency (or designated entity) staff in identifying and serving children and their families experiencing homelessness. **All Child Care Affordability Program staff are required to complete training as part of their onboarding process. MRTQ PDN technical assistance staff also complete training. We have also added this requirement for Maine Child Care Licensing Specialists. We have two training options for staff, either the MRTQ PDN on demand training or the on demand training, "Supporting Children and Families Experiencing Homelessness," offered at:**
<https://eclkc.ohs.acf.hhs.gov/family-support-well-being/article/supporting-children-families-experiencing-homelessness>

2.5 Promoting Continuity of Care

Lead Agencies must consider children's development and promote continuity of care when authorizing child care services and must establish a minimum 12-month period for each child, both at the initial eligibility determination and redetermination.

2.5.1 Children's development

Describe how the Lead Agency's eligibility, enrollment, reporting, and redetermination policies promote continuity of care in order to support children's development. **The lead agency policies promote continuity of care through a variety of options for families that are needing care through attendance policies, 12 -week job search, absent policies that include medical and maternity leave, parental choice, and provider education on expulsion and suspension.**

2.5.2 Minimum 12-month eligibility

Lead Agencies must establish a minimum 12-month eligibility period for each child, both at the initial eligibility determination and at redetermination to support continuity in child care assistance and reduce barriers to families retaining eligibility. This requirement is:

- Regardless of changes in income, Lead Agencies may not terminate CCDF assistance during the minimum 12-month period if a family has an increase in income that exceeds the Lead Agency's income eligibility threshold but not the federal threshold of 85 percent of SMI; and
- Regardless of temporary changes in participation in work, training, or educational activities.

- a. Does the Lead Agency certify that their policies or procedures provide a minimum 12-month eligibility period for each child at initial eligibility determination?

☒ Yes.

☐ No. If no, describe:

- b. Does the Lead Agency certify that its definition of "temporary change" includes each of the minimum required elements?

1. Any time-limited absence from work for an employed parent due to such reasons as the need to care for a family member or an illness.
2. Any interruption in work for a seasonal worker who is not working between regular industry work seasons.
3. Any student holiday or break for a parent participating in a training or educational program.
4. Any reduction in work, training, or education hours, as long as the parent is still working or attending a training or educational program.
5. Any cessation of work or attendance at a training or educational program not listed above. In these cases only, Lead Agencies may establish a period of 3 months or longer.
6. Any change in age, including a child turning 13 years old during the minimum 12-month eligibility period.
7. Any changes in residency within the State or Territory.

☒ Yes.

☐ No. If no, describe:

- c. Are the policies different for redetermination?

☒ No.

☐ Yes. If yes, provide the additional/varying policies for redetermination:

2.5.3 Job search and continued assistance

- a. Does the Lead Agency consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination and/or at the minimum 12-month eligibility redetermination? (Note: If yes, Lead Agencies must provide a minimum of 3 months of job search.) Check all that apply:

- i. ☐ Yes. The Lead Agency does consider seeking employment (engaging in a job

- search) as an eligible activity at initial eligibility determination. If yes, describe:
- ii. ☐ Yes. The Lead Agency does consider seeking employment (engaging in a job search) as an eligible activity at redetermination. If yes, describe:
 - iii. ☒ No. The Lead Agency does not consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination or redetermination.
- b. Does the Lead Agency continue assistance during the minimum 12-month eligibility period when a parent has a non-temporary loss or cessation of eligible activity?
- ☒ Yes. The Lead Agency continues assistance.
- ☐ No, the Lead Agency discontinues assistance.
- i. If no, describe the Lead Agency's policies for discontinuing assistance due to a parent's non-temporary change:
 - ii. If no, describe what specific actions/changes trigger the job-search period after each such loss or cessation:
 - iii. If no, how long is the job-search period where a family can continue assistance (must be at least 3 months)?
- c. The Lead Agency may discontinue assistance prior to the next minimum 12-month redetermination in the limited circumstances listed below. Check and provide the policy for all circumstances in which the Lead Agency chooses to discontinue assistance prior to the next minimum 12-month redetermination:
- i. ☐ Not applicable.
 - ii. ☒ Excessive unexplained absences despite multiple attempts by the Lead Agency or designated entity to contact the family and provider, including the prior notification of a possible discontinuation of assistance.
- Provide the Lead Agency's policy defining the number of unexplained absences identified as excessive: **A Parent will be determined to be ineligible for Child Care Affordability Program if: A Child Care Provider reports to the Department that a Child had twenty-five (25) or more Unacceptable Absences within the previous twelve (12) months.**
- Unacceptable Absence means a lack of attendance by a Child at the Child Care Service for more than two (2) calendar days per month without Reasonable Cause or previous approval from the Department. Reasonable Cause means Department-Approved reasons for a Child's absence from a Child Care Provider's program which may include, without limitation: Federal/State holidays; Parental vacation days; inclement weather defined by a snow day when local schools are closed; illness of the Child or other immediate Family member; appointments; transportation issues that affect the Parent's ability to transport the Child to care; Family visitations: Family emergencies, including, but not limited to, surgery, medical treatments, or accidents; or catastrophic events affecting the Family, including but not limited to fires, storms, or accidents.**
- iii. ☒ A change in residency outside of the State or Territory.

Provide the Lead Agency's policy for a change in residency outside the State or Territory: **Termination of Child Care Affordability Program to the Parent by the Department**

The Department will terminate a Parent's participation in the Child Care Affordability Program when:

b. The Parent participating in Child Care Affordability Program is no longer a Maine Resident;

CCAP Reporting Requirements: Within ten (10) calendar days of its occurrence, Parents receiving Child Care Affordability Program must report any of the following to the Department: Any Non-Temporary Change; and Change of Child Care Provider. If Parents fail to provide the requested information to support a reported change within ten (10) calendar days of the change, the Parent's Child Care Affordability Program will be terminated. It is the responsibility of the Parent to ensure that the Department has his/her current address and current email address. All notices sent to the last documented address provided by the Parent(s) and not returned will be considered to have been received, and as such the Department will consider the Parent(s) notified.

- iv. ☒ Substantiated fraud or intentional program violations that invalidate prior determinations of eligibility.

Provide the Lead Agency's definition of fraud/intentional program violations that lead to discontinued assistance: **Where the Department makes a preliminary determination that a Parent or Child Care Provider may have committed a Misrepresentation, the case may be referred to the DHHS Fraud Investigation Unit pursuant to 22 M.R.S. § 13 and the Department may pursue establishment of a Misrepresentation against the Parent and/or Child Care Provider administratively. A final determination that a Misrepresentation occurred shall be made only as the result of a decision by an Administrative Hearing, a court, or waiver of the Administrative Hearing by the Parent and/or Child Care Provider. Failure to request an Administrative Hearing constitutes a waiver.**

2.5.4 Reporting changes during the minimum 12-month eligibility period

Lead Agencies may only require families to report changes that impact a family's eligibility, including only if the family's income exceeds 85 percent of the SMI, taking into account irregular fluctuations in income, or there is a non-temporary change in the parent's work, training, or education status, during the 12-month eligibility period. Lead Agencies may also require families to report that enable the lead agency to contact the family or pay providers, such as a new telephone number or address.

Note: The response below should exclude reporting requirements for a graduated phase-out, which are described in question 2.5.5.

Does the Lead Agency limit what families must report during the 12-month eligibility period to the changes described above?

☒ Yes.

☐ No. If no, describe:

2.5.5 Policies and procedures for graduated phase-out of assistance at redetermination

Lead Agencies that establish initial family income eligibility below 85 percent of SMI must provide a graduated phase-out of assistance for families whose income has increased above the Lead Agency's initial income threshold at the time of redetermination but remains below the federal threshold of 85 percent of SMI.

Lead Agencies that provide a graduated phase-out must implement a two-tiered eligibility threshold, with the second tier of eligibility (used at the time of eligibility redetermination) to be set at:

- (i) 85 percent of SMI for a family of the same size; or,
- (ii) An amount lower than 85 percent of SMI for a family of the same size but above the Lead Agency's initial eligibility threshold that:
 - (A) Takes into account the typical household budget of a family with a low income
 - (B) Provides justification that the second eligibility threshold is:
 - (1) Sufficient to accommodate increases in family income over time that are typical for workers with low incomes and that promote and support family economic stability
 - (2) Reasonably allows a family to continue accessing child care services without unnecessary disruption

At redetermination, a child must be considered eligible if their parents are participating in an eligible activity even if their income exceeds the Lead Agency's initial eligibility income limit as long as their income does not exceed the second tier of eligibility. Note that once determined eligible, the child must be considered eligible for a full minimum 12-month eligibility period, even if the parents' income exceeds the second tier of eligibility during the eligibility period, as long as it does not exceed 85 percent of SMI.

A child eligible for services via the graduated phase-out of assistance is considered eligible under the same conditions as other eligible children with the exception of the co-payment restrictions, which do not apply to a graduated phase-out. To help families transition from child care assistance, Lead Agencies may gradually adjust co-payment amounts in proportion to a family's income growth for families whose children are determined eligible under a graduated phase-out. Lead Agencies may require additional reporting on changes in family income but must still ensure that any additional reporting requirements do not constitute an undue burden on families.

Check and describe the option that best identifies the Lead Agency's policies and procedures regarding the graduated phase-out of assistance.

- a. ☒ Not applicable. The Lead Agency sets its initial eligibility threshold at 85 percent of SMI and therefore is not required to provide a graduated phase-out period. (If checked, skip to question 3.1.1.)
- b. ☐ The Lead Agency sets the second tier of eligibility at 85 percent of SMI. If checked, describe the policies and procedures:
 - i. ☐ Lead Agency adjusts the family's co-pay during the graduated phase-out period. If checked, describe how the Lead Agency gradually adjusts co-payment for families under a graduated phase-out period in proportion to a family's

- income growth. Include information on the percentage or amount of change made in the co-payment during graduated phase-out:
- ii. ☐ Lead Agency requires additional reporting requirements during the graduated phase-out period. If checked, describe:
- c. ☐ The Lead Agency sets the second tier of eligibility at an amount lower than 85 percent of SMI for a family of the same size but above the Lead Agency's initial eligibility threshold. If checked, provide the following information:
 - i. Provide the income level (\$/month) and the percent of SMI for the second tier of eligibility for a family of three:
 - ii. Describe how the second eligibility threshold takes into account the typical household budget of a low-income family:
 - iii. Describe how the second eligibility threshold is sufficient to accommodate increases in family income over time that are typical for low-income workers and that promote and support family economic stability:
 - iv. Describe how the second eligibility threshold reasonably allows a family to continue accessing child care services without unnecessary disruption:
 - v. ☐ Lead Agency adjusts the family's co-pay during the graduated phase-out period. If checked, describe how the Lead Agency gradually adjusts co-payment for families under a graduated phase-out period in proportion to a family's income growth. Include information on the percentage or amount of change made in the co-payment during graduated phase-out:
 - vi. ☐ Lead Agency requires additional reporting requirements during the graduated phase-out period. If checked, describe:

3 Child Care Affordability

CCDF subsidies make child care more affordable for eligible families, providing access to a greater range of child care options that allow parents to work, go to school, or enroll in training and they allow parents to access higher quality care options that better support children's development. CCDF requires some families participating in CCDF to pay an affordable co-payment set by the Lead Agency to cover a part of their care. But co-payments can be a significant and destabilizing financial strain on family budgets and a barrier to parent employment, and the CCDBG Act requires that the co-payment amount not be a barrier to families participating in CCDF. Lead Agencies may not set parent co-payments above 7% of family income regardless of gradual phase-out policies and regardless of the number of children receiving assistance. Lead Agencies are encouraged to set co-payments much lower than 7% to make child care more affordable for more families and have broad flexibility to waive co-payments for too many participants. Lead Agencies must ensure that the total payment to a child care provider is not reduced because of family's lowered or waived co-payment.

In this section, Lead Agencies will identify how they determine an eligible family's co-payment, the policies in place to waive or ensure co-payments are affordable for families, and how the Lead Agency improves access for children and families in economically and/or socially marginalized communities.

3.1 Family Co-payments

Lead Agencies must establish and periodically revise a sliding-fee scale for families receiving CCDF services that varies based on income and the size of the family to determine each family's contribution (i.e., co-payment) and does not create a barrier to receiving CCDF assistance. In addition to income and the size of the family, the Lead Agency may use other factors as appropriate when determining family contributions/co-payments. Lead Agencies may not use price of care or amount of subsidy payment in determining co-payments. Lead Agencies must ensure that the total payment to a child care provider is not reduced because of family's lowered or waived co-payment.

3.1.1 Family co-payment

Lead Agencies may not charge any family more than 7% of a family's gross income, regardless of the number of children participating in CCDF.

- a. What is the maximum percent of a family's gross income any family could be charged as a co-payment? **10%. The Lead Agency intends to file for a waiver to implement 7% by FFY 2026.**
- b. Does the Lead Agency certify that their sliding fee scales are always based on income and family size (regardless of how many different scales they may use)?
☒ Yes.
☐ No. If no, describe:

3.1.2 Sliding fee scale

Provide the CCDF co-payments for eligible families in the table(s) below according to family size for one child in care.

- a. Is the sliding fee scale set statewide?
☒ Yes.
☐ No. If no, describe how the sliding fee scale is set:
- b. Complete the table below. If the sliding fee scale is not set statewide, complete the table for the most populous locality:

	<i>A</i>	<i>B</i>	<i>C</i>	<i>D</i>	<i>E</i>	<i>F</i>
Family Size	Lowest monthly income at initial eligibility where the family is first charged a co-pay (greater than \$0).	What is the monthly co-payment for a family of this size based on the income level in (A)?	What percentage of income is the co-payment in (B)?	Highest monthly income at initial eligibility where a family is charged a co-pay before a family is no longer eligible.	What is the monthly co-payment for a family of this size based on the income level in (D)?	What percentage of income is this co-payment in (E)?
1	50.00	1.00	2.00	5672.28	567.23	10.00
2	50.00	1.00	2.00	7417.60	741.76	10.00
3	50.00	1.00	2.00	9162.91	916.29	10.00
4	50.00	1.00	2.00	10908.23	1090.82	10.00
5	50.00	1.00	2.00	12653.55	1265.35	10.00

- c. What is the effective date of the sliding-fee scale(s)? **3/09/24**
- d. Provide the link(s) to the sliding-fee scale(s):
<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/sliding%20fee%20scale%2003092024.pdf>
- e. Does the Lead Agency allow providers to charge families additional amounts above the required co-payment in instances where the provider's price exceeds the subsidy payment?
☒ No.
☐ Yes.
If yes:
- i. Provide the rationale for the Lead Agency's policy to allow providers to charge families additional amounts above the required co-payment, including a demonstration of how the policy does not provide a barrier and promotes affordability and access for families:
- ii. Provide data (including data on the size and frequency of such amounts) on the extent to which CCDF providers charge additional amounts to families:

3.2 Calculation of Co-Payment

Lead agencies must calculate a family's contribution (or co-payment), taking into account income and family size, and Lead Agencies may choose to consider other factors in their calculation.

3.2.1 Family co-payment calculation

- a. How is the family's contribution calculated, and to whom is it applied? Check if the fee is a dollar amount or if the fee is a percent of income below, and then check all that apply

under the selection, as appropriate.

- i. ☐ The fee is a dollar amount and (check all that apply):
 - ☐ The fee is per child, with the same fee for each child.
 - ☐ The fee is per child and is discounted for two or more children.
 - ☐ The fee is per child up to a maximum per family.
 - ☐ No additional fee is charged after a certain number of children.
 - ☐ The fee is per family.
 - ☐ The contribution schedule varies because it is set locally/regionally (as indicated in 1.2.1). Describe:
 - ☐ Other. Describe:
- ii. ☒ The fee is a percent of income and (check all that apply):
 - ☐ The fee is per child, with the same percentage applied for each child.
 - ☐ The fee is per child, and a discounted percentage is applied for two or more children.
 - ☐ The fee is per child up to a maximum per family.
 - ☐ No additional percentage is charged after a certain number of children.
 - ☒ The fee is per family.
 - ☐ The contribution schedule varies because it is set locally/regionally (as indicated in 1.2.1). Describe:
 - ☐ Other. Describe:
- b. Does the Lead Agency use other factors in addition to income and family size to determine each family's co-payment? (Lead Agencies may not use price of care or amount of subsidy payment in determining co-payments).
 - ☒ No.
 - ☐ Yes.If yes, check and describe those additional factors below:
 - i. ☐ Number of hours the child is in care. Describe:
 - ii. ☐ Quality of care (as defined by the Lead Agency). Describe:
 - iii. ☐ Other. Describe:
- c. Describe any other policies the Lead Agency uses in the calculation of family co-payment to ensure it does not create a barrier to access. Check all that apply:
 - i. ☐ Base co-payments on only a portion of the family's income. For instance, only consider the family income over the federal poverty level.
 - ii. ☐ Base co-payments on the number of children in the family and reduce a portion of the co-payments as the number of children being served increases.
 - iii. ☐ Other. Describe:

3.3 Waiving Family Co-payment

3.3.1 Waiving family co-payment

The Lead Agency may waive family contributions/co-payments for many families to lower their costs and maximize affordability for families. Lead Agencies have broad flexibility in determining for which families they will waive co-payments.

Does the Lead Agency waive family contributions/co-payments?

☒ No, the Lead Agency does not waive any family contributions/co-payments. (Skip to question 4.1.1.)

☐ Yes. If yes, identify and describe which family contributions/co-payments waived.

- i. ☐ Families with an income at or below 100% of the Federal Poverty Level for families of the same size.
- ii. ☐ Families with an income above 100% but at or below 150% of the Federal Poverty Level for families of the same size.
- iii. ☐ Families experiencing homelessness.
- iv. ☐ Families with children with disabilities.
- v. ☐ Families enrolled in Head Start or Early Head Start.
- vi. ☐ Children in foster care or kinship care, or otherwise receiving or needing to receive protective services. Describe the policy:
- vii. ☐ Families meeting other criteria established by the Lead Agency. Describe the policy:

4 Parental Choice, Equal Access, Payment Rates, and Payment Practices

Core purposes of CCDF are to provide participating parents choice in their child care arrangements and provide their children with equal access to child care compared to those children not participating in CCDF. CCDF requirements approach equal access and parental choice comprehensively to meet these foundational program goals. Providing access to a full range of child care providers helps ensure that families can choose a child care provider that meets their family's needs. CCDF payment rates and practices must be sufficient to support equal access by allowing child care providers to recruit and retain skilled staff, provide high-quality care, and operate in a sustainable way. Supply-building strategies are also essential.

This section addresses many of the CCDF provisions related to equal access, including access to the full range of providers, payment rates for providers, co-payments for families, payment practices, differential payment rates, and other strategies that support parental choice and access by helping to ensure that child care providers are available to serve children participating in CCDF.

In responding to questions in this section, OCC recognizes that each Lead Agency identifies and defines its own categories and types of care. OCC does not expect Lead Agencies to change their definitions to fit the CCDF-defined categories and types of care. For these questions, provide responses that closely match the CCDF categories of care.

4.1 Access to Full Range of Provider Options

Lead Agencies must provide parents a choice of providers and offer assistance with child care services through a child care certificate (or voucher) or with a child care provider that has a grant or contract for the provision of child care services. Lead Agencies are reminded that policies and procedures should not restrict parental access to any type or category of care or provider (e.g., center care, home care, in-home care, for-profit provider, non-profit provider, or faith-based provider, etc.).

4.1.1 Parent choice

- a. Identify any barriers to provider participation, including barriers related to payment rates and practices, (including for family child care and in-home providers), based on provider feedback, public comment, and reports to the Lead Agency: **Barriers identified in Maine to provider participation are in regards to the processes for vendor codes that do not enable a provider to be reimbursed until one is established or if an update is needed. Similar is true for establishing direct deposits for provider reimbursement.**
- b. Does the Lead Agency offer child care assistance through vouchers or certificates?
☒ Yes.
☐ No.
- c. Does the Lead Agency offer child care assistance through grants or contracts?
☐ Yes.
☒ No.
- d. Describe how the parent is informed that the child care certificate allows the option to choose from a variety of child care categories, such as private, not-for-profit, faith-based providers; centers; family child care homes; or in-home providers: **The child care certificate (award letter) states:**
To access Maine child care providers' information, please call (888) 900-0055 or visit: www.earlycaremaine.org
To access the Child Care Affordability Program website, please visit:
<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>

The site earlycaremaine.org takes parents and families to a landing page where they can look for child care on Child Care Choices and learn about child care options, as well as learning about the quality rating and improvement system, Rising Stars for ME.
- e. Describe what information is included on the child care certificate: **The Lead Agency includes on the Parent Award Letter, the following is included, the award start and end dates, the parent fee amount, the names of the children the award is for the Child Care Provider's name, authorized hours, and all consumer statement information.**

4.2 Assess Market Rates and Analyze the Cost of Child Care

To establish subsidy payment rates that ensure equal access, Lead Agencies must collect and analyze statistically valid and reliable data and have the option to conduct either a (1) market rate survey (MRS) reflecting variations in the price to parents of child care services by geographic area,

type of provider, and age of child, or (2) an ACF pre-approved alternative methodology, such as a cost estimation model, which estimates the cost of care by incorporating both data and assumptions to estimate what expected costs would be incurred by child care providers and parents under different scenarios. All Lead Agencies must analyze the cost of providing child care through a narrow cost analysis or pre-approved alternative methodology.

Prior to conducting the MRS or pre-approved alternative, Lead Agencies must consult with the State Advisory Council on Early Childhood Education and Care (designated or established pursuant to the Head Start Act (42 U.S.C. 9837b(b)(1)(A)(i)) or similar coordinating body, local child care program administrators, local child care resource and referral agencies, and other appropriate entities; and organizations representing child care caregivers, teachers, and directors. Prior to conducting the MRS or pre-approved alternative methodology, Lead Agencies must consult with the State Advisory Council on Early Childhood Education and Care (designated or established pursuant to the Head Start Act (42 U.S.C. 9837b(b)(1)(A)(i)) or similar coordinating body, local child care program administrators, local child care resource and referral agencies, and other appropriate entities; and organizations representing child care caregivers, teachers, and directors.

Note: Any Lead Agency considering using an alternative methodology instead of a market rate survey to set payment rates, is required to submit a description of its proposed approach to OCC for pre-approval in advance of developing and conducting the alternative methodology. Advance approval is not required if the Lead Agency plans to implement both an MRS and an alternative methodology to set rates at a percentile of the market rate, but a Lead Agency conducting a limited market rate survey and using it to inform their cost model would need pre-approval for this approach. In its request for ACF pre-approval, a Lead Agency must provide details on the following elements of their proposed alternative methodology:

- Overall approach and rationale for using proposed methodology
- Description of stakeholder engagement
- Data collection timeframe (if applicable)
- Description of the data and assumptions included in the methodology, including how these elements will yield valid and reliable results from the model
- Description of how the methodology will capture the universe of providers, and reflect variations by provider type, age of children, geographic location, and quality

4.2.1 Completion of the market rate survey or ACF pre-approved alternative methodology

Did the Lead Agency conduct a statistically valid and reliable MRS or ACF pre-approved alternative methodology to meet the CCDF requirements to assess child care prices and/or costs and determine payment rates? Check only one based on which methodology was used to determine your payment rates.

a. ☒ Market rate survey.

- i. When were the data gathered (provide a date range; for instance, September – December 2023)? **The Lead Agency (DHHS, OCFS) conducted the 2024 Child Care Market Rate Survey between October 2023 and February 2024 with technical assistance from the Burns & Associates division of Health Management Associates (HMA-Burns). HMA supported the administration of the survey and HMA and the Center for Early Learning Funding Equity (CELFE) both supported the survey**

analysis. HMA provided the narrow cost analysis data from the survey to CELFE for the analysis.

- b. ☐ ACF pre-approved alternative methodology.
 - i. ☐ The alternative methodology was completed.
 - ii. ☐ The alternative methodology is in process.

If the alternative methodology was completed:

When were the data gathered and when was the study completed?

Describe any major differences between the pre-approved methodology and the final methodology used to inform payment rates. Include any major changes to stakeholder engagement, data, assumptions or proposed scenarios.

If the alternative methodology is in progress:

Provide a status on the alternative methodology and timeline (i.e., dates when the alternative methodology activities will be conducted, any completed steps to date, anticipated date of completion, and expected date new rates will be in effect using the alternative methodology).

- c. Consultation on data collection methodology.

Describe when and how the Lead Agency engaged the following partners and how the consultation informed the development and execution of the MRS or alternative methodology, as appropriate.

- iii. State Advisory Council or similar coordinating body: **The Lead Agency presented the draft instrument to the Children's Cabinet Early Childhood Advisory Council (CCECAC) in Fall of 2023. Council members then provided feedback and had questions answered regarding the MRS. Edits were made to the draft instrument based on the feedback provided during the meeting. The ECE Business Collaboratory is a group of stakeholders in Maine including state staff and external child care stakeholders. They also informed the development of the market rate survey and narrow cost analysis, since they are the lead advisory group in the development of a cost modeling tool in Maine.**
- iv. Local child care program administrators: **Local child care program administrators are represented on both the CCECAC and the ECE Business Collaboratory Team. The Lead Agency also sent communication regarding the upcoming survey to all licensed child care programs statewide.**
- v. Local child care resource and referral agencies: **Maine does not have CCR&Rs.**
- vi. Organizations representing child care caregivers, teachers, and directors from all settings and serving all ages: **The Lead Agency began sending communications via mail and email to all licensed providers in September 2024 that the survey was coming. Key stakeholders were also notified, including the Maine Association for the Education of Young Children, the Family Child Care Association of Maine, the YMCA Alliance of Northern New England, the Maine Head Start Director's Association, Maine Afterschool Network, and Maine Roads to Quality Professional Development Network, and asked to share with providers that are in their**

network. These key stakeholders informed the way the survey was executed, advising and supporting the multiple forms of survey distribution.

vii. Other. Describe: **N/A**

d. An MRS must be statistically valid and reliable.

An MRS can use administrative data, such as child care resource and referral data, if it is representative of the market. Please provide the following information about the market rate survey:

i. When was the market rate survey completed? **2/29/2024**

ii. What was the time period for collecting the information (e.g., all of the prices in the survey are collected within a three-month time period)? **All of the prices of the survey were collected within a five-month time period, between October 2023 and February 2024.**

iii. Describe how it represented the child care market, including what types of providers were included in the survey: **The Market Rate Survey (MRS) was conducted with licensed child care providers across Maine. To maximize representation, all licensed facility and family child care providers are invited to complete the survey via multiple modes, including the internet, postal survey, or by phone.**

iv. What databases are used in the survey? Are they from multiple sources, including licensing, resource and referral, and the subsidy program? **The source of licensed child care information comes from Maine's MACWIS database, which includes all licensed programs and the subsidy program.**

v. How does the survey use good data collection procedures, regardless of the method for collection (mail, telephone, or web-based survey)? **Contact with the providers was initiated with a prenotification letter that described the purpose and importance of the survey. Following this initial contact, the data collection was conducted in multiple phases to maximize response.**

vi. What is the percent of licensed or regulated child care centers responding to the survey? **62.50**

vii. What is the percent of licensed or regulated family child care homes responding to the survey? **66.80**

viii. Describe if the survey conducted in any languages other than English: **No**

ix. Describe if data were analyzed in a manner to determine price of care per child: **Yes, the market rate survey reflected variation in the price of cost of care per child.**

x. Describe if data were analyzed from a sample of providers and if so, how the sample was weighted: **Yes, to account for differences in service capacity and to appropriately recognize the relative prevalence of rates associated with providers that have more licensed capacity in a given county or county grouping, reported rates were weighted by total licensed capacity.**

e. Price variations reflected.

The market rate survey data or ACF pre-approved alternative methodology data must reflect variations in child care prices or cost of child care services in specific categories.

- i. Describe how the market rate survey or pre-approved alternative methodology reflected variation in geographic area (e.g., county, region, urban, rural). Include information on whether parts of the State or Territory were not represented by respondents and include information on how prices or costs could be linked to local geographic areas. **The 2024 Maine Child Care Market Rate Survey collected information regarding current child care rates for private-pay families, provider capacity by age level and current enrollment, details regarding participation in the CCAP, and other data to help OCFS to inform CCAP policies and funding.**

OCFS provided HMA-Burns with a roster of all licensed providers as of October 23, 2023. In total, 1,548 providers were listed. During the administration period of the survey, four providers indicated they were no longer providing child care services, resulting in a final count of 1,544 providers, including 834 child care facilities (child care centers, nursery schools, and small child care facilities), and 710 family child care providers.

Programs responded from every county in Maine. The highest participation was in Oxford County with 83% of child care facilities participating and 84% of family child care programs participating. The lowest participation was in Lincoln County with 48% of child care facilities and 40% of family child care programs.

Market rate data was collected by provider type, age group, attendance frequency (part and full-time), and rate type (hourly, daily, weekly, monthly, or annually). Administrative licensing data was used to analyze market rates and reported costs by county and by quality level as measured by a provider's Star rating from Rising Stars for ME, Maine's quality rating improvement system (QRIS).

- ii. Describe how the market rate survey or pre-approved alternative methodology reflected variation in type of provider (e.g., licensed providers, license-exempt providers, center-based providers, family child care home providers, home based providers). **The 2024 Maine Child Care Market Rate Survey collected information regarding current child care rates for private-pay families, provider capacity by age level and current enrollment, details regarding participation in the CCAP, and other data to help OCFS to inform CCAP policies and funding.**

All licensed child care facilities and family child care providers were invited to participate in the survey. Maine supports four primary types of child care licenses:

1. Child Care Centers - which may be a house or other place that provides care for 12 or more children under 13 years old.
2. Nursery School - which may be a house or other place that provides care for three or more children between 33 months and eight years old.
3. Small Child Care Facility – which may be a house (not the residence of the operator) or other place that cares for between three and 12 children under 13 years old.
4. Family Child Care Providers - which provide care in the operator's residence to up to 12 children under 13 years old.

OCFS provided HMA-Burns with a roster of all licensed providers as of October 23, 2023. In total, 1,548 providers were listed. During the administration period of the survey, four providers indicated they were no longer providing child care services, resulting in a final count of 1,544 providers, including 834 child care facilities (including child care centers, nursery schools, and small child care facilities), and 710 family child care providers. In total, 64.4 percent of providers participated in the survey, including 62.5 percent of child care facilities (including centers, nursery schools, and small facilities) and 66.8 percent of family child care providers.

Market rate data was collected by provider type, age group, attendance frequency (part and full-time), and rate type (hourly, daily, weekly, monthly, or annually). Administrative licensing data was used to analyze market rates and reported costs by county and by quality level as measured by a provider's Star rating from Rising Stars for ME, Maine's quality rating improvement system (QRIS).

- iii. Describe how the market rate survey or pre-approved alternative methodology reflected age of child (e.g., infant, toddler, preschool, school-age): **The 2024 Maine Child Care Market Rate Survey collected information regarding current child care rates for private-pay families, provider capacity by age level and current enrollment, details regarding participation in the CCAP, and other data to help OCFS to inform CCAP policies and funding.**

Market rate data was collected by provider type, age group, attendance frequency (part and full-time), and rate type (hourly, daily, weekly, monthly, or annually). The age groups included infant, toddler, preschool, and school-age children.

Administrative licensing data was used to analyze market rates and reported costs by county and by quality level as measured by a provider's Star rating from Rising Stars for ME, Maine's quality rating improvement system (QRIS).

- iv. Describe any other key variations examined by the market rate survey or ACF pre-approved alternative methodology, such as quality level: **The 2024 Maine Child Care Market Rate Survey collected information regarding current child care rates for private-pay families, provider capacity by age level and current enrollment, details regarding participation in the CCAP, and other data to help OCFS to inform CCAP policies and funding. Market rate data was collected by provider type, age group, attendance frequency (part and full-time), and rate type (hourly, daily, weekly, monthly, or annually). Administrative licensing data was used to analyze market rates and reported costs by county and by quality level as measured by a provider's Star rating from Rising Stars for ME, Maine's quality rating improvement system (QRIS).**

4.2.2 Cost analysis

If a Lead Agency does not complete a cost-based pre-approved alternative methodology, they must analyze the cost of providing child care services through a narrow cost analysis. A narrow cost analysis is a study of what it costs providers to deliver child care at two or more levels of

quality: (1) a base level of quality that meets health, safety, staffing, and quality requirements, and (2) one or more higher levels of quality as defined by the Lead Agency. The narrow cost analysis must estimate costs by levels of quality; include relevant variation by provider type, child's age, or location; and analyze the gaps between estimated costs and payment rates to inform payment rate setting. Lead agencies are not required to complete a separate narrow cost analysis if their pre-approved alternative methodology addresses all of the components required in the narrow cost analysis.

Describe how the Lead Agency analyzed the cost of child care through a narrow cost analysis or pre-approved alternative methodology for the FFY 2025–2027 CCDF Plan, including:

- a. How did the Lead Agency conduct a narrow cost analysis (e.g., a cost model, a cost study, existing data or data from the Provider Cost of Quality Calculator)? **Maine's Market Rate Survey used variation by geographic location, category of provider, and age of child for the 2024 MRS. The survey included a series of questions designed to assess the costs that providers incur in delivering child care. The survey requested details about the wages paid to staff by job type (director, assistant director, teacher, teachers' assistant, and substitute for child care facilities, and teachers and teachers' assistants for family child care providers), benefit packages, and other expenses such as facility costs. The survey included several questions to assess the time teaching staff spend in quality-based activities (such as curriculum development), and the availability of paid professional development days, tuition assistance benefits, and tuition discounts provided for child care provided to paid staff.**

The survey requested this information about provider costs and cost drivers, but analyses of the provider cost questions are not presented in Maine's 2024 MRS report. The data was sent to the Center for Early Learning Funding Equity to conduct the cost study, as OCFS is also using the results to support a future cost modeling tool and a broader child care financing analysis being performed by the CELFE.

- b. In the Lead Agency's analysis, were there any relevant variations by geographic location, category of provider, or age of child? **Before analyzing reported rate information, a variety of methodological decisions were made between OCFS, and contractors for the market rate survey and cost study, HMA and CELFE. A key determination was how to group counties for the purposes of analyzing data. Previous county groupings were established in the 2018 market rate survey where counties with fewer than 10 responding providers were combined with a neighboring county. As part of the cost analysis work, the Lead Agency identified two county groupings. In addition to recognizing similarities in providers' costs across counties, the shift to two county groupings is designed to reduce geographic rate disparities.**

CELFE helped OCFS assess the cost of care in centers and family child care homes for each age group based on staffing patterns, wages, and other costs. The most significant cost driver, wages, did not differ significantly across the state. The cost analysis showed little difference across counties in the cost of providing care, perhaps a 5% difference from the least expensive to most expensive areas for centers and a 10% difference from the least expensive to most expensive for homes.

The cost study did find higher rates being charged for preschool age children than the

preschool cost of care. When setting reimbursement rates for child care, CELFE recommended the state consider the way the child care market smooths prices across the infant, toddler, preschool and school-age age groups. OCFS did not want to set rates precisely at the estimated cost of care because setting infant and toddler subsidy rates at these very high costs (as much as \$467 per week) could inadvertently drive infant and toddler market rates too high for families who do not use subsidies to afford. However, the state rate setting can work incrementally to move providers away from an overreliance on profits from preschooler classrooms as the only way to support affordable infant-toddler programs.

As in most states, prices charged by Maine's child care providers differ significantly across rate regions. In 2021, Cumberland's 75th percentile of the market rate was 64% higher than in Aroostook. In 2024, this rose to the Cumberland rate being 83% higher than the rate in Aroostook, Piscataquis, and Somerset. This dramatic variation in prices charged has been driven by what parents in those communities can afford and by what the state has been paying providers through child care reimbursement rates.

CELFE recommend that the state work towards decreasing the number of CCAP subsidy rate regions. Based on the results from CELFE's cost study, the cost of care across the state was similar enough to support moving to one statewide region, or at most two regions (Cumberland, York & Sagadahoc in one county group and the rest of the state in the other county group).

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/Rate%20Setting%20Outreach%20Information%20from%20CELFE.pdf>

- c. What assumptions and data did the Lead Agency use to determine the cost of care at the base level of quality (e.g., ratios, group size, staff compensations, staff training, etc.)? **The survey requested information about provider costs and cost drivers, such as staffing by position type, average wages paid to each position, turnover rates, benefits offered to staff, and other expenses, including costs related to quality initiatives such as the cost of curriculum-related materials and program self-assessment tools and resources. Administrative licensing data was used to analyze market rates and reported costs by county and by quality level as measured by a provider's Rising Stars for ME rating, Maine's quality rating improvement system. Analyses of the provider cost questions supported the cost study.**
- d. How does the Lead Agency define higher quality and what assumptions and data did the Lead Agency use to determine cost at higher levels of quality (e.g., ratio, group size, staffing levels, staff compensation, professional development requirements)? **A Lead Agency can use a quality improvement system or other system of quality indicators (e.g., accreditation, pre-Kindergarten standards, Head Start Program Performance Standards, or State-defined quality measures). Maine defined higher quality through the state's QRIS system and the costs for Head Starts to meet their program performance standards. The MRS requested information about provider costs and cost drivers, such as staffing by position type, average wages paid to each position, turnover rates, benefits offered to staff, and other expenses, including costs related to quality initiatives such as the cost of curriculum-related materials and program self-assessment tools and resources. Administrative licensing data was used to analyze market rates and reported costs by county and by quality level as measured by a provider's Rising Stars for ME rating, Maine's**

quality rating improvement system. The analysis utilized costs for Star 4 programs and Head Starts, which are Star 5 programs in Rising Stars for ME to inform higher costs for higher quality. Analyses of the provider cost questions supported the cost study.

- e. What is the gap between cost and price, and how did the Lead Agency consider this while setting payment rates? Did the Lead Agency target any rate increases where gaps were the largest or develop any long-term plans to increase rates based on this information? The Lead Agency did a cost study with the Center for Early Learning Funding Equity (CELFE). The longer term goal is to use a cost modeling tool to inform the Lead Agency's efforts to provide reimbursement/funding for the true cost of care.

In 2021, Cumberland's 75th percentile of the market rate was 64% higher than in Aroostook. In 2024, this rose to the Cumberland rate being 83% higher than the rate in Aroostook, Piscataquis, and Somerset. This dramatic variation in prices charged has been driven by what parents in those communities can afford and by what the state has been paying providers through child care reimbursement rates.

CELFE helped OCFS assess the cost of care in centers and family child care homes for each age group based on staffing patterns, wages, and other costs. The most significant cost driver, wages, did not differ significantly across the state. The cost analysis shows little difference across counties in the cost of providing care, perhaps a 5% difference from the least expensive to most expensive areas for centers and a 10% difference from the least expensive to most expensive for family child care. When looking at cost overall, cost did not differ enough across the state to justify the significant variations in what providers charge. Instead, these variations are likely driven by what parents in local communities can afford to pay for care. The lack of differences in costs suggested that Maine should begin moving towards a single statewide rate (or perhaps a 2-level structure with only small differences) to more equitably fund child care services.

If Maine set the 2024 reimbursement rate at the 75th percentile of the market rate of Cumberland County, the typical center in Cumberland County—if serving all subsidized children and operating at 85% of full enrollment—would have total revenue that 32% higher than what the cost study estimates is the total cost of operating the center. At the same time, setting the reimbursement rate at the 75th percentile of Aroostook County would result in revenue 23% lower than the cost of care. To move toward a more equitable financing strategy, Maine would need to provide more significant rate increases to more rural and/or lower-income counties and substantially smaller rate increases to Cumberland and York Counties.

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/Rate%20Setting%20Outreach%20Information%20from%20CELFE.pdf>

To begin this process in 2024, Maine set differentiated reimbursement rates. The cost study showed the biggest gaps between price and cost of care in the more rural counties and/or those with lower median household incomes. Maine set rates to provide higher increases in those areas. The state set the 2024 reimbursement rates for child care centers at the 50th percentile in Cumberland, York, and Sagadahoc counties and the 70th percentile for centers in all other counties. The state set the 2024 reimbursement rates for family child care at the 65th percentile in Cumberland, York, and Sagadahoc counties and

the 75th percentile for all other counties. This rate structure provided a significant increase for the vast majority of age groups, types of providers, and regions of the state. There were a handful of rates, mainly some rates for preschool and school age children, and mostly in the Cumberland region, where the rate stayed the same as the 2021 rate.

4.2.3 Publicly available report on the cost and price of child care

The Lead Agency must prepare a detailed report containing the results of the MRS or ACF pre-approved alternative methodology and include the Narrow Cost Analysis if an ACF pre-approved alternative methodology was not conducted.

The Lead Agency must make this report widely available no later than 30 days after completion of the report, including posting the results on the Lead Agency website. The Lead Agency must describe in the detailed report how the Lead Agency took into consideration the views and comments of the public or stakeholders prior to conducting the MRS or ACF pre-approved alternative methodology.

a. Describe how the Lead Agency made the results of the market rate survey or ACF pre-approved alternative methodology report widely available to the public by responding to the questions below.

- i. Provide the date the report was completed: **4/29/2024**
- ii. Provide the date the report containing results was made widely available (no later than 30 days after the completion of the report): **5/10/2024**
- iii. Provide a link to the website where the report is posted and describe any other strategies the Lead Agency uses to make the detailed report widely available:
<https://www.maine.gov/dhhs/ocfs/provider-resources/child-care-subsidy-information-for-providers>

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/Rate%20Setting%20Outreach%20Information%20from%20CELFE.pdf>
- iv. Describe how the Lead Agency considered partner views and comments in the detailed report. Responses should include which partners were engaged and how partner input influenced the market rate survey or alternative methodology: **The Lead Agency engaged with staff from the Governor's office, the Department of Education's Early Learning Team and contractors HMA and CELFE to consider Maine's goals in the Office of Child and Family Services, in the Preschool Development Grant systems work, and in the Children's Cabinet. We had meetings and discussions related to the market rate survey and cost study and how the data would inform Maine's rate setting approach and final reports.**

4.3 Adequate Payment Rates

The Lead Agency must set CCDF subsidy payment rates in accordance with the results of the current MRS or ACF pre-approved alternative methodology and at a level to ensure equal access for eligible families to child care services comparable with those provided to families not receiving CCDF assistance. Lead Agencies are also required to provide a summary of data and facts to demonstrate how payment rates ensure equal access, which means the Lead Agency must also

consider the costs of base level care and higher quality care as part of its rate setting. Finally, the Lead Agency must re-evaluate its payment rates at least every 3 years.

The ages and types of care listed in the base payment rate tables are meant to provide a snapshot of the categories of rates and are not intended to be comprehensive of all categories that might exist or to reflect the terms used by the Lead Agency for particular ages. If rates are not statewide, please provide all variations of payment rates when reporting base payment rates below.

Base rates are the lowest, foundational rates before any differentials are added (e.g., for higher quality or other purposes) and must be sufficient to ensure that minimum health, safety, quality, and staffing requirements are covered. These are the rates that will be used to determine compliance with equal access requirements.

4.3.1 Payment rates

- a. Are the payment rates that the Lead Agency is reporting in 4.3.2 set statewide by the Lead Agency?

☒ Yes.

- i. If yes, check if the Lead Agency:

☐ Sets the same payment rates for the entire State or Territory.

☒ Sets different payment rates for different regions in the State or Territory.

☐ No.

- ii. If no, identify how many jurisdictions set their own payment rates:

- b. Provide the date the current payment rates became effective (i.e., date of last payment rate update based on most recent MRS or ACF pre-approved alternative methodology as reported in 4.2.1). **7/6/2024**

- c. If the Lead Agency does not publish weekly rates, then how were the rates reported in 4.3.2 or 4.3.3 calculated (e.g., were daily rates multiplied by 5 or monthly rates divided by 4.3)? **The Lead Agency does publish weekly rates.**

4.3.2 Base payment rates

- a. Provide the base payment rates in the tables below. If the Lead Agency completed a market rate survey (MRS), provide the percentiles based on the most recent MRS for the identified categories. If the Lead Agency sets different payment rates for different regions in the State or Territory (and checked 4.3.1aii), provide the rates for the most populous region as well as the region with payment rates set at the lowest percentile. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

The preamble to the 2016 final rule states that a benchmark for adequate payment rates is the 75th percentile of the most recent MRS. The 75th percentile benchmark applies to the base rates. The 75th percentile is the number separating the lowest 75 percent of rates from the highest 25 percent. Setting rates at the 75th percentile, while not a requirement, would ensure that eligible families can afford three out of four child care providers. In addition to reporting the 75th percentile in the tables below, the Lead Agency must also report the 50th percentile and 60th percentile for each identified category.

If the Lead Agency conducted an ACF pre-approved alternative methodology, provide the estimated cost of care for the identified categories, as well as the percentage of the cost of care covered by the established payment rate. If the Lead Agency indicated it sets different payment rates for different regions in the State or Territory in 4.3.1.a, provide the estimated cost of care and the percentage of the cost of care covered by the established payment rate for the most populous region as well as the region with rates established at the lowest percent of the cost of care.

For each identified category below, provide the percentage of providers who are receiving the base rate without any add-ons or differential payments.

Provide the full-time weekly base payment rates in the table below. If weekly payment rates are not published, then the Lead Agency will need to calculate its equivalent.

i. Table 1: Complete if rates are set statewide. If rates are not set statewide, provide rates for most populous region. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Infants (6 months)	330.00 Per Week	75.00	330.00	50.00	330.00	346.00	384.00		
Family Child Care for Infants (6 months)	245.00 Per Week	75.00	245.00	65.00	225.00	227.50	250.00		
Center Care for Toddlers (18 months)	320.00 Per Week	75.00	320.00	50.00	320.00	335.00	350.00		

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Family Child Care for Toddlers (18 months)	230.77 Per Week	75.00	230.77	65.00	220.00	225.00	250.00		
Center Care for Preschoolers (4 years)	300.00 Per Week	75.00	300.00	50.00	300.00	301.00	321.92		
Family Child Care for Preschoolers (4 years)	225.00 Per Week	75.00	225.00	65.00	205.00	220.00	230.77		
Center Care for School-Age (6 years)	214.00 Per Week	75.00	214.00	50.00	214.00	250.00	301.00		
Family Child Care for School-Age (6 years)	175.00 Per Week	75.00	175.00	65.00	150.00	170.00	195.00		

ii. Table 2: Do not complete if rates are set statewide. If rates are not set statewide, provide rates for region with payment rates set at the lowest percentile. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Infants (6 months)	330.00 Per Week	75.00	330.00	50.00	330.00	346.00	384.00		
Family Child Care for Infants (6 months)	245.00 Per Week	75.00	245.00	65.00	225.00	227.50	270.00		

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Toddlers (18 months)	320.00 Per Week	75.00	320.00	50.00	320.00	335.00	350.00		
Family Child Care for Toddlers (18 months)	230.77 Per Week	75.00	230.77	65.00	220.00	225.00	250.00		
Center Care for Preschoolers (4 years)	300.00 Per Week	75.00	300.00	50.00	300.00	301.00	321.92		
Family Child Care for Preschoolers (4 years)	225.00 Per Week	75.00	225.00	65.00	205.00	220.00	230.77		
Center Care for School-Age (6 years)	214.00 Per Week	75.00	214.00	50.00	214.00	250.00	301.00		
Family Child Care for School-Age (6 years)	175.00 Per Week	75.00	175.00	65.00	150.00	170.00	195.00		

- b. Does the Lead Agency certify that the percentiles reported in the table above are calculated based on their most recent MRS or ACF pre-approved Alternative Methodology?

☒ Yes.

☐ No. If no, what is the year of the MRS or ACF pre-approved alternative methodology that the Lead Agency used? What was the reason for not using the most recent MRS or ACF pre-approved alternative methodology? Describe:

4.3.3 Tiered rates, differential rates, and add-ons

Lead Agencies may establish tiered rates, differential rates, or add-ons on top of their base rates as a way to increase payment rates for targeted needs (e.g., a higher rate for serving children with special needs).

- a. Does the Lead Agency provide any rate add-ons above the base rate?
- ☒ Yes. If yes, describe the add-ons, including what they are, who is eligible to receive the add-ons, and how often are they paid: **The Lead Agency adds a \$100 stipend for any licensed CCAP provider caring for an infant for 20 hours or more per week.**
- ☐ No.
- b. Has the Lead Agency chosen to implement tiered reimbursement or differential rates?
- ☒ Yes.
- ☐ No. Tiered or differential rates are not implemented.

If yes, identify below any tiered or differential rates, and, at a minimum, indicate the process and basis used for determining the tiered rates, including if the rates were based on the MRS or an ACF pre-approved alternative methodology. Check and describe all that apply:

- i. ☒ Differential rate for non-traditional hours. Describe: **The Lead Agency reimburses for Non-traditional hours receive 35% additional reimbursement for 6PM-6AM and weekends.**
- ii. ☒ Differential rate for children with special needs, as defined by the Lead Agency. Describe: **Providers caring for a child receiving CCAP with a documented special need receive 35% increase in reimbursement.**
- iii. ☒ Differential rate for infants and toddlers. Note: Do not check if the Lead Agency has a different base rate for infants/toddlers with no separate bonus or add-on. Describe: **The Lead Agency reimburses for infant/toddler care receive 10% increase in reimbursement.**
- iv. ☐ Differential rate for school-age programs. Note: Do not check if the Lead Agency has a different base rate for school-age children with no separate bonus or add-on. Describe:
- v. ☒ Differential rate for higher quality, as defined by the Lead Agency. Describe: **Maine provides a percentage increase in payments on behalf of families utilizing the Child Care Subsidy Program for providers who are at various stars on the Quality Rating and Improvement System. The higher quality the program is, the more incentive or quality bump funds they receive. Providers at a star 3 receive 2% quality bump in payment. Providers at a star 4 receive a 5% quality bump and providers at the highest level, star 5, receive 10% quality bump.**
- vi. ☐ Other differential rates or tiered rates. For example, differential rates for geographic area or for type of provider. Describe:
- vii. If applicable, describe any additional add-on rates that you have besides those identified above.

Does the Lead Agency reduce provider payments if the price the provider charges to private-pay families not participating in CCDF is below the Lead Agency's established payment rate?

☒ Yes. If yes, describe: **The Department currently pays the state rate or the provider's private pay payment rate, whichever is lower. The Department is working to complete the**

child care management system that will support the implementation of the new Federal Rule, including payments at the established state rate.

[] No.

4.3.4 Establishing payment rates

Describe how the Lead Agency established payment rates:

- a. What was the Lead Agency's methodology or process for setting the rates or how did the Lead Agency use their data to set rates? **The Lead Agency set the rates based on the 2024 MRS and the cost analysis combined to provide an overall increase to rates, with differential percentiles, at a minimum of the 50th percentile and a maximum of the 75th percentile, working towards reimbursement closer to the true cost of care.**
- b. How did the Lead Agency determine that the rates are adequate to meet health, safety, quality, and staffing requirements under CCDF? **The Lead Agency utilized the cost analysis data completed by CELFE in conjunction with the 2024 MRS that considered factors to meet CCDF and licensing requirements.**

If Maine had set the reimbursement rate at the 75th percentile of the market rate in Cumberland County, the typical center in Cumberland County—if serving all subsidized children and operating at 85% of full enrollment—would have total revenue at 32% higher than what the cost study estimates is the total cost of operating the center. At the same time, setting the reimbursement rate at the 75th percentile of Aroostook County would result in revenue 23% lower than the cost of care. Maine is working toward a more equitable financing strategy, and that led to our 2024 rates giving more significant rate increases to more rural and/or lower-income counties and substantially smaller rate increases to Cumberland and nearby counties.

- c. How did the Lead Agency use the cost of care, either from the narrow cost analysis or the ACF pre-approved alternative methodology to inform rate setting, including how using the cost of care promotes the stabilization of child care providers? **The Lead Agency utilized the cost analysis data completed by CELFE in conjunction with the 2024 MRS that considered costs of care for both family child care and facilities. To move toward a more equitable rate setting strategy that would decrease the gap between price and cost of care, Maine set differentiated reimbursement rates. The cost study showed the biggest gaps between price and cost of care in the more rural counties and/or those with lower median household incomes. Maine set rates to provide higher increases in those areas and moved from 10 rate regions to 2 rate regions. The state set the 2024 reimbursement rates for child care centers at the 50th percentile in Cumberland, York, and Sagadahoc counties and the 70th percentile for centers in all other counties. The state set the 2024 reimbursement rates for family child care at the 65th percentile in Cumberland, York, and Sagadahoc counties and the 75th percentile for all other counties. This rate structure provided a significant increase for the vast majority of age groups, types of providers, and regions of the state. There were a handful of rates, mainly some rates for preschool and school age children, and mostly in the Cumberland region, where the 2024 rate stayed the same as the 2021 rate.**
- d. How did the Lead Agency account for the cost of higher quality while setting payment rates? **The Lead Agency utilized the cost analysis data completed by CELFE in conjunction**

with the 2024 MRS that considered costs at higher rates of care for both FCC and Facilities Providers at a star 3 receive 2% quality bump in payment. Providers at a star 4 receive a 5% quality bump and providers at the highest level, star 5, receive 10% quality bump, providing a tiered rate system for higher levels of quality.

- e. Identify and describe any additional facts (not covered in responses to 4.3.1 – 4.3.3) that the Lead Agency considered in determining its payment rates to ensure equal access. **The Lead Agency's cost analysis efforts with CELFE recommended the grouping of counties utilized for the market rates as the results indicated there was significant similarities in cost statewide versus several regions.**

4.4 Payment Practices to Providers

Lead Agencies must use subsidy payment practices that reflect practices that are generally accepted in the private pay child care market. The Lead Agency must ensure timeliness of payment to child care providers by paying in advance or at the beginning of delivery of child care services. Lead Agencies must also support the fixed cost of child care services based on paying by the child's authorized enrollment, or if impracticable, an alternative approach that will not undermine the stability of child care programs as justified and approved through this Plan.

Lead Agencies must also (1) pay providers based on established part-time or full-time rates rather than paying for hours of service or smaller increments of time, and (2) pay for reasonable, mandatory registration fees that the provider charges to private-paying parents. These policies apply to all provider types unless the Lead Agency can demonstrate that in limited circumstances the policies would not be considered generally-accepted payment practices.

In addition, Lead Agencies must ensure that child care providers receive payment for any services in accordance with a payment agreement or an authorization for services, ensure that child care providers receive prompt notice of changes to a family's eligibility status that could impact payment, and have timely appeal and resolution processes for any payment inaccuracies and disputes.

4.4.1 Prospective and enrollment-based payment practices

Lead Agencies must use payment practices for all CCDF child care providers that reflect generally-accepted payment practices of providers serving private-pay families, including paying providers in advance or at the beginning of the delivery of child care services and paying based on a child's authorized enrollment or an alternative approach for which the Lead Agency must demonstrate paying for a child's authorized enrollment is not practicable and it will not undermine the stability of child care programs. Lead Agencies may only use alternate approaches for subsets of provider types if they can demonstrate that prospective payments and authorized enrollment-based payment are not generally-accepted for a type of child care setting. Describe the Lead Agency payment practices for all CCDF child care providers:

- a. Does the Lead Agency pay all provider types prospectively (i.e., in advance of or at the beginning of the delivery of child care services)?

☐ Yes. If yes, describe:

☒ No, it is not a generally-accepted payment practice for each provider type. If no, describe the provider type not paid prospectively and the data demonstrating it is not a generally-accepted payment practice for that provider type, and describe the Lead

Agency's payment practice that ensures timely payment for that provider type: **The Lead Agency reimburses providers on a two-week billing cycle. The Department is working to complete an online child care management system that will support the implementation of prospective payments based on enrollment in the Child Care Affordability Program. The Department anticipates implementation of prospective enrollment-based payments in 2025.**

- b. Does the Lead Agency pay based on authorized enrollment for all provider types?

☐ Yes. The Lead Agency pays all providers by authorized enrollment and payment is not altered based on a child's attendance or the number of absences a child has.

☐ No, it is not a generally-accepted practice for each provider type. If no, describe the provider types not paid by authorized enrollment, including the data showing it is not a generally-accepted payment practice for that provider type, and describe how the payment policy accounts for fixed costs:

☒ It is impracticable. Describe provider type(s) for which it is impracticable, why it is impracticable, and the alternative approach the Lead Agency uses to delink provider payments from occasional absences, including evidence that the alternative approach will not undermine the stability of child care programs, and thereby accounts for fixed costs: **The Lead Agency is in the midst of building a new data management system with a estimated launch date in 2025. The Lead Agency is also building a fiscal plan to meet CCDF Rule.**

4.4.2 Other payment practices

Lead Agencies must (1) pay providers based on established part-time or full-time rates rather than paying for hours of service or smaller increments of time, and (2) pay for reasonable, mandatory registration fees that the provider charges to private-paying parents, unless the Lead Agency provides evidence that such practices are not generally-accepted for providers caring for children not participating in CCDF in its State or Territory.

- a. Does the Lead Agency pay all providers on a part-time or full-time basis (rather than paying for hours of service or smaller increments of time)?

☒ Yes.

☐ No. If no, describe the policies or procedures that are different than paying on a part-time or full-time basis and the Lead Agency's rationale for not paying on a part-time or full-time basis:

- b. Does the Lead Agency pay for reasonable mandatory registration fees that the provider charges to private-paying parents?

☐ Yes. If yes, identify the fees the Lead Agency pays for:

☒ No. If no, identify the data and how data were collected to show that paying for fees is not a generally-accepted payment practice: **The Lead Agency conducts a survey of all providers at the time of new provider agreement enrollment periods every three years. At that time, the majority of providers indicated that this was not practiced by the majority.**

- c. Describe how the Lead Agency ensures that providers are paid in accordance with a written payment agreement or an authorization for services that includes, at a minimum,

information regarding provider payment policies, including rates, schedules, any fees charged to providers, and the dispute-resolution process: **Along with the provider agreement all new interested child care providers are sent a packet with the agreement. The packet includes market rates, the CCAP bi-weekly billing schedule, and billing fact sheets. The Department will pay the Child Care Provider within twenty-one (21) calendar days of receipt of an invoice/bill. The Department will not pay a Child Care Provider who does not submit a bill within sixty (60) calendar days of the Department established submission deadline. The State of Maine observes 13 paid holidays. (No fees are charged to Providers)**

The Lead Agency notifies providers at the time of any changes to a family's CCAP award. Billing cycle dates are posted and sent to providers ahead of schedule. If there is a repayment needed for an improper payment, the Lead Agency notifies the provider and agrees to a payment schedule that is followed.

Here is the information providers receive for the dispute-resolution process. Providers have the right to ask for an Administrative Hearing. If you want an informal conference with the State Child Care Administrator or his/her agent, you must request within ten (10) days of the receipt of notification by submitting a written request to:

State Child Care Administrator
Office of Child and Family Services, DHHS
11 State House Station, 2 Anthony Ave
Augusta, Me 04333-0011

Services shall be continued throughout the entire hearing process when the written requests for a state Administrative Hearing are received within ten (10) calendar days. The Hearings Department will render a decision within 30 days of the hearing.

- d. Describe how the Lead Agency provides prompt notice to providers regarding any changes to the family's eligibility status that could impact payments, and such a notice is sent no later than the day that the Lead Agency becomes aware that such a change will occur: **The Lead Agency notifies providers at the time of any changes to a family's CCAP award A two-week notice is sent via mail and when possible email at the same time.**
- e. Describe the Lead Agency's timely appeal and resolution process for payment inaccuracies and disputes: **The Lead Agency's CCAP Rules provide providers the Lead Agency's information regarding provider payment policies and the dispute-resolution process. The CCAP Provider Agreement provides the written agreement on information regarding provider payment policies, including rates, and schedules.**

Here is the information providers receive for the dispute-resolution process: Providers have the right to ask for an Administrative Hearing. If you want an informal conference with the State Child Care Administrator or his/her agent, you must request within ten (10) days of the receipt of notification by submitting a written request to:

State Child Care Administrator
Office of Child and Family Services, DHHS
11 State House Station, 2 Anthony Ave

Services shall be continued throughout the entire hearing process when the written requests for a state Administrative Hearing are received within ten (10) calendar days. The Hearings Department will render a decision within 30 days of the hearing.

- f. Other. Describe any other payment practices established by the Lead Agency: **N/A**

4.4.3 Payment practices and parent choice

How do the Lead Agency's payment practices facilitate provider participation in all categories of care? **The Lead Agency's payment practices are statewide and across all types of care. The reimbursement rates are differentiated by region and include reimbursements at a higher percentile for family child care to support participation of home-based providers. Additional reimbursements added above weekly reimbursement rates include a 35% bump for serving children with special needs and a 35% bump for serving children in need of off hour care between 6p.m. to 6 a.m on weekdays and on weekends available to all licensed child care programs.**

4.5 Supply Building

Building a supply of high-quality child care that meets the needs and preferences of parents participating in CCDF is necessary to meet CCDF's core purposes. Lead Agencies must support parent choice by providing some portion of direct services via grants or contracts, including at a minimum for children in underserved geographic areas, infants and toddlers, and children with disabilities.

4.5.1 Child care services available through grants or contracts

Does the Lead Agency provide direct child care services through grants or contracts for child care slots?

☐ Yes, statewide. Describe how the Lead Agency ensures that parents who enroll with a provider who has a grant or contract have choices when selecting a provider:

☐ Yes, in some jurisdictions, but not statewide. Describe how many jurisdictions use grants or contracts for child care slots and how the Lead Agency ensures that parents who enroll with a provider who has a grant or contract have choices when selecting a provider:

☒ No. If no, describe any Lead Agency plans to provide direct child care services through grants and contracts for child care slots: **The Lead Agency plans to build a fiscal plan to incorporate direct child care services through grants and contracts for child care slots by FFY 2026.**

If no, skip to question 4.5.2.

- i. If yes, identify the populations of children served through grants or contracts for child care slots (check all that apply). For each population selected, identify the number of slots allocated through grants or contracts for direct service of children receiving CCDF.

☐ Children with disabilities. Number of slots allocated through grants or contracts:

☐ Infants and toddlers. Number of slots allocated through grants or

contracts:

☐ Children in underserved geographic areas. Number of slots allocated through grants or contracts:

☐ Children needing non-traditional hour care. Number of slots allocated through grants or contracts:

☐ School-age children. Number of slots allocated through grants or contracts:

☐ Children experiencing homelessness. Number of slots allocated through grants or contracts:

☐ Children in urban areas. Percent of CCDF children served in an average month:

☐ Children in rural areas. Percent of CCDF children served in an average month:

☐ Other populations. If checked, describe:

- ii. If yes, how are rates for slots funded by grants and contracts determined by the Lead Agency?

4.5.2 Care in the child's home (in-home care)

The Lead Agency must allow for in-home care (i.e., care provided in the child's own home) but may limit its use.

Will the Lead Agency limit the use of in-home care in any way?

☒ Yes.

☐ No.

If yes, what limits will the Lead Agency set on the use of in-home care? Check all that apply.

- i. ☐ Restricted based on the minimum number of children in the care of the in-home provider to meet the Fair Labor Standards Act (minimum wage) requirements. Describe:
- ii. ☒ Restricted based on the in-home provider meeting a minimum age requirement. Describe: **In-home provider must be 18.**
- iii. ☒ Restricted based on the hours of care (i.e., certain number of hours, non-traditional work hours). Describe: **Max of 50 hours in care and must meet Maine's minimum wage.**
- iv. ☐ Restricted to care by relatives. (A relative provider must be at least 18 years of age based on the definition of eligible child care provider.) Describe:
- v. ☐ Restricted to care for children with special needs or a medical condition. Describe:
- vi. ☐ Restricted to in-home providers that meet additional health and safety requirements beyond those required by CCDF. Describe:
- vii. ☐ Other. Describe:

4.5.3 Shortages in the supply of child care

Lead Agencies must identify shortages in the supply of child care providers that meet parents' needs and preferences.

What child care shortages has the Lead Agency identified in the State or Territory, and what is the plan to address the child care shortages?

- a. In infant and toddler programs:
 - i. Data sources used to identify shortages: **The Lead Agency in partnership with the Bipartisan Policy Center (BPC) completed a child care gap analysis in early 2023. The Lead Agency launched a monthly enrollment data collection program in May of the same year.**
 - ii. Method of tracking progress: **The Lead Agency continues to analyze the data of the enrollment against the capacity of the program to identify percentage of programs infant and toddler total enrollment.**
 - iii. What is the plan to address the child care shortages using family child care homes **The Lead Agency has the Child Care Infrastructure Grants (CCIG) utilizing funding through the Maine Jobs and Recovery Plan (MJRP) that includes priority points for programs caring for infants and toddlers and CCAP. The program is scheduled to end December 2024. Since the start in July of 2022 nearly 200 awards have been made with more than 3,100 slots created. The Lead Agency's goal is to incorporate the cost modeling analysis data along with the enrollment data to create additional efforts to incentivize providers to care for infants and toddlers statewide and increase the total number of infants and toddlers served on the CCAP by FFY2026.**
 - iv. What is the plan to address the child care shortages using child care centers? **The Lead Agency has the Child Care Infrastructure Grants (CCIG) utilizing funding through the Maine Jobs and Recovery Plan (MJRP) that includes priority points for programs caring for infants and toddlers and CCAP. The program is scheduled to end December 2024. Since the start in July of 2022 nearly 200 awards have been made with more than 3,100 slots created. The Lead Agency's goal is to incorporate the cost modeling analysis data along with the enrollment data to create additional efforts to incentivize providers to care for infants and toddlers statewide and increase the total number of infants and toddlers served on the CCAP by FFY2026.**
- b. In different regions of the State or Territory:
 - i. Data sources used to identify shortages: **The Lead Agency in partnership with the Bipartisan Policy Center (BPC) completed a child care gap analysis in 2019 and 2022, with some updated data analyzed in early 2023.**
<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/Maine-Child-Care-Gaps-January2023.pdf>
 - ii. Method of tracking progress: **Our gap analysis work with BPC has shown that our largest gap by number of children is in Aroostook County, followed by Penobscot County, York County, Waldo County, Oxford County, Hancock County, and**

Washington County. Using a distanced-based methodology our largest gap is in York County, followed by Androscoggin, Penobscot, Aroostook, Oxford, Somerset, Hancock, and Waldo counties. 115 of Maine's Child Care Infrastructure Grant awards, over 57% of the awards, have gone to these counties, to help address child care shortages.

- iii. What is the plan to address the child care shortages using family child care homes? The Lead Agency has the Child Care Infrastructure Grants (CCIG) utilizing funding through the Maine Jobs and Recovery Plan (MJRP) that includes priority points for programs in areas identified as having the highest gaps. The funding ends December 2024. Since the start in July of 2022 over 200 awards have been made with over 5,000 anticipated slots.

Over 90 of the Child Care Infrastructure Grant awards have gone to family child care start ups and family child care expansions, with about 945 anticipated new slots in family child care homes. The Lead Agency has partnered with CEI on this grant program. CEI continues to run a Child Care Business Lab in Maine that is helping to start new child care programs with a focus on small facilities and family child care programs.

The Lead Agency plans to utilize the gap analysis, updated with enrollment data, along with CELFE's cost modeling analysis to ensure an increase in access.

- iv. What is the plan to address the child care shortages using child care centers? The Lead Agency has the Child Care Infrastructure Grants (CCIG) utilizing funding through the Maine Jobs and Recovery Plan (MJRP) that includes priority points for programs in areas identified as having the highest gaps. The funding ends December 2024. Since the start in July of 2022 over 200 awards have been made with over 5,000 anticipated slots.

118 Child Care Infrastructure Grant awards have gone to child care facility start ups and child care facility expansions, with 4,126 anticipated new slots in child care centers. The Lead Agency has partnered with CEI on this grant program. CEI continues to run a Child Care Business Lab in Maine that is helping to create new child care programs, with a focus on small facilities, including child care centers.

The Lead Agency plans to utilize the gap analysis, updated with enrollment data, along with CELFE's cost modeling analysis to ensure an increase in access.

- c. In care for special populations:

- i. Data sources used to identify shortages: The CCDF Administrator receives a monthly report from MACWIS that tracks identified special populations that include children with a special needs or experiencing homelessness.
- ii. Method of tracking progress: Maine is seeing a steady increase in the number of children with special needs served in child care programs, from an average of 140 a few years ago, to 230 in recent months. This past year the Child Care Affordability Program served anywhere from 13 to 21 families each month who were experiencing homelessness.

- iii. What is the plan to address the child care shortages using family child care homes? **The Lead Agency has ongoing efforts of offering support to family child care programs through technical assistance on caring for children in special populations. This includes the Early Childhood Consultation Partnership (ECCP) program, MRTQ PDN's technical assistance system, and support from the Child Care Affordability Program team.**

The Lead Agency is working with Maine DOE staff and Maine Children's Cabinet staff to look at how to grow access to child care family child care programs and publicly funded preschool for children with special needs. We are partnering on mixed delivery preschool strategies and initiatives as Maine transitions the Free and Appropriate Public Education responsibility from a quasi-governmental agency to local school districts.

- iv. What is the plan to address the child care shortages using child care centers? **The Lead Agency has ongoing efforts of offering support to child care centers through technical assistance on caring for children in special populations. This includes the Early Childhood Consultation Partnership (ECCP) program, MRTQ PDN's technical assistance system, and support from the Child Care Affordability Program team.**

The Lead Agency is working with Maine DOE staff and Maine Children's Cabinet staff to look at how to grow access to child care centers and publicly funded preschool for children with special needs. We are partnering on mixed delivery preschool strategies and initiatives as Maine transitions the Free and Appropriate Public Education responsibility from a quasi-governmental agency to local school districts.

4.5.4 Strategies to increase the supply of and improve quality of child care

Lead Agencies must develop and implement strategies to increase the supply of and improve the quality of child care services. These strategies must address child care in underserved geographic areas; infants and toddlers; children with disabilities, as defined by the Lead Agency; and children who receive care during non-traditional hours.

How does the Lead Agency identify any gaps in the supply and quality of child care services and what strategies are used to address those gaps for:

- a. Underserved geographic areas. Describe: **Our gap analysis work with BPC has shown that our largest gap by number of children is in Aroostook County, followed by Penobscot County, York County, Waldo County, Oxford County, Hancock County, and Washington County. Using a distanced-based methodology our largest gap is in York County, followed by Androscoggin, Penobscot, Aroostook, Oxford, Somerset, Hancock, and Waldo counties. 115 of Maine's Child Care Infrastructure Grant awards, over 57% of the awards, have gone to these counties, to help address child care shortages.**

In looking at Maine's Rising Stars for ME data, we have opportunities to grow quality in several regions of the state where there are less programs with Star 3, Star 4, and/or Star 5 quality ratings. We will focus efforts in several counties, including Aroostook, Hancock, Piscataquis, Penobscot, Waldo and York counties. Some of these counties overlap with

those identified above in regions with higher needs for increased child care supply.

Overall quality updates: The Lead Agency revised the state's quality rating and improvement system in the spring of 2023 called Rising Stars for ME (RSM). RSM's system changes included nonrelative license-exempt providers to the star rating as a Star 1 and encumbering QRIS rules. Through PDG B-5 funds, MRTQ PDN has Quality Awards for providers moving up Star ratings. In addition, Maine has the Early Childhood Educator Workforce Salary Supplement System that provides salary supplements to direct child care staff based on a tiered system for higher levels on the MRTQ Registry.

- b. **Infants and toddlers. Describe:** In Maine's Child Care Affordability program, from July 2023-June 2024, 4-5% of children served each month were infants. 21-23% were toddlers, 29-36% were preschoolers, and 38-45% were school age. Over the past year, the Office of Child and Family Services has been collecting enrollment data and has found that of children enrolled in licensed child care in Maine approximately 8% are infants, 23% are toddlers, 40% are preschoolers, and 29% are school age. Maine is working to increase the supply of infant and toddler early care and education.

The Lead Agency has an infant and toddler initiative through the Child Care Affordability Program with early childhood bumps for increased reimbursement to honor the higher costs of serving infants and toddlers. In addition, there is a weekly stipend of up to \$100 per infant to all licensed providers participating in the Quality Rating and Improvement System (QRIS), who are in compliance with health and safety requirements and are providing full- or part-time care through the Child Care Affordability Program. This infant and toddler initiative has supported between 800 and 1,000 infants and toddlers per month, with 992 infants and toddlers in CCAP in June 2024 whose providers were receiving these additional funds.

Overall quality updates: The Lead Agency revised the state's quality rating and improvement system in the spring of 2023 called Rising Stars for ME (RSM). RSM's system changes included nonrelative license-exempt providers to the star rating as a Star 1 and encumbering QRIS rules. Through PDG B-5 funds, MRTQ PDN has Quality Awards for providers moving up Star ratings. In addition, Maine has the Early Childhood Educator Workforce Salary Supplement System that provides salary supplements to direct child care staff based on a tiered system for higher levels on the MRTQ Registry.

- c. **Children with disabilities. Describe:** Maine provides services for both Early Intervention (birth through 2 years, sometimes referred to as Part C services) and Appropriate Public Education (for ages 3-5 years, sometimes referred to as Part B/Section 619 services) through Child Development Services (CDS), a separate intermediate unit under the supervision of the Maine Department of Education. CDS consists of nine regional sites and a state office. The state CDS office maintains a central data management system and systemwide policies and procedures, and provides centralized fiscal services for regional CDS sites. Through the regional sites, CDS conducts Child Find to identify children eligible for services, addresses referrals, conducts evaluations to determine diagnoses, and provides services for eligible children.

In FY2237 (the most recently available summary data), of the individual 8,979 children receiving direct services through CDS, 29% were in the 0-2 age group receiving early

intervention for developmental delays. The total represents about a 6% increase in children served over the previous year. In the same time period, 3,716 children entered the CDS system (about 40% of the total are ages 0–2) and 3,240 exited the system. The most frequent primary disability for children ages 3–5 was speech or language impairment (46%) followed by autism (24%). About half of children served had Maine Care coverage for all or part of services.

CDS services for young children are provided directly by CDS staff, through contracted providers, and in a small number of preschool programs operated at regional CDS sites. Increasingly, 4-year-old children with special needs are served in public pre-K programs provided by school districts.

Through a relatively recent initiative, CDS has established nine preschools in Southern Maine designed to provide an inclusive education (with others under exploration) and has been working with SAUs to provide services to children in public pre-K through a growing number of contracted arrangements, now partnering with over one third of Maine’s SAUs. CDS reports that staffing these sites and encouraging participation from nondisabled peers remains challenging.

Staff from Maine DHHS and DOE have been focused on developing strategies for increase the supply and improve the quality of early care and education for children with disabilities. The following inclusion and special education needs of children, their families, and early educators were identified in our Preschool-Development Grant needs assessment, developed from 2023-2024.

- While efforts have been made on several fronts to expand early screening, Maine still has a relatively low rate of early identification compared to other states.
- Parents of children with delays and disabilities find systems of support difficult to access and navigate, facing long wait times at each stage of the process as well as frustrations associated with inaccurate diagnoses, and lack of service access and availability.
- Transitions can be especially difficult for children with special needs and their families (from Part C to Part B; preschool to K) because of gaps in receiving services.
- New American families in Maine with children with delays and disabilities have unique needs.
- All teachers need training in working with children with special needs and understanding appropriate behavioral expectations.
- Shortages of staff with special needs preparation and certification is at the root of capacity to serve students adequately, in part the result of low compensation and underfunding.

Through our Preschool Develop Grant (PDG) work, we are increasing inclusion training in Maine for staff working in child care, including a free on demand inclusion training and increased support for cohorts of early childhood educators to earn their Inclusions Credential.

The Lead Agency is working with Maine DOE staff and Maine Children's Cabinet staff to look at how to grow access to child care family child care programs and publicly funded preschool for children with special needs. We are partnering on mixed delivery preschool strategies and initiatives as Maine transitions the Free and Appropriate Public Education

responsibility from a quasi-governmental agency to local school districts.

- d. Children who receive care during non-traditional hours. Describe: **Staff from Maine DHHS and DOE have been focused on developing strategies for increasing the supply and improving the quality of early care and education for children and families in Maine. The following key takeaway in regard to meeting the early care and education needs of children and families came from Maine's Preschool-Development Grant needs assessment, developed from 2023-2024.**
- While there has been progress in reducing the overall gap between demand and supply of early care and education slots, more options are needed for rural areas, care outside traditional daytime hours, and students with special needs.

Maine data on non-traditional hours offered within the Child Care Affordability Program are as follows: In FFY 2024, 6284 children were served by 1268 CCAP providers across the state for off-hours care for a total of 26170.4 hours. This total off-hours care paid for by CCAP in FFY24 has a cost of \$38,095.99.

In 2024, year to date, Maine had 893 child care providers providing care during non-traditional hours through the Child Care Affordability Program, 78% of providers in the program. The Lead Agency tracks every month the providers and children served through off-hours care in CCAP. We includes additional payments to support non-traditional hours and are working to expand access. The Lead Agency will be analyzing the availability of non-traditional hours by region and by quality level to help focus targeted outreach and support to specific areas of the state.

- e. Other. Specify what population is being focused on to increase supply or improve quality. Describe: **N/A**

4.5.5 Prioritization of investments in areas of concentrated poverty and unemployment

Lead Agencies must prioritize investments for increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment and do not currently have sufficient numbers of such programs.

Describe how the Lead Agency prioritizes increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment and that do not have access to high-quality programs. **The Lead Agency has the Child Care Infrastructure Grants (CCIG) utilizing funding through the Maine Jobs and Recovery Plan (MJRP) that includes priority points for programs in areas identified as having the highest gaps and identified as areas of concentration of poverty. Since the start in July of 2022 nearly 200 awards have been made with more than 3,100 slots created. Data is then compared to other known sources like MACWIS, Maine's Children's Alliance Annual Report, and the Child Care Enrollment Data Project.**

5 Health and Safety of Child Care Settings

Child care health and safety standards and enforcement practices are essential to protect the health and safety of children while out of their parents' care. CCDF provides a minimum threshold for child care health and safety policies and practices but leaves authority to [Lead Agencies](#) to

design standards that appropriately protect children’s safety and promote nurturing environments that support their healthy growth and development. Lead Agencies should set standards for ratios, group size limits, and provider qualifications that help ensure that the child care environment is conducive to safety and learning and enable caregivers to promote all domains of children’s development.

CCDF health and safety standards help set clear expectations for CCDF providers, form the foundation for health and safety training for child care workers, and establish the baseline for monitoring to ensure compliance with health and safety requirements. These health and safety requirements apply to all providers serving children receiving CCDF services – whether the providers are licensed or license-exempt, must be appropriate to the provider setting and age of the children served, must include specific topics and training on those topics, and are subject to monitoring and enforcement procedures by the [Lead Agency](#). CCDF-required annual monitoring and enforcement actions help ensure that CCDF providers are adopting and implementing health and safety requirements.

Through child care licensing, [Lead Agencies](#) set minimum requirements, including health and safety requirements, that child care providers must meet to legally operate in that State or Territory. In some cases, CCDF health and safety requirements may be integrated within the licensing system for licensed providers and may be separate for CCDF providers who are license-exempt.

This section addresses CCDF health and safety requirements, [Lead Agency](#) licensing requirements and exemptions, and comprehensive background checks.

When responding to questions in this section, OCC recognizes that each [Lead Agency](#) identifies and defines its own categories of care. OCC does not expect [Lead Agencies](#) to change their definitions to fit the CCDF-defined categories of care. For these questions, provide responses that best match the CCDF categories of care.

5.1 Licensing Requirements

Each Lead Agency must ensure it has in effect licensing requirements applicable to all child care services provided within the State/Territory (not restricted to providers receiving CCDF funds).

5.1.1 Providers subject to licensing

For each category of care listed below, identify the type of providers subject to licensing and describe the licensing requirements.

- a. Identify the center-based provider types subject to child care licensing: **Per 22 M.R.S. §8301-A(1-A)(B), means a Child care center, nursery school, or small Child care facility, as defined in §§8301-A(1-A)(A), (F), and (E), respectively, as well as any program for Children under 5 located in a private school and programs that contract with one or more Child Development Services Systems sites (22 M.R.S. §8301-A(1-A)(B)).**

Are there other categories of licensed, regulated, or registered center providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

- b. Identify the family child care providers subject to licensing: **Family Child Care Licensure is required for any individual providing care and protection for three (3) or more children under thirteen (13) years of age who are not the children of the provider or who are not residing in the provider's home.**

Are there other categories of regulated or registered family child care providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

- c. Identify the in-home providers subject to licensing: **N/A**

Are there other categories of regulated or registered in-home providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

5.1.2 CCDF-eligible providers exempt from licensing

Identify the categories of CCDF-eligible providers who are exempt from licensing requirements, the types of exemptions, and describe how these exemptions do not endanger the health, safety, and development of children. -Relative providers, as defined in CCDF, are addressed in subsection 5.8.

- a. License-exempt center-based child care. Describe by answering the questions below.

- i. Identify the categories of CCDF-eligible center-based child care providers who are exempt from licensing requirements. **10-148 CMR Ch. 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

A License-Exempt Child Care Provider is a Recreational program through a municipality and has enrolled in Rising Stars for ME.

G. License-Exempt Child Care Providers: Recreational Programs

1.Recreational programs participating in CCAP as License-Exempt Child Care Providers must comply with all requirements of this rule.

2.Recreational Programs must also comply with the following requirements:

a. Maintain a maximum Group size of fifty (50) for School Age children.

b. Maintain a ratio of one staff for every 15 Children (1:15).

- ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **A License-Exempt Child Care Provider is a Recreational program through a municipality.**

Statute exempts the Recreation program from licensing requirements when it is operated by a municipality. Recreation programs in Maine operated by municipalities provide out of school time care onsite in school district buildings.

The recreation programs follow the health and safety guidelines set by the school district for school-age care.

- iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **A License-Exempt Child Care Provider is a Recreational program through a municipality. Statute exempts the Recreation program from licensing requirements when it is operated by a municipality. Recreation programs in Maine operated by municipalities provide out of school time care onsite in school district buildings. The recreation programs follow the health and safety guidelines set by the school district for school-age care.**

Recreational programs participating in CCAP as License-Exempt Child Care Providers must comply with all requirements of the Child Care Affordability Program Rule, including CCDF required health and safety training and an annual site visit from Child Care Licensing.

- b. License-exempt family child care. Describe by answering the questions below.
 - i. Identify the categories of CCDF-eligible family child care providers who are exempt from licensing requirements. **License-exempt family child care is for any individual providing care for one or two children under thirteen (13) years of age who are not the children of the provider or who are not residing in the provider's home. If care is provided for 3 or more children, who are not the children of the provider, than licensure is required.**
 - ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **The rules are implemented through Lead Agency. Statute exempts an individual from licensure who is caring for one or two children, who are not their own, on a regular basis.**
 - iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **Nonrelative CCAP License-Exempt providers must complete the CCDF required health and safety training and receive an annual site visit from Child Care Licensing.**
- c. In-home care (care in the child's own home by a non-relative). Describe by answering the questions below.
 - i. Identify the categories of CCDF-eligible in-home care (care in the child's own home by a non- relative) providers who are exempt from licensing requirements. **A Non-Relative In-home Child Care Provider, who provides Child Care Services in the Child's home and must be: eighteen (18) years of age or older, a Maine resident, and providing Child Care Services to a family unit receiving CCAP. In addition, the provider cannot care for more than two (2) of their own children while providing services.**

A Relative In-home Child Care Provider, who is a Child(ren)'s grandparent, great grandparent, sibling (if living in a separate residence), aunt, or uncle who provides Child Care Services in the Child's home and must be: eighteen (18) years of age or older, a Maine resident, and providing Child Care Services to a family unit receiving CCAP. In addition, the provider cannot care for more than two (2) of their own children while providing services

- ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **The rules are implemented through the Lead Agency. Current statute exempts any individual caring for two (2) or fewer children on a regular basis and for consideration from licensure.**
- iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **Nonrelative CCSP License-Exempt providers must complete the CCDF required health and safety training and receive an annual site visit from Child Care Licensing.**

5.2 Ratios, Group Size, and Qualifications for CCDF Providers

Lead Agencies must have child care standards for providers receiving CCDF funds, appropriate to the type of child care setting involved, that address appropriate staff:child ratios, group size limits for specific age populations, and the required qualifications for providers. Lead Agencies should map their categories of care to the CCDF categories. Exemptions for relative providers will be addressed in subsection 5.8.

5.2.1 Age classifications

Describe how the **Lead Agency** defines the following age classifications (e.g., Infant: 0 – 18 months).

- a. Infant. Describe: **6 weeks to 12 months old**
- b. Toddler. Describe: **13 - 36 months old**
- c. Preschool. Describe: **3 - 5 years old**
- d. School-Age. Describe: **at least 5 and under 13 years old**

5.2.2 Ratio and group size limits

Provide the ratio and group size limits for settings and age groups below.

- a. Licensed CCDF center-based care:
 - i. Infant.
 - Ratio: **1:4**
 - Group size: **8**
 - ii. Toddler.
 - Ratio: **1:4 or 1:5**
 - Group size: **10-12**
 - iii. Preschool.
 - Ratio: **1:7, 1:8, or 1:10**
 - Group size: **21, 24, or 20**
 - iv. School-Age.

Ratio: **1:13**

Group size: **50**

- v. Mixed-Age Groups (if applicable).

Ratio: **It is based on the age of the youngest child in attendance.**

Group size: **It is based on the age of the youngest child in attendance.**

- b. If different, provide the ratios and group size requirements for the license-exempt center-based providers who receive CCDF funds under the following age groups:

- i. **[x]** Not applicable. There are no differences in ratios and group size requirements.

- ii. Infant:

- iii. Toddler:

- iv. Preschool:

- v. School-Age:

- vi. Mixed-Age Groups:

- c. Licensed CCDF family child care home providers:

- i. Infant (if applicable)

Ratio: **1:4, or one provider may care for three infants, three preschoolers and two school age children. Two providers may care for eight infants, or three providers can care for twelve children.**

Group size: **4, 8, 12**

- ii. Toddler (if applicable)

Ratio: **One provider may care for three toddlers, three preschoolers and two school age children, two providers can care for six toddlers and six preschoolers.**

Group size: **8 or 12**

- iii. Preschool (if applicable)

Ratio: **One provider may care for eight preschoolers and two school age children; two providers may care for twelve preschoolers.**

Group size: **10 or 12**

- iv. School-Age (if applicable)

Ratio: **1:12**

Group size: **12**

- v. Mixed-Age Groups

Ratio: **One provider may care for three infants, three preschoolers and two school age children; or one provider may care for eight preschoolers and two school age children. Two providers may care for eight infants, two providers can care for twelve preschool children, or two providers can**

care for six children under two and six children over two, or three providers can care for twelve children.

Group size: **8, 10, 12**

- d. Are any of the responses above different for license-exempt family child care homes?

☐ No.

☒ Yes. If yes, describe how the ratio and group size requirements for license-exempt providers vary by age of children served. **There are no rules established for exempt providers with the exception that a License- Exempt Provider may not care for more than two children that do not reside in the home.**

☐ Not applicable. The Lead Agency does not have license-exempt family child care homes.

- e. Licensed in-home care (care in the child's own home):

- i. Infant (if applicable)

Ratio: **N/A**

Group size: **N/A**

- ii. Toddler (if applicable)

Ratio: **N/A**

Group size: **N/A**

- iii. Preschool (if applicable)

Ratio: **N/A**

Group size: **N/A**

- iv. School-Age (if applicable)

Ratio: **N/A**

Group size: **N/A**

- v. Mixed-Age Groups (if applicable)

Ratio: **N/A**

Group size: **N/A**

- f. Are any of the responses above different for license-exempt in-home care?

☐ No.

☒ Yes. If yes, describe how the ratio and group size requirements for license-exempt in-home care vary by age of children served. **A Non-Relative In-home Child Care Provider, who provides Child Care Services in the Child's home and must be: eighteen (18) years of age or older, a Maine resident, and providing Child Care Services to a family unit receiving CCAP. In addition, the provider cannot care for more than two (2) of their own children while providing services.**

5.2.3 Teacher/caregiver qualifications for licensed, regulated, or registered care

Provide the teacher/caregiver qualifications for each category of care.

a. Licensed center-based care

- i. Describe the teacher qualifications for licensed CCDF center-based care (e.g., degrees, credentials, etc.), including any variations based on the ages of children in care: **All staff shall have a high school diploma or equivalent or be attending high school or be enrolled in a GED (General Educational Development) preparation program or HiSET (Highschool Equivalency Test). Lead Teacher: The lead teacher or person having the primary responsibility for a group of children in a program with thirteen (13) or more children shall be at least eighteen (18) years of age and meet one of the following; 1. Child Development Associate (CDA) as awarded by the CDA National Credentialing Program or a Maine State-approved credential, or 2. Twelve (12) months of direct Child Care experience or, 3. One year (30 credit hours) of college work including one course in a child related subject and 6 months experience.**
- ii. Describe the director qualification for licensed CCDF center-based care, including any variations based on the ages of children in care or the number of staff employed: **Facilities licensed for 13-20 Children must employ a minimum of one director or lead teacher. The Director/lead teacher may be the owner of the Child Care Facility. The director and/or lead teacher must be at least 21 years of age and must meet one of the following requirements: Graduation from an accredited high school or its equivalent, and 12 months of employment in a licensed Child Care Program and 12 hours of training in healthy, safe environments; Child development; observation and assessment; developmentally appropriate practice; guidance; relationships with families; individual and cultural diversity; Children with special needs; business and professional development, Child care, or early Childhood education; or 30 credit hours of college courses including six credit hours in early Childhood education or closely related subjects and six months of experience (employment or college practicum) in a licensed Child Care Program; or Current Child Development Associate (CDA) credential as awarded by the Council for Professional Recognition, or a Maine State-approved credential, and six months of direct Child care experience (employment or college practicum). Facilities licensed for 21 – 49 Children must employ a Director and/or lead teacher who is at least 21 years of age, and meets one of the following requirements: An Associate Degree in Arts/Associate in Science (AA/AS) in Early Childhood Education; or An AA/AS with 12 credits in Early Childhood Education or a Department-approved related field, and two years of direct Childcare experience; or 30 college credits in Early Childhood Education, and one year of direct Child care experience; or Child Development Associate (CDA) as awarded by the Council for Professional Recognition or a Maine State-approved credential, and three years' direct Child care experience; or Five years of direct Child care experience, and 135 hours of training in early childhood education including healthy, safe, and inclusive environments; Child development; observation and assessment; developmentally appropriate practice; guidance; relationships with families; and cultural diversity. Facilities licensed for 50 or more Children must employ a Director and/or lead teacher who is at least 21 years of age, and meets one of the following requirements: A Bachelor of Science/Bachelor of Arts**

(BA/BS) in Early Childhood Education; or A BA/BS in a Department-approved related field with 18 credit hours in Early Childhood Education and three years of direct Child care experience; or An Associate in Arts/Associate in Science (AA/AS) in Early Childhood Education and three years of direct Child care experience; or An AA/AS in a related field with 18 credit hours in Early Childhood Education and three years of direct Childcare experience; Current CDA as awarded by the Council for Professional Recognition or a Maine State-approved credential, with five years of direct Childcare experience; Seven years of experience and 180 hours of training in the topics listed in Section 7(F)(4)(e) above; or Meet Level 5, 6, 7, or 8 on the Maine Roads to Quality Career Lattice. Directors and/or lead teachers of school-age Child Care Facilities must have an AA/AS in Early Childhood Education or 30 college credits in a closely related field such as elementary education, Child development, or recreation management.

b. Licensed family child care

Describe the provider qualifications for licensed family child care homes, including any variations based on the ages of children in care: **The applicant must be at least 18 years old. A provider must be at least 16 years old, and, if younger than 18 years old, must be under the direct supervision of an adult. The provider and all Staff Members must have a high school diploma or equivalent, be attending high school, or be enrolled in a General Educational Development (GED) or HISET (High School Equivalency Test) preparation program. Either the provider or at least one staff member must hold a current certification in adult and pediatric first aid and Cardio-Pulmonary Resuscitation (CPR) and have completed at least 6 hours of pre-licensing training in healthy, safe environments; Child development; observation and assessment; developmentally appropriate practice; guidance; relationships with families; individual and cultural diversity; children with special needs, business, and professional development; or childcare practices.**

c. Licensed, regulated, or registered in-home care (care in the child's own home by a non-relative)

Describe the provider qualifications for licensed, regulated, or registered in-home care providers (care in the child's own home) including any variations based on the ages of children in care: **N/A**

5.2.4 Teacher/caregiver qualifications for license-exempt providers

Provide the teacher/provider qualification requirements (for instance, age, high school diploma, specific training, etc.) for the license-exempt providers under the following categories of care:

a. License-exempt center-based child care. **License-exempt CCAP providers providing center-based care are recreation programs operated by municipalities and are required to have staff that are Maine Residents, 18 years of age or older. The programs must meet all CCDF requirements, including background checks for staff, training requirements, health and safety standards, and annual inspection visits.**

Recreational Programs must provide a copy of the owner or Director's background check approval letter and a statement that they will have all staff background checks on file for viewing during inspections.

Allow the Department to perform at least annual health and safety monitoring visit. The Department may also inspect License-Exempt Child Care Providers' facilities upon receipt of a report or complaint, and to conduct random health and safety inspections, all with or without notice at the Department's sole discretion.

- b. License-exempt home-based child care. License-exempt CCAP providers providing home-based care are required to be eighteen (18) years of age or older, a Maine Resident, and meet CCDF requirements, including background checks, training requirements, and health and safety standards.
- c. License-exempt in-home care (care in the child's own home). License-exempt CCAP providers providing in-home care are required to be eighteen (18) years of age or older, a Maine Resident, and meet CCDF requirements, including background checks, training requirements, and health and safety standards.

5.3 Health and Safety Standards for CCDF Providers

Lead Agencies must have health and safety standards for providers serving children receiving CCDF assistance relating to the required health and safety topics as appropriate to the provider setting and age of the children served. This requirement is applicable to all child care programs receiving CCDF funds regardless of licensing status (i.e., licensed or license-exempt). The only exception to this requirement is for relative providers, as defined by CCDF. Lead Agencies have the option of exempting certain relatives from any or all CCDF health and safety requirements.

Exemptions for relative providers' standards requirements will be addressed in question 5.8.1.

Describe the following health and safety standards for programs serving children receiving CCDF assistance on the following topics (note that monitoring and enforcement will be addressed in subsection 5.5):

5.3.1 Prevention and control of infectious diseases (including immunizations) health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the prevention and control of infectious diseases for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 2

F. Administration

6. A copy of the Child Care Facility handbook will be made available to all parents or legal guardians at the time of enrollment. The handbook must contain, at a minimum:

o. Management of communicable illnesses;

9. The Child Care Facility must maintain a Child Care Facility Staff manual that addresses the following:

a. Policies and their implementation. This must include, at a minimum, policies for the following:

iii. Child illness,

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 12

D. Health monitoring. The Child Care Facility must observe the Child each day at the time of arrival and throughout the Child's stay for obvious signs of illness such as fever, diarrhea, vomiting, or skin rashes.

2. When a Child Care Facility knows or suspects that a Child has contracted a notifiable communicable disease or condition the Child Care Facility must notify the Maine Center for Disease Control and Prevention (MECDC).

a. The Child Care Facility must notify the MECDC immediately by phone for Category 1 conditions, and within 48 hours for Category 2 conditions. MECDC contacts: Phone: 1-800-821-5821 (24 hours a day); FAX: 1-800-293-7534 (24 hours

a day); or TTY: Maine relay 711 (24 hours a day).

b. For a list of Category 1 and 2 notifiable conditions, see 10-144 CMR Chapter 258, Rules for the Control of Notifiable Conditions, Chapter 2(l).
<http://www.maine.gov/sos/cec/rules/10/144/144c258.doc>

E. Dismissal of Children due to illness. When a Child becomes ill, but does not require immediate medical help, the Child Care Facility must determine if the Child should be sent home. The Child Care Facility must notify the parent/guardian of the Child who has symptoms that require exclusion.

I. Handwashing. Handwashing must be done with soap and running water. The provider must ensure that all Adults and all Children wash their hands in at least the following circumstances:

1. Immediately before and after eating snacks and meals, including washing the hands of all Infants and toddlers before all feedings;
2. After each diaper change or toileting;
3. Before and after handling food; and
4. Before and after administering Medication.

SECTION 14

A. General condition of the building and surrounding premises. The facility must take immediate steps to correct any condition in the physical building or on the premises, which poses a danger to Children's life, health or safety.

10. Toys and equipment must be kept clean and in safe working order.

13. The Child Care Facility must Clean surfaces daily in all areas where Children are present. Cleaning products must not be used in close proximity to Children and adequate ventilation must be maintained during use.

L. Cribs, cots, and bedding.

3. Cribs, bassinets and play yards for Infants less than 12 months old must not include soft or loose bedding including, but not limited to: bumper pads, pillows, quilts, comforters, blankets, sleep positioning devices, bibs or stuffed toys. Appropriately fitted arms-free wearable blankets are permitted.

a. Cots, mats, play yards, bassinets, and cribs used by any Infant or toddler must be Cleaned and sanitized between use by different Children, when contaminated, or at a minimum of once per week.

c. Cribs, cots, beds, mats, or play yards used for sleeping must be spaced at least 2 feet apart or separated by a solid divider on one side.

4. A separate mat, bed or cot, with a blanket or sleeping bag, must be assigned to each Child under the age of five in care for longer than four hours. All bedding must be cleaned before being used by another Child. Bedding must be cleaned as needed, or at least weekly.

a. Blankets, sleeping bags, bedding, cots, and mats must be stored in a manner which ensures that sleeping surfaces are not touching or must be washed and disinfected before re-use if stored in a manner that allows sleeping surfaces to touch during storage.

M. Toilet facilities. Washing and toilet facilities with running water and soap must

be provided.

5. Either disposable paper products, or clean, individually assigned wash cloths and towels may be used. Individually assigned washcloths and towels must be laundered daily.

N. Diapering/toileting.

2. Wet or soiled disposable diapers must be placed in a lined, lidded container, separate from other trash or garbage and separate from play or food-preparation areas, or individually bagged and tied within a lidded shared garbage container.

3. Diapering must be done on a changing table or washable vinyl mat which is Cleaned after each use or which has a disposable single-use covering.

4. The changing table or mat must not be located in the kitchen or food preparation area.

5. Staff must wash their hands and the Child's hands with soap and running water after each diaper change.

8. Potty chair receptacles must be emptied immediately after each use, rinsed, and sanitized in a sink not used for food preparation.

SECTION 16

B. Handwashing. All adults and all Children must wash their hands immediately prior to preparing or consuming snacks and meals.

D. Child Care Staff Member Responsibilities

1. Child Care Staff Members who are ill must not work in the food preparation area. Child Care Staff Members with open sores that cannot be covered must not handle or prepare food.

G. Equipment and Utensils

1. All food preparation and eating surfaces must be Cleaned before and after use.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 2

G. Administration

4. A copy of the Provider handbook will be made available to all Parents or Legal Guardians at the time of enrollment. The handbook must contain, at a minimum:

o. Management of communicable illnesses;

7. The Provider must maintain a staff manual that addresses the following:

a. Policies and their implementation. This must include, at a minimum, policies for the following:

iii. Child illness,

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff

Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.
2. This Rule;
3. The Provider's policies and procedures;
4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
6. Prevention of and response to food and allergy emergencies;
7. Building and physical premise safety;
8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
11. Adult and pediatric first aid with hands-on skills demonstration;
12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 12

D. Health monitoring. The Provider must observe Children in care each day at the time of arrival and throughout the Child's stay for obvious signs of illness such as fever, diarrhea, vomiting, or skin rashes. In the event of an apparent illness of a Child, the Provider must comply with the guidance in the manual described in Section 14(C)(1) below.

When a Provider knows or suspects that a Child has contracted a notifiable disease or condition, the Provider must notify the Maine Center for Disease Control and Prevention (MECDC). The Provider must notify the MECDC immediately by phone for Category 1 conditions, and within 48 hours for Category 2 conditions. MECDC contacts: Phone: 1-800-821-5821 (24 hours a day); FAX: 1-800-293-7534 (24 hours a day); or TTY: Maine relay 711 (24 hours a day). For a list of Category 1 and 2 notifiable diseases and conditions, see 10-144 CMR Chapter 258, Rules for the Control of Notifiable Conditions, Chapter 2(I).
<http://www.maine.gov/sos/cec/rules/10/144/144c258.doc>

E. Dismissal of children due to illness. When a Child becomes ill, but does not require immediate medical help, the provider must determine if the Child should be sent home. The provider must notify the Parent/Legal Guardian of the Child who has symptoms that require exclusion.

I. Handwashing. Handwashing must be done with soap and running water. The provider must ensure that all Adults and all Children wash their hands in at least the following circumstances:

1. Immediately before and after eating snacks and meals, including washing the hands of all Infants and toddlers before all feedings;
2. After each diaper change or toileting;
3. Before and after handling food; and
4. Before and after administering Medication.

SECTION 14

A. General condition of the building and surrounding Premises. A Provider must take immediate steps to correct any condition in the physical building or on the Premises, which poses a danger to Children's life, health or safety.

10. Toys and equipment must be kept clean and in safe working order.

13. The provider must Clean surfaces daily in all areas where Children are present. Cleaning products must not be used in close proximity to Children and adequate ventilation must be maintained during use.

L. Cribs, cots, and bedding.

3. Cribs, bassinets and play yards for Infants less than 12 months old must not include soft or loose bedding including, but not limited to: bumper pads, pillows, quilts, comforters, blankets, sleep positioning devices, bibs or stuffed toys.

Appropriately fitted arms-free wearable blankets are permitted.

a. Cots, mats, play yards, bassinets, and cribs used by any Infant or toddler must be Cleaned and sanitized between use by different Children, when contaminated, or at a minimum of once per week.

c. Cribs, cots, beds, mats, or play yards used for sleeping must be spaced at least 2 feet apart or separated by a solid divider on one side.

4. A separate mat, bed or cot, with a blanket or sleeping bag, must be assigned to each Child under the age of five in care for longer than four hours. All bedding must be cleaned before being used by another Child. Bedding must be cleaned as needed, or at least weekly.

a. Blankets, sleeping bags, bedding, cots, and mats must be stored in a manner which ensures that sleeping surfaces are not touching or must be washed and disinfected before re-use if stored in a manner that allows sleeping surfaces to touch during storage.

M. Toilet facilities. Washing and toilet facilities with running water and soap must be provided.

3. Either disposable paper products, or clean, individually assigned wash cloths and towels may be used. Individually assigned washcloths and towels must be laundered daily.

N. Diapering/toileting.

- 2. Wet or soiled disposable diapers must be placed in a lined, lidded container, separate from other trash or garbage and separate from play or food-preparation areas, or individually bagged and tied within a lidded shared garbage container.**
- 3. Diapering must be done on a changing table or washable vinyl mat which is Cleaned after each use or which has a disposable single-use covering.**
- 4. The changing table or mat must not be located in the kitchen or food preparation area.**
- 5. Staff must wash their hands and the Child's hands with soap and running water after each diaper change.**
- 8. Potty chair receptacles must be emptied immediately after each use, rinsed, and sanitized in a sink not used for food preparation.**
- 9. Potty chair receptacles may not be used in a food preparation area.**

SECTION 16

A. Handwashing. All Adults and all Children must wash their hands immediately prior to preparing or consuming snacks and meals.

D. Provider Responsibilities

- 1. Providers or Staff Members who are ill must not work in the food preparation area. Providers or Staff Members with open sores that cannot be covered must not handle or prepare food.**

G. Equipment and Utensils

- 1. All food preparation and eating surfaces must be Cleaned before and after use.**

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

- 1. All Child Care Providers, Teachers, and directors associated with Child Care**

Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: Maine License-Exempt Standards

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

10-148 Chapter 6, Child Care Affordability Program Rules

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a. Prevention and Control of Infectious Diseases is defined by following proper

methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

10-148 Chapter 6, Child Care Affordability Program Rules

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a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety

Standards listed below as required by federal law (45 C.F.R. § 98.41):

- a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

a. Prevention and Control of Infectious Diseases is defined by following proper methods of handwashing cleaning and sanitizing, and disinfecting surface areas, bedding, and toys/objects, by not attending Child Care Services when ill, and to have Children stay current on vaccinations which are nationally recommended;

- b. Provide the standards, appropriate to the provider setting and age of children, that address that children attending child care programs under CCDF are age-appropriately

immunized, according to the latest recommendation for childhood immunizations of the respective State public health agency, for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 12: HEALTH AND MEDICAL

A. Immunization. Immunization records must be maintained to ensure proper medical treatment is determined and given in the event of a disease outbreak or public health emergency.

1. All children in care must meet the following requirements.

a. The Child Care Facility must have a current record of immunization on file for each Child, which clearly documents each Child's present immunization status based on the Department's Day Care Immunization Standards (available at <https://www.maine.gov/dhhs/mecdc/infectious-disease/immunization/documents/Daycare-Immunization-standards-revised.pdf> and published August 8, 2021), within 30 days of the Child's first admission to the Child care and updated as needed thereafter or,

b. A blood test documenting immunity to measles, mumps, rubella and varicella (chickenpox), placed in the Child's record and updated in a timely manner.

c. Pursuant to P.L. 2019, Ch. 154, § 11, effective September 1, 2021, Children must be immunized as set forth herein and exemption from immunization requirements are limited to medical reasons. The Child's physician, nurse practitioner, or physician assistant must provide documentation that immunization is medically inadvisable.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 12. HEALTH AND MEDICAL

A. Immunization. Immunization records must be maintained to ensure proper medical treatment is determined and given in the event of a disease outbreak or public health emergency.

1. All Children in care must meet the following requirements:

a. The Provider must have a current record of immunization on file for each child, which clearly documents each child's present immunization status based on the Department's Childcare Immunization Standards (available <https://www.maine.gov/dhhs/mecdc/infectious-disease/immunization/documents/Daycare-Immunization-standards-revised.pdf> and published on August 8, 2021 within 30 days of the child's first admission to the child care and updated as needed thereafter or,

b. A blood test documenting immunity to measles, mumps, rubella and varicella (chickenpox), placed in the Child's record and updated in a timely manner.

c. No Child may be required to be immunized if religious, philosophical or medical reasons are documented. Effective September 1, 2021, exemption from immunization requirements are limited to medical reasons. The Child's physician, nurse practitioner, or physician assistant must provide documentation that immunization is medically inadvisable.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**
☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards
7. Upon request, provide documentation that the Children receiving Child Care Affordability Program subsidy are age-appropriately immunized and meet the latest recommendation for Childhood immunizations in Maine, as recommended by the Department's Center for Disease Control (CDC). A ninety (90) calendar day grace period shall be granted while Parents are taking the necessary actions to comply with the immunization requirement. Children who receive care in his/her own home may be exempted from the immunization requirement.
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards
7. Upon request, provide documentation that the Children receiving Child Care Affordability Program subsidy are age-appropriately immunized and meet the latest recommendation for Childhood immunizations in Maine, as recommended by the Department's Center for Disease Control (CDC). A ninety (90) calendar day grace period shall be granted while Parents are taking the necessary actions to comply with the immunization requirement. Children who receive care in his/her own home may be exempted from the immunization requirement.
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards
7. Upon request, provide documentation that the Children receiving Child Care Affordability Program subsidy are age-appropriately immunized and meet the latest recommendation for Childhood immunizations in Maine, as recommended by the Department's Center for Disease Control (CDC). A ninety (90) calendar day grace period shall be granted while Parents are taking the necessary actions to comply with the immunization requirement. Children who receive care in his/her own home may be exempted from the immunization requirement.
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards
7. Upon request, provide documentation that the Children receiving Child Care Affordability Program subsidy are age-appropriately immunized and meet the latest recommendation for Childhood immunizations in Maine, as recommended by the Department's Center for Disease Control (CDC). A ninety (90) calendar day grace period shall be granted while Parents are taking the necessary actions to

comply with the immunization requirement. Children who receive care in his/her own home may be exempted from the immunization requirement.

5.3.2 Prevention of sudden infant death syndrome and the use of safe-sleep practices health and safety standard

Provide the standards, appropriate to the provider setting and age of children, that address the prevention of sudden infant death syndrome and use of safe sleeping practices for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 2

F. Administration

9. The Child Care Facility must maintain a Child Care Facility Staff manual that addresses the following:

- a. Policies and their implementation. This must include, at a minimum, policies for the following:
- x. Safe sleep policy,

SECTION 7

D. Supervision.

3. The Child Care Facility must provide direct supervision, by awake Child Care Staff Members, to Children while maintaining staff-Child ratios during napping and/or sleeping.

- b. Dimmed, but adequate, lighting to allow visual supervision of all Children must be maintained at all times.
- c. Monitors providing both video and audio may be considered as an acceptable form of supervision in an adjacent nap space. Sleeping Children must be checked in person at a minimum every 30 minutes.

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

- 1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

- 1. **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

2. The program's policies and procedures;
3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
5. Prevention of and response to food and allergy emergencies;
6. Building and physical premise safety;
7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
10. Adult and pediatric first aid with hands-on skills demonstration;
11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
12. Department-approved mandated reporter training;
13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.
 - a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 14

L. Cribs, cots, and bedding.

1. A crib or play yard that meets Consumer Product Safety Commission (CPSC) safety standards, available at <https://www.cpsc.gov/s3fs-public/5023.pdf>, must be provided for each Child up to 18 months of age.
 - d. Each crib must be equipped with a waterproof, firm, snug fitting mattress which is positioned within two finger widths on all sides of the crib frame.
 - e. Devices of any type that restrict the movements of Children must not be used in cribs.
 - f. Toys including mobiles and other types of play equipment that is designed to be attached to any part of the crib must not be used. Each play yard must be used with the original mattress or a replacement specified by the manufacturer.
2. Bassinets that meet CPSC safety standards may be used for infants up to five months of age, within appropriate weight limits. A Child Care Facility may not use a bassinet for an infant who can independently lift their chest off the sleep surface.
3. Cribs, bassinets and play yards for infants less than 12 months old must not include soft or loose bedding including, but not limited to: bumper pads, pillows, quilts, comforters, blankets, sleep positioning devices, bibs or stuffed toys. Appropriately fitted arms-free wearable blankets are permitted.
- b. All crib, bassinet, and play yard mattresses will be covered by a snugly-fitted

sheet.

SECTION 18

E. Napping/Resting. The facility must observe the provisions of Section 14(L) of this rule, and:

- 1.** Infants must be placed on their backs in an approved bassinet, crib, or play yard for sleeping, unless the Child's parent provides a physician's recommendation. The written documentation from the physician must be in the Child's file.
- 3.** Bibs and garments with ties or hoods must be removed before placing an infant in the crib to sleep.
- 4.** Swaddling of Infants and Toddlers is prohibited.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 2

G. Administration

7. The Provider must maintain a staff manual that addresses the following:

- a. Policies and their implementation. This must include, at a minimum, policies for the following:

X. Safe sleep policy,

SECTION 7

D. Supervision.

- 2.** During napping and/or sleeping hours, the Provider or Staff Member must be awake and supervising all Children, and Child-staff ratios must be maintained. Dimmed, but adequate, lighting to allow visual supervision of all Children must be maintained at all times.
- 3.** Monitors providing both video and audio may be considered as an acceptable form of supervision during quiet indoor activities.

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

- 1.** Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.
- 2.** This Rule;
- 3.** The Provider's policies and procedures;
- 4.** Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
- 5.** Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
- 6.** Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;
8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
11. Adult and pediatric first aid with hands-on skills demonstration;
12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 14

L. Cribs, cots, and bedding.

1. A crib or play yard that meets Consumer Product Safety Commission (CPSC) safety standards, available at <https://www.cpsc.gov/s3fs-public/5023.pdf>, must be provided for each Child up to 18 months of age.
 - d. Each crib must be equipped with a waterproof, firm, snug fitting mattress which is positioned within two finger widths on all sides of the crib frame. Each play yard must be used with the original mattress or a replacement specified by the manufacturer.
 - e. Devices of any type that restrict the movements of Children must not be used in cribs.
 - f. Toys including mobiles and other types of play equipment that is designed to be attached to any part of the crib must not be used.
2. Bassinets that meet CPSC safety standards may be used for Infants up to five months of age, within appropriate weight limits. A provider may not use a bassinet for an Infant who can independently lift their chest off the sleep surface.
3. Cribs, bassinets and play yards for Infants less than 12 months old must not include soft or loose bedding including, but not limited to: bumper pads, pillows, quilts, comforters, blankets, sleep positioning devices, bibs or stuffed toys. Appropriately fitted arms-free wearable blankets are permitted.
- b. All crib, bassinet, and play yard mattresses must be covered by a snugly fitted sheet.

SECTION 18

D. Napping/Resting. The Provider must observe the provisions of Section 14 (L) of

this rule, and:

1. Infants must be placed on their backs in an approved bassinet, crib, or play yard for sleeping, unless the Child's Parent provides a physician's recommendation. The written documentation from the physician must be in the Child's file.
3. Bibs and garments with ties or hoods must be removed before placing an infant in the crib to sleep.
4. Swaddling of Infants and Toddlers is prohibited.
5. Sleeping Infants must be checked in person at least every 30 minutes.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Safe sleep practices implemented: Children are placed on their backs for sleeping. Cribs and/or play yards are free of objects that could cause suffocation or strangulation.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome (SIDS) and use of Safe Sleep Practices is defined as following practices to reduce the risk of SIDS:

Placing a baby to sleep on his or her back on a firm mattress using a safety-approved crib;

Removal of any loose bedding; and

While sleeping making sure the baby's head stays uncovered and the baby does not get overheated.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome and use of Safe Sleep Practices;

v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Maine License-Exempt Standards**

Safe sleep practices implemented: Children are placed on their backs for sleeping.

Cribs and/or play yards are free of objects that could cause suffocation or strangulation.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

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b. Prevention of Sudden Infant Death Syndrome (SIDS) and use of Safe Sleep Practices is defined as following practices to reduce the risk of SIDS:

Placing a baby to sleep on his or her back on a firm mattress using a safety-approved crib;

Removal of any loose bedding; and

While sleeping making sure the baby's head stays uncovered and the baby does not get overheated.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome and use of Safe Sleep Practices;

vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Safe sleep practices implemented: Children are placed on their backs for sleeping. Cribs and/or play yards are free of objects that could cause suffocation or strangulation.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome (SIDS) and use of Safe

Sleep Practices is defined as following practices to reduce the risk of SIDS:

Placing a baby to sleep on his or her back on a firm mattress using a safety-approved crib;

Removal of any loose bedding; and

While sleeping making sure the baby's head stays uncovered and the baby does not get overheated.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome and use of Safe Sleep Practices;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Safe sleep practices implemented: Children are placed on their backs for sleeping. Cribs and/or play yards are free of objects that could cause suffocation or strangulation.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome (SIDS) and use of Safe Sleep Practices is defined as following practices to reduce the risk of SIDS:

Placing a baby to sleep on his or her back on a firm mattress using a safety-approved crib;

Removal of any loose bedding; and

While sleeping making sure the baby's head stays uncovered and the baby does not get overheated.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

b. Prevention of Sudden Infant Death Syndrome and use of Safe Sleep Practices;

5.3.3 Administration of medication, consistent with standards for parental consent health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the administration of medication for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 2

F. Administration

6. A copy of the Child Care Facility handbook will be made available to all parents or legal guardians at the time of enrollment. The handbook must contain, at a minimum:

p. Medication administration;

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

- 1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**
- 2. The program's policies and procedures;**
- 3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;**
- 4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);**
- 5. Prevention of and response to food and allergy emergencies;**
- 6. Building and physical premise safety;**
- 7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;**
- 8. Handling and storage of Hazardous material and disposal of Bio-contaminants;**
- 9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);**
- 10. Adult and pediatric first aid with hands-on skills demonstration;**
- 11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;**
- 12. Department-approved mandated reporter training;**
- 13. For programs serving 13 or more Children, training in the program's health**

care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 12

C. Health care consultation.

1. For Child Care Facilities licensed for 3-12 Children:

a. The Child Care Facility must have a manual of written guidelines for the prevention and control of communicable diseases and other appropriate health practices for Child Care Facilities of this size.

2. For Child Care Facilities licensed for 13 or more Children:

b. The Child Care Facility must have a written plan approved by the health care consultant which must include:

iii. Policy on administration of medication including identification of Child Care Staff Members permitted to dispense medication and procedures for documentation of the administration or dispensing of medication; and

H. Medication administration. A Child Care Facility may give a Child prescription medication only when the Child Care Facility has written, signed, and dated permission from a Parent.

1. The Child Care Facility must only give medication prescribed for the specified Child, according to the label instructions on the original container.

2. The Child Care Facility must not give any nonprescription medications to a Child without written permission from the parent. Telephone, text, or email permission is allowed in emergencies, if the Child Care Facility documents giving the medication and obtains written permission from the parent as soon as possible.

3. The Child Care Facility must keep a written record, noting each time a prescription and nonprescription medication is given to a Child.

4. All medications, refrigerated or non-refrigerated, must be:

- a. Completely inaccessible to Children,
- b. Stored at the proper temperature, and
- c. Discarded upon expiration.

I. Handwashing. Handwashing must be done with soap and running water. The Child Care Facility must ensure that all adults and all Children wash their hands in at least the following circumstances:

4. Before and after administering medication.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 2

G. Administration

- 4. A copy of the Provider handbook will be made available to all Parents or Legal Guardians at the time of enrollment. The handbook must contain, at a minimum:**
 - p. Medication administration;**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

- 1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.**
- 2. This Rule;**
- 3. The Provider's policies and procedures;**
- 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;**
- 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);**
- 6. Prevention of and response to food and allergy emergencies;**
- 7. Building and physical premise safety;**
- 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;**
- 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;**
- 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);**
- 11. Adult and pediatric first aid with hands-on skills demonstration;**
- 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;**
- 13. Mandated Reporter Training.**

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

- 1. Annual training must include a review of health and safety topics.**
- 2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.**
- 3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.**

SECTION 12

H. Medication administration. A Provider may give a Child prescription medication only when the Provider has written, signed, and dated permission from a Parent.

- 1. The Provider must only give Medication prescribed for the specified Child, according to the label instructions on the original container.**
- 2. The Provider must not give any nonprescription Medications to a Child without**

written permission from the Parent. Telephone, text, or email permission is allowed in emergencies, if the provider documents giving the Medication and obtains written permission from the Parent as soon as possible.

3. The provider must keep a written record, noting each time a prescription and nonprescription Medication is given to a Child.

4. All Medications, refrigerated or non-refrigerated, must be:

- a. Completely inaccessible to Children,
- b. Stored at the proper temperature, and
- c. Discarded upon expiration.

I. Handwashing. Handwashing must be done with soap and running water. The provider must ensure that all Adults and all Children wash their hands in at least the following circumstances:

4. Before and after administering Medication.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a

minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

10-148 Chapter 6, Child Care Affordability Program Rules

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c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a

signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

- b. Provide the standards, appropriate to the provider setting and age of children, that address obtaining permission from parents to administer medications to children for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child**

Care Facilities

SECTION 5

D. Child records. Child records must be stored in a central location and may only be accessed by authorized Child Care Staff Members. Child records must be updated annually and maintained in an organized fashion.

1. A record must be completed at the time of admission and maintained in the Child Care Facility on each Child in care (including relatives) and must include:

k. Signed authorization to dispense medication, if applicable;

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by

Maine's state-approved Professional Development Network.

SECTION 12

H. Medication administration. A Child Care Facility may give a Child prescription medication only when the Child Care Facility has written, signed, and dated permission from a Parent.

2. The Child Care Facility must not give any nonprescription medications to a Child without written permission from the parent. Telephone, text, or email permission is allowed in emergencies, if the Child Care Facility documents giving the medication and obtains written permission from the parent as soon as possible.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 5

D. Child records. Child records must be updated annually and maintained in an organized fashion.

1. A record must be completed at the time of admission and maintained on the Premises for each Child in care (including relatives) and must include:

k. Signed authorization to dispense Medication, if applicable;

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine’s Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 12

H. Medication administration. A Provider may give a Child prescription medication only when the Provider has written, signed, and dated permission from a Parent.

2. The Provider must not give any nonprescription Medications to a Child without written permission from the Parent. Telephone, text, or email permission is allowed in emergencies, if the provider documents giving the Medication and obtains written permission from the Parent as soon as possible.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**
[x]Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care

Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

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F. Health and Safety Standards

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c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered

with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

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SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have

current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

Medicines are not expired, permission documented: Any over-the-counter medication must be current. Prescription medication may only be administered with written authorization from the parent/guardian and must be documented.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with parental consent is defined as Child Care Providers and staff must be aware of and follow state regulations, laws, and program policies and procedures. Directors, supervisors, and owner/operators are responsible to prepare and enforce policies for accurate medication administration procedures. They must also make sure that identified staff are well trained to administer medication to Children by following State-approved medication administration training guidelines. Medicines administered in Child Care Centers, family group homes, and Family Child Care programs should be limited to prescription or nonprescription (over-the-counter or OTC) medications. All medication administration must include parental/guardian written, documented permission, and medication logs. Medications must be ordered by a prescribing health professional for a specific Child. Orders from the prescribing health professional should specify the medical need, medication, dosage, and length of time to give medication;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

c. Administration of medication, consistent with standards for parental consent;

5.3.4 Prevention of and response to emergencies due to food and allergic reactions health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the *prevention* of emergencies due to food and allergic reactions for the following

CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 5

D. Child records. Child records must be stored in a central location and may only be accessed by authorized Child Care Staff Members. Child records must be updated annually and maintained in an organized fashion.

1. A record must be completed at the time of admission and maintained in the Child Care Facility on each Child in care (including relatives) and must include:
 - p. Known allergies and other health conditions, including related health plans as needed,

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities
2. The program's policies and procedures;
3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
5. Prevention of and response to food and allergy emergencies;
6. Building and physical premise safety;
7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
10. Adult and pediatric first aid with hands-on skills demonstration;
11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
12. Department-approved mandated reporter training;
13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number

of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 16

C. Allergies. Documentation regarding Children's food allergies/food intolerance as verified by a physician must be readily accessible and reviewed by all staff responsible for food preparation and distribution. Food must be prepared according to a written plan for each Child with a food allergy or food intolerance. The plan must be reviewed and updated as the Child's needs change or a minimum of once a year.

SECTION 18

B. Promoting developmental needs. The developmental needs of Infants and Toddlers must be promoted in the following ways:

3. Infants and Toddlers must not be allowed to use toys or objects which could be swallowed or cause choking. Foods that are choking hazards due to the size, shape or texture must not be served to Infants and toddlers;

C. Feeding

6. Providers must hold Infants who are not independently bottle-feeding in their arms for feeding.

7. Children who are independently bottle-feeding must be in a seated position within reach of a Provider.

8. If breast milk or formula is to be warmed, bottles must be placed in hot (not boiling) water for no longer than five minutes, after which the bottle must be shaken well, and the temperature tested before feeding. Bottles of formula or breast milk must never be warmed in a microwave oven.

9. A chair or table used for feeding Infants and Toddlers must be designed to prevent the Child from slipping or falling.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 5

D. Child records. Child records must be updated annually and maintained in an organized fashion.

1. A record must be completed at the time of admission and maintained on the Premises for each Child in care (including relatives) and must include:

p. Known allergies and other health conditions, including related health plans as needed,

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

- 1. Fire drill procedures, disaster procedures, and all other emergency procedures.** This training must be completed within the first week of service.
- 2. This Rule;**
- 3. The Provider's policies and procedures;**
- 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;**
- 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);**
- 6. Prevention of and response to food and allergy emergencies;**
- 7. Building and physical premise safety;**
- 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;**
- 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;**
- 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);**
- 11. Adult and pediatric first aid with hands-on skills demonstration;**
- 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;**
- 13. Mandated Reporter Training.**

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

- 1. Annual training must include a review of health and safety topics.**
- 2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.**
- 3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.**

SECTION 16

C. Allergies. Documentation regarding Children's food allergies/food intolerance as verified by a physician must be readily accessible and reviewed by all Staff Members responsible for food preparation and distribution. Food must be prepared according to a written plan for each Child with a food allergy or food intolerance. The plan must be reviewed and updated as the Child's needs change and at a minimum of once a year.

SECTION 18

B. Promoting developmental needs. The developmental needs of Infants and Toddlers must be promoted in the following ways:

- 3. Infants and Toddlers must not be allowed to use toys or objects which could be swallowed or cause choking. Foods that are choking hazards due to the size, shape**

or texture must not be served to Infants and toddlers;

C. Feeding

6. Providers must hold Infants who are not independently bottle-feeding in their arms for feeding.

7. Children who are independently bottle-feeding must be in a seated position within reach of a Provider.

8. If breast milk or formula is to be warmed, bottles must be placed in hot (not boiling) water for no longer than five minutes, after which the bottle must be shaken well, and the temperature tested before feeding. Bottles of formula or breast milk must never be warmed in a microwave oven.

9. A chair or table used for feeding Infants and Toddlers must be designed to prevent the Child from slipping or falling.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one-and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one-and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies

available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or

mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

- b. Provide the standards, appropriate to the provider setting and age of children, that address the *response* to emergencies due to food and allergic reactions for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32,**

Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

SECTION 5

D. Child records. Child records must be stored in a central location and may only be accessed by authorized Child Care Staff Members. Child records must be updated annually and maintained in an organized fashion.

- 1.** A record must be completed at the time of admission and maintained in the Child Care Facility on each Child in care (including relatives) and must include:
 - p.** Known allergies and other health conditions, including related health plans as needed,

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

- 1.** All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

- 1.** 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities
- 2.** The program's policies and procedures;
- 3.** Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
- 4.** Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
- 5.** Prevention of and response to food and allergy emergencies;
- 6.** Building and physical premise safety;
- 7.** Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
- 8.** Handling and storage of Hazardous material and disposal of Bio-contaminants;
- 9.** Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
- 10.** Adult and pediatric first aid with hands-on skills demonstration;
- 11.** Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
- 12.** Department-approved mandated reporter training;
- 13.** For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

- 1.** All Child Care Staff Members must register with Maine's Professional Development Network.
 - a.** Annual training must include a review of health and safety topics, as offered by

Maine's state-approved Professional Development Network.

SECTION 12

K. Emergency procedures. Providers must have a procedure for responding to situations when an immediate emergency medical response is required. Staff-Child ratio must be maintained during all medical emergencies. Staff Members may be called in as necessary to maintain the required ratio. Every Staff Member must be trained to manage an emergency until emergency medical care becomes available.

SECTION 14

Q. Emergency preparedness plan. Providers must have a written emergency preparedness plan in the event of a fire, natural disaster, or other threatening situation that may pose a health or safety hazard to the Children. Providers and Staff Members must review the emergency preparedness plan annually and update as needed. Providers must retain the updated copy of the plan for evacuation and provide details of the emergency relocation procedures to each Parent or Legal Guardian of the Child at the time of the Child's enrollment with the Provider and whenever the plan is updated.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 5

D. Child records. Child records must be updated annually and maintained in an organized fashion.

1. A record must be completed at the time of admission and maintained on the Premises for each Child in care (including relatives) and must include:

p. Known allergies and other health conditions, including related health plans as needed,

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;
8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
11. Adult and pediatric first aid with hands-on skills demonstration;
12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 12

K. Emergency procedures. Providers must have a procedure for responding to situations when an immediate emergency medical response is required. Staff-Child ratio must be maintained during all medical emergencies. Staff Members may be called in as necessary to maintain the required ratio. Every Staff Member must be trained to manage an emergency until emergency medical care becomes available.

SECTION 14

Q. Emergency preparedness plan. Providers must have a written emergency preparedness plan in the event of a fire, natural disaster, or other threatening situation that may pose a health or safety hazard to the Children. Providers and Staff Members must review the emergency preparedness plan annually and update as needed. Providers must retain the updated copy of the plan for evacuation and provide details of the emergency relocation procedures to each Parent or Legal Guardian of the Child at the time of the Child's enrollment with the Provider and whenever the plan is updated.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**
☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food

preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

- d. Prevention of and response to emergencies due to food and allergic reactions;
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

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Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing

symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully

completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Hand washing and diapering requirements followed: Adults and children must wash their hands with soap and running water before and after toileting, food preparation, and diapering.

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Nutritious meals/snacks available: The provider must have a supply of nutritious foods and snacks available for the number of children in care. .

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

Written authorization for emergency care/medications: The provider must show a signed release allowing emergency medical care for all children, and a signed release for any medications being administered. Note any allergies and treatment required (if needed).

First aid supplies and manual available: There must be adequate first aid supplies available to treat common injuries and illnesses and a reference manual that describes response to common injuries.

CPR/First Aid current: The Licensed-Exempt Provider caring for the child must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR). In Recreation programs all staff must have current Pediatric first-aid and cardiopulmonary resuscitation (CPR).

10-148 Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions is defined by having classroom procedures for policies, food preparation and food label reading, food services, cleaning and sanitizing, field trips, and recognizing symptoms. By also having a food allergy action plan or emergency care plan in place for a Child with severe food allergies;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

d. Prevention of and response to emergencies due to food and allergic reactions;

5.3.5 Building and physical premises safety, including the identification of and protection from hazards, bodies of water, and vehicular traffic health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from building and physical premises hazards for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained

- Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
 5. Prevention of and response to food and allergy emergencies;
 6. Building and physical premise safety;
 7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
 8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
 9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
 10. Adult and pediatric first aid with hands-on skills demonstration;
 11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 12. Department-approved mandated reporter training;
 13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.
- C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.
1. All Child Care Staff Members must register with Maine's Professional Development Network.
 - a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 13

A. Drinking water. A Child Care Facility must provide documentation of water safe for drinking by Child Care Staff Members and Children in care. If a satisfactory supply cannot be provided, a license or renewal of a license may not be issued.

SECTION 14

- A. General condition of the building and surrounding premises. The facility must take immediate steps to correct any condition in the physical building or on the premises, which poses a danger to Children's life, health or safety.
1. Stairs used by Children must be equipped with solidly mounted handrails.
 2. Stairs must be enclosed by walls or railings. Stairs in areas used by infants, toddlers and preschool age Children must be protected by a gate or door.
 3. Electrical outlets in areas used by infants, toddlers and preschool Children must be protected by safety caps, plugs, tamper-resistant outlets, or other means.
 4. Any Medications, Toxic Substances, Hazardous Materials, and other items dangerous to Children such as matches, lighters, fireworks, and power tools must be kept where Children cannot access them.
 5. Smoking is prohibited on the premises when Children are present.
 6. Strings and cords long enough to encircle a Child's neck (six inches or more) must not be accessible to Children unless the Child is engaged in a recreational or educational activity with an adult.
 7. Equipment with an electrical cord must be anchored to the wall to prevent falling, or the cord must be inaccessible to Children under five years of age.

9. A facility may lock the main entrance to prevent access from the exterior, provided that the door can be freely opened from the interior and there is a means to inform Child Care Staff Members of the arrival of parents/guardians or other visitors.

11. The fall zone under and around all indoor play equipment which would buffer a fall from a height of more than 29 inches must be covered with shock-absorbing mats or materials which extend at least 39 inches beyond the outside edge of the equipment in all directions. If the play structure exceeds 48 inches in height, the shock absorbing mat must be at least two inches thick.

12. Furnishings, other heavy items, and/or items that could easily tip over or are unstable must be secured to the wall or floor.

13. The Child Care Facility must Clean surfaces daily in all areas where Children are present. Cleaning products must not be used in close proximity to Children and adequate ventilation must be maintained during use.

14. Children are not allowed to be near power equipment while it is being operated and power equipment must be stored out of reach or have safeguards to prevent any Child from being injured when not in use.

16. All entrances and exits must be clear for evacuation at all times.

C. Satisfactory lead levels. Child Care Facilities must comply with 22 MRS Ch. 252, Lead Poisoning Control Act, regarding the use of lead-based paint on toys, furniture, or any interior or exterior surfaces.

D. Carbon monoxide. Child Care Facilities must be equipped with a working carbon monoxide detection system.

E. Building temperature.

3. Air conditioners, electric fans, and heaters must be mounted and anchored out of all Children's reach or have safeguards that keep any Child from being injured.

F. Lighting and Ventilation.

1. Every room used by Children must be properly lighted to allow safe movement through the space. Natural light is required in any room used by Children for more than four hours per day.

2. All indoor space used by Children must be adequately ventilated.

a. Windows should be open when outside temperature and air quality allow.

b. Doors and windows used for ventilation must be equipped with securely fastened screens.

c. The facility must resolve any damp conditions which result in visible mold, mildew, or musty odors before Children may occupy the space.

H. Outdoor play areas. The facility must have access to an outdoor play area, with sufficient space for safe play for all Children, with drainage that prevents standing water.

2. Where hazardous conditions exist in the outdoor play area, the Children must be protected from those conditions by fencing or other appropriate barriers.

a. Hazardous conditions include, but are not limited to, roads, any body of water, active railroad tracks, sharp inclines or embankments, or any other dangerous area.

4. Appropriately fitted helmets must be worn for all activities when head injury risks are present (e.g. bicycles, scooters, rollerblades, skateboards, snowboard, and/or power wheels).
5. Equipment that exceeds 36 inches in height at the climbable or standing surface shall have energy-absorbing materials beneath it.
7. The play area must be free of hazards and visually inspected daily, prior to Children having access to the area.
8. Sandboxes or sand piles must be maintained in a safe and sanitary condition.
9. Outdoor play areas must provide shade.
 - a. The Child Care Facility shall ensure sun safety for Children by limiting sun exposure when UV rays are strongest, wearing hats, or applying sunscreen (unless directed otherwise by the Child's parent).
 - b. Infants younger than six months must stay out of direct sunlight, and Child Care Staff Members may not apply sunscreen to an infant unless the Child's parent provides a physician's recommendation. Written documentation from the physician must be stored in the Child's file.

I. Weapons. Firearms, hunting knives, bows and arrows, and other weapons are prohibited on the premises of the Child Care Facility.

J. Materials, toys, equipment and activities. Materials, toys, equipment and activities must be Developmentally and culturally appropriate for Children enrolled.

2. Toys and equipment that have been recalled through the manufacturer or the United States Consumer Product Safety Commission (CPSC) must be removed from the Premises, repaired, or replaced based on the recall information available.

Q. Emergency preparedness plan. Child Care Facilities must have a written emergency preparedness plan in the event of a fire, natural disaster, or other threatening situation that may pose a health or safety hazard to the Children in the Child Care Facility. Child Care Staff Members must review the emergency preparedness plan annually and update as needed. Child Care Facilities must retain the updated copy of the plan for evacuation and provide details of the emergency relocation procedures to each Parent or Legal Guardian of the Child at the time of the Child's enrollment with the Child Care Facility and whenever the plan is updated

1. The plan must provide for the following:
 - a. Evacuation, including a designated relocation site and evacuation route;
 - b. Lockdown of the facility or shelter-in-place;

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days

of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.

2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.

3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 13

A. Drinking water. A Provider must provide documentation of water safe for drinking by staff and Children in care. If a satisfactory supply cannot be provided, a License or renewal of a License may not be issued.

SECTION 14

A. General condition of the building and surrounding Premises. A Provider must take immediate steps to correct any condition in the physical building or on the Premises, which poses a danger to Children's life, health or safety.

1. Stairs used by Children must be equipped with solidly mounted handrails.

2. Stairs must be enclosed by walls or railings. Stairs in areas used by Infants, toddlers and preschool Children must be protected by a gate or door.

3. Electrical outlets in areas used by Infants, toddlers and preschool Children must be protected by safety caps, plugs, tamper-resistant outlets, or other means.

4. Any Medications, Toxic Substances, Hazardous Materials, and other items dangerous to Children such as matches, lighters, fireworks, and power tools must be kept where Children cannot access them.

5. Smoking is prohibited on the Premises when Children are present.

6. Strings and cords long enough to encircle a Child's neck (six inches or more) must not be accessible to Children unless the Child is engaged in a recreational or educational activity with an Adult.

7. Equipment with an electrical cord must be anchored to the wall to prevent falling, or the cord must be inaccessible to Infants, toddlers and preschool Children.

9. A Provider may Lock the main entrance of their home and/or the Premises to prevent access from the exterior, provided that the door can be freely opened from the interior and there is a means to inform the Provider of the arrival of Parents/Legal Guardians or other visitors.

11. The fall zone under and around all indoor play equipment which would buffer a fall from a height of more than 29 inches must be covered with shock-absorbing mats or materials which extend at least 39 inches beyond the outside edge of the equipment in all directions. If the play structure exceeds 48 inches in height, the shock absorbing mat must be at least two inches thick.

12. Furnishings, other heavy items, and/or items that could easily tip over or are unstable must be secured to the wall or floor.

14. Children are not allowed to be near power equipment while it is being operated and power equipment must be stored out of reach or have safeguards to prevent any Child from being injured when not in use.

16. All entrances and exits must be clear for evacuation at all times.

C. Satisfactory lead levels. Providers must comply with 22 MRS Ch. 252, Lead Poisoning Control Act, regarding the use of lead-based paint on toys, furniture, or any interior or exterior surfaces.

D. Carbon monoxide. Providers must be equipped with a working carbon monoxide detection system.

E. Building temperature.

3. Air conditioners, electric fans, and heaters must be mounted and anchored out of all Children's reach or have safeguards that keep any Child from being injured.

F. Lighting and Ventilation.

1. Every room used by Children must be properly lighted to allow safe movement through the space. Natural light is required in any room used by Children for more than four hours per day.

2. All indoor space used by Children must be adequately ventilated.

a. Windows should be open when outside temperature and air quality allow.

b. Doors and windows used for ventilation must be equipped with securely fastened screens.

c. The facility must resolve any damp conditions which result in visible mold, mildew, or musty odors before Children may occupy the space.

H. Outdoor play areas. The facility must have access to an outdoor play area, with sufficient space for safe play for all Children, with drainage that prevents standing water.

1. Where hazardous conditions exist in the outdoor play area, the Children must

be protected from those conditions by fencing or other appropriate barriers.

a. Hazardous conditions include, but are not limited to, roads, any bodies of water, active railroad tracks, sharp inclines or embankments, or any other dangerous area.

3. Appropriately fitted helmets must be worn for all activities when head injury risks are present (e.g. bicycles, scooters, rollerblades, skateboards, snowboard, and/or power wheels).

4. Equipment that exceeds 36 inches in height at the climbable or standing surface shall have energy-absorbing materials beneath it.

6. The play area must be free of hazards and visually inspected by a Provider daily, prior to Children having access to the area.

7. Sandboxes or sand piles must be maintained in a safe and sanitary condition.

8. Outdoor play areas must provide shade. The provider shall ensure sun safety for Children by limiting sun exposure when UV rays are strongest, wearing hats, or applying sunscreen (unless directed otherwise by the Child's Parent). Infants younger than six months must stay out of direct sunlight, and a Provider may not apply sunscreen to an Infant unless the Child's Parent provides a physician's recommendation. Written documentation from the physician must be stored in the Child's file.

I. Weapons. Firearms, hunting knives, bows and arrows, and other weapons kept on the Premises must remain in a Locked case, closet, or cabinet inaccessible to Children during all hours of operation.

1. Firearms must be kept unloaded. Ammunition must be Locked in a separate location from weapons.

J. Materials, toys and equipment. Materials, toys, and equipment must be Developmentally Appropriate for Children enrolled.

4. Toys and equipment that have been recalled through the manufacturer or the United States Consumer Product Safety Commission (CPSC) must be removed from the Premises, repaired, or replaced based on the recall information available.

Q. Emergency preparedness plan. Providers must have a written emergency preparedness plan in the event of a fire, natural disaster, or other threatening situation that may pose a health or safety hazard to the Children. Providers and Staff Members must review the emergency preparedness plan annually and update as needed. Providers must retain the updated copy of the plan for evacuation and provide details of the emergency relocation procedures to each Parent or Legal Guardian of the Child at the time of the Child's enrollment with the Provider and whenever the plan is updated.

1. The plan must provide for the following:

- a. Evacuation, including a designated relocation site and evacuation route;
- b. Lockdown of the residence and/or shelter-in-place;

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

[x] Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Outdoor area is free from hazardous conditions: The play area must be free of broken or damaged equipment, insect nests, and poisonous plants.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Working smoke detectors and fire extinguisher: There must be working smoke detectors in all areas occupied by children, and at least one working fire extinguisher readily available.

Two or more unblocked exits: In addition to the main entrance to the site, there must be another usable exit.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

Firearms and ammunition locked separately: All firearms must be locked and unloaded, with ammunition locked separately and out of the reach of children. Verification by viewing of storage is necessary.

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the

family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Outdoor area is free from hazardous conditions: The play area must be free of broken or damaged equipment, insect nests, and poisonous plants.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Working smoke detectors and fire extinguisher: There must be working smoke detectors in all areas occupied by children, and at least one working fire extinguisher readily available.

Two or more unblocked exits: In addition to the main entrance to the site, there must be another usable exit.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

Firearms and ammunition locked separately: All firearms must be locked and unloaded, with ammunition locked separately and out of the reach of children. Verification by viewing of storage is necessary.

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements:
Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or

mold.

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Outdoor area is free from hazardous conditions: The play area must be free of broken or damaged equipment, insect nests, and poisonous plants.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Working smoke detectors and fire extinguisher: There must be working smoke detectors in all areas occupied by children, and at least one working fire extinguisher readily available.

Two or more unblocked exits: In addition to the main entrance to the site, there must be another usable exit.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

Firearms and ammunition locked separately: All firearms must be locked and unloaded, with ammunition locked separately and out of the reach of children. Verification by viewing of storage is necessary.

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a

minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements:
Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Evidence of routine maintenance and cleaning: The site must be free of hazards that could cause injury to a child such as missing well cover, bare electrical wires, or broken glass. The areas for food preparation must be free of dried food or mold.

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Outdoor area is free from hazardous conditions: The play area must be free of broken or damaged equipment, insect nests, and poisonous plants.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Working smoke detectors and fire extinguisher: There must be working smoke detectors in all areas occupied by children, and at least one working fire extinguisher readily available.

Two or more unblocked exits: In addition to the main entrance to the site, there must be another usable exit.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

Firearms and ammunition locked separately: All firearms must be locked and unloaded, with ammunition locked separately and out of the reach of children. Verification by viewing of storage is necessary.

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

All choking/swallowing hazards are out of reach: Any objects smaller than one- and one-half inch in diameter are not within reach of children.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care

Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- b. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from bodies of water for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional

Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 14

H. Outdoor play areas. The facility must have access to an outdoor play area, with sufficient space for safe play for all Children, with drainage that prevents standing water.

2. Where hazardous conditions exist in the outdoor play area, the Children must be protected from those conditions by fencing or other appropriate barriers.

a. Hazardous conditions include, but are not limited to, roads, any body of water, active railroad tracks, sharp inclines or embankments, or any other dangerous area.

SECTION 15

F. Pool Accessibility.

1. All Wading Pools must be emptied when not in use.

2. Hot tubs must be protected by fencing or locked barriers to prevent access by Children.

3. All in-ground Swimming Pools located in areas accessible to Children must be fenced. The fence must be at least four feet in height and secured with a Locked gate.

4. All above-ground Swimming Pools must have non-climbable sidewalls that are at least four feet high or must be enclosed with a fence at least four feet in height and secured with a Locked gate. When the Pool is not in use, steps must be removed from the Pool or otherwise protected to prevent unsupervised access.

H. Staff-Child ratios during water activities. Staff-Child ratios must be maintained at all times during swimming and wading.

1. Wading Pool attendants may count toward staff-Child ratios. Water safety attendants, lifeguards, and swimming instructors do not count toward staff-Child ratios. Water safety attendants and Certified Lifeguards must be stationed outside and next to the Swimming Pool.

2. When three or fewer Children are in the Swimming Pool, a water safety attendant may be included in staff-Child ratios. The water safety attendant must be solely responsible for supervision of the Children in the Swimming Pool.

3. If the pool is more than four feet deep at any point, only Child Care Staff Members who are able to swim may be counted in the staff-Child ratios.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days

of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.

2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.

3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 14

H. Outdoor play areas. The facility must have access to an outdoor play area, with sufficient space for safe play for all Children, with drainage that prevents standing water.

1. Where hazardous conditions exist in the outdoor play area, the Children must be protected from those conditions by fencing or other appropriate barriers.

a. Hazardous conditions include, but are not limited to, roads, any bodies of water, active railroad tracks, sharp inclines or embankments, or any other dangerous area.

SECTION 15

F. Pool accessibility

1. All Wading Pools must be emptied when not in use.

2. Hot tubs must be protected by fencing or Locked barriers to prevent access by Children.

3. All in-ground Swimming Pools located in areas accessible to Children must be

fenced. The fence must be at least four feet in height and secured with a Locked gate.

4. All above-ground Swimming Pools must have non-climbable sidewalls that are at least four feet high or must be enclosed with a fence at least four feet in height and secured with a Locked gate. When the Pool is not in use, steps must be removed from the Swimming Pool or otherwise protected to prevent unsupervised access.

H. Staff-Child ratios during water activities. Staff-Child ratios must be maintained at all times during swimming and wading.

1. Wading Pool attendants may count toward staff-Child ratios. Water safety attendants, Certified Lifeguards, and swimming instructors do not count toward staff-Child ratios. Water safety attendants and Certified Lifeguards must be stationed outside and next to the swimming Pool.

2. When three or fewer Children are in the swimming Pool, a water safety attendant may be included in staff-Child ratios. The water safety attendant must be solely responsible for supervision of the Children in the swimming Pool.

3. If the Pool is more than four feet deep at any point, only providers who are able to swim may be counted in the staff-Child ratios.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements:
Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements:
Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15)

Swimming pool adequately safeguarded: Swimming pool must be inaccessible to children when not in use.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- c. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from vehicular traffic hazards for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 7

D. Supervision.

1. Children must be supervised at all times.

a. In Child Care Facilities serving 3-12 Children, Child Care Staff Members must have knowledge of the activity and whereabouts of each Child in care.

- i. Child Care Staff Members must be able to see or hear all Children at all times and be able to provide prompt intervention when needed.
- ii. Child Care Staff Members must be physically present outside when Children under the age of eight outdoors.
- iii. If Children over the age of eight are outdoors and a Child Care Staff Member is not physically present, the play area must be enclosed by fencing and the facility must remain in compliance with Section 7(D)(1)(a)(i) of this rule.
- b. In Child Care Facilities serving 13 or more Children, a Child Care Staff Member must be able to see and hear all Children, be responsible for the ongoing activity of the Children, and be able to provide prompt intervention when needed.

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

- 1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

- 1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities
- 2. The program's policies and procedures;
- 3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
- 4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
- 5. Prevention of and response to food and allergy emergencies;
- 6. Building and physical premise safety;
- 7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
- 8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
- 9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
- 10. Adult and pediatric first aid with hands-on skills demonstration;
- 11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
- 12. Department-approved mandated reporter training;
- 13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

- 1. All Child Care Staff Members must register with Maine's Professional Development Network.
 - a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 14

H. Outdoor play areas. The facility must have access to an outdoor play area, with sufficient space for safe play for all Children, with drainage that prevents standing water.

1. In playgrounds intended to serve Children of all ages, the layout of pathways, fencing, and the landscaping of the playground must show the distinct areas for the different age groups. The areas should be separated by a buffer zone, which may include, but is not limited to, shrubs or benches.

2. Where hazardous conditions exist in the outdoor play area, the Children must be protected from those conditions by fencing or other appropriate barriers.

a. Hazardous conditions include, but are not limited to, roads, any body of water, active railroad tracks, sharp inclines or embankments, or any other dangerous area.

SECTION 17

D. Field Trips

1. Child Care Facilities must have measures in place to account for all Children at all times.

2. Children must be accounted for when leaving and re-entering the Premises, and a head count should be performed periodically during a Field Trip.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 7

D. Supervision

1. Children must be supervised at all times by Provider or a Staff Member. The Provider or Staff Member must be present and interacting, intervening, providing direction, feedback and assistance at all times.

a. Providers must have knowledge of the activity and whereabouts of each Child in care.

i. A Provider or Staff Member must be able to see or hear all Children at all times and be able to provide prompt intervention when needed.

ii. A Provider or Staff Member must be physically present outside when Children under the age of eight are outdoors.

iii. If Children over the age of eight are outside, and a Provider or Staff Member is not physically present, the play area must be enclosed by fencing.

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;
 3. The Provider's policies and procedures;
 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
 6. Prevention of and response to food and allergy emergencies;
 7. Building and physical premise safety;
 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
 11. Adult and pediatric first aid with hands-on skills demonstration;
 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 13. Mandated Reporter Training.
- B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.
1. Annual training must include a review of health and safety topics.
 2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
 3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 14

- H. Outdoor play areas. The facility must have access to an outdoor play area, with sufficient space for safe play for all Children, with drainage that prevents standing water.
1. Where hazardous conditions exist in the outdoor play area, the Children must be protected from those conditions by fencing or other appropriate barriers.
 - a. Hazardous conditions include, but are not limited to, roads, any bodies of water, active railroad tracks, sharp inclines or embankments, or any other dangerous area.

SECTION 17

D. Field Trips

1. Providers must have measures in place to account for all Children at all times.
2. Children must be accounted for when leaving and re-entering the Premises, and a head count should be performed periodically during a Field Trip.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCAP needs to be provided during the inspection for all staff who transport.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of

water, and vehicular traffic;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

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Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCAP needs to be provided during the inspection for all staff who transport.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

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e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection

from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCAP needs to be provided during the inspection for all staff who transport.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Children are adequately supervised: Ensure all children are always supervised by the provider, to include at a minimum, inside supervision, which can be sight or sound, and outdoor supervision (including swimming), which must be by sight and close enough to intervene.

Fencing or other appropriate barriers in place: There must be a barrier adequate to restrict child access if the site is near a street, body of water, or other danger.

Emergency preparedness plans in place: The provider must have an accessible site-specific emergency plan (YIKES or equivalent).

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCAP needs to be provided during the inspection for all staff who transport.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic is defined as a safe space where age appropriate planning and checks take place inside and outside for toys, furniture, materials, and equipment. Proper supervision takes place even during naptime. Prepare and prevent to ensure Children in care can move around the space and explore;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety

Standards listed below as required by federal law (45 C.F.R. § 98.41):

e. Building and physical premises safety, including identification of and protection from hazards that can cause bodily injury such as electrical hazards, bodies of water, and vehicular traffic;

5.3.6 Prevention of shaken baby syndrome, abusive head trauma, and maltreatment health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the prevention of shaken baby syndrome and abusive head trauma and indicate the age of children it applies to for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 9 (ALL AGES)

D. Detrimental practice. Actions that have a reasonable likelihood to be harmful to Children are strictly prohibited. The Child Care Facility must ensure that no Child is subjected to an action or practice detrimental to the welfare of Children, including, but not limited to:

1. Corporal punishment. Corporal punishment means physical actions harmful to a Child's body, including, but not limited to, the following:

a. Slapping, striking, shaking, shoving, spanking, pinching, twisting, kicking, biting, ear pulling or ear twisting;

SECTION 10 (ALL AGES)

A. Rights of Children. Children receiving Child care from Child Care Facilities have the following rights.

1. Children must be free from emotional, physical and/or sexual abuse, neglect and exploitation.

2. Each Child has the right to freedom from harmful actions or practices that are detrimental to the Child's welfare, and to practices that are potentially harmful to the Child.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the

transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.

2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.

3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 9 (ALL AGES)

D. Detrimental practices. Actions that have a reasonable likelihood to be harmful to Children are strictly prohibited. The Provider must ensure that no Child is subjected to an action or practice detrimental to the welfare of Children, including, but not limited to:

1. Corporal punishment. Corporal punishment means physical actions harmful to a Child's body, including, but not limited to, the following

a. Slapping, striking, shaking, shoving, spanking, pinching, twisting, kicking, biting, ear pulling or ear twisting;

SECTION 10 (ALL AGES)

A. Rights of children. Children receiving childcare from Providers have the following rights.

1. Children must be free from emotional, physical, sexual abuse, neglect and exploitation.

2. Each Child has the right to freedom from harmful actions or practices that are detrimental to the Child's welfare, and to practices that are potentially harmful to the Child.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards:**

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking, shaking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the

family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

- i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;
- ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;
- iii. Learning strategies for coping with a crying, fussing or distraught Child; and
- iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: Maine License-Exempt Standards:

Positive methods of child guidance observed/described: The provider must be

able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking, shaking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

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- i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;
- ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;
- iii. Learning strategies for coping with a crying, fussing or distraught Child; and
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2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards:**

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking, shaking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

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f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;

ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;

iii. Learning strategies for coping with a crying, fussing or distraught Child; and

iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of

Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards:**

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking, shaking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;

ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;

iii. Learning strategies for coping with a crying, fussing or distraught Child; and

- iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.
- 2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):
 - f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;
- b. Provide the standards, appropriate to the provider setting and age of children, that address the prevention of child maltreatment and indicate the age of children it applies to for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

- 1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities
 - 2. The program's policies and procedures;
 - 3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
 - 4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
 - 5. Prevention of and response to food and allergy emergencies;
 - 6. Building and physical premise safety;
 - 7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
 - 8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
 - 9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
 - 10. Adult and pediatric first aid with hands-on skills demonstration;
 - 11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 - 12. Department-approved mandated reporter training;
 - 13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.
- C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine’s Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine’s state-approved Professional Development Network.

SECTION 9 (ALL AGES)

D. Detrimental practice. Actions that have a reasonable likelihood to be harmful to Children are strictly prohibited. The Child Care Facility must ensure that no Child is subjected to an action or practice detrimental to the welfare of Children, including, but not limited to:

1. Corporal punishment. Corporal punishment means physical actions harmful to a Child’s body, including, but not limited to, the following:

a. Slapping, striking, shaking, shoving, spanking, pinching, twisting, kicking, biting, ear pulling or ear twisting;

b. Forcing a Child to taste or eat spicy, bitter or otherwise distasteful products for the purpose of Discipline;

c. Spraying with water as a means of controlling behavior;

d. Placing tape over a Child’s mouth;

e. Mechanical restraints, such as tying a Child to a chair;

f. Requiring or forcing a Child to take an uncomfortable position such as: squatting, kneeling, standing and holding arms outstretched at sides or overhead, bending, or requiring or forcing a Child to repeat physical movements; or

2. Other forms of aggressive contact harmful to a Child’s body. Cruel or severe punishment, humiliation, or verbal abuse, including but not limited to:

a. Shaming and embarrassing; or

b. Punishment for soiling, wetting or not using the toilet;

c. Humiliating or verbally abusing a Child in any other way.

3. Withholding food, drink, rest or outdoor playtime as punishment or threat of punishment, or being forced to eat or drink against the Child’s will;

4. Exposing a Child to profane language, depictions of violence, use of illicit drugs or sexual content;

5. Requiring a Child to be silent or inactive as a disciplinary measure for a period exceeding that Child’s age in minutes;

6. Unusual confinement, including but not limited to developmentally inappropriate use of high chairs, play yards, cribs or leaving a Child unattended in any room;

7. Withholding any adaptive equipment that would result in loss of a Child’s independence;

8. Using physical restraints, unless trained, and with the review and approval of a Licensed clinician;

9. Lack of supervision;

10. Derogatory remarks to or about Children or parents; or

11. Rough handling.

SECTION 10 (ALL AGES)

A. Rights of Children. Children receiving Child care from Child Care Facilities have the following rights.

1. Children must be free from emotional, physical and/or sexual abuse, neglect

and exploitation.

2. Each Child has the right to freedom from harmful actions or practices that are detrimental to the Child's welfare, and to practices that are potentially harmful to the Child.

3. Each Child has a right to an environment that meets the health and safety standards in this rule.

4. Each Child must be provided Child care services without discrimination to race, age, national origin, religion, disability, sex or family composition.

5. Children must be treated with dignity, consideration and respect in full recognition of their individuality. This includes the use of developmentally appropriate practices by the Child Care Facility.

6. Each Child has the right to the implementation of any plan of service that has been developed for that Child in conjunction with community or state agencies by the Child Care Facility.

7. Each Child has the right to Developmentally Appropriate activities, materials, and equipment.

8. Children with disabilities have the right to reasonable modifications to Child Care Facility policies and practices.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's

Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 9 (ALL AGES)

D. Detrimental practices. Actions that have a reasonable likelihood to be harmful to Children are strictly prohibited. The Provider must ensure that no Child is subjected to an action or practice detrimental to the welfare of Children, including, but not limited to:

1. Corporal punishment. Corporal punishment means physical actions harmful to a Child's body, including, but not limited to, the following
 - a. Slapping, striking, shaking, shoving, spanking, pinching, twisting, kicking, biting, ear pulling or ear twisting;
 - b. Forcing a Child to taste or eat spicy, bitter or otherwise distasteful products for the purpose of Discipline;
 - c. Spraying with water as a means of controlling behavior;
 - d. Placing tape over a Child's mouth;
 - e. Mechanical restraints, such as tying a Child to a chair;
 - f. Requiring or forcing a Child to take an uncomfortable position such as: squatting, kneeling, standing and holding arms outstretched at sides or overhead, bending, or requiring or forcing a Child to repeat physical movements; or
2. Other forms of aggressive contact harmful to a Child's body. Cruel or severe punishment, humiliation, or verbal abuse, including but not limited to:
 - a. Shaming and embarrassing;
 - b. Punishing for soiling, wetting or not using the toilet; or
 - c. Humiliating or verbally abusing a Child in any other way.
3. Withholding food, drink, rest or outdoor playtime as punishment or threat of punishment, or being forced to eat or drink against the Child's will;
4. Exposing a Child to profane language, depictions of violence, use of illicit drugs or sexual content;
5. Requiring a Child to be silent or inactive as a disciplinary measure for a period exceeding that Child's age in minutes;
6. Unusual confinement, including but not limited to developmentally inappropriate use of high chairs, play yards, cribs or leaving a Child unattended in any room;
7. Withholding any adaptive equipment that would result in loss of a Child's independence;
8. Using physical restraints, unless trained, and with the review and approval of a Licensed clinician;
9. Lack of supervision;
10. Derogatory remarks to or about Children or Parents; or
11. Rough handling.

SECTION 10 (ALL AGES)

A. Rights of children. Children receiving childcare from Providers have the

following rights.

1. Children must be free from emotional, physical, sexual abuse, neglect and exploitation.
2. Each Child has the right to freedom from harmful actions or practices that are detrimental to the Child's welfare, and to practices that are potentially harmful to the Child.
3. Each Child has a right to an environment that meets the health and safety standards in this rule.
4. Each Child must be provided childcare services without regard to race, age, national origin, religion, disability, sex or family composition.
5. Children must be treated with dignity, consideration and respect in full recognition of their individuality. This includes the use of Developmentally Appropriate practices by the Provider and Staff Members.
6. Each Child has the right to the implementation of any plan of service that has been developed for that Child in conjunction with community or state agencies by the Provider.
7. Each Child has a right to Developmentally Appropriate activities, materials, and equipment.
8. Children with disabilities have the right to reasonable modifications to Provider policies and practices.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards:**

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;

ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;

iii. Learning strategies for coping with a crying, fussing or distraught Child; and

iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
Maine License-Exempt Standards:

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine

State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

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f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;

ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;

iii. Learning strategies for coping with a crying, fussing or distraught Child; and

iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards:**

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements:
Maintain a maximum group size of fifty (50) for School Age children and maintain

a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

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f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;

ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;

iii. Learning strategies for coping with a crying, fussing or distraught Child; and

iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards:**

Positive methods of child guidance observed/described: The provider must be able to describe how they respond to challenging behaviors in ways that are non-punitive and that do not involve physical discipline (spanking).

Number of children cared for in the home: No more than 2 children that do not reside in the providers home can be cared for. If caring for a family unit in the

family's home, the provider can also care for no more than 2 of their own children.

Recreational Programs must also comply with the following requirements: Maintain a maximum group size of fifty (50) for School Age children and maintain a ratio of one staff for every 15 Children (1:15).

Mandated reporter training: The provider and anyone providing direct care for children must have completed mandated reporter training, and the Mandated Reporter hotline number must be available on site. In accordance with Maine State Law this training must be updated every four (4) years.

Background Checks: A copy of the letter of eligibility that will be provided to the individual by the Office of Child & Family Services (OCFS) will be verified during the on-site inspection.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment is defined as all caregivers/Teachers who are in direct contact with Children including substitute caregivers/Teachers and volunteers:

- i. Receiving training on how to prevent shaken baby syndrome/abusive head trauma;
- ii. Recognizing the potential signs and symptoms of shaken baby syndrome/abusive head trauma;
- iii. Learning strategies for coping with a crying, fussing or distraught Child; and
- iv. Learning the development and vulnerabilities of the brain in infancy and early childhood.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

f. Prevention of Shaken Baby Syndrome, Abusive Head Trauma, and Child maltreatment;

5.3.7 Emergency preparedness and response planning standard

Identify by checking below that the emergency preparedness and response planning due to natural disasters and human-caused events standard includes procedures in the following areas:

- i. ☒ Evacuation
- ii. ☒ Relocation
- iii. ☒ Shelter-in-place
- iv. ☒ Lock down
- v. Staff emergency preparedness
 - ☒ Training
 - ☒ Practice drills
- vi. Volunteer emergency preparedness
 - ☒ Training
 - ☒ Practice drills
- vii. ☒ Communication with families
- viii. ☒ Reunification with families
- ix. ☒ Continuity of operations
- x. Accommodation of
 - ☒ Infants
 - ☒ Toddlers
 - ☒ Children with disabilities
 - ☒ Children with chronic medical conditions
- xi. If any of the above are not checked, describe: **N/A**

5.3.8 Handling and storage of hazardous materials and the appropriate disposal of biocontaminants health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the handling and storage of hazardous materials for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities
 2. The program's policies and procedures;
 3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
 4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);
 5. Prevention of and response to food and allergy emergencies;
 6. Building and physical premise safety;
 7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
 8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
 9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
 10. Adult and pediatric first aid with hands-on skills demonstration;
 11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 12. Department-approved mandated reporter training;
 13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.
- C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.
1. All Child Care Staff Members must register with Maine's Professional Development Network.
 - a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 14

- A. General condition of the building and surrounding premises. The facility must take immediate steps to correct any condition in the physical building or on the premises, which poses a danger to Children's life, health or safety.
4. Any Medications, Toxic Substances, Hazardous Materials, and other items dangerous to Children such as matches, lighters, fireworks, and power tools must be kept where Children cannot access them.
13. The Child Care Facility must Clean surfaces daily in all areas where Children are present. Cleaning products must not be used in close proximity to Children and adequate ventilation must be maintained during use.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

- A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:
 1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;
 3. The Provider's policies and procedures;
 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
 6. Prevention of and response to food and allergy emergencies;
 7. Building and physical premise safety;
 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
 11. Adult and pediatric first aid with hands-on skills demonstration;
 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 13. Mandated Reporter Training.
- B. Ongoing Training.** All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.
1. Annual training must include a review of health and safety topics.
 2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
 3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 14

- A. General condition of the building and surrounding Premises.** A Provider must take immediate steps to correct any condition in the physical building or on the Premises, which poses a danger to Children's life, health or safety.
4. Any Medications, Toxic Substances, Hazardous Materials, and other items dangerous to Children such as matches, lighters, fireworks, and power tools must be kept where Children cannot access them.
 13. The provider must Clean surfaces daily in all areas where Children are present. Cleaning products must not be used in close proximity to Children and adequate ventilation must be maintained during use.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**
☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per

manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio-contaminants is defined as any material that either contains poison or is poisonous, and possibly can cause serious problems or even death, or blood, body fluids or excretions that may spread infectious disease. Exposure can take place through inhalation, skin contact, or ingestion.

i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

1) Wash their hands after contact, even if gloves are worn;

2) Ensure safe waste management by immediately discarding contaminated single use items; and

3) Immediately Clean and disinfect surfaces and reusable equipment.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: Maine License-Exempt Standards**

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

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i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

- 1) Wash their hands after contact, even if gloves are worn;
- 2) Ensure safe waste management by immediately discarding contaminated single use items; and
- 3) Immediately Clean and disinfect surfaces and reusable equipment.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

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h. Handling and storage of hazardous materials and the appropriate disposal of bio-contaminants is defined as any material that either contains poison or is poisonous, and possibly can cause serious problems or even death, or blood, body fluids or excretions that may spread infectious disease. Exposure can take place through inhalation, skin contact, or ingestion.

i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

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- 1) Wash their hands after contact, even if gloves are worn;
- 2) Ensure safe waste management by immediately discarding contaminated single use items; and
- 3) Immediately Clean and disinfect surfaces and reusable equipment.

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h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Hazardous materials handled properly, dangers and toxins inaccessible: All hazardous materials must be stored in the original container and inaccessible to children. Chemicals are stored separately from food and medications, used per manufacturer's instructions, pesticides and chemicals should not be used when children are present. The site must be free of materials and conditions that could cause serious injury to an unsupervised child.

10-148 Chapter 6, Child Care Affordability Program Rules

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h. Handling and storage of hazardous materials and the appropriate disposal of bio-contaminants is defined as any material that either contains poison or is poisonous, and possibly can cause serious problems or even death, or blood, body fluids or excretions that may spread infectious disease. Exposure can take place through inhalation, skin contact, or ingestion.

i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

- 1) Wash their hands after contact, even if gloves are worn;**
- 2) Ensure safe waste management by immediately discarding contaminated single use items; and**
- 3) Immediately Clean and disinfect surfaces and reusable equipment.**

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the

Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

- b. Provide the standards, appropriate to the provider setting and age of children, that address the disposal of bio contaminants for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 12

J. Prevention of exposure to blood and bodily fluids. Child Care Staff Members must take measures to prevent potential exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, Care Staff Members must:

1. Wash their hands after contact, even if gloves are worn;
2. Ensure safe waste management by immediately discarding contaminated single use items; and
3. Immediately clean and disinfect surfaces and reusable equipment.

SECTION 14

N. Diapering/Toileting

2. Wet or soiled disposable diapers must be placed in a lined, lidded, container, separate from other trash or garbage and separate from play or food-preparation areas, or individually bagged and tied within a lidded shared garbage container.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.
2. This Rule;
3. The Provider's policies and procedures;
4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
6. Prevention of and response to food and allergy emergencies;
7. Building and physical premise safety;
8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
11. Adult and pediatric first aid with hands-on skills demonstration;
12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours

of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 12

J. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

1. Wash their hands after contact, even if gloves are worn;
2. Ensure safe waste management by immediately discarding contaminated single use items; and
3. Immediately Clean and disinfect surfaces and reusable equipment.

SECTION 14

N. Diapering/toileting.

2. Wet or soiled disposable diapers must be placed in a lined, lidded container, separate from other trash or garbage and separate from play or food-preparation areas, or individually bagged and tied within a lidded shared garbage container.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R.

§ 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio-contaminants is defined as any material that either contains poison or is poisonous, and possibly can cause serious problems or even death, or blood, body fluids or excretions that may spread infectious disease. Exposure can take place through inhalation, skin contact, or ingestion.

i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

1) Wash their hands after contact, even if gloves are worn;

2) Ensure safe waste management by immediately discarding contaminated single use items; and

3) Immediately Clean and disinfect surfaces and reusable equipment.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **Maine License-Exempt Standards**

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

10-148 Chapter 6, Child Care Affordability Program Rules

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iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

1) Wash their hands after contact, even if gloves are worn;

2) Ensure safe waste management by immediately discarding contaminated single use items; and

3) Immediately Clean and disinfect surfaces and reusable equipment.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum

of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio-contaminants is defined as any material that either contains poison or is poisonous, and possibly can cause serious problems or even death, or blood, body fluids or excretions that may spread infectious disease. Exposure can take place through inhalation, skin contact, or ingestion.

i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

1) Wash their hands after contact, even if gloves are worn;

2) Ensure safe waste management by immediately discarding contaminated single use items; and

3) Immediately Clean and disinfect surfaces and reusable equipment.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Proper disposal of bio-contaminants: When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin adults must wash their hands after contact, even if gloves are worn, immediately discard contaminated single use items and immediately clean and disinfect affected surfaces and reusable equipment.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio-contaminants is defined as any material that either contains poison or is poisonous, and possibly can cause serious problems or even death, or blood, body fluids or excretions that may spread infectious disease. Exposure can take place through inhalation, skin contact, or ingestion.

i. When not in use, all hazardous materials should be stored in the original container in a locked cabinet or room that has a Child-resistant lock and is not accessible to Children.

ii. Chemicals should be stored separately from food and medications. All hazardous materials should be used per the manufacturer's instructions on the label. Pesticides and other chemicals should not be used when Children are present.

iii. Chemicals used to treat lawns should be restricted to chemicals that are approved for use in areas where Children will be present.

iv. Prevention of exposure to blood and body fluids. The Provider must take measures to prevent exposure to blood and other potentially infectious fluids, which may include use of disposable gloves. When touching blood, body fluids, secretions, excretions, mucous membranes, or non-intact skin, providers must:

1) Wash their hands after contact, even if gloves are worn;

2) Ensure safe waste management by immediately discarding contaminated single use items; and

3) Immediately Clean and disinfect surfaces and reusable equipment.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

h. Handling and storage of hazardous materials and the appropriate disposal of bio contaminants;

5.3.9 Precautions in transporting children health and safety standard

Provide the standards, appropriate to the provider setting and age of children, that address precautions in transporting children for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members

are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 17. TRANSPORTATION

A. Driver requirements. Any person driving Children receiving Child care from a Child Care Facility must have the proper license to drive the class of motor vehicle being used, annually submit a signed release authorizing the Department to check the records of the Bureau of Motor Vehicles, receive a qualifying result pursuant to the Bureau of Motor Vehicle Check, and complete Department-approved training for transportation of Children prior to driving Children and at least every two years thereafter.

1. The following offenses disqualify from driving Children receiving Childcare at a Child Care Facility, whenever committed:

a. Motor vehicle violation resulting in death,

b. Aggravated refusing to stop for a law enforcement officer,

c. Criminal homicide in operation of a motor vehicle.

2. The following offenses disqualify from driving Children receiving Childcare at a

Child Care Facility if committed within the last five years:

- a. Criminal OUI,
- b. Operating while license suspended or revoked,
- c. Driving to endanger,
- d. Refusing to stop for a law enforcement officer,
- e. Criminal speeding

B. Vehicle capacity not to be exceeded. The number of persons transported in any vehicle must not exceed the seating capacity of the vehicle, except in the event of emergency evacuation and relocation to a previously established relocation site.

C. Safety in vehicles.

- 1. No Child will be permitted in any vehicle without Adult supervision.
- 2. Children must be seated with seat belts fastened or in Child safety seats in accordance with 29-A M.R.S. § 2081.
- 3. The vehicle requirements above shall not apply to vehicles not operated by the Child Care Facility, such as school buses, taxis, and/or public transportation agencies.
- 4. Children being transported on a school bus that are under the age of 4 must be in Child safety seats in accordance with 29-A M.R.S. § 2081.
- 5. Weapons must not be transported in any vehicle in which Children are riding.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

- 1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.
- 2. This Rule;
- 3. The Provider's policies and procedures;
- 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
- 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
- 6. Prevention of and response to food and allergy emergencies;
- 7. Building and physical premise safety;
- 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
- 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
- 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
- 11. Adult and pediatric first aid with hands-on skills demonstration;
- 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
- 13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 17. TRANSPORTATION

A. Driver requirements. Any person driving Children receiving Child care from a Provider must have the proper license to drive the class of motor vehicle being used, annually submit a signed release authorizing the Department to check the records of the Bureau of Motor Vehicles, receive a qualifying result pursuant to the Bureau of Motor Vehicle Check, and complete Department-approved training for transportation of Children prior to driving Children and at least every two years thereafter.

1. The following offenses disqualify a person from driving Children receiving childcare from a Provider, whenever committed:

- a. Motor vehicle violation resulting in death,
- b. Aggravated refusing to stop for a law enforcement officer,
- c. Criminal homicide in operation of a motor vehicle.

2. The following offenses disqualify a person from driving Children receiving childcare from a Provider if committed within the last five years:

- a. Criminal OUI,
- b. Operating while license is suspended or revoked,
- c. Driving to endanger,
- d. Refusing to stop for a law enforcement officer,
- e. Criminal speeding

B. Vehicle capacity not to be exceeded. The number of persons transported in any vehicle must not exceed the seating capacity of the vehicle, except in the event of emergency evacuation and relocation to a previously established relocation site.

C. Safety in vehicles

1. No Child will be permitted in any vehicle without Adult supervision.
2. Children must be seated with seat belts fastened or in Child safety seats in accordance with 29-A M.R.S. §2081.
3. The vehicle requirements above shall not apply to vehicles not operated by the Provider, such as school buses, taxis, and/or public transportation agencies.
4. Children being transported on a school bus that are under the age of 4 must be in Child safety seats in accordance with 29-A M.R.S. §2081.
5. Weapons must not be transported in any vehicle in which Children are riding.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **Maine License-Exempt Standards**

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCSP needs to be provided during the inspection for all staff who transport.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

i. Appropriate precautions in transporting Children (if applicable) are defined as all providers and staff follow state laws and regulations, program policies, liability, and insurance. Written transportation policies should be in place and should address the safe transport of Children by vehicle to and from the facility, home pickups and deliveries, and special outings such as field trips. Policies should also address the safe care of Children around vehicles, such as during drop-off and pickup times, in parking lots, or anywhere that children may be exposed to moving vehicles;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

i. Appropriate precautions in transporting Children (if applicable);

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: Maine License-Exempt Standards

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCSP needs to be provided during the inspection for all staff who transport.

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i. Appropriate precautions in transporting Children (if applicable);

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **Maine License-Exempt Standards**

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCSP needs to be provided during the inspection for all staff who transport.

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§ 98.41):

i. Appropriate precautions in transporting Children (if applicable) are defined as all providers and staff follow state laws and regulations, program policies, liability, and insurance. Written transportation policies should be in place and should address the safe transport of Children by vehicle to and from the facility, home pickups and deliveries, and special outings such as field trips. Policies should also address the safe care of Children around vehicles, such as during drop-off and pickup times, in parking lots, or anywhere that children may be exposed to moving vehicles;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

i. Appropriate precautions in transporting Children (if applicable);

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **Maine License-Exempt Standards**

Safe transportation practices: Providers that have been approved by the Child Care Affordability Program to transport children must use appropriate restraints (car seats or seat belts, based on the size of the child) if transportation is provided. <http://www.mainelegislature.org/legis/statutes/29-a/title29-asec2081.html>. Transporting on buses does not require appropriate restraints for children age 4 and older. Providers must also hold a valid driver's license and refrain from smoking in the motor vehicle with any child(ren) under the age of sixteen (16), as required by Maine State Law. The Transportation Approval letter from CCSP needs to be provided during the inspection for all staff who transport.

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SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

i. Appropriate precautions in transporting Children (if applicable) are defined as all providers and staff follow state laws and regulations, program policies, liability, and insurance. Written transportation policies should be in place and should address the safe transport of Children by vehicle to and from the facility, home pickups and deliveries, and special outings such as field trips. Policies should also address the safe care of Children around vehicles, such as during drop-off and pickup times, in parking lots, or anywhere that children may be exposed to moving vehicles;

2. All Child Care Providers, Teachers, and directors associated with Child Care

Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

i. Appropriate precautions in transporting Children (if applicable);

5.3.10 Pediatric first aid and pediatric cardiopulmonary resuscitation (CPR) health and safety standard

a. Provide the standards, appropriate to the provider setting and age of children, that address pediatric first aid for all staff for the following CCDF-eligible providers:

i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours

of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.
2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **10-148**

Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):
j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

v. All CCDF-eligible license-exempt family child care homes. Provide the standard:

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

vi. All CCDF-eligible license-exempt in-home care. Provide the standard: 10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- b. Provide the standards, appropriate to the provider setting and age of children, that address pediatric cardiopulmonary resuscitation for all staff for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;
 7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;
 8. Handling and storage of Hazardous material and disposal of Bio-contaminants;
 9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);
 10. Adult and pediatric first aid with hands-on skills demonstration;
 11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 12. Department-approved mandated reporter training;
 13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.
- C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.
1. All Child Care Staff Members must register with Maine's Professional Development Network.
 - a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**
SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;
 3. The Provider's policies and procedures;
 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
 6. Prevention of and response to food and allergy emergencies;
 7. Building and physical premise safety;
 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
 11. Adult and pediatric first aid with hands-on skills demonstration;
 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;
 13. Mandated Reporter Training.
- B. Ongoing Training.** All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.
1. Annual training must include a review of health and safety topics.
 2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.
 3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):
 - j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;
2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):
 - j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**
 - ☒ Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards
 - 1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):
 - j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;
 - 2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):
 - j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules
SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS
F. Health and Safety Standards
 - 1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):
 - j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;
 - 2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):
 - j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **10-148**
Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR) is defined as Providers have learned the priorities, roles, and responsibilities of a rescuer providing first aid or CPR to a Child or an Infant. Included is how to help when a Child or Infant is choking. Proper certification is gained through training;

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

j. Pediatric first-aid and cardiopulmonary resuscitation (CPR);

5.3.11 Identification and reporting of child abuse and neglect health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the identification of child abuse and neglect for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R.

§ 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**
SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.

2. This Rule;

3. The Provider's policies and procedures;

4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);

6. Prevention of and response to food and allergy emergencies;

7. Building and physical premise safety;

8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;

9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;

10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);

11. Adult and pediatric first aid with hands-on skills demonstration;

12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.

2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.

3. All training, including Adult and Pediatric First Aid and CPR certification hours,

will be counted toward the requirement for the year it was completed.

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care

Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- b. Provide your standards, appropriate to the provider setting and age of children, that address the reporting of child abuse and neglect for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities**

SECTION 2

F. Administration.

6. A copy of the Child Care Facility handbook will be made available to all parents or legal guardians at the time of enrollment. The handbook must contain, at a minimum:

m. Incident reporting and mandatory reporting of Child Abuse or Neglect;

9. The Child Care Facility must maintain a Child Care Facility Staff manual that addresses the following:

a. Policies and their implementation. This must include, at a minimum, policies for

the following:

i. Mandated reporting,

10. The facility must adopt written policy for handling all suspected instances of Child abuse or neglect in accordance with Maine statute. Written policy must include, but is not limited to, the following:

a. Internal notification procedure of suspected abuse or neglect;

b. Conditions that require internal notification of the owner, director, or designee;

c. The requirement that any suspicion of abuse and/or neglect must be immediately shared with the owner, director, or designee;

d. The requirement that suspected Child abuse and neglect must be immediately reported upon suspicion to Child Protective Intake hotline;

e. Identification of personnel responsible for contacting the Child Protective Intake hotline;

f. Protocol to notify all relevant parties that suspected Child abuse and neglect has been reported to Child Protective Intake;

g. Completion of incident reports, including: the details of the allegation or suspicion, the date Child Protective Intake was called, which relevant parties were notified the report was filed, and whether or not the parent/guardian was notified of the allegation; and

h. Parental notification, including the determination how and what information will be shared with a parent/legal guardian when Child Protective Intake has been contacted.

11. The Child Care Facility must develop a written policy to follow if an allegation of Child abuse or neglect is made against any Child Care Staff Member at the Child Care Facility. The written policy must include but is not limited to the following:

a. Prevention measures to protect from potential allegations;

d. The requirement to report suspected Child abuse and neglect occurring at any location in accordance with statute;

SECTION 5

A. Record management. Facilities must comply with the following:

4. The Child Care Facility must maintain a separate record of all reports of alleged Child abuse and/or neglect made in accordance with 22 MRS §4011-A.

SECTION 6

B. Mandatory report of suspected Child abuse and/or neglect. Any Child Care Staff Member suspecting abuse and/or neglect must immediately report this information to Child Protective Intake, 1-800-452-1999, TTY 1-800-963-9490 which is staffed 24 hours a day, 7 days a week.

1. The Child Care Facility must maintain documentation, not maintained in the Child's record, that a report has been made.

2. All reporting of suspected Child abuse and/or neglect, and serious injuries of a Child who is under six months of age or otherwise non-ambulatory, must comply with 22 MRS §4011-A.

F. Other Notifications. The Child Care Facility must notify the Department of the following within 24 hours of occurrence:

2. Any Child abuse and neglect allegations naming any Child Care Staff Member(s) associated with the Child Care Facility, in addition to the immediate report to

Child Protective Intake required in Section 6(B) above;

SECTION 8. TRAINING

A. Child Care Facility Responsibility.

The Child Care Facility is responsible for ensuring that all Child Care Staff Members are familiar with the operation of the Child care and understand the relevant requirements of this rule.

1. All Child Care Staff Members must receive training regarding fire drill and other emergency and disaster procedures within the first week of service.

B. Orientation training. All Child Care Staff Members must receive training in the following areas within the first ninety days of service:

1. 10-148 CMR Ch. 32, Child Care Facility Licensing Rule - Child Care Center, Nursery Schools, Small Child Care Facilities

2. The program's policies and procedures;

3. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;

4. Administration of Medication and parental consent (Child Care Staff Members may not administer Medication until training has occurred);

5. Prevention of and response to food and allergy emergencies;

6. Building and physical premise safety;

7. Communicable diseases, infectious disease prevention and control Universal Safety Precautions and Blood Borne Pathogens;

8. Handling and storage of Hazardous material and disposal of Bio-contaminants;

9. Transportation of Children (if the individual will be responsible for the transportation of Children; required biannually thereafter);

10. Adult and pediatric first aid with hands-on skills demonstration;

11. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

12. Department-approved mandated reporter training;

13. For programs serving 13 or more Children, training in the program's health care plan as developed by the program's Health Care Consultant.

C. Ongoing Training. All Child Care Facilities must ensure the appropriate number of hours of ongoing training for Child Care Staff Members.

1. All Child Care Staff Members must register with Maine's Professional Development Network.

a. Annual training must include a review of health and safety topics, as offered by Maine's state-approved Professional Development Network.

SECTION 11

A. Requirement to report Child abuse or neglect. All Child Care Staff Members must immediately contact the Department of Health and Human Service's Child Protective Intake hotline (800-452-1999) when they suspect Child Abuse or Neglect.

B. Child Care Facility requirement to inform. The Director must inform all Child Care Staff Members during orientation of their status as Mandated Reporters and their responsibility to report to the Department of Health and Human Services

whenever there is reasonable cause to suspect Abuse or Neglect of a Child, and comply with ongoing Mandated Reporter training for all Child Care Staff Members as provided in Section 8 of this Rule.

C. Written instruction provided. Child Care Staff Members must receive written instructions regarding Child Abuse or Neglect reporting that contains a summary of the State of Maine Child abuse reporting statute and a statement that they will not be discharged or disciplined solely because they have made a Child Abuse or Neglect report.

D. Hotline number posted. The Child Care Facility must ensure that the telephone number of the Department's Child Protective Intake hotline is posted in a readily accessible central location on the Premises.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **10-148 CMR Ch. 33, Family Child Care Provider Licensing Rule**

SECTION 2

G. Administration.

4. A copy of the Provider handbook will be made available to all Parents or Legal Guardians at the time of enrollment. The handbook must contain, at a minimum:

m. Incident reporting and mandatory reporting of Child Abuse or Neglect;

7. The Provider must maintain a staff manual that addresses the following:

a. Policies and their implementation. This must include, at a minimum, policies for the following:

i. Mandated reporting,

8. The Provider must adopt a written policy for handling all suspected instances of Child Abuse or Neglect in accordance with Maine statute. Written policy must include, but is not limited to, the following:

a. Internal notification procedure of suspected Abuse or Neglect,

b. Conditions that require internal notification of the Provider,

c. The requirement that any suspicion of Abuse or Neglect must be immediately shared with the Provider,

d. The requirement that suspected Child Abuse or Neglect must be immediately reported upon suspicion to Child Protective Intake hotline,

e. Identification of personnel responsible for contacting the Child Protective Intake hotline,

f. Protocol to notify all relevant parties (Parents, Staff Members, interns, or Household Members) that suspected Child Abuse or Neglect has been reported to Child Protective Intake,

g. Completion of Incident reports, including: the details of the allegation or suspicion, the date Child Protective Intake was called, which Staff Members and/or interns were notified the report was filed, and whether or not the Parent/Legal Guardian were notified of the allegation, and

h. Parental notification, including the determination of how and what information will be shared with a Parent/Legal Guardian when Child Protective Intake has been contacted.

9. The Provider must develop a written policy to follow if an allegation of Child Abuse or Neglect is made against the Provider, any Staff Member, or Household Member in the Provider's home. The written policy must include but is not limited

to the following:

- a. Prevention measures to protect providers from potential allegations;
- d. The requirement to report suspected Child Abuse or Neglect occurring at any location in accordance with statute;

SECTION 5

A. Record management. Providers must comply with the following:

- 4. Maintain a separate record of all reports of alleged Child Abuse or Neglect made in accordance with 22 MRS §4011-A.

SECTION 6

B. Mandatory report of suspected Abuse or Neglect. Any provider suspecting Abuse or Neglect must immediately report this information to Child Protective Intake Services, 1-800-452-1999, TTY 1-800-963-9490 which is staffed 24 hours a day, 7 days a week.

- 1. The provider must maintain documentation, not maintained in the Child's record, that a report has been made.

- 2. All reporting of suspected Child Abuse or Neglect, and serious injuries of a Child who is under six months of age or otherwise non-ambulatory, must comply with 22 MRS §4011-A.

F. Other notifications. The Provider must notify the Department of the following within 24 hours of occurrence:

- 2. Any Child Abuse or Neglect allegations naming any Provider or Staff Member associated with the Provider, in addition to the immediate report to Child Protective Intake required in Section 6(B) above;

SECTION 8. TRAINING

A. Orientation training. Except when another time period is indicated, all Staff Members must receive training in the following areas within the first ninety days of service:

- 1. Fire drill procedures, disaster procedures, and all other emergency procedures. This training must be completed within the first week of service.
- 2. This Rule;
- 3. The Provider's policies and procedures;
- 4. Prevention of Sudden Infant Death Syndrome (SIDS) or Sudden Unexplained Infant Death (SUID), Shaken Baby Syndrome, and safe sleep practices;
- 5. Administration of Medication and Parental consent (providers may not administer Medication until training has occurred);
- 6. Prevention of and response to food and allergy emergencies;
- 7. Building and physical premise safety;
- 8. Communicable diseases, infectious disease prevention and control, Universal Safety Precautions and Blood Borne Pathogens;
- 9. Handling and storage of Hazardous Material and disposal of Bio-contaminants;
- 10. Transportation of Children (if the individual will be responsible for the transportation of Children; required biennially thereafter);
- 11. Adult and pediatric first aid with hands-on skills demonstration;
- 12. Adult and pediatric Cardio-Pulmonary Resuscitation (CPR) with hands-on skills demonstration;

13. Mandated Reporter Training.

B. Ongoing Training. All Providers must ensure the appropriate number of hours of ongoing training for themselves and Staff Members. Twelve hours of training is required annually. All Providers and Staff Members must register with Maine's Professional Development Network.

1. Annual training must include a review of health and safety topics.

2. Ongoing training must include Department-approved Mandated Reporter retraining at least once every four years.

3. All training, including Adult and Pediatric First Aid and CPR certification hours, will be counted toward the requirement for the year it was completed.

SECTION 11

A. Requirement to report child abuse or neglect. Every Provider or Staff Member must immediately contact the Department of Health and Human Service's Child Protective Intake hotline (800-452-1999) when they suspect Child Abuse or Neglect.

B. Provider requirement to inform. The Provider must inform all Staff Members during orientation of their status as Mandated Reporters and their responsibility to report to the Department of Health and Human Services whenever there is reasonable cause to suspect Abuse or Neglect of a Child, and comply with ongoing Mandated Reporter training for all Staff Members as provided in Section 8 of this Rule.

C. Written instruction provided. Staff members must receive written instructions regarding Child Abuse or Neglect reporting that contains a summary of the State of Maine Child abuse reporting statute and a statement that they will not be discharged or disciplined solely because they have made a Child Abuse or Neglect report.

D. Hotline number posted. The Provider must ensure that the telephone number of the Department's Child Protective Intake hotline is posted in a readily accessible central location on the Premises.

iii. All CCDF-eligible licensed in-home care. Provide the standard: **N/A**

☒ Not applicable.

iv. All CCDF-eligible license-exempt center care. Provide the standard: **10-148**

Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection

from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:

10-148 Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **10-148**

Chapter 6, Child Care Affordability Program Rules

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment,

sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **10-148 Chapter 6, Child Care Affordability Program Rules**

SECTION 8: CHILD CARE PROVIDER QUALIFICATIONS AND REQUIREMENTS

F. Health and Safety Standards

1. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must have attained a minimum of a Star One (1) quality rating from Maine Roads to Quality and successfully completed the Department-Approved orientation training that includes, at a minimum, the Health and Safety Standards below, within ninety (90) calendar days of beginning his/her work with Children as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect is defined as a threat to a Child's health or welfare by physical, mental or emotional injury or impairment, sexual abuse or exploitation, deprivation of essential needs or lack of protection from these, by a person responsible for the Child. Any provider suspecting abuse or neglect must report this information to Child Protective Intake Services, which is staffed twenty-four (24) hours a day, seven (7) days a week. The Provider must maintain documentation that a report has been made.

2. All Child Care Providers, Teachers, and directors associated with Child Care Providers (excluding Relative Child Care Providers) must successfully complete the Department-Approved annual training, that includes, at a minimum, inclusion of Children with Special Needs, Homelessness, and the minimum Health and Safety Standards listed below as required by federal law (45 C.F.R. § 98.41):

k. Recognition and reporting of Child abuse or neglect.

- c. Confirm if child care providers must comply with the [Lead Agency's](#) procedures for reporting child abuse and neglect as required by the Child Abuse Prevention and Treatment Act (42 U.S.C. 5106a(b)(2)(B)(i):

☒ Yes, confirmed.

☐ No. If no, describe:

5.3.12 Additional optional standards

In addition to the required health and safety standards, does the Lead Agency require providers to comply with the following optional standards?

☐ Yes.

☒ No. If no, skip to Section 5.4

If yes, describe the standard(s).

- i. Nutrition. Describe:
- ii. Access to physical activity. Describe:
- iii. Caring for children with special needs. Describe:
- iv. Any other areas determined necessary to promote child development or to protect children's health and safety. Describe:

5.4 Pre-Service or Orientation Training on Health and Safety Standards

Lead Agencies must have requirements for all caregivers, teachers, and directors at CCDF providers to complete pre-service or orientation training (within 3 months of starting) on all CCDF health and safety standards and child development. The training must be appropriate to the setting and the age of children served. This training must address the required health and safety standards and the content area of child development. Lead Agencies have flexibility in determining the minimum number of training hours to require, and are encouraged to consult with Caring for our Children Basics for best practices.

Exemptions for relative providers' training requirements are addressed in question 5.8.1.

5.4.1 Health and safety pre-service/orientation training requirements

Lead Agencies must certify staff have pre-service or orientation training on each standard that is appropriate to different settings and age groups. Lead Agencies may require pre-service or orientation to be completed before staff can care for children unsupervised. In the table below, check the boxes for which you have training requirements.

	Is this standard addressed in the pre-service or orientation training?	Is the pre-service or orientation training on this standard appropriate to different settings and age groups?	Does the Lead Agency require staff to complete the training before caring for children unsupervised?
a. Prevention and control of infectious diseases (including immunizations)	☒	☒	☐
b. SIDS prevention and use of safe sleep practices	☒	☒	☐
c. Administration of	☒	☒	☐

medication			
d. Prevention and response to food and allergic reactions	☒	☒	☐
e. Building and physical premises safety, including identification of and protection from hazards, bodies of water, and vehicular traffic	☒	☒	☐
f. Prevention of shaken baby syndrome, abusive head trauma and child maltreatment	☒	☒	☐
g. Emergency preparedness and response planning and procedures	☒	☒	☐
h. Handling and storage of hazardous materials and disposal of biocontaminants	☒	☒	☐
i. Appropriate Precautions in transporting children, if applicable	☒	☒	☐
j. Pediatric first aid and pediatric CPR (age-appropriate)	☒	☒	☐
k. Child abuse and neglect recognition and reporting	☒	☒	☐
l. Child development including major domains of cognitive, social, emotional, physical development and approaches to learning.	☒	☒	☐

- m. If the Lead Agency does not certify implementation of all the health and safety pre-service/orientation training requirements for staff in programs serving children receiving CCDF assistance, please describe: **The Lead Agency certifies implementation of all the health and safety pre- service/orientation training requirements for staff in programs serving children receiving CCDF assistance.**

- n. Are there any provider categories to whom the above pre-service or orientation training requirements do not apply?

☐ No

☒ Yes. If yes, describe: **CCAP License-Exempt relative providers.**

5.5 Monitoring and Enforcement of Licensing and Health and Safety Requirements

5.5.1 Inspections for licensed CCDF providers

Licensing inspectors must perform at least one annual, unannounced inspection of each licensed CCDF provider for compliance with all child care licensing standards, including an inspection for compliance with health and safety and fire standards. Lead Agencies must conduct at least one pre-licensure inspection for compliance with health, safety, and fire standards of each child care provider and facility in the State/Territory.

- a. Licensed CCDF center-based providers

- i. Does your pre-licensure inspection for licensed center-based providers assess compliance with health standards, safety standards, and fire standards?

☒ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed center-based providers addressing compliance with health, safety, and fire standards:

☐ Annually.

☒ More than once a year. If more than once a year, describe: **Child Care providers are inspected based on a differential monitoring system. Providers receive unannounced inspections based on their compliance history in 1, 3, 6 month or 1-year intervals. The frequency of inspections changes based on the results of inspections, implementation of health and safety requirements per CCDF Rule, complaint investigations and any subsequent licensing action.**

☐ Other. If other, describe:

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed center-based providers?

☒ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements. **Child Care providers are inspected based on a differential monitoring system. Providers receive unannounced inspections based on their compliance history in 1, 3, 6 month or 1-year intervals. The frequency of inspections changes based on the results of inspections, implementation of health and safety requirements per CCDF Rule, complaint investigations and any subsequent licensing action.**

☐ No. If no, describe:

- iv. Identify which department or agency is responsible for completing the inspections for licensed center-based providers. **The Lead Agency's Children's Licensing and Investigation Services oversees the site inspections for licensed center-based**

providers.

b. Licensed CCDF family child care providers

- i. Does your pre-licensure inspection for licensed family child care homes assess compliance with health standards, safety standards, and fire standards?

☒ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed family child care homes addressing compliance with health, safety, and fire standards:

☐ Annually.

☒ More than once a year. If more than once a year, describe: **Child Care providers are inspected based on a differential monitoring system. Providers receive unannounced inspections based on their compliance history in 1, 3, 6 month or 1-year intervals. The frequency of inspections changes based on the results of inspections, implementation of health and safety requirements per CCDF Rule, complaint investigations and any subsequent licensing action.**

☐ Other. If other, describe:

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed family child care providers?

☒ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements. **Child Care providers are inspected based on a differential monitoring system. Providers receive unannounced inspections based on their compliance history in 1, 3, 6 month or 1-year intervals. The frequency of inspections changes based on the results of inspections, implementation of health and safety requirements per CCDF Rule, complaint investigations and any subsequent licensing action.**

☐ No. If no, describe:

- iv. Identify which department or agency is responsible for completing the inspections for licensed family child care providers. **The Lead Agency's Children's Licensing and Investigation Services oversees the site inspections for licensed family child care providers.**

c. Licensed in-home CCDF child care providers

- i. Does your Lead Agency license CCDF in-home child care (care in the child's own home) providers?

☒ No.

☐ Yes. If yes, does your pre-licensure inspection for licensed in-home providers assess compliance with health, safety, and fire standards?

☐ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed in-home

child care providers for compliance with health, safety, and fire standards completed:

☐ Annually.

☐ More than once a year. If more than once a year, describe:

☒ Other. If other, describe: **N/A**

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed in-home child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iv. Identify which department or agency is responsible for completing the inspections for licensed in-home providers. **N/A**

5.5.2 Inspections for license-exempt providers

Licensing inspectors must perform at least one annual monitoring visit of each license-exempt CCDF provider for compliance with health, safety, and fire standards. Inspections for relative providers will be addressed in subsection 5.8.

Describe the policies and practices for the annual monitoring of:

- a. License-exempt CCDF center-based child care providers

- i. Identify the frequency of inspections for compliance with health, safety, and fire standards for license-exempt center-based providers:

☒ Annually.

☐ More than once a year. If more than once a year, describe:

☐ Other. If other, describe:

- ii. Does the Lead Agency implement a differential monitoring approach when monitoring license-exempt center-based providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iii. Identify which department or agency is responsible for completing the inspections for license-exempt center-based CCDF providers. **The Lead Agency's Children's Licensing and Investigation Services oversees the annual site inspections for all CCAP License-Exempt Providers.**

- b. License-exempt CCDF family child care providers

- i. Identify the frequency of the inspections of license-exempt family child care providers to determine compliance with health, safety, and fire standards:

☒ Annually.

☐ More than once a year. If more than once a year, describe:

☐ Other. If other, describe:

- ii. Does the Lead Agency implement a differential monitoring approach when monitoring license-exempt family child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iii. Identify which department or agency is responsible for completing the inspections for license-exempt family child care providers. **The Lead Agency's Children's Licensing and Investigation Services oversees the annual site inspections for all CCAP License-Exempt Providers.**

5.5.3 Inspections for CCDF license-exempt in-home child care providers

Lead Agencies may develop alternate monitoring requirements for care provided in the child's home that are appropriate to the setting. This flexibility cannot be used to bypass the monitoring requirement altogether.

- a. Describe the requirements for the annual monitoring of CCDF license-exempt in-home child care (care in the child's own home) providers, including if monitoring is announced or unannounced, occurs more frequently than once per year, and if differential monitoring procedures are used. **The Lead Agency's Children's Licensing and Investigation Services oversees the annual site visits for all CCAP License-Exempt Providers that are announced.**
- b. List the entity(ies) in your State/Territory responsible for conducting inspections of license-exempt CCDF in-home child care (care in the child's own home) providers: **The Lead Agency's Children's Licensing and Investigation Services oversees the annual site inspections for all CCAP License-Exempt Providers.**

5.5.4 Posting monitoring and inspection reports

Lead Agencies must post monitoring and inspection reports on their consumer education website for each licensed and CCDF child care provider, except in cases where the provider is related to all the children in their care. These reports must include the results of required annual monitoring visits and visits due to major substantiated complaints about a provider's failure to comply with health and safety requirements and child care policies. A full report covers everything in the monitoring visit, including areas of compliance and non-compliance. If the Lead Agency does not produce any reports that include areas of compliance, the website must include information about all areas covered by a monitoring visit.

The reports must be in plain language or provide a plain language summary Lead Agency and be timely to ensure that the results of the reports are available and easily understood by parents when they are deciding on a child care provider. Lead Agencies must post at least 3 years of monitoring and inspection reports.

- a. Does the Lead Agency post:
- i. ☒ Pre-licensing inspection reports for licensed programs.
- ii. ☒ Full monitoring and inspection reports that include areas of compliance and

- non-compliance for all non-relative providers eligible to provide CCDF services.
- iii. ☐ Monitoring and inspection reports that include areas of non-compliance only, with information about all areas covered by a monitoring visit posted separately on the website (e.g., a blank checklist used by monitors) for all non-relative providers eligible to provide CCDF services. If checked, provide a direct URL/website link to the website where a blank checklist is posted:
 - iv. ☐ Other. Describe:
- b. Check if the monitoring and inspection reports and any related plain language summaries include:
- i. ☒ Date of inspection.
 - ii. ☒ Health and safety violations, including those violations that resulted in fatalities or serious injuries occurring at the provider. Describe how these health and safety violations are prominently displayed: **All monitoring reports are posted on www.childcarechoices.me, which includes inspection reports and noncompliance reports. The inspection reports include the date of inspection, any health and safety violations marked, and summaries of next steps to address any items that are in noncompliance.**
 - iii. ☒ Corrective action plans taken by the Lead Agency and/or child care provider. Describe: **The Non-Compliance reports are posted in the same public site as the inspection report www.childcarechoices.me, and are part of the list of all monitoring reports. The Non-Compliance reports stay posted on the site for three years. Non-compliance reports have corrective action plans, called Directed Plans of Action. These action plans must be prominently displayed at the child care program when issued, and until further notice by a Licensor. The corrective action plans outline all health and safety violations with citations of the corresponding Rule section. The Directed Plans of Action are posted publicly on**
 - iv. ☒ A minimum of 3 years of results, where available.
 - v. If any of the components above are not selected, please explain: **N/A**
- c. Lead Agencies must post monitoring and inspection reports and/or any related summaries in a timely manner.
- i. Provide the direct URL/website link to where the reports are posted:
<https://www.childcarechoices.me/>
 - ii. Identify the Lead Agency's established timeline for posting monitoring reports and describe how it is timely: **Inspection reports are posted after supervisory review and generally within 30 days of the inspection.**
- d. Does the Lead Agency certify that the monitoring and inspection reports or the summaries are in plain language that is understandable to parents and other consumers?
- ☒ Yes.
- ☐ No. If no, describe:
- e. Does the Lead Agency certify that there is a process for correcting inaccuracies in the monitoring and inspection reports?

☒ Yes.

☐ No. If no, describe:

- f. Does the Lead Agency maintain monitoring and inspection reports on the consumer education website?

☒ Yes.

☐ No. If no, describe:

5.5.5 Qualifications and training of licensing inspectors

Lead Agencies must ensure that individuals who are hired as licensing inspectors (or qualified monitors designated by the Lead Agency) are qualified to inspect child care providers and facilities and have received health and safety training appropriate to the provider setting and age of the children served.

Describe how the Lead Agency ensures that licensing inspectors (or qualified monitors designated by the Lead Agency) are qualified and have received training on health and safety requirements that are appropriate to the age of the children in care and the type of provider setting. **Child Care Licensing Specialists receive licensing certification training to include National Association for Regulatory Administration's credential following 1-year of employment and required health and safety training within the 90 days of hire. The SSPS II - Child Care Licensing, Children's Licensing Supervisor ensures all trainings are complete during the three-month employee review.**

5.5.6 Ratio of licensing inspectors

Lead Agencies must ensure the ratio of licensing inspectors to child care providers and facilities in the State/Territory are maintained at a level sufficient to enable the Lead Agency to conduct effective inspections of child care providers and facilities on a timely basis in accordance with federal, State, and local laws.

Provide the ratio of licensing inspectors to child care providers (i.e., number of inspectors per number of child care providers) and facilities in the State/Territory and include how the ratio is sufficient to conduct effective inspections on a timely basis. **The State of Maine currently has eighteen (18) Child Care Licensing Specialists within CLIS with caseloads that average 75 to 100 providers based on geographic location. While the statutory requirement is one unannounced inspection per year, CLIS is currently utilizing a risk based differential monitoring algorithm to ensure the ratio is sufficient to conduct inspections on a timely basis. CLIS management team comprised of the Children's Licensing & Investigation Manager and the Children's Licensing Supervisors review a monthly report on each Licensing Specialist's caseload and move or add cases as needed to ensure they can meet inspection timeframe.**

5.6 Ongoing Health and Safety Training

Lead Agencies must have ongoing training requirements for all caregivers, teachers, and directors of eligible CCDF providers for health and safety standards but have discretion on frequency and training content (e.g., pediatric CPR refresher every year and recertification every 2 years). Lead Agencies have discretion on which health and safety standards are subject to ongoing training. Lead Agencies may exempt relative providers from these requirements.

5.6.1 Required ongoing training of health and safety standards

Describe any required ongoing training of health and safety standards for caregivers, teachers, and directors of the following CCDF eligible provider types.

- a. Licensed child care centers: **The annual state approved 2-hour training always addresses at least one of the following CCDF required health and safety standards:**
 - 98.41(a)(1)(i): H&S Requirements - Prevention and Control of Infectious Diseases (including immunizations)**
 - 98.41(a)(1)(ii): H&S Requirements - SIDS Prevention**
 - 98.41(a)(1)(iii): H&S Requirements - Medication Administration**
 - 98.41(a)(1)(iv): H&S Requirements - Food & Allergic Reactions**
 - 98.41(a)(1)(v): H&S Requirements - Building and Physical Premises**
 - 98.41(a)(1)(vi): H&S Requirements ☑ Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment**
 - 98.41(a)(1)(vii): H&S Requirements - Emergency Preparedness and Response Planning**
 - 98.41(a)(1)(viii): H&S Requirements - Hazardous Materials and Bio-contaminants**
 - 98.41(a)(1)(ix): H&S Requirements - Transporting Children**
 - 98.41(a)(1)(x): H&S Requirements - Pediatric First Aid and CPR**
 - 98.41(a)(1)(xi): H&S Requirements - Child Abuse and Neglect Recognition and Reporting**
- b. License-exempt child care centers: **The annual state approved 2-hour training always addresses at least one of the following CCDF required health and safety standards:**
 - 98.41(a)(1)(i): H&S Requirements - Prevention and Control of Infectious Diseases (including immunizations)**
 - 98.41(a)(1)(ii): H&S Requirements - SIDS Prevention**
 - 98.41(a)(1)(iii): H&S Requirements - Medication Administration**
 - 98.41(a)(1)(iv): H&S Requirements - Food & Allergic Reactions**
 - 98.41(a)(1)(v): H&S Requirements - Building and Physical Premises**
 - 98.41(a)(1)(vi): H&S Requirements ☑ Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment**
 - 98.41(a)(1)(vii): H&S Requirements - Emergency Preparedness and Response Planning**
 - 98.41(a)(1)(viii): H&S Requirements - Hazardous Materials and Bio-contaminants**
 - 98.41(a)(1)(ix): H&S Requirements - Transporting Children**
 - 98.41(a)(1)(x): H&S Requirements - Pediatric First Aid and CPR**
 - 98.41(a)(1)(xi): H&S Requirements - Child Abuse and Neglect Recognition and Reporting**
- c. Licensed family child care homes: **The annual state approved 2-hour training always addresses at least one of the following CCDF required health and safety standards:**
 - 98.41(a)(1)(i): H&S Requirements - Prevention and Control of Infectious Diseases (including immunizations)**
 - 98.41(a)(1)(ii): H&S Requirements - SIDS Prevention**
 - 98.41(a)(1)(iii): H&S Requirements - Medication Administration**
 - 98.41(a)(1)(iv): H&S Requirements - Food & Allergic Reactions**
 - 98.41(a)(1)(v): H&S Requirements - Building and Physical Premises**
 - 98.41(a)(1)(vi): H&S Requirements ☑ Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment**
 - 98.41(a)(1)(vii): H&S Requirements - Emergency Preparedness and Response Planning**
 - 98.41(a)(1)(viii): H&S Requirements - Hazardous Materials and Bio-contaminants**
 - 98.41(a)(1)(ix): H&S Requirements - Transporting Children**
 - 98.41(a)(1)(x): H&S Requirements - Pediatric First Aid and CPR**

- 98.41(a)(1)(xi): H&S Requirements - Child Abuse and Neglect Recognition and Reporting**
- d. License-exempt family child care homes: **The annual state approved 2-hour training always addresses at least one of the following CCDF required health and safety standards:**
98.41(a)(1)(i): H&S Requirements - Prevention and Control of Infectious Diseases (including immunizations)
98.41(a)(1)(ii): H&S Requirements - SIDS Prevention
98.41(a)(1)(iii): H&S Requirements - Medication Administration
98.41(a)(1)(iv): H&S Requirements - Food & Allergic Reactions
98.41(a)(1)(v): H&S Requirements - Building and Physical Premises
98.41(a)(1)(vi): H&S Requirements ☐ Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment
98.41(a)(1)(vii): H&S Requirements - Emergency Preparedness and Response Planning
98.41(a)(1)(viii): H&S Requirements - Hazardous Materials and Bio-contaminants
98.41(a)(1)(ix): H&S Requirements - Transporting Children
98.41(a)(1)(x): H&S Requirements - Pediatric First Aid and CPR
98.41(a)(1)(xi): H&S Requirements - Child Abuse and Neglect Recognition and Reporting
 - e. Regulated or registered in-home child care: **N/A**
 - f. Non-regulated or registered in-home child care: **The annual state approved 2-hour training always addresses at least one of the following CCDF required health and safety standards:**
98.41(a)(1)(i): H&S Requirements - Prevention and Control of Infectious Diseases (including immunizations)
98.41(a)(1)(ii): H&S Requirements - SIDS Prevention
98.41(a)(1)(iii): H&S Requirements - Medication Administration
98.41(a)(1)(iv): H&S Requirements - Food & Allergic Reactions
98.41(a)(1)(v): H&S Requirements - Building and Physical Premises
98.41(a)(1)(vi): H&S Requirements ☐ Shaken Baby Syndrome, Abusive Head Trauma, and Child Maltreatment
98.41(a)(1)(vii): H&S Requirements - Emergency Preparedness and Response Planning
98.41(a)(1)(viii): H&S Requirements - Hazardous Materials and Bio-contaminants
98.41(a)(1)(ix): H&S Requirements - Transporting Children
98.41(a)(1)(x): H&S Requirements - Pediatric First Aid and CPR
98.41(a)(1)(xi): H&S Requirements - Child Abuse and Neglect Recognition and Reporting

5.7 Comprehensive Background Checks

Lead Agencies must conduct comprehensive background checks for all child care staff members (including prospective staff members) of all child care providers that are (1) licensed, regulated, or registered under State/Territory law, regardless of whether they receive CCDF funds; or (2) all other child care providers eligible to deliver CCDF services (e.g., license-exempt CCDF eligible child care providers). Family child care home providers must also submit background check requests for all household members age 18 or older.

A comprehensive background check must include: three in-state checks, two national checks, and three interstate checks if the individual resided in another State or Territory in the preceding 5 years. The background check components must be completed at least once every five years.

All child care staff members must receive a qualifying result from either the FBI criminal background check or an in-state fingerprint criminal history check before working (under supervision) with or near children. Lead Agencies must apply a CCDF-specific list of disqualifying crimes for child care providers serving families participating in CCDF.

These background check requirements do not apply to individuals who are related to all children for whom child care services are provided. Exemptions for relative providers will be addressed in subsection 5.8.

5.7.1 In-state criminal history check with fingerprints

- a. Does the Lead Agency conduct in-state criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct in-state criminal background checks with fingerprints.

- b. Does the Lead Agency conduct in-state criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers) other than relative providers?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct in-state criminal background checks with fingerprints.

- c. Does the Lead Agency conduct the in-state criminal background check with fingerprints for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an in-state criminal background check with fingerprints.

5.7.2 National Federal Bureau of Investigation (FBI) criminal history check with fingerprints

- a. Does the Lead Agency conduct FBI criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct FBI criminal background checks with fingerprints.

- b. Does the Lead Agency conduct FBI criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct FBI criminal background checks.

- c. Does the Lead Agency conduct the FBI criminal background check with fingerprints for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an FBI criminal background check with fingerprints.

5.7.3 National Crime Information Center (NCIC) National Sex Offender Registry (NSOR) name-based check

The majority of NCIC NSOR records are fingerprint records and are automatically included in the FBI fingerprint criminal background check. But a small percentage of NCIC NSOR records are only name-based records and must be accessed through the required name-based search of the NCIC NSOR.

- a. Does the Lead Agency conduct NCIC NSOR name-based background checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct NCIC NSOR name-based background checks.

- b. Does the Lead Agency conduct NCIC NSOR name-based background checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct NCIC NSOR name-based background checks.

- c. Does the Lead Agency conduct the NCIC NSOR name-based background check for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive a NCIC NSOR name-based background check.

5.7.4 In-state sex offender registry (SOR) check

- a. Does the Lead Agency conduct in-state SOR checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct in-state SOR background checks.

- b. Does the Lead Agency conduct in-state SOR background checks for all child care staff

members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct in-state SOR background checks.

- c. Does the Lead Agency conduct the in-state SOR background check for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an in-state SOR background check.

5.7.5 In-state child abuse and neglect (CAN) registry check

- a. Does the Lead Agency conduct CAN registry checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct CAN registry checks.

- b. Does the Lead Agency conduct CAN registry checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct CAN registry checks.

- c. Does the Lead Agency conduct the CAN registry check for all individuals age 18 or older who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive a CAN registry check.

5.7.6 Interstate criminal history check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate criminal history background checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate criminal history background checks.

- b. Does the Lead Agency conduct interstate criminal history background checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate criminal history background checks.

- c. Does the Lead Agency conduct interstate criminal history background checks for all individuals age 18 or older who reside in a family child care home and resided in other state(s) in the past 5 years.

☒ Yes.

☐ No. If no, describe why individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive an interstate criminal history background check.

5.7.7 Interstate Sex Offender Registry (SOR) check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate SOR checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate SOR checks.

- b. Does the Lead Agency conduct interstate SOR checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate SOR checks.

- c. Does the Lead Agency conduct the interstate SOR checks for all individuals age 18 or older who resided in other state(s) in the past 5 years who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive an interstate SOR check.

5.7.8 Interstate child abuse and neglect (CAN) registry check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate CAN registry checks for any staff member (or prospective staff member) that resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate CAN registry checks.

- b. Does the Lead Agency conduct interstate CAN registry checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☒ Yes.

☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate CAN registry checks.

- c. Does the Lead Agency conduct the interstate CAN registry checks for all individuals age 18 or older who resided in other state(s) in the past 5 years who reside in a family child care home?

☒ Yes.

☐ No. If no, describe individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive interstate CAN registry checks.

5.7.9 Disqualifications for child care employment

The Lead Agency must prohibit employment of individuals with child care providers receiving CCDF subsidy payment if they meet any of the following disqualifying criteria:

- Refused to consent to a background check.
- Knowingly made materially false statements in connection with the background check.
- Are registered, or are required to be registered, on the State/Territory sex offender registry or repository or the National Sex Offender Registry.
- Have been convicted of a felony consisting of murder, child abuse or neglect, crimes against children (including child pornography), spousal abuse, crimes involving rape or sexual assault, kidnapping, arson, physical assault, or battery.
- Have a violent misdemeanor committed as an adult against a child, including the following crimes: child abuse, child endangerment, sexual assault, or any misdemeanor involving child pornography.
- Convicted of a felony consisting of a drug-related offense committed during the preceding 5 years.

- a. Does the Lead Agency disqualify the employment of child care staff members (including

prospective staff members) by child care providers receiving CCDF subsidy payment for CCDF-identified disqualifying criteria?

☒ Yes.

☐ No. If no, describe the disqualifying criteria:

- b. Does the Lead Agency use the same criteria for licensed, regulated, and registered child care providers regardless of CCDF participation?

☒ Yes.

☐ No. If no, describe any disqualifying criteria used for licensed, regulated, and registered child care providers:

- c. How does the Lead Agency use results from the in-state child abuse and neglect registry check?

☐ Does not use them to disqualify employment.

☒ Uses them to disqualify employment. If checked, describe: **The disqualification is a substantiation and on the State Child Abuse and Neglect Registry. This means that DHHS has determined, by a preponderance of evidence, that child abuse or child neglect occurred by a person responsible for the child's health, welfare, or care.**

The Lead Agency utilizes a Matrix of Disqualifying Criminal Convictions for employment 45 C.F.R. 98.43(c)(1).

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/documents/ocfs/documents/Matrix%20of%20Disqualifying%20Criminal%20Convictions%20for%20Child%20Care%20Providers.pdf>

- d. How does the Lead Agency use results from the interstate child abuse and neglect registry check?

☐ Does not use them to disqualify employment.

☒ Uses them to disqualify employment. If checked, describe: **For out of state, we use that state's guidelines on what constitutes a substantiation. Typically the process involves the person being on the child abuse registry in that state. The disqualification is a substantiation, which means that DHHS has determined, by a preponderance of evidence, that child abuse or child neglect occurred by a person responsible for the child's health, welfare, or care.**

The Lead Agency utilizes a Matrix of Disqualifying Criminal Convictions for employment 45 C.F.R. 98.43(c)(1).

<https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/documents/ocfs/documents/Matrix%20of%20Disqualifying%20Criminal%20Convictions%20for%20Child%20Care%20Providers.pdf>

5.7.10 Privacy

Lead Agencies must ensure the privacy of a prospective staff member by notifying child care providers of the individual's eligibility or ineligibility for child care employment based on the results of the comprehensive background check without revealing any documentation of criminal history or disqualifying crimes or other related information regarding the individual.

Does the Lead Agency certify they ensure the privacy of child care staff members (including prospective child care staff member) when providing the results of the comprehensive background check?

☒ Yes.

☐ No. If no, describe the current process of notification:

5.7.11 Appeals processes for background checks

Lead Agencies must provide for a process that allows child care provider staff members (and prospective staff members) to appeal the results of a background check to challenge the accuracy or completeness of the information contained in the individual's background check report.

Does the appeals process:

- i. Provide the affected individual with information related to each disqualifying crime in a report, along with information/notice on the opportunity to appeal.

☒ Yes.

☐ No. Describe:

- ii. Provide the affected individual with clear instructions about how to complete the appeals process for each background check component if they wish to challenge the accuracy or completeness of the information contained in such individual's background report.

☒ Yes.

☐ No. Describe:

- iii. Ensure the Lead Agency attempts to verify the accuracy of the information challenged by the individual, including making an effort to locate any missing disposition information related to the disqualifying crime.

☒ Yes.

☐ No. Describe:

- iv. Get completed in a timely manner.

☒ Yes.

☐ No. Describe:

- v. Ensure the affected individual receives written notice of the decision. In the case of a negative determination, the decision must indicate (1) the Lead Agency's efforts to verify the accuracy of information challenged by the individual, (2) any additional appeals rights available to the individual, and (3) information on how the individual can correct the federal or State records at issue in the case.

☒ Yes.

☐ No. Describe:

- vi. Facilitate coordination between the Lead Agency and other agencies in charge of background check information and results (such as the Child Welfare office and the State Identification Bureau), to ensure the appeals process is conducted in

accordance with the Act.

☒ Yes.

☐ No. Describe:

5.7.12 Provisional hiring of prospective staff members

Lead Agencies must at least complete and receive a qualifying result for either the FBI criminal background check or a fingerprint-based in-state criminal background check where the individual resides before prospective staff members may provide services or be in the vicinity of children.

Until all the background check components have been completed, the prospective staff member must be supervised at all times by someone who has already received a qualifying result on a background check within the past five years.

Check all background checks for which the Lead Agency requires a qualifying result before a prospective child care staff member begins work with children.

a. FBI criminal background check.

☒ Yes.

☐ No. If no, describe:

b. In-state criminal background check with fingerprints.

☒ Yes.

☐ No. If no, describe:

c. In-state Sex Offender Registry.

☒ Yes.

☐ No. If no, describe:

d. In-state child abuse and neglect registry.

☒ Yes.

☐ No. If no, describe:

e. Name-based national Sex Offender Registry (NCIC NSOR).

☒ Yes.

☐ No. If no, describe:

f. Interstate criminal background check, as applicable.

☐ Yes.

☒ No. If no, describe: **While interstate checks are being conducted individuals are sent a provisional letter of eligibility requiring them to be supervised for up to 90-days while the out of state checks are being processed and received. This indicates individuals are working (under the supervision of a staff member who has already received a qualifying result on a background check within the past five years) prior to receiving the results of the Interstate criminal background check.**

g. Interstate Sex Offender Registry check, as applicable.

☐ Yes.

☒ No. If no, describe: **While interstate checks are being conducted individuals are sent a provisional letter of eligibility requiring them to be supervised for up to 90-days while the out of state checks are being processed and received. This indicates individuals are working (under the supervision of a staff member who has already received a qualifying result on a background check within the past five years) prior to receiving the results of the Interstate criminal background check.**

- h. Interstate child abuse and neglect registry check, as applicable.

☐ Yes.

☒ No. If no, describe: **While interstate checks are being conducted individuals are sent a provisional letter of eligibility requiring them to be supervised for up to 90-days while the out of state checks are being processed and received. This indicates individuals are working (under the supervision of a staff member who has already received a qualifying result on a background check within the past five years) prior to receiving the results of the Interstate criminal background check.**

- i. Does the Lead Agency require provisional hires to be supervised by a staff member who received a qualifying result on the comprehensive background check while awaiting results from the provisional hire's full comprehensive background check?

☒ Yes.

☐ No. If no, describe:

5.7.13 Completing the criminal background check within a 45-day timeframe

The Lead Agency must carry out a request from a child care provider for a criminal background check as expeditiously as possible, and no more than 45 days after the date on which the provider submitted the request

- a. Does the Lead Agency ensure background checks are completed within 45 days (after the date on which the provider submits the request)?

☒ Yes.

☐ No. If no, describe the timeline for completion for categories of providers, including which background check components take more than 45 days.

- b. Does the Lead Agency ensure child care staff receive a comprehensive background check when they work in your State but reside in a different State?

☒ Yes.

☐ No. If no, describe the current policy:

5.7.14 Responses to interstate background check requests

Lead Agencies must respond as expeditiously as possible to requests for interstate background checks from other States/Territories/Tribes in order to meet the 45-day timeframe.

- a. Does your State participate in the National Crime Prevention and Privacy Compact or National Fingerprint File programs?

☒ Yes.

☐ No.

- b. Describe how the State/Territory responds to interstate criminal history, Sex Offender Registry, and Child Abuse and Neglect Registry background check requests from another state. **SBI (Maine criminal background) checks requests are online <https://www5.informe.org/online/pcr/> . On average 1-5 business days. The Child Abuse and Neglect Registry Checks request are done through our online portal <https://apps1.web.maine.gov/cgi-bin/online/carbc/user/start>. Lead Agency has requirements, policies, and, procedures in place to respond as expeditiously as possible to other states', territories', and tribes' requests for background check results from interstate criminal history, Sex Offender Registry, and Child Abuse and Neglect Registry, to accommodate the 45 day timeframe, including any agencies/entities responsible for responding to requests from other states. It is in statute in Maine to respond to a request within 48 hours and the results will be mailed through the US Postal Service.**
- c. Does your State/Territory have a law or policy that prevents a response to CCDF interstate background check requests from other States/Territories/Tribes?
- ☐ Yes. If yes, describe the current policy.
- ☒ No.

5.7.15 Consumer education website links to interstate background check processes

Lead Agencies must include on their consumer education website and the website of local Lead Agencies if the CCDF program is county-run, the policies and procedures related to comprehensive background checks. This includes the process by which a child care provider or other State or Territory may submit a background check request.

- a. Provide the direct URL/website link that contains instructions on how child care providers and other States and Territories should initiate background check requests for prospective and current child care staff members: **<https://www.maine.gov/dhhs/ocfs/provider-resources/child-care-licensing/becoming-a-childcare-provider/child-care-provider-background-checks>**

<https://www.childcarechoices.me/wp-content/uploads/2024/01/1.19.2024-Consumer-Education-Statement.pdf>

The Child Abuse and Neglect Registry Checks request are done through our online portal <https://apps1.web.maine.gov/cgi-bin/online/carbc/user/start>. Lead Agency has requirements, policies, and, procedures in place to respond as expeditiously as possible to other states', territories', and tribes' requests for background check results to accommodate the 45 day timeframe, including any agencies/entities responsible for responding to requests from other states is in statute to respond to a request within 48 hours and the results will be mailed through the US Postal Service.

Check to certify that the required elements are included on the Lead Agency's consumer and provider education website for each interstate background check component.

- b. Interstate criminal background check:

- i. ☒ Agency name
 - ii. ☒ Address
 - iii. ☒ Phone number
 - iv. ☒ Email
 - v. ☒ Website
 - vi. ☒ Instructions
 - vii. ☒ Forms
 - viii. ☒ Fees
 - ix. ☒ Is the State a National Fingerprint File (NFF) State?
 - x. ☒ Is the State a National Crime Prevention and Privacy Compact State?
 - xi. If not all boxes above are checked, describe: **N/A**
- c. Interstate sex offender registry (SOR) check:
- i. ☒ Agency name
 - ii. ☒ Address
 - iii. ☒ Phone number
 - iv. ☒ Email
 - v. ☒ Website
 - vi. ☒ Instructions
 - vii. ☒ Forms
 - viii. ☒ Fees
 - ix. If not all boxes above are checked, describe: **N/A**
- d. Interstate child abuse and neglect (CAN) registry check:
- i. ☒ Agency name
 - ii. ☒ Is the CAN check conducted through a county administered registry or centralized registry?
 - iii. ☒ Address
 - iv. ☒ Phone number
 - v. ☒ Email
 - vi. ☒ Website
 - vii. ☒ Instructions
 - viii. ☒ Forms
 - ix. ☒ Fees
 - x. If not all boxes above are checked, describe: **N/A**

5.7.16 Background check fees

The Lead Agency must ensure that fees charged for completing the background checks do not exceed the actual cost of processing and administration.

Does the Lead Agency certify that background check fees do not exceed the actual cost of processing and administering the background checks?

☒ Yes.

☐ No. If no, describe what is currently in place and what elements still need to be implemented:

5.7.17 Renewal of the comprehensive background check

Does the Lead Agency conduct the background check at least every 5 years for all components?

☒ Yes.

☐ No. If no, what is the frequency for renewing each component?

5.8 Exemptions for Relative Providers

Lead Agencies may exempt relatives (defined in CCDF regulations as grandparents, great-grandparents, siblings if living in a separate residence, aunts, and uncles) from certain health and safety requirements. This exception applies only if the individual cares only for relative children.

5.8.1 Exemptions for relative providers

Does the Lead Agency exempt any federally defined relative providers from licensing requirements, the CCDF health and safety standards, preservice/orientation training, ongoing training, inspections, or background checks?

☐ No.

☒ Yes. If yes, which type of relatives do you exempt, and from what requirements (licensing requirements, CCDF health and safety standards, preservice/orientation training, ongoing training, inspections, and/or background checks) do you exempt them?

A License-Exempt Relative Child Care Provider, who is a Child(ren)'s grandparent, great grandparent, sibling (if living in a separate residence), aunt, or uncle, who provides Child Care Services for no more than two (2) children, is eighteen (18) years of age or older, and is a Maine Resident.

A Relative In-home Child Care Provider, who is a Child(ren)'s grandparent, great grandparent, sibling (if living in a separate residence), aunt, or uncle who provides Child Care Services in the Child's home and must be: eighteen (18) years of age or older, a Maine resident, and providing Child Care Services to a family unit receiving CCAP. In addition, the provider cannot care for more than two (2) of their own children while providing services.

Relative providers are exempt from the following requirements:
all licensing requirements
all health and safety standard requirements

all health and safety training requirements
all monitoring and enforcement requirements
all background check requirements

6 Support for a Skilled, Qualified, and Compensated Child Care Workforce

A skilled child care workforce with adequate wages and benefits underpins a stable high-quality child care system that is accessible and reliable for working parents and that meets their needs and promotes equal access. Positive interactions between children and caregivers provide the cornerstone of quality child care experiences. Responsive caregiving and rich interactions support healthy socio-emotional, cognitive, and physical development in children. Strategies that successfully support the child care workforce address key challenges, including low wages, poor benefits, and difficult job conditions. Lead Agencies can help mitigate some of these challenges through various CCDF policies, including through ongoing professional development and supports for all provider types and embedded in the payment policies and practices covered in Section 4. Lead Agencies must have a framework for training, professional development, and post-secondary education. They must also incorporate health and safety training into their professional development. Lead Agencies should also implement policies that focus on improving wages and access to benefits for the child care workforce. When implemented as a cohesive approach, the initiatives support the recruitment and retention of a qualified and effective child care workforce, and improve opportunities for caregivers, teachers, and directors to advance on their progression of training, professional development, and postsecondary education.

This section addresses Lead Agency efforts to support the child care workforce, the components and implementation of the professional development framework, and early learning and developmental guidelines.

6.1 Supporting the Child Care Workforce

Lead Agencies have broad flexibility to implement policies and practices to support the child care workforce.

6.1.1 Strategies to improve recruitment, retention, compensation, and well-being

- a. Identify any Lead Agency activities related to strengthening workforce recruitment and retention of child care providers. Check all that apply:
 - i. ☒ Providing program-level grants to support investments in staff compensation.
 - ii. ☒ Providing bonuses or stipends paid directly to staff, like sign-on or retention bonuses.
 - iii. ☐ Connecting family child care providers and center-based child care staff to health insurance or supporting premiums in the Marketplace.
 - iv. ☐ Subsidizing family child care provider and center-based child care staff retirement benefits.
 - v. ☐ Providing paid sick, personal, and parental leave for family child care providers and center-based child care staff.
 - vi. ☐ Providing student loan debt relief or loan repayment for family child care providers and center-based child care staff.

- vii. ☒ Providing scholarships or tuition support for center-based child care staff and family child care providers.
 - viii. ☐ Other. Describe:
- b. Describe any Lead Agency ongoing efforts and future plans to assess and improve the compensation of the child care workforce in the State or Territory, including increasing wages, bonuses, and stipends. **The Maine Early Childhood Workforce Salary Supplement System assists eligible child care providers in paying competitive salaries to address challenges in the child care workforce and ensure access to affordable, high-quality child care options for Maine families. The system provides salary supplements to providers for eligible workers based on the workers' level of education and experience. This system is available to child care providers and early childhood educators who provide direct child care services to children in licensed child care facilities or are licensed family child care providers. As an activity of the PDG B-5 grant, the Lead Agency is contracting out to conduct an evaluation of the Maine Early Childhood Workforce Salary Supplement System. The evaluation will collect data from the program and individual staff levels.**
 - c. Describe any Lead Agency ongoing efforts and future plans to expand access to benefits, including health insurance, paid sick, personal, and parental leave, and retirement benefits. **The Lead Agency partnered with CELFE to analyze Maine's investment in the early care and education system statewide including public PreK. The outcomes of this analysis will guide the Lead Agency in access and/or expansion in the future of access to benefits, including health insurance, paid sick, personal, and parental leave, and retirement benefits. The two cost modeling tools created indicate the true cost of care when benefits are included.**
 - d. Describe any Lead Agency ongoing efforts and future plans to support the mental health and well-being of the child care workforce. **The Lead Agency houses Maine's Salary Supplement program, Early Childhood Consultation Partnership (ECCP), and contracted training and technical assistance through MRTQ PDN. ECCP offers free early childhood mental health consultation to child care programs statewide in Maine. These programs and services contribute to a supportive work environment and the wellbeing of the child care workforce.**
 - e. Describe any other strategies the Lead Agency is developing and/or implementing to support providers' recruitment and retention of the child care workforce. **In addition to the Maine Early Childhood Workforce Salary Supplement System, the Lead Agency offers a variety of professional development opportunities and pathways to developing and/or implementing to support providers' recruitment and retention of the child care workforce. The newest strategy launched by pilot in the fall of 2023 and now statewide is in partnership with Jobs for Maine Graduates (JMG) and MRTQ PDN for high school students in JMG and at least 16 years old to earn the JMG Child Care Career Badge. This is a forty-hour job shadowing and training experience. Once completed, a student receives the badge along with a \$500 stipend funded through MRJP. The student is also then ready for employment by meeting Maine Licensing requirements and CCDF complaint.**

6.1.2 Strategies to support provider business practices

- a. Describe other strategies that the Lead Agency is developing and/or implementing to strengthen child care providers' business management and administrative practices.

Through the Lead Agency's partnership with MRTQ PDN, a review of all trainings geared towards business practices are being reviewed for needed updates and to identify gap areas for additional trainings needed with the Strengthening Business Practices for Child Care Programs. The Lead Agency has a newly developed position called Child Care Program Navigator whose main role will be to aid programs in navigating the system with a focus on programs and resources that ultimately can assist in stronger business practices and increased revenue.

- b. Check the topics addressed in the Lead Agency's strategies for strengthening child care providers' administrative business practices. Check all that apply:
 - i. ☒ Fiscal management.
 - ii. ☒ Budgeting.
 - iii. ☒ Recordkeeping.
 - iv. ☒ Hiring, developing, and retaining qualified staff.
 - v. ☒ Risk management.
 - vi. ☒ Community relationships.
 - vii. ☒ Marketing and public relations.
 - viii. ☒ Parent-provider communications.
 - ix. ☒ Use of technology in business administration.
 - x. ☒ Compliance with employment and labor laws.
 - xi. ☐ Other. Describe any other efforts to strengthen providers' administrative business:

6.1.3 Strategies to support provider participation

Lead Agencies must facilitate participation of child care providers and staff with limited English proficiency and disabilities in the child care subsidy system. Describe how the Lead Agency will facilitate this participation, including engagement with providers to identify barriers and specific strategies used to support their participation:

- a. Providers and staff with limited English proficiency: **MRTQ PDN works with practitioners and offers specialized resources and supports. French and English are the primary languages in Maine. However, some pockets of Maine have over 16 different languages. MRTQ PDN works individually with programs and practitioners needing assistance. Interpreters are available on an as needed basis. The MRTQ PDN website can be read in multiple languages. Interpreters are utilized as needed during trainings in person or on demand. The Lead Agency supports recruitment and participation of providers with limited English proficiency to participate as a CCSP provider through targeted outreach and communications. This includes translated materials and outreach to stakeholder groups within identified communities.**
- b. Providers and staff who have disabilities: **The Center for Community Inclusion and Disability Studies (CCIDS) is a partner of the MRTQ PDN. Their work is specific to Inclusion. The DCs workplans include outreach to all providers and will assist with obtaining resources or additional support as needed for any provider who has a disability in order to**

help assist with meeting their needs. The Lead Agency supports recruitment and participation of providers with disabilities to participate as a CCSP provider through statewide outreach to all licensed providers or individuals identified as possible CCSP license-exempt providers.

6.2 Professional Development Framework

A Lead Agency must have a professional development framework for training, professional development, and post-secondary education for caregivers, teachers, and directors in child care programs that serve children of all ages. The framework must include these components:

(1) professional standards and competencies, (2) career pathways, (3) advisory structures, (4) articulation, (5) workforce information, and (6) financing. CCDF provides Lead Agencies flexibility on the strategies, breadth, and depth of the framework. The professional development framework must be developed in consultation with the State Advisory Council on Early Childhood Education and Care or a similar coordinating body.

6.2.1 Updates and consultation

- a. Did the Lead Agency make any updates to the professional development framework since the FFY 2022-2024 CCDF Plan was submitted?

☒ Yes. If yes, describe the elements of the framework that were updated and describe if and how the State Advisory Council on Early Childhood Education and Care (if applicable) or similar coordinating body was consulted: **In March of 2023, the Lead Agency released the revised QRIS system for Maine, Rising Stars for ME. The revisions include updated standards and introduced the Quality Rating and Improvement System Rules C.M.R 10-148 Chapter 31. In the Spring of 2024, the Maine Early Learning Development Standards (MELDS) were revised. Coordination with the CCECAC occurred early on in the process with updates and feedback occurring at the monthly meetings.**

☐ No.

- b. Did the Lead Agency consult with other key groups in the development of their professional development framework?

☒ Yes. If yes, identify the other key groups: **The Lead Agency established the QRIS Revision Team with members from a variety of Lead Agency and DOE staff whose areas of expertise was necessary for standard updates and alignment across agencies.**

☐ No.

6.2.2 Description of the professional development framework

- a. Describe how the Lead Agency's framework for training and professional development addresses the following required elements:

- i. Professional standards and competencies. For example, Lead Agencies can include information about which roles in early childhood education are included (such as teachers, directors, infant and toddler specialists, mental health consultants, coaches, licensors, QIS assessors, family service workers, home visitors). **The Lead Agency's partnership with MRTQ PDN offers professional development (PD) to ECE practitioners in Maine. Professional development as defined by NAEYC and**

NACCRRRA is "a continuum of learning and support activities designed to prepare individuals for work with and on behalf of young children and their families, as well as ongoing experiences to enhance this work. Professional Development encompasses education, training, and technical assistance." (NAEYC & NACCRRRA, Early Childhood, 2011). The framework that MRTQ PDN uses is that of Education, Training and Technical assistance where the focus is on using all three for more positive outcomes. Beginning in 1992, the DHHS, through its Early Childhood Division, convened 100 stakeholders in a two-year process that resulted in the development of recommendations for a career development system for early childhood professionals. The recommendations were published in 1994 in a document titled Pathways to Quality: Toward the Development of a Comprehensive Training Plan for Child Care Practitioners in Maine. From this point, the Department put several initiatives in place, core knowledge curriculum, and a professional Registry and Career Lattice. In 1999, the Department's Office of Child Care and Head Start contracted with the Muskie School of Public Service, University of Southern Maine to establish Maine Roads to Quality, a new program tasked to manage the comprehensive, coordinated career development system. In 2012, the State of Maine's professional development framework was transitioned into an intentionally designed network in order to improve efficiency and effectiveness in providing professional development that was coordinated with the goal to offer a seamless, integrated and transdisciplinary approach to supporting the field. Today the professional development framework is known as the MRTQ Professional Development Network (MRTQ PDN) and is a partnership between the University of Southern Maine Muskie School of Public Service, Cutler Institute, The University of Maine Center for Community Inclusion and Disabilities Studies and the Maine After School Network. Within this professional development framework, each curriculum that has been created for the MRTQ Core Knowledge training is based on standards and competencies.

Maine has developed several competency and standards guides including:

Core Knowledge: [https://www.impact-](https://www.impact-publications.com/assets/me_coreknowledgecompetencies.pdf)

[publications.com/assets/me_coreknowledgecompetencies.pdf](https://www.impact-publications.com/assets/me_coreknowledgecompetencies.pdf)

Family Child Care: [https://www.mrtq.org/wp-content/uploads/2022/12/FCC-](https://www.mrtq.org/wp-content/uploads/2022/12/FCC-Competencies.pdf)
[Competencies.pdf](https://www.mrtq.org/wp-content/uploads/2022/12/FCC-Competencies.pdf)

Child Care Directors: [https://www.mrtq.org/wp-content/uploads/2019/09/FINAL-](https://www.mrtq.org/wp-content/uploads/2019/09/FINAL-Maine-Director-Credential-Information-Packet.pdf)
[Maine-Director-Credential-Information-Packet.pdf](https://www.mrtq.org/wp-content/uploads/2019/09/FINAL-Maine-Director-Credential-Information-Packet.pdf)

Early Childhood Technical Assistance: [https://ccids.umaine.edu/wp-](https://ccids.umaine.edu/wp-content/uploads/sites/26/2019/06/ME_TA_comp-20190627-accessible.pdf)
[content/uploads/sites/26/2019/06/ME_TA_comp-20190627-accessible.pdf](https://ccids.umaine.edu/wp-content/uploads/sites/26/2019/06/ME_TA_comp-20190627-accessible.pdf)

Each training and credential developed in Maine has been created, piloted, and revised with a group of stakeholders. New trainings and revisions have been made by authors who hold ECE Master's degrees. The MRTQ PDN uses current standards and practices that are vetted by NAEYC, NAFCC, COA, Head Start and uses the national centers resources.

The MRTQ PDN ECE Workforce Registry will continue to support providers' ongoing professional development by tracking education, training, and experience and recognizing advancement along the Career Lattices. Any early childhood

education provider, public school educator, administration/management professional, or family education and support professional may participate in the Maine Roads to Quality Registry. Directors, teachers, and direct care staff in licensed child care programs in Maine are required to join the registry.

- ii. Career pathways. For example, Lead Agencies can include information about professional development registries, career ladders, and levels. **The MRTQ Registry and Career Lattices provide a clear pathway for providers to advance and document their professional development. Any professional wishing to participate in the MRTQ Registry will submit an application form; both paper and online forms will be available. There is no fee to join the MRTQ Registry. All applications will be logged as they are received and will be reviewed and entered on a timely basis. All providers submitting a complete application will be assigned placement on the Career Lattice that reflects their education, training, and experience in the field. Applicants will choose one Career Lattice from the following options: Licensed Exempt providers, Direct Care (for center-based staff, family child care, and school age providers), Administrative/Management/Coordination, Family Support Professional, or Public School. MRTQ PDN revises and updates the Career Lattices regularly to ensure that they are responsive to the needs of professionals in the field. The MRTQ registry tracks education, training, and experiences and recognizes advancement along the Career Lattice. Any early childhood education provider including licensed exempt providers, public school educators, administration/management professionals or family education and support provider participating in the MRTQ Registry.**
- iii. Advisory structure. For example, Lead Agencies can include information about how the professional development advisory structure interacts with the State Advisory Council on Early Childhood Education and Care. **MRTQ PDN has an advisory council which meets quarterly and reviews the work of the PDN. The membership is comprised of representatives from FCC, Center based, School Age programs, Inclusion and Disabilities, Head Start, Cooperative Extension, private schools, department of education, and the OCFS**
- iv. Articulation. For example, Lead Agencies can include information about articulation agreements, and collaborative agreements that support progress in degree acquisition. **The MRTQ core knowledge 180-hour program is articulated into all eight (8) Maine community college ECE programs. Current work is being done to articulate the MRTQ PDN developed State of Maine credentials. MRTQ PDN staff the Higher Education Committee which is comprised of all the ECE department chairs around the state.**
- v. Workforce information. For example, Lead Agencies can include information about workforce demographics, educator well-being, retention/turnover surveys, actual wage scales, and/or access to benefits. **MRTQ PDN Registry houses data on all staff working in licensed child care programs in Maine as well as many other ECE professionals who serve in coaching, consultation, and public PreK roles.**

Over 7,000 teachers, staff, and administrators lead and work in Maine licensed child care programs.

On a recent survey the ECE workforce in Maine reported speaking and communicating in the following languages at home or at work:

English 99%

American Sign Language (ASL) 5%

French 3%

Spanish 2%

Portuguese 1%

Lingala 1%

Passamaquoddy 1%

Arabic, Maltese, Italian, Hebrew, Korean, and German less than 1%

Maine's PDG Needs Assessment highlighted the following core ECE Workforce challenges:

- Shortage of qualified staff is a persistent and pervasive problem for providers. Staff turnover has increased post pandemic.
- Even though there have been major advancements in providing salary stipends, low wages remain a primary barrier to attracting and retaining staff.
- While the size of the workforce has rebounded post pandemic, turnover rates are higher, especially for the lowest wage workers.
- The pathways for entry into the profession are not easy to navigate (i.e., understanding the options and financial supports); the certification process can be lengthy.
- Maine does not have a predictable system of articulation among postsecondary institutions.
- Post pandemic, young children are presenting greater emotional and behavioral needs which many staff are not prepared to address; the resulting stress is contributing to high turnover.

Workforce shortages were reflected in all encounters during the needs assessment process, and almost always, as a major concern. Almost 85% of respondents to the early care and education survey indicated that recruiting qualified staff was one of their greatest challenges (4 or 5 on a 5-point scale). Similarly, 75% rated staff turnover as a great challenge on the same scale. 48% of programs cited workforce shortages as a reason for their child care program being under enrolled.

- vi. Financing. For example, Lead Agencies can include information about strategies including scholarships, apprenticeships, wage enhancements, etc. MRTQ PDN trainings range from free to \$1 per training hour. Technical assistance support for a practitioner to obtain a credential is free. Registry is free to all. The MRTQ PDN training courses are articulated into the Community College system and the cost for a practitioner to articulate into college coursework is free. The apprenticeship program funds the cost of earning a credential. The Maine TEACH Early Childhood Scholarship Program funds the costs associated with earning associate and bachelor's degrees including tuition, books, travel, and supplies. Maine also has a salary supplement system where teachers and direct care staff working in licensed child care receive a monthly stipend that is tiered based on years of experience and credentials and degrees earned.

b. Does the Lead Agency use additional elements?

☒ Yes.

If yes, describe the element(s). Check all that apply.

- i. ☒ Continuing education unit trainings and credit-bearing professional development. Describe: **The MRTQ PDN articulation agreements allow a practitioner to take MRTQ PDN training and turn that into credit-bearing professional development at the community colleges in Maine. MRTQ/PDN staff can assist ECE practitioners in the process for doing this and MRTQ/PDN staff the ECE Higher Education committee where discussions lead to better communications and systems for articulation.**
- ii. ☒ Engagement of training and professional development providers, including higher education, in aligning training and educational opportunities with the Lead Agency's framework. Describe: **The MRTQ PDN Advisory Committee includes members from a variety of training and professional development providers, including higher education. The committee meets at a minimum quarterly. Through PDG B-5 Grant, the Lead Agency and its partners are working to increase opportunities for expanded alignment and articulation agreements.**
- iii. ☐ Other. Describe:
- ☐ No.

6.2.3 Impact of the Professional Development Framework

Describe how the framework improves the quality, stability, and retention of caregivers, teachers, and directors and identify what data are available to assess the impact.

- a. Professional standards and competencies. For example, do the professional standards and competencies reflect the range of providers across role, child care setting, or age of children served? **Maine's professional standards and competencies aim to reflect the diversity of providers across roles, child care setting, and age of child. Competencies, trainings, and credentials have been developed for those teaching and caring for infants and toddlers, for child care directors, owners, and administrators, for early childhood technical assistance professionals, and coming soon, for those teaching and caring for preschoolers.**

Maine Roads to Quality Professional Development Network (MRTQ PDN) conducts a biennial Professional Development Needs Assessment to better understand the training, technical assistance, and overall needs of early childhood education (ECE) and out-of-school time (OST) professionals in the state of Maine and to inform any potential revisions to standards or trainings. Staff analyze the PD needs assessment to improve training and technical assistance offerings that in turn improves the quality and stability of the workforce. Approximately 85% of the ECE workforce in Maine is accessing and completing trainings through Maine Roads to Quality Professional Development Network (MRTQ PDN). There were over 20,000 registrations for on demand trainings at MRTQ PDN in the past year and 97% of registrants completed the training. The prior year had over 13,000 registrations with the same high rate of completion. There were 113 facilitated trainings offered this past year through MRTQ PDN, including 24 deliveries of the Infant and

Toddler Maine Early Learning and Development Standards (MELDS) course. Each training enrolled between 10-20 participants.

The Professional Development Needs Assessment survey consisted of seven sections which were compiled and reviewed by MRTQ PDN staff, MRTQ PDN Advisory Council, and state partners. The demographic information of the ECE workforce was gathered to better understand the lived experience of the workforce, including languages spoken, race, and ethnicity. Maine's ECE Workforce is becoming more diverse and there is an increase in translations available and support from interpreters to access information and complete trainings. Continuing to improve access will be core to the work at OCFS moving forward.

- b. Career pathways. For example, has the Lead Agency developed a wage ladder that provides progressively higher wages as early educators gain more experience and credentials? What types of child care settings and staff roles are addressed in career pathways, such as licensed centers and family child care homes? **In partnership with stakeholders, OCFS developed a wage scale to use in cost modeling the cost of child care. The lead agency also developed a tiered salary supplement system for the ECE workforce to receive monthly stipends. The MRTQ Registry with the corresponding Career Lattice and Salary Supplement tiers provide a pathway for early educators to advance, document their professional development, and increase their compensation. Since the salary supplements started, there has been an increase in the number of early educators receiving the supplement, pointing to better rates of recruitment and retention, and more early educators moving up to higher levels on the Career Lattice and earning a higher monthly stipend, pointing to credentials and degrees earned and documented. Salary supplements went to 7,045 staff in October 2023, when the supplements moved to being tiered by education and experience, to 7,571 staff in June 2024. In October 2023, 772 staff were at Levels 5 or 6 on the Career Lattice and earned a higher salary supplement and 772 staff were at Levels 7 or 8 on the lattice, earning the highest salary supplement. In June 2024, 1,331 staff were at Levels 5 or 6 and 1,075 were at Levels 7 or 8.**

All ECE professional working in licensed child care are required to join the registry and upon submitting a complete application are assigned placement on the Career Lattice that reflects their education, training, and experience in the field. Applicants choose a Career Lattice from the following options: Licensed Exempt providers, Direct Care (for center-based staff, family child care, and school age providers), Administrative / Management / Coordination, Family Support Professional, or Public School. MRTQ PDN revises and updates the Career Lattices regularly to ensure that they are responsive to the needs of professionals in the field. The ECE Workforce registry tracks education, training, and experiences and recognizes advancement along the Career Lattice. Any early childhood education professional including licensed exempt providers, public school educators, administration/management professionals or family education and support providers may participate in the Registry.

- c. Advisory structure. For example, has the advisory structure identified goals for child care workforce compensation, including types of staff and target compensation levels? Does the Lead Agency have a Preschool Development Birth-to-Five grant and is part of its scope of work child care compensation activities? Are they represented in the advisory structure? **The PDN's advisory structure includes both formal and informal opportunities for stakeholders to advise MRTQ PDN on services and delivery through multiple measures**

that include but are not limited to training evaluations, technical assistance evaluations, formal meetings, and anecdotal records. The PDN convene an advisory meeting quarterly to provide expertise and guidance for successful implementation of professional development in early care and education. The council reviews the work of the PDN, sets goals, and supports work toward a more supported, trained, and compensated workforce. Over the past two years the council has advised on the work of the PDN and the state in the Preschool Development Grant. The implementation of grant activities includes more scholarships to earn associate and bachelor's degrees, several ways for professional to increase their knowledge and skills in inclusive practices, and quality improvement awards for licensed child care programs with TA to set goals and develop plans for quality improvement. The membership is comprised of representatives from family child care, center-based child care, school age programs, professional supporting inclusion, Head Start, higher education, Maine DOE, and the Office of Child and Family Services. The advisory activities are also a way to coordinate efforts among partners and to explore ideas for leveraging resources and partners to increase the overall effectiveness of MRTQ PDN statewide.

- d. Articulation. For example, how does the advisory structure include training and professional development for providers, including higher education, to assist in aligning training and education opportunities? **The MRTQ core knowledge 180-hour program is articulated into all eight (8) Maine community college ECE programs. Current work is being done to articulate the State of Maine credentials for early childhood educators. MRTQ PDN convenes and staffs the Higher Education Committee which is comprised of all the ECE department chairs around the state.**

The PDN has added a new staff position, a Statewide Higher Education Coordinator focused on expanding the Department's efforts to align programming and agreements across the professional development system. Maine's goal over the next few years is to strengthen and build articulation agreements across the ECE professional development and educational system, creating pathways for students from Maine's Career and Technical Education (CTE)'s and MRTQ PDN trainings into Maine's Institutes of Higher Education. In 2024, within the scope of the preschool development grant.

In fiscal year 2024, 60 credentials were awarded to graduating high school students who completed Early Childhood Education CTE programs and met testing and portfolio requirements, representing about a 36% increase in applicants from FY 2023. Attainment of the CECA credential allows these students to begin at a Level 3 on the Direct Care Career Lattice as they enter employment in the field of early care and education.

- e. Workforce information. For example, does the Lead Agency have data on the existing wages and benefits available to the child care workforce? Do any partners such as the Quality Improvement System, child care resource and referral agencies, Bureau of Labor Statistics, and universities and research organizations collect compensation and benefits data? Does the Lead Agency monitor child care workforce wages and access to benefits through ongoing data collection and evaluation? Can the data identify any disparities in the existing compensation and benefits (by geography, role, child care setting, race, ethnicity, gender, or age of children served)? **MRTQ PDN Registry houses data on all the MRTQ registry members. The Registry provides Maine extensive data on the ECE workforce, including role, setting, professional learning activities, credentials and degrees**

earned, and years of experience. The Registry does not provide the state with comprehensive salary or benefit data at this time. This past year OCFS collected and analyzed additional data on the ECE workforce, including salary and benefit data, as part of the development of a child care cost estimation tool. From June 2023 to June 2024, OCFS worked with the Center for Early Learning Funding Equity (CELFE) to develop a cost estimation study to calculate the cost of early learning experiences in the state using a cost modeling tool.

The cost model design required inputs related to the ECE workforce, including personnel costs. Personnel cost inputs included the number of staff (tied to the number of classrooms and overall program size) and staff wage rates and benefits. The model calculates estimated program costs as the system currently exists, with costs based on current wages paid by early care and education programs. The model also calculates program costs with higher “target” wages using a salary scale developed by CELFE and OCFS. The wage and benefit information collected, and work with stakeholders, helped to develop target salary scales.

Considering the challenge of low early care and education workforce compensation (and resulting staffing shortages) in Maine and nationally, OCFS was interested in understanding what the cost of care would be if salaries and benefits were high enough to attract and retain an adequate workforce. CELFE developed a set of “desired” salary inputs for each geographic region that were anchored by estimates of a “living wage” (for entry-level assistants) and parity with public school teachers (for BA-level teachers) to illustrate the resources that would be needed to raise compensation across the field. While the desired salaries are not intended to be directly included in policy at this time, they help inform the “north-star” vision for the early childhood workforce while also recognizing the current compensation (and related recruitment and retention) challenges that the early childhood field faces.

- f. Financing. For example, has the Lead Agency set a minimum or living wage as a floor for all child care staff? Do Lead Agency-provider subsidy agreements contain requirements for staff compensation levels? Do Lead Agencies provide program-level compensation grants to support staff base salaries and benefits? Does the Lead Agency administer bonuses or stipends directly to workers? **Maine finances a professional development system to lower barriers and increase access to professional learning. MRTQ PDN trainings range from free to \$1 per training hour. Technical assistance support for a practitioner to obtain a credential is free. Registry is free to all. The MRTQ PDN trainings are articulated into the Community College system and the cost for a practitioner to articulate into college coursework is free. The Lead Agency funds scholarships and apprenticeships to support access to credentials and degrees, and with the Preschool Development Grant is also funding quality improvement awards that can support costs for trainings and higher education.**

In addition, the professional development system and corresponding career lattice and workforce registry are connected to the state's salary supplement system. ECE staff can earn a \$275 monthly stipend if they are at Levels 1-4 on the career lattice in the Registry, \$415 a month at Levels 5-6, and \$625 a month at Levels 7-8. Salary supplements went to 7,045 staff in October 2023, when the supplements moved to being tiered by education

and experience, to 7,571 staff in June 2024. In October 2023, 772 staff were at Levels 5 or 6 on the Career Lattice and earned a higher salary supplement and 772 staff were at Levels 7 or 8 on the lattice, earning the highest salary supplement. In June 2024, 1,331 staff were at Levels 5 or 6 and 1,075 were at Levels 7 or 8.

6.3 Ongoing Training and Professional Development

6.3.1 Required hours of ongoing training

Provide the number of hours of ongoing training required annually for CCDF-eligible providers in the following settings:

- a. Licensed child care centers: **Under 13 children, 12 hours annually. Over 13 children, 18 hours annually for weekly employment less than 20 hours and 30 hours annually for those weekly employed for over 20 hours (these hours include the 2-hour annual health and safety training).**
- b. License-exempt child care centers: **2-hour annual health and safety training and when necessary, recertification of Pediatric first aid and pediatric cardiopulmonary resuscitation (CPR) and Mandated Reporter trainings.**
- c. Licensed family child care homes: **12 hours annually (this includes the 2-hour annual health and safety training).**
- d. License-exempt family child care homes: **2-hour annual health and safety training and when necessary, recertification of Pediatric first aid and pediatric cardiopulmonary resuscitation (CPR) and Mandated Reporter trainings.**
- e. Regulated or registered in-home child care: **N/A**
- f. Non-regulated or registered in-home child care: **2-hour annual health and safety training and when necessary, recertification of Pediatric first aid and pediatric cardiopulmonary resuscitation (CPR) and Mandated Reporter trainings.**

6.3.2 Accessibility of professional development for Tribal organizations

Describe how the Lead Agency's training and professional development are accessible to providers supported through Indian tribes or Tribal organizations receiving CCDF funds (as applicable). **All child care programs and professionals, including Tribal organizations, can access the entire PDN training and development framework that includes the initial and annual on demand health and safety trainings. MRTQ PDN has seven (7) District Coordinators (DCs), who are consultants assigned to specific regions and one statewide inclusion consultant. Consultants provide resources and virtual or onsite technical assistance. Outreach occurs by the consultants to child care programs in their region.**

6.3.3 Professional development appropriate for the children, families, and child care providers

Describe how the Lead Agency's training and professional development requirements reflect the range of children, families, and child care providers participating in CCDF. To the extent practicable, how does professional development include specialized training or credentials for providers who care for infants or school-age children; individuals with limited English proficiency; children who are bilingual; children with developmental delays or disabilities; and/or Native

Americans, including Indians, as the term is defined in Section 900.6 in subpart B of the Indian Self-Determination and Education Assistance Act (including Alaska Natives) and Native Hawaiians? **The system of professional development supports from MRTQ PDN include training, technical assistance and education. The MRTQ PDN trainings can be taken to obtain early care and education setting and age specific credentials. Currently those credentials are: Child Development Associates, Infant Toddler Credential, Youth Development Credential, Director Credential, and Inclusion Credential. The training to meet the credential requirements are offered through the yearly offerings at MRTQ PDN. Technical assistance in the form of phone and email, on site consultation, peer to peer groups are offered to support practitioners.**

On demand trainings include Supporting Inclusion: Laws and Practices and Caring for Children and Families Experiencing Homelessness. Facilitated trainings include Inclusive Child Care, Foundations of Universal Design, and Creating Inclusive Youth Development Settings. Maine has also developed an Early Childhood Settings Inclusion Toolkit (<https://ccids.umaine.edu/resources/inclusion-toolkit/>). This toolkit and related trainings were designed to support early childhood educators to work with children with limited English proficiency, children who are bilingual, children with developmental delays or disabilities, Native American and American Indian children, children in LGBTQ families, children arriving in Maine with immigrant, refugee, and asylee status, and children with all types of lived experiences and identities.

English and French are the primary languages in Maine. However, some pockets of Maine have over 16 different languages. MRTQ PDN works individually with programs and practitioners needing assistance. Interpreters are available on an as needed basis. The MRTQ PDN website can be read in multiple languages.

6.3.4 Child developmental screening

Describe how all providers receive, through training and professional development, information about: (1) existing resources and services the State/Territory can make available in conducting developmental screenings and providing referrals to services when appropriate for children who receive assistance under this part, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program (42 U.S.C. 1396 et seq.) and developmental screening services available under section 619 and part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.); and (2) how child care providers may utilize these resources and services to obtain developmental screenings for children who receive assistance and who may be at risk for cognitive or other developmental delays, which may include social, emotional, physical, or linguistic delays: **Efforts have been made over the past few years to improve developmental screening to increase the number of children identified early for services. Maine's Child Development Services (CDS) has made a major effort in the past few years to increase outreach statewide about Part C services, contacting all primary referral sources (e.g., physicians, Early Head Start, home health agencies and so forth) at least annually; in FY22, CDS reached almost 1,500 referral sources. Special efforts were made to reach referral sources in touch with families of infants (e.g., birthing hospitals). Data suggests those outreach services are having an effect: In one year (FY21–FY22) there was an 88% increase in Part C referrals from hospitals and an almost 50% increase in the number of infants with Individualized Family Service Plans (IFSP). Data from MaineCare shows an improvement over the past several years from 30% to 36% of children with MaineCare receiving developmental screenings which is similar to the comparable national median but lower than other New England states. Help Me Grow has also**

begun to conduct ASQ screenings at child care sites; during 2023, 121 screenings were conducted through Help Me Grow.

The Lead Agency's Help Me Grow Maine program is a free information line available to service providers, pregnant parents, and families with children up to eight years of age. HMG's staff provide developmental screening and information to access development screenings. HMG's staff, materials, and website provide developmental screening information and an array of resources. While efforts have been made on several fronts to expand early screening, Maine still has a relatively low rate of early identification compared to other states.

Maine provides information to child care programs and early childhood professionals and inclusive practices and supporting children with diverse needs through the Early Childhood Consultation Partnership program, Help Me Grow, and MRTQ PDN. MRTQ PDN has 2 dedicated staff that focus on Inclusion and Diversity. As a partner of the MRTQ PDN, the Center for Community Inclusion and Disabilities Studies provides expertise in inclusion, diversity and equity. Maine's ECCP program focuses on inclusion and Early Childhood Mental Health with statewide capacity. Also, the MRTQ PDN offers the following core knowledge trainings that align with a State Inclusionary Credential: Inclusive Child Care (30 hours), Creating Inclusive Youth Development Settings (30 hours), Foundations of Inclusion: Relevant Laws, Featuring the Americans with Disabilities Act (ADA) (6 hours) Foundations of Universal Design and Individualizing (12 hours), Collaborating with Others to Support Inclusion (12 hours), Positive Supports and Challenging Behavior.

The Lead Agency will be working toward increased outreach around developmental screenings and more specific guidance to child care programs on conducting developmental screenings and providing referrals to services when appropriate, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program (42 U.S.C. 1396 et seq.) and developmental screening services available under section 619 and part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.).

6.4 Early Learning and Developmental Guidelines

Lead Agencies must develop, maintain, or implement early learning and developmental guidelines appropriate for children from birth to kindergarten entry. Early learning and developmental guidelines should describe what children should know and be able to do at different ages and cover the essential domains of early childhood development, which at a minimum includes cognition, including language arts and mathematics; social, emotional, and physical development; and approaches toward learning.

6.4.1 Early learning and developmental guidelines

- a. Check the boxes below to certify the Lead Agency's early learning and developmental guidelines are:
 - i. ☒ Research-based.
 - ii. ☒ Developmentally appropriate.
 - iii. ☒ Culturally and linguistically appropriate.
 - iv. ☒ Aligned with kindergarten entry.

- v. ☒ Appropriate for all children from birth to kindergarten entry.
 - vi. ☒ Implemented in consultation with the educational agency and the State Advisory Council on Early Childhood Education and Care or similar coordinating body.
 - vii. If any components above are not checked, describe: **N/A**
- b. Check the boxes below to certify that the required domains are included in the Lead Agency's early learning and developmental guidelines.
- i. ☒ Cognition, including language arts and mathematics.
 - ii. ☒ Social development.
 - iii. ☒ Emotional development.
 - iv. ☒ Physical development.
 - v. ☒ Approaches toward learning.
 - vi. ☐ Other optional domains. Describe any optional domains:
 - vii. If any components above are not checked, describe: **N/A**
- c. When were the Lead Agency's early learning and developmental guidelines most recently updated and for what reason? **The Preschool Maine Early Learning and Development Standards (P-MELDS) were updated in fall of 2023. Previously, in 2021 the supporting Infant Toddler Guidelines were updated and renamed the Infant and Toddler Maine Early Learning and Development Standards (I/T-MELDS).**
- d. Provide the Web link to the Lead Agency's early learning and developmental guidelines.
<https://www.maine.gov/doe/learning/earlychildhood/publicpreschool/standards>

<https://www.maine.gov/doe/sites/maine.gov.doe/files/inline-files/PreschoolMELDS2024.pdf>

https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/Infant%20and%20Toddler%20MELDS_2021.pdf

6.4.2 Use of early learning and developmental guidelines

- a. Describe how the Lead Agency uses its early learning and developmental guidelines. **The early learning guidelines are imbedded into Maine's quality rating system. The intent of Maine's Early Learning and Development Standards is to:**
- Represent the communities, values, perspectives and recommended practices of the early childhood community in the State of Maine;
 - Provide early childhood educators with guidance as they design inclusive environments, shape curriculum, lead professional development initiatives, build intentionality into teaching practice, engage families, and support children's learning at home. Since effective early childhood learning environments for young children incorporate an integrated, holistic approach to teaching children and address each child's social emotional, physical and intellectual development, Maine's Early Learning and Development Standards' eight domains and their standards cannot be addressed in isolation;

- Facilitate personalized learning goals to accommodate each child’s unique learning pathway. Learning goals are based on predictable developmental stages, yet include an individualized approach to each child as s/he develops at her/his own rate. Development is influenced by many factors: genetics, prenatal care, birth and temperament, attachment to families, caregivers, and teachers and early experiences;
 - Provide early childhood educators with tools to support children who may be at-risk; 7 • Serve as a guide for best practices in inclusive environments for all children including those who are culturally, linguistically and ability diverse;
 - Be neither a curriculum nor an assessment, but should align with and inform both in early childhood settings;
 - Connect the learning that occurs in the preschool years (age three through kindergarten entry) with the essential learning and development that occurs both before and after this age span. Maine’s Early Learning and Development Standards is aligned with the Supporting Maine’s Infants and Toddlers: Guidelines for Learning & Development and Maine’s College and Career Ready Standards (K-12), as appropriate, to demonstrate the continuous learning pathways for children as they progress from birth through all subsequent development.
 - Incorporate and reflect current research on early education and care, school readiness, and culturally embedded practices
- b. Check the boxes below to certify that CCDF funds are not used to develop or implement an assessment for children that:
- i. ☒ Will be the primary or sole basis to determine a child care provider ineligible to participate in the CCDF.
 - ii. ☒ Will be used as the primary or sole basis to provide a reward or sanction for an individual provider.
 - iii. ☒ Will be used as the primary or sole method for assessing program effectiveness.
 - iv. ☒ Will be used to deny children eligibility to participate in CCDF.
 - v. If any components above are not checked, describe: **N/A**

7 Quality Improvement Activities

The quality of child care directly affects children’s safety and healthy development while in care settings, and high-quality child care can be foundational across the lifespan. Lead Agencies may use CCDF for quality improvement activities for all children in care, not just those receiving child care subsidies. OCC will collect the most detailed Lead Agency information about quality improvement activities in annual reports instead of this Plan.

Lead Agencies must report on CCDF child care quality improvement investments in three ways:

1. In this Plan, Lead Agencies will describe the types of activities supported by quality investments over the 3-year period.
2. An annual expenditure report (the ACF-696). Lead Agencies will provide data on how much CCDF funding is spent on quality activities. This report will be used to determine compliance with the required quality and infant and toddler

spending requirements.

3. An annual Quality Progress Report (the ACF-218). Lead Agencies will provide a description of activities funded by quality expenditures, the measures used to evaluate its progress in improving the quality of child care programs and services within the State/Territory, and progress or barriers encountered on those measures.

In this section of the Plan, Lead Agencies will describe their quality activities needs assessment and identify the types of quality improvement activities where CCDF investments are being made using quality set-aside funds.

7.1 Quality Activities Needs Assessment

7.1.1 Needs assessment process and findings

- a. Describe the Lead Agency needs assessment process for expending CCDF funds on activities to improve the quality of child care, including the frequency of assessment, how a range of parents and providers were consulted, and how their views are incorporated: **MRTQ PDN conducts a biennial Professional Development Needs Assessment. The survey is conducted primarily online. In addition to demographic questions (county of residence, type of program, job title, years of experience), questions are asked about individuals' knowledge and training and technical assistance activities offered by MRTQ PDN, where individuals access training (including why they do not access training from MRTQ PDN, if applicable), and their interest in receiving training on various topics. The survey link is sent out to all MRTQ registry members with web access. It is publicized in MRTQ PDN publications (newsletters and social media). Postcards are sent to all licensed programs that do not have a Registry member on staff. The link is shared with child care licensing and other partners.**

This past year the Lead Agency focused on consultation with parents and providers to inform the early care and education needs assessment conducted for the Preschool Development Grant. This needs assessment included a focus on quality improvement activities.

Over the past few years, Maine has expanded its programming and initiatives to improve the quality of child care adding in the Early Childhood Consultation Partnership (ECCP) Program and piloting First4ME, an innovative program that is modeled after Head Start child care partnerships and regional hubs for early care and education. In the coming years, Maine will be building on the biennial needs assessment through MRTQ PDN to conduct a broader assessment of all quality activities and to include parents and family consultation in the assessment process.

- b. Describe the findings of the assessment, including any findings related to needs of different populations and types of providers, and if any overarching goals for quality improvement were identified: **Key takeaways from the PDG needs assessment on quality activities were:**
 - Improvements to Rising Stars for ME have the potential to make it easier and more feasible for programs to improve their ratings but the value of making the required

investments needs further demonstration.

- Programs need incentives for improving program quality given competing challenges, e.g., staff shortages.
- The greatest needs for professional development have not changed much in 5 years but may be felt even more acutely post pandemic: addressing challenging behaviors; working with children who have special needs; and learning about trauma-informed practices.

Key takeaways from the biennial MRTQ PDN needs assessment were:

- 85% providers access training through MRTQ PDN, 85% were satisfied with trainings offered by MRTQ PDN, and 85% value access to on demand training.
- About 33% want to participate in facilitated trainings online and 33% want to access in person training as part of their professional learning
- The top 5 training topics that professionals are most interested in include support for challenging behaviors, social and emotional learning, outdoor and nature-based programming, serving children with disabilities, and engaging and communicating with families.

The Lead Agency has identified some key goals that can be delivered through CCDF funds and PDG funds, including quality improvement funding and technical assistance, increased outreach connecting programs to onsite technical assistance, including through ECCP, and growth in training and T.A. for inclusion and serving children with disabilities.

7.2 Use of Quality Set-Aside Funds

Lead Agencies must use a portion of their CCDF expenditures for activities designed to improve the quality of child care services and to increase parental options for and access to high-quality child care. They must use the quality set-aside funds on at least one of 10 activities described in CCDF and the quality activities must be aligned with a Statewide or Territory-wide assessment of the State's or Territory's need to carry out such services and care.

7.2.1 Quality improvement activities

- a. Describe how the Lead Agency will make its Quality Progress Report (ACF – 218) and expenditure reports, available to the public. Provide a link if available.
<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>
- b. Identify Lead Agency plans, if any, to spend CCDF funds for each of the following quality improvement activities. If an activity is checked “yes”, describe the Lead Agency’s current and/or future plans for this activity.
 - i. Supporting the training and professional development of the child care workforce, including birth to five and school-age providers.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **The Lead Agency’s partnership with MRTQ PDN which provides Maine’s system of professional development and technical assistance is funded through blended funding that includes CCDF funds. The Lead Agency supports the credentialing and degree attainment of child care providers and educators through apprenticeships, scholarships, badging, and articulation agreements.**

- ii. Developing, maintaining, or implementing early learning and developmental guidelines.
☐ No plans to spend in this category of activities at this time.
☒ Yes. If yes, describe current and future investments. **The Lead Agency utilizes CCDF funds for the Preschool Maine Early Learning and Development Standards (P-MELDS) and the Infant Toddler Maine Early Learning and Development Standards (I/T MELDS) training and maintenance of those standards.**
- iii. Developing, implementing, or enhancing a quality improvement system.
☐ No plans to spend in this category of activities at this time.
☒ Yes. If yes, describe current and future investments. **The Lead Agency's partnership with MRTQ PDN which provides Maine's system of professional development and technical assistance including Maine's Rising Stars for ME QRIS is funded through CCDF.**
- iv. Improving the supply and quality of child care services for infants and toddlers.
☐ No plans to spend in this category of activities at this time.
☒ Yes. If yes, describe current and future investments. **The Lead Agency's partnership with MRTQ PDN which provides Maine's system of professional development and technical assistance is funded through blended funding that includes CCDF funds. The Lead Agency supports the Infant and Toddler Credential through this partnership. The Lead Agency utilizes CCDF funds for the Infant Toddler Maine Early Learning and Development Standards (I/T MELDS) training and maintenance of those standards. The Child Care Affordability Program (CCAP) in Maine provides an infant stipend of \$100 per week for 20 or more hours of care. The program also offers an infant/toddler bump based on QRIS Rising Star levels. The bump is as follows:
 Star 3: 2%
 Star 4: 5%
 Star 5: 10%**
- v. Establishing or expanding a statewide system of CCR&R services.
☒ No plans to spend in this category of activities at this time.
☐ Yes. If yes, describe current and future investments.
- vi. Facilitating compliance with Lead Agency child care licensing, monitoring, inspection and health and safety standards.
☐ No plans to spend in this category of activities at this time.
☒ Yes. If yes, describe current and future investments. **The Lead Agency houses Children's Licensing and Investigation Services (CLIS). CLIS is funded through CCDF for child care licensing activities.**
- vii. Evaluating and assessing the quality and effectiveness of child care services within the State/Territory.
☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **The Lead Agency's oversight of QRIS and the role of the Early Child Care and Education Quality Specialist is to assess the quality and effectiveness of child care services through site visits and review of a program's online portfolio.**

viii. Accreditation support.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **The Lead Agency's partnership with MRTQ PDN which provides Maine's system of professional development and technical assistance is funded through blended funding that includes CCDF funds. Accreditation supports is part of the PDN system.**

ix. Supporting State/Territory or local efforts to develop high-quality program standards relating to health, mental health, nutrition, physical activity, and physical development.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **The Lead Agency's partnership with MRTQ PDN which provides Maine's system of professional development and technical assistance is funded through blended funding that includes CCDF funds. The QRIS standards are maintained within the system and include standards relating to health, mental health, nutrition, physical activity, and physical development.**

x. Other activities determined by the Lead Agency to improve the quality of child care services and the measurement of outcomes related to improved provider preparedness, child safety, child well-being, or kindergarten entry.

☒ No plans to spend in this category of activities at this time.

☐ Yes. If yes, describe current and future investments.

8 Lead Agency Coordination and Partnerships to Support Service Delivery

Coordination and partnerships help ensure that the Lead Agency's efforts accomplish CCDF goals effectively, leverage other resources, and avoid duplication of effort. Such coordination and partnerships can help families better access child care, can assist in providing consumer education to parents, and can be used to improve child care quality and the stability of child care providers. Such coordination can also be particularly helpful in the aftermath of disasters when the provision of emergency child care services and the rebuilding and restoring of child care infrastructure are an essential part of ensuring the well-being of children and families in recovering communities.

This section identifies who the Lead Agency collaborates with to implement services, how match and maintenance-of-effort (MOE) funds are used, coordination with child care resource and referral (CCR&R) systems, and efforts for disaster preparedness and response plans to support continuity of operations in response to emergencies.

8.1 Coordination with Partners to Expand Accessibility and Continuity of Care

Lead Agencies must coordinate child care services supported by CCDF with other federal, State/Territory, and local level programs. This includes programs for the benefit of Indian

children, infants and toddlers, children with disabilities, children experiencing homelessness, and children in foster care.

8.1.1 Coordination with required and optional partners

Describe how the Lead Agency coordinates and the results of this coordination of the provision of child care services with the organizations and agencies to expand accessibility and continuity of care and to assist children enrolled in early childhood programs in receiving full-day services that meet the needs of working families.

The Lead Agency must coordinate with the following agencies:

- a. State Advisory Council on Early Childhood Education and Care or similar coordinating body (pursuant to 642B(b)(1)(A)(i) of the Head Start Act). Describe the coordination and results of the coordination: **The Children's Cabinet Early Childhood Advisory Council (CCECAC) operates as the State Advisory Council on Early Education and Care. Coordination between the Lead Agency's CCDF Plan and the CCECAC's strategic plan, occurs through the ongoing monthly meeting to ensure alignment on increasing access to affordable quality early care and to recruit, prepare, and retain a diverse early childhood workforce.**

Members of the council have provided feedback and recommendations to the Lead Agency on the implementation and expansion of the Child Care Affordability Program, Child Care Employment Awards, the Early Childhood Consultation Partnership (ECCP) Program, Help Me Grow Maine, First4ME pilots, the work of MRTQ PDN, and the connected work on the Preschool Development Grant. Feedback led to revised implementation of quality improvement awards and the addition of quality bumps to the state's child care stability grants.

- b. Indian Tribe(s) and/or Tribal organization(s), at the option of the Tribe or Tribal organization. Describe the coordination and results of the coordination, including which Tribe(s) was (were) involved: **Each of the five Maine Tribal CCDF administrators are contacted on a regular basis by the State Administrator with updates to and invitations concerning child care policies and rules in Maine. The five CCDF tribes recognized in Maine are the Aroostook Band of Micmac Indians, the Houlton Band of Maliseet Indians, Penobscot Nation, Passamaquoddy Tribe at Indian Township and Passamaquoddy Tribe at Pleasant Point. In May of 2023, the CCDF Administrator was invited to meet with the five CCDF Tribal Administrators during the Onsite Visit for Tribal Nationals in Maine along with ACF representatives. The goal was to share updates and barriers/successes from all. Communication between individual administrators continues based on the Tribal goals. The CCDF Administrator also met individually with a Tribal CCDF Administrator to discuss any updates or concerns regarding their tribe's early care and education systems. Several topics of discussion included data management systems, health and safety standards and training, and other programs available.**

As a result of connection and collaboration, the Lead Agency is working to expand the state-funded Head Start grants to Tribal Head Starts and to fund a home visiting project through PDG funds.

[] Not applicable. Check here if there are no Indian Tribes and/or Tribal organizations in the State/Territory.

- c. State/Territory agency(ies) responsible for programs for children with disabilities, including early intervention programs authorized under the Individuals with Disabilities Education Act. Describe the coordination and results of the coordination: **The Lead Agency and Child Development Services (CDS) through Maine's Department of Education, coordinate on several cross departmental efforts including the Children's Cabinet and Children's Cabinet Early Childhood Advisory Council (CCECAC). The results of our coordination is currently focused on transitioning Part B special education Free and Appropriate Public Education (FAPE) responsibility to school districts through a mixed delivery model of implementation where children can attend their preschool programming in family child care programs, child care centers, Head Starts, and school district classrooms.**

In partnership with the Lead Agency, MRTQ PDN and the Center for Community Inclusion and Disability Studies (CCDIS) support the Lead Agency's Quality Rating and Improvement System (QRIS). Maine has the Inclusion Credential, an advanced credential for practitioners serving children with special needs. Maine also has the Early Childhood Consultation Partnership (ECCP) Program supporting early childhood practitioners with early childhood mental health consultation and inclusive practices. The Lead Agency, Maine DOE, MRTQ PDN and CCDIS work together to ensure providers get needed support around serving children with special needs. Our coordination has led to efforts to expand access to the Inclusion Credential and ECCP.

- d. State/Territory office/director for Head Start State collaboration. Describe the coordination and results of the coordination: **The Lead Agency holds contracts with the 11 Head Start grantees for state-funded Head Start grants. Maine's DOE Head Start (HS) Collaboration Director facilitates the use of federal funds with the 11 Head Start grantees throughout the State by providing resources and clarifications of State policies. The Lead Agency and HS Collaboration Director meet regularly to discuss and strategize topics and goals on accessibility and continuity of care and changes of policies that are impacting areas throughout the State and are on several cross-agency groups. MRTQ PDN and the HS Collaboration director also coordinate services through the Maine Head Start State Collaboration Office. The HS Collaboration Director holds a seat as a member on the Children's Cabinet Early Childhood Advisory Council (CCECAC). In addition, the Lead Agency attends Head Start Directors meeting every other month to address the early care and education goals of the State, policies, and services provided through Head Start.**

Through our Head Start collaboration efforts, both Maine DOE and DHHS are working on how to best support mixed delivery preschool expansion with Head Stars and child care and how potential updates to the Salary Supplement system could best work with Head Start policies and practices.

- e. State/Territory agency responsible for public health, including the agency responsible for immunizations. Describe the coordination and results of the coordination: **The Lead Agency houses Maine's Center for Disease Control and Prevention (CDC), which is the office responsible for public health and immunizations. Collaboration is ongoing between the CDC and OCFS through several cross-agency groups including the Children's Cabinet Early Childhood Advisory Council (CCECAC), and emergency preparedness. The coordination between offices helps the Lead Agency provide the most up to date public health and immunization information to child care programs.**

- f. State/Territory agency responsible for employment services/workforce development. Describe the coordination and results of the coordination: **The Lead Agency collaboration with the Maine Department of Labor (DOL) and Office of Family Independence (OFI) provides access to families by linking the services through Maine's Career Center. The Career Centers offer resources on job seeking, child care needs, and financial needs online, at Career Center events, and locations. DOL offers grant funding to Maine residents and assist workers to learn new skills. Maine businesses gain access to a qualified workforce to succeed in the changing economy with the Competitive Skills Scholarship Program. Grant money can be used to pay for child care. The Lead Agency and DOL communicate with each other on funding, rules, and child care options for grant recipients. As well as attending an annual DOL Employer Summit to provide information on Maine's Child Care Employer Toolkit and CCAP. One result of our office's coordination is the development of the Early Childhood Apprenticeships through MRTQ PDN and ongoing work to coordinate and expand apprenticeships across the state.**
- g. State/Territory agency responsible for public education, including pre-Kindergarten. Describe the coordination and results of the coordination: **The Lead Agency and Maine Department of Education (DOE) meet regularly to discuss the goals of both departments and to collaborate on many aspects of early care and education in the state. Additionally, collaboration crosses over between the two departments with attendance of several committees, projects, and meeting State goals. The results of the cross-agency coordination include several successful and ongoing collaborative projects:**
- partnering on revisions to the Maine Early Learning and Development Standards,
 - coordinating work of First4ME at DHHS and First 10 at DOE,
 - working together on building a mixed delivery preschool system,
 - and the joint implementation of Maine's PDG B-5 grant.
- h. State/Territory agency responsible for child care licensing. Describe the coordination and results of the coordination: **The Children's Licensing and Investigation Services Unit (CLIS) is a division at OCFS. The CCDF Administrator and CLIS work closely together on coordinating goals and policies for child care, rulemaking, and meeting guidelines for CCDF Rules, and other Lead Agency activities. A recent result of the coordination between our divisions is centered on outdoor and nature based early care and education. Staff across divisions have shared information and collaborated on ways to expand training and adapt practices to better support outdoor and nature based programming. This has led to new resources, training and Communities of Practice at MRTQ PDN, training for child care licensors, and work to develop policies for licensing outdoor and nature based programs.**
- i. State/Territory agency responsible for the Child and Adult Care Food Program (CACFP) and other relevant nutrition programs. Describe the coordination and results of the coordination: **DOE is responsible for the Child and Adult Care Food Program (CACFP) and School Nutrition program. Resource information to the Child and Adult Food Program is provided in the form of links on the CCAP web page as well on our consumer education statement. Resources for families has links for physical activity and nutrition and to USDA chose My Plate web site. CCAP offers to disseminate information to Child Care Providers about CACFP with inserts accompanying paper billing. CCAP Supervisor contacts the Child Nutrition Director at DOE at a minimum once per year for inserts. License-exempt Child Care Providers who care for children receiving child care subsidy may also participate in CACFP. CCAP staff coordinate with CACFP staff so they may keep Sponsor up to date on**

rules regarding CCAP and the number of children Unlicensed Providers may care for and in turn claim for meals. The results of this coordination in 2024 include contracts with sponsoring agencies to increase FCC providers participating in CACFP.

- j. McKinney-Vento State coordinators for homeless education and other agencies providing services for children experiencing homelessness and, to the extent practicable, local McKinney-Vento liaisons. Describe the coordination and results of the coordination: **The Lead Agency and DOE's Homeless Education Consultant (McKinney-Vento state coordinator) continue to have interdepartmental exchanges that require updates and coordination between the two departments including work on the PDG B-5 grant. Results of coordination include the development of an on demand training at MRTQ PDN and support for building policies and practices that support children experiencing homelessness, including Child Care Affordability Program rules.**
- k. State/Territory agency responsible for the TANF program. Describe the coordination and results of the coordination: **The Office for Family Independence (OFI) is within the Lead Agency which determines a family's financial eligibility for a wide range of public assistance program that include TANF, MaineCare, Food Supplement, and Emergency Assistance. The Lead Agency and OFI's substantial coordination on policy and practice increases the assurance that families seeking assistance needs are met. This coordination includes involving CLIS to keep the Licensing Specialists updated on the programs OFI has. In addition, members of both offices sit on several committees and projects together. A result of coordination between offices has been updates to policies and practices with market rates and child care reimbursement, including decisions to pay the state rate through TANF.**
- l. State/Territory agency responsible for Medicaid and the State Children's Health Insurance Program. Describe the coordination and results of the coordination: **The Lead Agency is responsible for MaineCare and coordination within the Lead Agency's offices continues to ensure the accessibility of the Department is consumer friendly. OFI is responsible for Medicaid and the state Children's Health Insurance Program. The collaboration between offices has helped OCFS to share current and accurate health insurance information with families through child care provider outreach and to ensure Help Me Grow Maine has accurate information for parents and families.**
- m. State/Territory agency responsible for mental health services. Describe the coordination and results of the coordination: **The Lead Agency is responsible for mental health services with the Office of Aging and Disability Services (OADS) and the Office of Behavioral Health (OBH). High levels of coordination occur between all offices in order to align the Lead Agency's goals. OCFS meets with staff from Children's Behavioral Health services at OBH as part of sharing information about the statewide Early Childhood Consultation Partnership (ECCP) at OCFS and resources available to families through OBH. This ensures that OCFS staff and Help Me Grow Maine have current and accurate information for parents and families seeking support for their children.**
- n. Child care resource and referral agencies, child care consumer education organizations, and providers of early childhood education training and professional development. Describe the coordination and results of the coordination: **Maine does not have a resource and referral agency. The Lead Agency is in constant coordination and partnership with Maine Roads to Quality Professional Development Network (MRTQ PDN) around**

early childhood education training and professional development, child care consumer education, and support for families to find and access child care. MRTQ PDN helps set goals for the Child Care Choices website for locating Child Care Providers in Maine and for training and technical assistance. Recent goals have been to develop and provide access to trainings on Homelessness, Health and Safety topics, and access to statewide technical assistance. The Lead Agency also houses Help ME Grow Maine linking families and professionals to information about child development, pregnancy, and community resources for children all over Maine up to the age of eight years old. Coordination with Help Me Grow Maine has led to a focus on updating information on the Child Care Choices Maine website, including accurate listings of child care openings.

- o. Statewide afterschool network or other coordinating entity for out-of-school time care (if applicable). Describe the coordination and results of the coordination: **The Lead Agency is affiliated with Maine's After School Network (MASN) through MRTQ PDN and the University of Southern Maine as a primary collaborator as supports for trainings, technical assistance, and advocacy of out-of- school time programs across the State. Coordination between Maine's CCDF administrator and MASN takes place regularly with meetings with MRTQ PDN to achieve the Lead Agency's goals regarding the school age population in Maine. One of the main focuses is on supporting recreation al programs to become Child Care Affordability Program providers and/or licensed facilities. In the coming year we will be evaluating data to look at the results of this coordination and how many programs have joined CCAP and/or became licensed.**
- p. Agency responsible for emergency management and response. Describe the coordination and results of the coordination: **The Lead Agency is responsible for emergency management and response for the State with coordination taking place between divisions of the Department that include the Division of Environmental and Community Health (DECH), Office of Children and Family Services (OCFS), Division of Support Enforcement and Recovery (DSER), Maine Center for Disease Control (MeCDC), Office of MaineCare Services, and the Office of Behavioral Health (OBH). Coordination continues to take place with regular meetings with the Emergency Preparedness Work Group (EPWG) having representation from each of the above departments to ensure in the event of an emergency or disaster, service would continue or be implemented for those that are newly in need of services. This coordination has helped OCFS to create and update the Child Care Emergency Plan in Maine and to create and update the YIKES guide that support child care programs to create their own emergency plans.**
- q. The following are examples of optional partners a Lead Agency might coordinate with to provide services. Check which optional partners the Lead Agency coordinates with and describe the coordination and results of the coordination.
 - i. ☒ State/Territory/local agencies with Early Head Start – Child Care Partnership grants. Describe: **The Lead Agency is overseeing a pilot program built on the Early Head Start Child Care Partnership model with 4 sites in Maine that are implementing child care partnerships that bring access to full wrap around Head Start supports, but where the coordinating agency may be a Head Start or may be another nonprofit. OCFS is currently evaluating the results of this coordination. In the first year, the pilot has led to the Lead Agency being more connected to local challenges and the different needs being experienced by region in the state. For example, in one area there is more support needed for multilingual children and**

children experiencing homelessness and in another area there is more support needed for child care business practices and opening new child care programs.

- ii. ☒ State/Territory institutions for higher education, including community colleges. Describe: **The Lead Agency contracts with the Maine Association for the Education of Young Children to deliver the T.E.A.C.H. Early Childhood Maine Scholarship Program. This program coordinates with community colleges and the University of Maine system. The scholarship program and resulting higher education collaborations have led to the expansion of online course offerings and a model where students can attend classes nights and weekends at a local community college working toward their 4-year degree with the UMaine system.**
- iii. ☐ Other federal, State, local, and/or private agencies providing early childhood and school-age/youth-serving developmental services. Describe:
- iv. ☐ State/Territory agency responsible for implementing the Maternal, Infant, and Early Childhood Home Visiting (MIECHV) programs grant. Describe:
- v. ☒ Agency responsible for Early and Periodic Screening, Diagnostic, and Treatment Program. Describe: **The Lead Agency is responsible for child developmental monitoring and screening at Maine CDC. The result of this collaboration has been work between Maine CDC and Help Me Grow Maine to develop strategies to increase access to developmental screening in Maine.**
- vi. ☒ State/Territory agency responsible for child welfare. Describe: **The Lead Agency is responsible for Child Welfare. The result of ongoing internal office collaboration is reflected in new connections and strategies being developed between Child Welfare and Help Me Grow Maine and the Early Childhood Consultation Partnership (ECCP) Program. OCFS is working to increase referrals to Help Me Grow and ECCP from Child Welfare. We will be analyzing the results of this coordination in the coming year.**
- vii. ☒ Child care provider groups or associations. Describe: **Representation from the Family Child Care Association of Maine (FCCAM) and the Maine Association for the Education of Young Children (MaineAEYC) are on the Children's Cabinet Early Childhood Advisory Council (CCECAC). The Lead Agency works to have high levels of outreach and communication with both groups, as well as the Maine Head Start Directors Association, the YMCA Alliance of Northern New England, and the Maine Afterschool Network regarding key CCDF topics and OCFS updates, conferences, and the PDG grant activities. One result of the collaboration between groups, association, and OCFS is an annual statewide conference planned and funded by ECE partners, including MaineAEYC, FCCAM, Maine Head Start Directors, Educare Central Maine, YMCAs, Maine DOE, and the Lead Agency.**
- viii. ☒ Parent groups or organizations. Describe: **The Lead Agency partners with Educare Central Maine, which is a Head Start housed at Kennebec Valley Community Action Program (KVCAP), to support statewide Parent Ambassador groups. As a result of this collaboration, OCFS and KVCAP identified a need to expand parent leadership programs beyond Head Start parents and with targeted support for New American families in Maine. New cohorts are being developed and piloted for parents in Maine.**

- ix. ☐ Title IV B 21st Century Community Learning Center Coordinators. Describe:
- x. ☒ Other. Describe: **OCFS is represented at the Physical Activity and Nutrition in Early Care and Education (PAN in ECE) Committee that is made up of members of early care and education stakeholders and State agencies. The result of this coordination was updated physical health and nutrition guidelines in the Maine's Quality Rating and Improvement System (QRIS) revision to create Rising Stars for ME.**

8.2 Optional Use of Combined Funds, CCDF Matching, and Maintenance-of-Effort Funds

Lead Agencies may combine CCDF funds with other Federal, State, and local child care and early childhood development programs, including those in 8.1.1. These programs include preschool programs, Tribal child care programs, and other early childhood programs, including those serving infants and toddlers with disabilities, children experiencing homelessness, and children in foster care.

Combining funds may include blending multiple funding streams, pooling funds, or layering funds from multiple funding streams to expand and/or enhance services for infants, toddlers, preschoolers, and school-age children and families to allow for the delivery of comprehensive quality care that meets the needs of children and families. For example, Lead Agencies may use multiple funding sources to offer grants or contracts to programs to deliver services; a Lead Agency may allow a county/local government to use coordinated funding streams; or policies may be in place that allow local programs to layer CCDF funds with additional funding sources to pay for full-day, full-year child care that meets Early Head Start/Head Start Program Performance Standards or State/Territory pre-Kindergarten requirements in addition to State/Territory child care licensing requirements.

As a reminder, CCDF funds may be used in collaborative efforts with Head Start and Early Head Start programs to provide comprehensive child care and development services for children who are eligible for both programs.

8.2.1 Combining funding for CCDF services

Does the Lead Agency combine funding for CCDF services with Title XX of the Social Services Block Grant (SSBG), Title IV B 21st Century Community Learning Center Funds, State-only child care funds, TANF direct funds for child care not transferred into CCDF, Title IV-B, IV-E funds, or other federal or State programs?

☒ No. (If no, skip to question 8.2.2)

☐ Yes.

- i. If yes, describe which funds you will combine. Combined funds may include, but are not limited to:

☐ Title XX (Social Services Block Grant, SSBG)

☐ Title IV B 21st Century Community Learning Center Funds (Every Student Succeeds Act)

☐ State- or Territory-only child care funds

☐ TANF direct funds for child care not transferred into CCDF

☐ Title IV-B funds (Social Security Act)

☐ Title IV-E funds (Social Security Act)

☐ Other. Describe:

- ii. If yes, what does the Lead Agency use combined funds to support, such as extending the day or year of services available (i.e., full-day, full-year programming for working families), smoothing transitions for children, enhancing and aligning quality of services, linking comprehensive services to children in child care, or developing the supply of child care for vulnerable populations?

8.2.2 Funds used to meet CCDF matching and MOE requirements

Lead Agencies may use public funds and donated funds to meet CCDF match and maintenance of effort (matching MOE) requirements.

Note: Lead Agencies that use State pre-Kindergarten funds to meet matching requirements must check State pre-Kindergarten funds and public and/or private funds.

Use of private funds for match or maintenance-of-effort: Donated funds do not need to be under the administrative control of the Lead Agency to qualify as an expenditure for federal match. However, Lead Agencies must identify and designate in the State/Territory CCDF Plan the donated funds given to public or private entities to implement the CCDF child care program.

☐ Not applicable. The Lead Agency is a Territory (skip to 8.3.1).

- a. Does the Lead Agency use public funds to meet match requirements?

☒ Yes. If yes, describe which funds are used: **State General Funds**

☐ No.

- b. Does the Lead Agency use donated funds to meet match requirements?

☐ Yes. If yes, identify the entity(ies) designated to receive donated funds:

i. ☐ Donated directly to the state.

ii. ☐ Donated to a separate entity(ies) designated to receive donated funds. If checked, identify the name, address, contact, and type of entities designated to receive private donated funds:

☒ No.

- c. Does the Lead Agency certify that, if State expenditures for pre-Kindergarten programs are used to meet the MOE requirements, the following is true:

- The Lead Agency did not reduce its level of effort in full-day/full-year child care services.
- The Lead Agency ensures that pre-Kindergarten programs meet the needs of working parents.
- The estimated percentage of the MOE requirement that will be met with pre-Kindergarten expenditures (does not to exceed 20 percent).
- If the percentage is more than 10 percent of the MOE requirement, the State will coordinate its pre-Kindergarten and child care services to expand the availability of child care.

Public pre-Kindergarten funds may also serve as MOE funds as long as the State can describe how it will coordinate pre-Kindergarten and child care services to expand the availability of child care while using public pre-Kindergarten funds as no more than 20 percent of the State's MOE or 30 percent of its matching funds in a single fiscal year.

If expenditures for pre-Kindergarten services are used to meet the MOE requirement, does the Lead Agency certify that the State or Territory has not reduced its level of effort in full-day/full-year child care services?

☒ Yes.

☐ No. If no, describe:

8.3 Coordination with Child Care Resource and Referral Systems

Lead Agencies may use CCDF funds to establish or support a system or network of local or regional child care resource and referral (CCR&R) organizations that is coordinated, to the extent determined by the Lead Agency, by a statewide public or private non-profit, community-based or regionally based, lead child care resource and referral organization (such as a statewide CCR&R network).

If Lead Agencies use CCDF funds for local CCR&R organizations, the local or regional CCR&R organizations supported by those funds must, at the direction of the Lead Agency:

- Provide parents in the State with consumer education information concerning the full range of child care options (including faith-based and community-based child care providers), analyzed by provider, including child care provided during non-traditional hours and through emergency child care centers, in their area.
- To the extent practicable, work directly with families who receive assistance to offer the families support and assistance to make an informed decision about which child care providers they will use to ensure that the families are enrolling their children in the most appropriate child care setting that suits their needs and one that is of high quality (as determined by the Lead Agency).
- Collect data and provide information on the coordination of services and supports, including services under Part B, Section 619 and Part C of the Individuals with Disabilities Education Act.
- Collect data and provide information on the supply of and demand for child care services in areas of the State and submit the information to the Lead Agency.
- Work to establish partnerships with public agencies and private entities, including faith-based and community-based child care providers, to increase the supply and quality of child care services in the State and, as appropriate, coordinate their activities with the activities of the Lead Agency and local agencies that administer funds made available through CCDF.

8.3.1 Funding a system or network of CCR&R organization(s)

Does the Lead Agency fund a system or network of local or regional CCR&R organization(s)?

☒ No. The Lead Agency does not fund a system or network of local or regional CCR&R organization(s) and has no plans to establish one.

☐ No, but the Lead Agency has plans to develop a system or network of local or regional

CCR&R organization(s).

☐ Yes. The Lead Agency funds a system or network of local or regional CCR&R organization(s) with all the responsibilities outlined above. If yes, describe the activities outlined above carried out by the CCR&R organization(s), as directed by the Lead Agency:

8.4 Public-Private Partnerships

Lead Agencies must demonstrate how they encourage partnerships among other public agencies, Tribal organizations, private entities, faith-based organizations, businesses, or organizations that promote business involvement, and/or community-based organizations to leverage existing service delivery (i.e., cooperative agreement among providers to pool resources to pay for shared fixed costs and operation) to leverage existing child care and early education service delivery systems and to increase the supply and quality of child care services for children younger than age 13.

8.4.1 Lead Agency public-private partnerships

Identify and describe any public-private partnerships encouraged by the Lead Agency to leverage public and private resources to further the goals of CCDF: **The Lead Agency and MRTQ/PDN partnered with Coastal Enterprises, Inc to assist in the outreach, TA, and available resources to child care programs across the state. As new funding opportunities arise, collaboration continues to distribute information and support in a streamlined and concise manner.**

8.5 Disaster Preparedness and Response Plan

Lead Agencies must establish a Statewide Child Care Disaster Plan and demonstrate how they will address the needs of children—including the need for safe child care before, during, and after a state of emergency declared by the Governor or a major disaster or emergency (as defined by Section 102 of the Robert T. Stafford Disaster Relief and Emergency Assistance Act, 42 U.S.C. 5122)—through a Statewide Disaster Plan.

8.5.1 Statewide Disaster Plan updates

- a. When was the Lead Agency's Child Care Disaster Plan most recently updated and for what reason? **The Lead Agency maintains the Child Care Emergency Plan. The plan had been revised in 2019. In 2024 the state did a thorough review of the plan and made a few language updates, renaming the Child Care Subsidy Program (CCSP) to Child Care Affordability Program (CCAP).**
- b. Please certify compliance by checking the required elements the Lead Agency includes in the current State Disaster Preparedness and Response Plan.
 - i. The plan was developed in collaboration with the following required entities:
 - ☒ State human services agency.
 - ☒ State emergency management agency.
 - ☒ State licensing agency.
 - ☒ State health department or public health department.
 - ☐ Local and State child care resource and referral agencies.

- ☒ State Advisory Council on Early Childhood Education and Care or similar coordinating body.
- ii. ☒ The plan includes guidelines for the continuation of child care subsidies.
- iii. ☒ The plan includes guidelines for the continuation of child care services.
- iv. ☒ The plan includes procedures for the coordination of post-disaster recovery of child care services.
- v. The plan contains requirements for all CCDF providers (both licensed and license-exempt) to have in place:
 - ☒ Procedures for evacuation.
 - ☒ Procedures for relocation.
 - ☒ Procedures for shelter-in-place.
 - ☒ Procedures for communication and reunification with families.
 - ☒ Procedures for continuity of operations.
 - ☒ Procedures for accommodations of infants and toddlers.
 - ☒ Procedures for accommodations of children with disabilities.
 - ☒ Procedures for accommodations of children with chronic medical conditions.
- vi. ☒ The plan contains procedures for staff and volunteer emergency preparedness training.
- vii. ☒ The plan contains procedures for staff and volunteer practice drills.
- viii. If any of the above are not checked, describe: **Maine does not have local or regional CCR&Rs.**
- ix. If available, provide the direct URL/website link to the website where the Statewide Child Care Disaster Plan is posted:
<https://www.maine.gov/dhhs/ocfs/provider-resources/child-care-subsidy-information-for-providers>

9 Family Outreach and Consumer Education

CCDF consumer education requirements facilitate parental choice in child care arrangements, support parents as child care consumers who need information to make informed choices regarding the services that best suit their family's needs, and the delivery of resources that can support child development and well-being. Lead Agency consumer education activities must provide information for parents receiving CCDF assistance, the general public, and, when appropriate, child care providers. Lead Agencies should use targeted strategies for each group to ensure tailored consumer education information and take steps to ensure they are effectively reaching all individuals, including those with limited English proficiency and those with disabilities.

In this section, Lead Agencies address their consumer education practices, including details about their child care consumer education website, and the process for collecting and maintaining a record of parental complaints.

9.1 Parental Complaint Process

Lead Agencies must maintain a record of substantiated parental complaints against child care providers and make information regarding such complaints available to the public on request. Lead Agencies must also provide a detailed description of the hotline or similar reporting process for parents to submit complaints about child care providers; the process for substantiating complaints; the manner in which the Lead Agency maintains a record of substantiated parental complaints; and ways that the Lead Agency makes information on such parental complaints available to the public on request. Lead Agencies are not required to limit the complaint process to parents.

9.1.1 Parental complaint process

- a. Describe the Lead Agency's hotline or similar reporting process through which parents can submit complaints about child care providers, including a link if it is a Web-based process: **Complaints can be called into the statewide Child Protective Services Intake hotline. In addition, Child Care Licensing Specialists are assigned to the daily complaint intake on a rotating basis. Those calls come in to the main number for Child Care Licensing and are transferred to the Child Care Licensing Specialist assigned to the complaint intake. In addition, any parental complaints can be reported to the Department by means of phone, fax, or email.**
<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/reporting-child-care-concerns>
- b. Describe how the parental complaint process ensures broad access to services for families that speak languages other than English: **The Lead Agency provides all staff with Language Link, a telephone language line for assistance with translation. The State of Maine provides translators as needed.**
- c. Describe how the parental complaint process ensures broad access to services for persons with disabilities: **Child care licensing investigates all complaints received. Intake is available 24 hours a day, 7 days a week, including weekends and holidays. For those that are hearing impaired dialing the extension 711 for support.**

In addition the Lead Agency contracts with the Center for Community Inclusion and Disability Studies and the University of Maine to support children, families, early educators staff and state staff in navigating support for children with disabilities and special needs.

- d. For complaints about providers, including CCDF providers and non-CCDF providers, does the Lead Agency have a process and timeline for screening, substantiating, and responding to complaints, including information about whether the process includes monitoring?
[x] Yes. If yes, describe: The report comes into intake and is forwarded to licensing once assigned. Once it is assigned to a Licensing Specialist, they have 5 business days to begin the investigation (sometimes a more immediate response is necessary). The Licensing Specialist then has 35 days from the date of assignment to complete the investigation and send their summary to their supervisor. Almost all reports are responded to by going to the program.

Complaints are received by Child Protective Intake. Complaint information is recorded in Katahdin and submitted to the Out of Home Investigation Supervisor or designee. The Out of Home Investigation Supervisor or designee determines which complaints are referred to child care licensing or to Out of Home to investigate. Complaints alleging high risk or other serious situations will be brought to the attention of management as soon as the information is received. Child care licensing investigates all complaints received.

The timeframe for investigation begins from the date assigned. Investigations must begin within five (5) business days. Children's Licensing Supervisors (CLS) may request more immediate response, when necessary.

There are times other agencies are involved in investigating a complaint including but not limited to law enforcement, town code enforcement officers, and Child Protective Services if there is a companion report and Maine Center for Disease Control. If the report is assigned to our Out of Home Investigation team to investigate allegations of abuse or neglect the Investigator will reach out to the Child Care Licensing Specialist (CCLS) upon beginning their investigation to learn about the program's history (in addition to doing a review of the program's record), and to see if the CCLS has any relevant information in regard to the complaint.

The CCLS begins the investigation process by contacting the referent when they are known. The referent is asked about the report they made, and if they are aware of any other witnesses that might be able to corroborate their allegations. They are also asked if there is any new information that has come to their attention or any concerns they have that they did not include in their original report.

Since each complaint is unique there is no set format for how to conduct the investigation. The CCLS will review the program's history and determine if there is anything in their history that may be relevant to the investigation.

Complaint investigations are unannounced unless circumstances indicate a phone call or appointment would be more effective. The majority of complaint investigations involve the CCLS going to the program to investigate the complaint. On very rare occasions, based on the nature of the complaint and with the approval of a CLS, the investigation may be conducted by phone, Zoom or Microsoft Teams.

Upon entering the home or facility to investigate the complaint, the CCLS will explain the reason for the visit without stating the exact nature of the complaint. The CCLS interviews all relevant individuals. These individuals may include, but not limited to: persons present during a reported incident, witnesses, the person in charge of the program, parents of involved child(ren), and persons with knowledge of program operation. Permission is obtained for any Child Care Staff Members who are minors (sixteen- and seventeen-year-olds) from their parent or legal guardian. Interviews are conducted in private whenever possible. The CCLS plans an appropriate sequence of interviews based on the nature of the complaint.

In addition to interviews the CCLS will view pertinent written materials. These materials may include but are not limited to: a written complaint by a referent, prior complaints,

provider licensing history, program policies and records, personnel records, child records and attendance records. The CCLS may also conduct observations as part of the investigation process if the nature of the complaint warrants them to.

If at any point during a complaint investigation the CCLS becomes aware of or observes behavior or conditions in the child care setting that poses an immediate threat of harm to a child's safety or wellbeing, the CCLS immediately contacts a CLS to develop a plan to remediate the threat of harm.

At the conclusion of the on-site complaint investigation, the CCLS may discuss any possible rule violations with the program, unless such a discussion would place children at risk. It is optional for the CCLS to use the site record form to document they were on-site to conduct a complaint investigation and any discussion that occurred with the program.

Once all relevant information has been gathered the CCLS will document the information obtained and results of the investigation on our investigation summary form, which includes any potential rule violations and the supporting evidence. The completed investigation summary form is submitted to the CLS for review and approval. The investigation will be conducted, and documentation completed within 35 days of assignment to the CCLS. Extensions may be granted with prior approval from the CLS.

In the case of an Out of Home Investigation the investigator has up to 90 days to complete the investigation and provide the results of the investigation using the Out of Home Investigation report form. The report is submitted to the Out of Home Supervisor for review and approval of the report and any identified rule violations.

If the rule violations indicate a need for potential licensing action (Directed Plan of Action (DPOA), Notice of Conditional License, Denial of license, etc.). The CCLS and/or Out of Home Investigator (if assigned to OOH) and CLS will present the investigation results to the Compliance Advisory Panel (CAP) for decision. Within ten (10) business days of CLS approval or CAP decision, a closing letter, Letter of Deficiencies (LOD) or licensing action is sent to the provider stating the outcome of the investigation.

The CCLS documents the results of the investigation in MACWIS and supporting materials are forwarded to the Office Associate II (OAll) for scanning into DocuWare.

[] No.

- e. For substantiated parental complaints, who maintains the record for CCDF and non-CCDF providers? **Child care Licensing for licensed providers and the Child Care Affordability Program for license-exempt providers.**
- f. Describe how information about substantiated parental complaints is made available to the public; this information can include the consumer education website discussed in subsection 9.2: **Children's Licensing and Investigation Services provides monitoring inspection reports for each on-site visit to licensed and license-exempt Early Care and Education Programs. This report outlines any health and safety violations and includes action step(s) the child care program took to correct the violation(s). To find the licensing**

history and inspection reports of an Early Care and Education Program, go to:
<https://search.childcarechoices.me/>.

If it is determined by the Children's Licensing Supervisors that a Letter of Deficiencies or licensing action is warranted a Child Care Licensing Specialist will draft the appropriate follow up documentation for review and approval by the supervisors. Letter of Deficiencies and any licensing action is posted to the Consumer Education website, also known as Child Care Choices, in the program's inspection history. In addition, the program is required to post any licensing action at the program in a conspicuous location to be viewed on-site.

9.2 Consumer Education Website

Lead Agencies must provide information to parents, the general public, and child care providers through a State or Territory website, which is consumer-friendly and easily accessible for families who speak languages other than English and persons with disabilities. The website must:

- Include information to assist families in understanding the Lead Agency's policies and procedures, including licensing child care providers;
- Include monitoring and inspection reports for each provider and, if available, the quality of each provider;
- Provide the aggregate number of deaths, serious injuries, and the number of cases of substantiated child abuse that have occurred in child care settings;
- Include contact information for local CCR&R organizations to help families access additional information on finding child care; and
- Include information on how parents can contact the Lead Agency and other organizations to better understand the information on the website.

9.2.1 Consumer-friendly website

Does the Lead Agency ensure that its consumer education website is consumer-friendly and easily accessible?

- Provide the URL for the Lead Agency's consumer education website homepage:
<https://www.childcarechoices.me/>
- Does the Lead Agency certify that the consumer education website ensures broad access to services for families who speak languages other than English?
☒ Yes.
☐ No. If no, describe:
- Does the Lead Agency certify that the consumer education website ensures broad access to services for persons with disabilities?
☒ Yes.
☐ No. If no, describe:

9.2.2 Additional consumer education website links

Provide the direct URL/website link for the following:

- i. Provide the direct URL/website link to how the Lead Agency licenses child care providers: <https://www.maine.gov/dhhs/ocfs/provider-resources/child-care-licensing/becoming-a-childcare-provider>
- ii. Provide the direct URL/website link to the processes for conducting monitoring and inspections of child care providers::
<https://www.maine.gov/dhhs/ocfs/provider-resources/child-care-licensing/becoming-a-childcare-provider>

Children’s Licensing and Investigations Services website includes links to the Family Child Care Provider Licensing Rule and the Child Care Facility Licensing Rule. Section 4. Inspections and Investigations of each rule outlines the process for inspections and investigations.

- iii. Provide the direct URL/website link to the policies and procedures related to criminal background checks for staff members of child care providers:
https://www.maine.gov/dhhs/sites/maine.gov.dhhs/files/inline-files/10-148%20Ch.%2034%20CC%20Background%20Check%20Licensing%20Rule%20Final%20Adoption%20Effective%205-12-22_1.pdf
- iv. Provide the direct URL/website link to the offenses that prevent individuals from being employed by a child care provider:
<https://www.maine.gov/dhhs/ocfs/provider-resources/child-care-licensing/becoming-a-childcare-provider/child-care-provider-background-checks>

9.2.3 Searchable list of providers

- a. The consumer education website must include a list of all licensed providers searchable by ZIP code.
 - i. Does the Lead Agency certify that the consumer education website includes a list of all licensed providers searchable by ZIP code?
☒ Yes.
☐ No. If no, describe:
 - ii. Provide the direct URL/website link to the list of child care providers searchable by ZIP code: <https://www.childcarechoices.me/>
 - iii. In addition to the licensed child care providers that must be included in the searchable list, are there additional providers included in the Lead Agency’s searchable list of child care providers? Check all that apply:
☒ License-exempt center-based CCDF providers.
☒ License-exempt family child care CCDF providers.
☐ License-exempt non-CCDF providers.
☐ Relative CCDF child care providers.

☐ Other (e.g., summer camps, public pre-Kindergarten). Describe:

- b. Identify what additional (optional) information, if any, is available in the searchable results by ZIP code. Check the box when information is provided.

Provider Information Available in Searchable Results					
	All licensed providers	License-exempt CCDF center-based providers	License-exempt CCDF family child care home providers	License-exempt non-CCDF providers	Relative CCDF providers
Contact information	☒	☒	☒	☒	☐
Enrollment capacity	☐	☐	☐	☐	☐
Hours, days, and months of operation	☐	☐	☐	☐	☐
Provider education and training	☐	☐	☐	☐	☐
Languages spoken by the caregiver	☐	☐	☐	☐	☐
Quality information	☒	☒	☒	☒	☐
Monitoring reports	☒	☒	☒	☒	☐
Willingness to accept CCDF certificates	☒	☒	☒	☒	☐
Ages of children served	☐	☐	☐	☐	☐
Specialization or training for certain populations	☐	☐	☐	☐	☐
Care provided during nontraditional hours	☐	☐	☐	☐	☐

- c. Identify any other information searchable on the consumer education website for the child care provider type listed below and then, if checked, describe the searchable information included on the website.

- i. ☒ All licensed providers. Describe: **Licensed providers can update their profiles to show how many openings they have by age group.**
- ii. ☐ License-exempt CCDF center-based providers. Describe:
- iii. ☐ License-exempt CCDF family child care providers. Describe:
- iv. ☐ License-exempt, non-CCDF providers. Describe:
- v. ☐ Relative CCDF providers. Describe:

- vi. ☒ Other. Describe: **Directions to programs are available and printable.**

9.2.4 Provider-specific quality information

Lead Agencies must identify specific quality information on each child care provider for whom they have this information. Provider-specific quality information must only be posted on the consumer education website if it is available for the individual child care provider.

- a. What specific quality information does the Lead Agency provide on the website?
 - i. ☒ Quality improvement system.
 - ii. ☒ National accreditation.
 - iii. ☐ Enhanced licensing system.
 - iv. ☒ Meeting Head Start/Early Head Start Program Performance Standards.
 - v. ☐ Meeting pre-Kindergarten quality requirements.
 - vi. ☐ School-age standards.
 - vii. ☐ Quality framework or quality improvement system.
 - viii. ☐ Other. Describe:
- b. For what types of child care providers is quality information available?
 - i. ☒ Licensed CCDF providers. Describe the quality information: **QRIS Star Level, national accreditation, and if meeting Head Start/Early Head Start Program Performance Standards.**
 - ii. ☒ Licensed non-CCDF providers. Describe the quality information: **QRIS Star Level, national accreditation, and if meeting Head Start/Early Head Start Program Performance Standards.**
 - iii. ☒ License-exempt center-based CCDF providers. Describe the quality information: **QRIS Star Level**
 - iv. ☒ License-exempt FCC CCDF providers. Describe the quality information: **QRIS Star Level**
 - v. ☐ License-exempt non-CCDF providers. Describe the quality information:
 - vi. ☐ Relative child care providers. Describe the quality information:
 - vii. ☐ Other. Describe:

9.2.5 Aggregate data on serious injuries, deaths, and substantiated abuse

Lead Agencies must post aggregate data on serious injuries, deaths, and substantiated cases of child abuse that have occurred in child care settings each year on the consumer education website. This aggregate data must include information about any child in the care of a provider eligible to receive CCDF, not just children receiving subsidies.

This aggregate information on serious injuries and deaths must be separated by category of care (e.g., centers, family child care homes, and in-home care) and licensing status (i.e., licensed or license-exempt) for all eligible CCDF child care providers in the State/Territory. The information on instances of substantiated child abuse does not have to be organized by category of care or

licensing status. Information must also include the total number of children in care by provider type and licensing status, so that families can better understand the data presented on serious injuries, deaths, and substantiated cases of abuse.

- a. Certify by checking below that the required elements are included in the Aggregate Data Report on serious incident data that have occurred in child care settings each year.
 - i. **[x]** The total number of serious injuries of children in care by provider category and licensing status.
 - ii. **[x]** The total number of deaths of children in care by provider category and licensing status.
 - iii. **[x]** The total number of substantiated instances of child abuse in child care settings.
 - iv. **[x]** The total number of children in care by provider category and licensing status.
 - v. If any of the above elements are not included, describe: **N/A**
- b. Certify by providing:
 - i. The designated entity to which child care providers must submit reports of any serious injuries or deaths of children occurring in child care and describe how the Lead Agency obtains the aggregate data from the entity: **The Lead Agency is the designated entity.**
 - ii. The definition of “substantiated child abuse” used by the Lead Agency for this requirement: **The Lead Agency defines substantiated child abuse as it is more likely than not that high severity abuse or neglect did happen.**
 - iii. The definition of “serious injury” used by the Lead Agency for this requirement: **Serious physical injury or impairment as defined in 22 M.R.S.A. § 4002(11). Examples of serious injuries to be considered for this internal review procedure include: an injury resulting from Shaken Baby Syndrome, any injury to a child under six (6) months of age, abusive head trauma, skull fracture, inflicted head injury, subdural hematoma, multiple fractures, severe beating resulting in extensive contusions or welts, any injury resulting from Munchausen Syndrome by Proxy, drowning, non-organic failure to thrive, and other significant injuries which may have been inflicted by a person responsible for the child and /or are not consistent with the explanation offered.**
- c. Provide the direct URL/website link to the page where the aggregate number of serious injuries, deaths, and substantiated child abuse, and the total number of children in care by provider category and licensing status are posted:
<https://www.childcarechoices.me/index.php/mainedata/>

9.2.6 Contact information on referrals to local child care resource and referral organizations

The Lead Agency consumer education website must include contact information on referrals to local CCR&R organizations.

- a. Does the consumer education website include contact information on referrals to local CCR&R organizations?

☐ Yes.

☐ No.

☒ Not applicable. The Lead Agency does not have local CCR&R organizations.

b. Provide the direct URL/website link to this information:

9.2.7 Lead Agency contact information for parents

The Lead Agency consumer and provider education website must include information on how parents can contact the Lead Agency or its designee and other programs that can help the parent understand information included on the website.

a. Does the website provide directions on how parents can contact the Lead Agency or its designee and other programs to help them understand information included on the website?

☒ Yes.

☐ No.

b. Provide the direct URL/website link to this information:

<https://www.childcarechoices.me/> - Select "Subsidy Program"

<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>

9.2.8 Posting sliding fee scale, co-payment amount, and policies for waiving co-payments

The consumer education website must include the sliding fee scale for parent co-payments, including the co-payment amount a family may expect to pay and policies for waiving co-payments.

a. Does the Lead Agency certify that their consumer education website includes the sliding fee scale for parent co-payments, including the co-payment amount a family may expect to pay and policies for waiving co-payments?

☒ Yes.

☐ No.

b. Provide the direct URL/website link to the sliding fee scale.

<https://www.childcarechoices.me/> - Select "Subsidy Program"

<https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/paying-for-child-care>

9.3 Increasing Engagement and Access to Information

Lead Agencies must collect and disseminate information about the full range of child care services to promote parental choice to parents of children eligible for CCDF, the general public, and child care providers.

9.3.1 Information about CCDF availability and eligibility

Describe how the Lead Agency shares information with eligible parents, the general public, and child care providers about the availability of child care services provided through CCDF and other programs for which the family may be eligible. The description should include, at a minimum, what is provided (e.g., written materials, the website, and direct communications) and what

approaches are used to tailor information to parents, the general public, and child care providers. **The Lead Agency shares information with eligible parents, the general public, and Child Care Providers about programs and availability on the DHHS website. Links are provided to programs and services. Written materials can be requested from the Lead Agency through the website, telephone, or e-mail. Information is tailored for the audience by linking Providers to Provider specific webpages and linking programs and services tailored specifically for families, children, teens, adults, elders, or health. Help ME Grow (HMG) model as a system for improving access to existing resources and services by linking families those that they need at the community-based level statewide.**

9.3.2 Information about child care and other services available for parents

Does the Lead Agency certify that it provides information described in 9.3.1 for the following required programs?

- Temporary Assistance for Needy Families (TANF) program.
- Head Start and Early Head Start programs.
- Low Income Home Energy Assistance Program (LIHEAP)
- Supplemental Nutrition Assistance Program (SNAP).
- Women, Infants, and Children Program (WIC) program.
- Child and Adult Care Food Program (CACFP).
- Medicaid and Children's Health Insurance Program (CHIP).
- Programs carried out under IDEA Part B, Section 619 and Part C.

☒ Yes.

☐ No. If no, describe:

9.3.3 Consumer statement for parents receiving CCDF services

Lead Agencies must provide parents receiving CCDF services with a consumer statement in hard copy or electronically that contains general information about the CCDF program and specific information about the child care provider they select.

Please certify if the Lead Agency provides parents receiving CCDF services a consumer statement that contains the following 8 requirements:

1. Health and safety requirements met by the provider
2. Licensing or regulatory requirements met by the provider
3. Date the provider was last inspected
4. Any history of violations of these requirements
5. Any voluntary quality standards met by the provider
6. How CCDF subsidies are designed to promote equal access
7. How to submit a complaint through the hotline
8. How to contact a local resource and referral agency or other community-based

organization to receive assistance in finding and enrolling in quality child care

Does the Lead Agency provide to families, either in hard copy or electronically, a consumer statement that contains the required information about the provider they have selected, including the eight required elements above?

☒ Yes.

☐ No. If no, describe:

9.3.4 Informing families about best practices on child development

Describe how the Lead Agency makes information available to parents, providers, and the general public on research and best practices concerning children's development, including physical health and development, and information about successful parent and family engagement. At a minimum, the description should include what information is provided; how the information is provided; any distinct activities for sharing this information with parents, providers, the general public; and any partners in providing this information. **The Help Me Grow Maine program's website provides information and resources through links, contact information, and documents on children's development. The information provided includes health and safety topics on a variety of subjects. <https://www.maine.gov/dhhs/ocfs/support-for-families/child-development>** The 2-1-1 Maine website is also included that leads to an array of information and for a variety of audiences.

Child Care Choices also has important information for families. <https://childcarechoices.me>

9.3.5 Unlimited parental access to their children

Does the Lead Agency have procedures to ensure that parents have unlimited access to their children whenever their children are in the care of a provider who receives CCDF funds:

☒ Yes.

☐ No. If no, describe:

9.3.6 Informing families about best practices in social and emotional health

Describe how the Lead Agency shares information with families, providers, and the general public regarding the social-emotional and behavioral and mental health of young children, including positive behavioral intervention and support models based on research and best practices for those from birth to school age: **The Help Me Grow Maine program's website provides information and resources through links, contact information, and documents on children's development. The information provided includes health and safety topics on a variety of subjects. <https://www.maine.gov/dhhs/ocfs/support-for-families/child-development>**

Maine's Early Childhood Consultation Partnership (ECCP) Program is an excellent social emotional and mental health resource for families. <https://www.maine.gov/dhhs/ocfs/support-for-families/child-care/early-childhood-consultation-partnership>

The 2-1-1 Maine website is also included that leads to an array of information and for a variety of audiences.

Child Care Choices also has important information for families. <https://childcarechoices.me>

9.3.7 Policies on the prevention of the suspension and expulsion of children

- a. The Lead Agency must have policies to prevent the suspension and expulsion of children from birth to age 5 in child care and other early childhood programs receiving CCDF funds. Describe those policies and how those policies are shared with families, providers, and the general public: **The Lead Agency's policy is to provide information on suspension and expulsion through Help ME Grow, ECCP and MRTQ PDN. The Lead Agency provides the link and contact information to ECCP and MRTQ PDN on both website pages for parents and providers. The CCAP Parent Agreement provides links to developmental screenings. The CCAP Provider Agreement provides a link to MRTQ PDN's Inclusion Warm Line. Lead Agency staff provide the link and contact information for ECCP and MRTQ PDN when an individual inquires, including a parent reaching out in response to a denial of services.**

The Lead Agency's ECCP program is a free statewide infant and early childhood mental health consultation to early care programs in Maine. The MRTQ PDN District Coordinators have been trained in Relationship Based Consultation, Communities of Practice facilitation, Adult Learning Practices, Coaching and Mentoring. They have also been trained on the Rising Stars for ME, Maine Quality Rating and Improvement System. All trainings and TA incorporate the knowledge and application of its early learning and developmental guidelines (where applicable); its health and safety standards and social-emotional/behavioral and early childhood mental health intervention models, which can include positive behavior intervention and support models in the training and professional development framework and are ongoing. The information is shared with families, providers, and the general public in a variety of ways that include through the services of Help Me Grow, the Lead Agency's website, and through the many partnerships' media outlets.

- b. Describe what policies, if any, the Lead Agency has to prevent the suspension and expulsion of school-age children from child or youth care settings receiving CCDF funds: **The Lead Agency's policy is to provide information on suspension and expulsion through Help ME Grow, ECCP and MRTQ PDN. The Lead Agency provides the link and contact information to ECCP and MRTQ PDN on both website pages for parents and providers. The CCAP Parent Agreement provides links to developmental screenings. The CCAP Provider Agreement provides a link to MRTQ PDN's Inclusion Warm Line. CCAP staff provide the link and contact information for ECCP and MRTQ PDN when an individual inquires.**

9.4 Providing Information on Developmental Screenings

Lead Agencies must provide information on developmental screenings to parents as part of the intake process for families participating in CCDF and to child care providers through training and education. This information must include:

- Existing resources and services that the State can make available in conducting developmental screenings and providing referrals to services when appropriate for children who receive child care assistance, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program under the Medicaid program carried out under Title XIX of the Social Security Act and developmental screening services available under IDEA

Part B, Section 619 and Part C; and,

- A description of how a family or child care provider can use these resources and services to obtain developmental screenings for children who receive subsidies and who might be at risk of cognitive or other developmental delays, which can include social, emotional, physical, or linguistic delays.

Information on developmental screenings, as in other consumer education information, must be accessible for individuals with limited English proficiency and individuals with disabilities.

9.4.1 Developmental screenings

Does the Lead Agency collect and disseminate information on the following:

- a. Existing resources and services available for obtaining developmental screening for parents receiving CCDF, the general public, and child care providers.

☒ Yes.

☐ No. If no, describe:

- b. Early and Periodic Screening, Diagnosis, and Treatment program under the Medicaid program—carried out under Title XIX of the Social Security Act (42 U.S.C. 1396 et seq.)—and developmental screening services available under Part B, Section 619 and Part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.).

☒ Yes.

☐ No. If no, describe:

- c. Developmental screenings to parents receiving a subsidy as part of the intake process.

☒ Yes. If yes, include the information provided, ways it is provided, and any partners in this work: **The Lead Agency's CCAP application process provides the following information, If you would like information on developmental screenings, please go to the following link:**

<https://www.maine.gov/dhhs/ocfs/support-for-families/child-development/developmental-screening>

The CCAP staff have the links and resources to give out, and it is in the Staff Procedure Manual. Links are also included in the Lead Agency's Consumer Education webpage on Child Care Choices.

☐ No. If no, describe:

- d. How families receiving CCDF services or child care providers receiving CCDF can use the available resources and services to obtain developmental screenings for children at risk for cognitive or other developmental delays.

☒ Yes.

☐ No. If no, describe:

10 Program Integrity and Accountability

Program integrity and accountability activities are integral to the effective administration of the CCDF program. As stewards of federal funds, Lead Agencies must ensure strong and effective internal controls to prevent fraud and maintain continuity of services to meet the needs of children and families. In order to operate and maintain a strong CCDF program, regular evaluation of the program's internal controls as well as comprehensive training for all entities involved in the administration of the program are imperative. In this section, Lead Agencies will describe their internal controls and how those internal controls effectively ensure integrity and accountability. These accountability measures should address reducing fraud, waste, and abuse, including program violations and administrative errors and should apply to all CCDF funds.

10.1 Effective Internal Controls

Lead Agencies must ensure the integrity of the use of CCDF funds through effective fiscal management and must ensure that financial practices are in place. Lead Agencies must have effective fiscal management practices in place for all CCDF expenditures.

10.1.1 Organizational structure to support integrity and internal controls

Describe how the Lead Agency's organizational structure ensures the oversight and implementation of effective internal controls that promote and support program integrity and accountability. Describe: **The organizational structure implemented ensures the oversight and implementation of effective internal controls that promote and support program integrity and accountability through separation of duties, transparency, oversight and auditing.**

The organization is separated into four primary areas

- Executive Management
- Program Management
- Financial Management
- Quality Assurance and Auditing

Executive Management

The highest organizational leadership level of the decision makers, this group is responsible for the oversight and setting strategic priorities for the program, planning for the service array, administering resources and budget, and final approval of the strategic plan for the array of services being offered by the Early Childhood and Education (ECE) division.

Program Management

Responsible for all programmatic activities within the ECE Division. Program management staff oversee the efficient and effective implementation of the program and has direct oversight of the policies and practice supporting the program's strategic plan. This group manages the direct and indirect activities and contracts within the program to ensure services are meeting the expectations of the strategic plan.

Financial Management

Includes the Program Financial Officer (PFO) and Maine Department of Administrative and Financial Service (DAFS) which are separate and independent entities and not part of the OCFS Program Management team. As separate entities for budgeting, financial oversight, and financial reporting, they are able to provide transparency for program fiscal activities. The Lead Agency has developed procedures such as the State Accounting Manual and a CCDF Grant Manual for Service

Center management and reporting.

Quality Assurance and Auditing

The Quality Assurance Unit and Data Management Unit are supervised by the OCFS Associate Director of Technology and Support allowing them to provide impartial review of the CCDF program. This also allows QA to provide clear and accurate feedback as they are specially trained in providing the services. The Program undergoes a separate independent annual audit conducted by the State of Maine Department of Audit.

Coordination and Communication

The Executive Management Team of the Office of Child and Family Services meets weekly and discusses any areas of need regarding the Child Care Affordability Program, CCDF funding, and CCDF activities. This meeting is a key time of coordination, as it includes representatives from executive management, program management, financial management and quality assurance. Quality Assurance staff coordinate with CCDF Administrators and program staff regularly through email and information connection and at monthly meetings. In addition, there are quarterly meetings led by the Office's grants coordinator and chief operations officer, where program and financial staff review expenses, grant balances, audits, CCDF activities, and like items.

Include the following elements in your description:

1. Assignment of authority and responsibilities related to program integrity.
2. Delegation of duties.
3. Coordination of activities.
4. Communication between fiscal and program staff.
5. Segregation of duties.
6. Establishment of checks and balances to identify potential fraud risks.
7. Other activities that support program integrity.

10.1.2 Fiscal management practices

Describe how the Lead Agency ensures effective fiscal management practices for all CCDF expenditures, including:

- a. Fiscal oversight of CCDF funds, including grants and contracts. Describe: **The Lead Agency's PFO oversees program fiscal oversight of grant funds to ensure reasonable and allowable costs. Funds are tracked through Weekly Budget Variance Reports that enable Program Administrators to monitor revenue, expenditures, and grant balance this includes CCAP, contract, and administrative cost spending.**
- b. Tracking systems that ensure reasonable and allowable costs and allow for tracing of funds to a level of expenditure adequate to establish that such funds have not been used in violation of the provision of this part. Describe: **The administration and tracking of funds are made through two separate systems. The State of Maine's financial accounting system (Advantage) is administered through the Maine Department of Administrative and Financial Service (DAFS). DAFS manages state budgeting and payment processes. Advantage is able to provide full accounting of all financial transactions made to any provider. This includes the management of all contract encumbrances and payments.**

OCFS stores all client level detail of payments made through advantage in its Child Care Management System known as MACWIS. The Department of Audit can trace expenditures from submission, through approval, through final payment across both systems.

- c. Processes and procedures to prepare and submit required state and federal fiscal reporting. Describe: **Reports and information are reviewed by the program team, data team, and the Program Financial Officer, and then provided to the Department of Administrative and Financial Services (DAFS) Service Center. The Service Center reviews the data and reporting for accuracy and generates and transmits required state and federal financial reports.**

Department of Administrative and Financial Services prepares and submits quarterly financial reports on behalf of The Office of Child and Family Services. CCDF reports are subject to a multi-level review. The accountant who manages the grant in the Service Center prepares the report, their manager/managing staff accountant, reviews the accuracy of the report, and the associate director of financial reporting provides a second review. After all reviews have been completed, they are then certified by the Service Center director in the OLDC system.

- d. Other. Describe: **N/A**

10.1.3 Effectiveness of fiscal management practices

Describe how the Lead Agency knows there are effective fiscal management practices in place for all CCDF expenditures, including:

- a. How the Lead Agency defines effective fiscal management practices. Describe: **Effective fiscal management starts with a sound program strategic plan that defines and creates the budget. The budget is managed through cash flows that are monitored by the Program Financial Officer and the Department of Administrative and Financial Services. The PFO with support from the Data team, reviews cash flows to generate budget forecast, allowing OCFS to analyze the programs services and expenditures to ensure they align with the program priorities. Accounts are managed using the State of Maine's internal control structure which provides separation of duties, approval, and oversight within the State's Budget and Financial Management System. All of this is completed under the oversight of both DHHS Executive Management and internal State Audit to ensure transparency and accountability and that resources are used efficiently, ethically, and in alignment with the budget priorities and goals.**

The Lead Agency's PFO oversees program fiscal oversight of grant funds to ensure reasonable and allowable costs. Funds are tracked through Weekly Budget Variance Reports that enable Program Administrators to monitor revenue, expenditures, and grant balance, this includes CCAP, contract, and administrative cost spending.

- b. How the Lead Agency measures and tracks results of their fiscal management practices. Describe: **Effective fiscal management starts with a sound program strategic plan that defines and creates the budget. The budget is managed through cash flows that are monitored by the Program Financial Officer and the Department of Administrative and**

Financial Services. Services defined in the plan are monitored by the program leads, the PFO and OCFS Contract Management unit. Monitoring occurs through expense and contract management practices to ensure the outcomes established in the contract or agreement are met.

- c. How the results inform implementation. Describe: **The PFO works with the program team to develop the budget and forecasted expenditures. This information guides annual financial reporting used by DHHS management in support of its budget strategy. OCFS creates annual and biannual budgets required to support planned expenses and contracting needs. These budgets are reviewed and approved by executive management prior to being sent forward for approval. Funding allotment and allocations requests are approved by the legislature based on the program's expense plan and supporting budget information. We know that our fiscal management practices are effective as they all start with a program strategic plan that defines and creates the budget and the budget is managed through cash flows that are monitored by the Program Financial Officer and the Department of Administrative and Financial Services.**

Program Administrators and management regularly meet with the financial team for updates and any discrepancies. Program also takes results from the QA Team's reviews and state audit findings to make necessary corrective actions.

- d. Other. Describe: **N/A**

10.1.4 Identifying risk

Describe the processes the Lead Agency uses to identify risk in the CCDF program including:

- a. Each process used by the Lead Agency to identify risk (including entities responsible for implementing each process). Describe: **OCFS and Maine Office of Information Technology have a risk assessment protocols and OCFS has a Business Continuity Plan and a Continuity of Operations Plan. MACWIS as part of the CCWIS structure falls under the plan.**

Additionally, the OCFS establishes checks and balances to ensure program integrity through the PFO's oversight of expenditures of CCDF funds, supervisory approval on expenditures at departmental level, and through auditing measures taken by the State. Program works in coordination with the QA Team to ensure accuracy in eligibility determination. Coordination with the QA team includes;

- Reviewing 23 files a month, using the approved federal tool.
- Reviewing MACWIS financial information used by the Financial Resource Specialists (FRS) to make determinations
- Reviewing documents stored in Docuware (document repository) related to the families and childcare providers used to determine family eligibility. Based on the financial documents used by the child care affordability program specialists, QA uses those same documents to calculate and confirm that a family met the eligibility requirements. QA is required to use the federally approved Record Review Worksheet to document if eligibility met and, if not, clearly document what the issues were using the federally approved language. QA reaches out with the Child Care Subsidy Team when there are questions related to not finding documentation in Docuware or understanding what documents were used.

- b. The frequency of each risk assessment. Describe: **The Lead Agency meets regularly and has quarterly meetings with the PFO and members of the financial team. The QA Team meets monthly.**
- c. How the Lead Agency uses risk assessment results to inform program improvement. Describe: **The Lead Agency takes the results from multiple sources for overall risk assessments and utilizes those sources to inform program improvement. The Lead Agency establishes checks and balances to ensure program integrity through the PFO's oversight of expenditures of CCDF funds, supervisory approval on expenditures at departmental level, and through auditing measures taken by the State. Program works in coordination with the QA Team to ensure accuracy in eligibility determination. Coordination with the QA team includes;**
 - Reviewing 23 files a month, using the approved federal tool.
 - Reviewing MACWIS financial information used by the Financial Resource Specialists (FRS) to make determinations
 - Reviewing documents stored in Docuware (document repository) related to the families and childcare providers used to determine family eligibility. Based on the financial documents used by the child care affordability program specialists, QA uses those same documents to calculate and confirm that a family met the eligibility requirements. QA is required to use the federally approved Record Review Worksheet to document if eligibility met and, if not, clearly document what the issues were using the federally approved language.

QA reaches out to the Child Care Subsidy Team when there are questions related to not finding documentation in Docuware or understanding what documents were used.

The risk assessment results help inform program structure, such as staffing levels and division of responsibilities. In addition, the results help inform staff training and professional development.

- d. How the Lead Agency knows that the risk assessment processes utilized are effective. Describe: **The Lead Agency knows the assessment processes are utilized effectively based on measurable goals for either increasing or decreasing variables being measured. For instance, monthly results from the QA team would see a decrease in improper payments of the same category in future months.**
- e. Other. Describe: **N/A**

10.1.5 Processes to train about CCDF requirements and program integrity

Describe the processes the Lead Agency uses to train staff of the Lead Agency and other agencies engaged in the administration of CCDF, and child care providers about program requirements and integrity.

- a. Describe how the Lead Agency ensures that all staff who administer the CCDF program (including through MOUs, grants, and contracts) are informed and trained regarding program requirements and integrity.
 - i. Describe the training provided to staff members around CCDF program requirements and program integrity: **The CCDF Administrators, staff at Maine Roads to Quality Professional Development Network, Child Care Licensing staff,**

and Child Care Affordability Program staff have regular meetings focused on the CCDF Program and CCDF funded initiatives that are being administered by the Lead Agency to discuss CCDF information, program requirements, checks and balances in program administration, and updates. New CCDF Administrators and new Child Care Affordability Program staff have more detailed CCDF onboarding meetings and training in CCDF implementation and integrity. These trainings include a manual for their CCDF or CCAP related role that details program implementation and compliance.

- ii. Describe how staff training is evaluated for effectiveness: **The training in a specific area of program implementation is determined to be effective if the monthly results from the QA team show a decrease in improper payments in that same area in future months.**
 - iii. Describe how the Lead Agency uses program integrity data (e.g., error rate results, risk assessment data) to inform ongoing staff training needs: **The error rate results, audit findings, and risk assessment data from the QA team inform staff training, by highlighting areas of challenges or confusion, where specific training can build coordination and increase accuracy. The CCDF Administrators review and evaluate findings, determine proper actions, and take corrective actions within established timeframes. The results are discussed in program manager meetings in the Early Care and Education Division to ensure that staff across the division is trained to support CCDF program integrity.**
- b. Describe how the Lead Agency ensures all providers for children receiving CCDF funds are informed and trained regarding CCDF program requirements and program integrity:
- i. Describe the training for providers around CCDF program requirements and program integrity: **The Lead Agency provides every new Child Care Affordability Program Provider with a provider packet that includes, CCAP Rules, Provider Agreements by Provider type, QRIS information, billing dates, and billing instructions and a short Program Integrity Training video, which is also available and posted on the website. The training video includes an overview of program requirements, such as billing and reimbursement and an overview of the most common program integrity issues, such as parents not providing two-week notice to their provider ending child care enrollment.**
 - ii. Describe how provider training is evaluated for effectiveness: **The Lead Agency monitors errors by category and type. Reminders and/or TA is provider either across all providers or individually based on the monitoring.**
 - iii. Describe how the Lead Agency uses program integrity data (e.g., error rate results, risk assessment data) to inform ongoing provider training needs: **The Lead Agency monitors errors by category and type. Reminders and/or TA is provider either across all providers or individually based on the monitoring. The training video is updated annually to add new information and correct outdated information. The Program Integrity Training video shares the most common program integrity issues with providers that lead to recoupment of funds and/or program disqualification, including misrepresentation of household size, nonreporting of job loss, and provider not collecting correct weekly parent fees.**

10.1.6 Evaluate internal control activities

Describe how the Lead Agency uses the following to regularly evaluate the effectiveness of Lead Agency internal control activities for all CCDF expenditures.

- a. Error rate review triennial report results (if applicable). Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **The Lead Agency shares the results of the report internally with, Leadership, CCAP staff, AQ Team, and across departments with the Department of Administrative and Financial Services and the PFO to determine necessary policy and procedures changes needed to meet requirement or funding for repayments. The report is also shared with the State Auditor for guiding findings from a state audit.**
- b. Audit results. Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **Audits inform changes to program, policy and overall budget strategy. The CCDF budget is managed through cash flows that are monitored by the Program Financial Officer and the Department of Administrative and Financial Services. Services defined in the plan are monitored by the program leads, the PFO and OCFS Contract Management unit. Monitoring occurs through expense and contract management practices to ensure the outcomes established in the contract or agreement are met. The Lead Agency shares the audit results internally with Executive Management, CCDF Administrators, the QA Team, the Department of Administrative and Financial Services and the Program Financial Office (PFO) to determine necessary program, policy, or budget strategy changes.**
- c. Other. Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **N/A**

10.1.7 Identified weaknesses in internal controls

Has the Lead Agency or other entity identified any weaknesses in its internal controls?

- a. ☐ No. If no, describe when and how it was most recently determined that there were no weaknesses in the Lead Agency's internal controls.
- b. ☒ Yes. If yes, what were the indicators? How did you use the information to strengthen your internal controls? **The Lead Agency had a state audit completed for SFY 2023. The findings resulted in a significant deficiency regarding payment practices. The results require the Lead Agency to establish and maintain effective internal control over Federal awards that provides reasonable assurance that the Department is managing awards in compliance with Federal statutes, regulations, and the terms and conditions of awards.**

10.2 Fraud Investigation, Payment Recovery, and Sanctions

Lead Agencies must have the necessary controls to identify fraud and other program violations to ensure program integrity. Program violations can include both intentional and unintentional client and/or provider violations, as defined by the Lead Agency. These violations and errors, identified through the error-rate review process and other review processes, may result in payment or nonpayment (administrative) errors and may or may not be the result of fraud, based on the Lead Agency definition.

10.2.1 Strategies used to identify and prevent program violations

Check the activities the Lead Agency employs to ensure program integrity, and for each checked activity, identify what type of program violations the activity addresses, describe the activity and the results of these activities based on the most recent analysis.

- a. **[x]** Share/match data from other programs (e.g., TANF program, Child and Adult Care Food Program, Food and Nutrition Service (FNS), Medicaid) or other databases (e.g., State Directory of New Hires, Social Security Administration, Public Assistance Reporting Information System (PARIS)).

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Utilizing shared data from the Office of Family Independence (OFI) and Division of Support Enforcement and Recovery (DSER), the Lead Agency has identified application discrepancies between public assistance programs. Parents withdraw the application when they are unable to correct eligibility information. Programs that OCFS shares data with, such as TANF, will proceed with Fraud/Program violation if necessary.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. We pull data from our state audit process to help us identify overarching program needs while we continue to find efficient ways to monitor program-wide data. For FFY 24, a state audit identified 0 intentional program violations. We do not, at this time, track how many program violations were prevented by the above described activities of cross-checking program application information with other public assistance applications.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **CCAP recovers monies in the amount \$10.00 and over. Staff training on use and recovery process of share match is updated within the CCAP standard operating procedures.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. Of the state auditing data captured in FFY 2024, 9 unintentional program violations were identified and addressed. Of those 9, most had to do with missing or incorrect documentation. Our program specialists address and correct this as soon as the information is discovered.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Utilizing shared data from OFI and DSER, the Lead Agency has been able to identify discrepancies between family data in the two programs and correct agency errors. CCAP recovers monies over the amount of \$10.00 per agency error. Staff training is utilized when agency errors occur. Staff receive updates and training through information in emails, at staff meetings,**

and/or in updated standard operating procedures.

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. State auditing data showed 50 agency errors were identified in FFY 24. We will continue to track this information. In each of these cases, we identified the origin of the error, corrected it directly, determined any relevant improper payment associated with it, and trained staff on how to prevent any future errors. State Auditing agency errors in FFY 2024 broke down into these topic areas: 9 missed documentation related to application and redetermination, 2 missed documentation of residency, 6 related to improper documentation of qualifying care, 15 related to improper documentation of financial requirements, and 18 related in improper calculation of award amounts/parent co-pays. These results have helped the management team shape both individual supervision and professional development work as well as group topics for common issues and errors across the team.

- b. ☒ Run system reports that flag errors (include types).
 - i. ☐ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice:
 - ii. ☐ Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice:
 - iii. ☒ Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Lead Agency runs a report to flag any cases that do not have a parent fee assigned with a \$0 value or higher. Lead Agency Runs a report if a parent fee exceeds Child Care Provider Rate.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. In FFY 24, all errors were tracked with their appropriate element number and notes on the cause of the error. From data pulled in state auditing processes, there were 50 agency errors total and broke down into these topic areas: 9 missed documentation related to application and redetermination, 2 missed documentation of residency, 6 related to improper documentation of qualifying care, 15 related to improper documentation of financial requirements, and 18 related in improper calculation of award amounts/parent co-pays. These results have helped the management team shape both individual supervision and professional development work as well as group topics for common issues and errors across the team.

- c. ☒ Review enrollment documents and attendance or billing records.
 - i. ☒ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency requires CCAP**

providers to provide attendance records for audit upon request.

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. In FFY 24, state auditing data showed no intentional program violations were identified. We do take preventative measures to address program violations, including utilizing shared data from OFI/DSER. If the Lead Agency has identified application discrepancies between CCAP applications with applications for other public assistance programs, we would research the situation, identify errors or discrepancies and work to correct them. Parents withdraw the application when they are unable to correct eligibility information. Programs that OCFS shares data with, such as TANF, will proceed with Fraud/Program violation if necessary.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency requires CCAP providers to provide attendance records for audit upon request.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. In FFY 2024, state auditing data showed 9 unintentional program violations were identified and addressed. Of those 9, most had to do with missing or incorrect documentation. Our program specialists address and correct this as soon as the information is discovered.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency requires CCAP providers to provide attendance records for audit upon request.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. In FFY 24, state auditing data highlighted 9 administrative errors related to enrollment documents, attendance or billing records. Those were tracked with notes on the origin of the issue and reviewed to address the specific issues with both individual eligibility specialists as well as inform management plans for team-wide professional development and education.

- d. **[x]** Conduct supervisory staff reviews or quality assurance reviews.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency's Quality Assurance Team audits a monthly random sample of cases. From the finding the Lead Agency conducts trainings and update the SOP.**

CCAP is in the process of coding all the over/underpayments, intentional, and

nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. As discussed above, for FFY 24, 0 intentional program violations were identified in our state auditing process. Our supervisory staff have and continue to identify opportunities for individual and group training on how to prevent intentional program violations.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency's Quality Assurance Team audits a monthly random sample of cases. From the finding the Lead Agency conducts trainings and update the SOP.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. For FFY 24 state auditing data, 9 unintentional program violations were identified. Our supervisory staff have and continue to identify opportunities for individual and group training on how to prevent unintentional program violations.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency's Quality Assurance Team audits a monthly random sample of cases. From the finding the Lead Agency conducts trainings and update the SOP.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. State Auditing data in FFY 24, 50 agency errors were identified and supervisory staff have addressed each with individual eligibility specialists as well as utilized this information to create preventative staff training.

- e. **[x]** Audit provider records.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency audits random provider records to look at intentional program violations and accuracy in the eligibility and award process.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. State auditing data For FFY 24, 0 intentional program violations were found.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency audits random**

provider records to look at unintentional program violations and accuracy in the eligibility and award process.

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. In state auditing data for FFY 24, 9 unintentional program violations were found.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency audits random provider records to look at agency accuracy.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. In state auditing data for FFY 24, 50 agency errors were identified.

- f. **[x]** Train staff on policy and/or audits.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency has a SOP for all child care affordability program staff and quality assurance team members. Training is also completed with information shared by email and at staff meetings. Training around intentional program violations helps staff members identify this violation correctly and consistently, understanding the most common types of intentional program violations.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. With information and data from state auditing in FFY 24, we are focused on training for continued prevention of intentional program violations.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency has a SOP for all child care affordability program staff and quality assurance team members. Training is also completed with information shared by email and at staff meetings. Training around unintentional program violations helps staff members identify this violation correctly and consistently, understanding the most common type of unintentional program violations.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. With the information from state auditing in FFY 24, staff supervisors are

working to update SOPs and training opportunities to support prevention of future unintentional program violations.

- iii. ☒ Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **The Lead Agency has a SOP for all child care affordability program staff and quality assurance team members. Training is also completed with information shared by email and at staff meetings. Training is during onboarding and regularly with program staff to share knowledge of the most common agency errors and to improve coordination and program implementation and integrity to reduce those errors.**

CCAP is in the process of coding all the over/underpayments, intentional, and nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs. With the information from state audit findings in FFY 24, all 50 agency errors have been noted, addressed directly with individual eligibility specialists and topics have been integrated into ongoing staff training opportunities.

- g. ☐ Other. Describe the activity(ies):
 - i. ☐ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice:
 - ii. ☐ Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice:
 - iii. ☐ Agency errors. Describe the activities, the results of these activities, and how they inform better practice:

10.2.2 Identification and recovery of misspent funds

Lead Agencies must identify and recover misspent funds that are a result of fraud, and they have the option to recover any misspent funds that are a result of unintentional program violations or agency errors.

- a. Identify which agency is responsible for pursuing fraud and overpayments (e.g., State Office of the Inspector General, State Attorney): **DHHS Fraud Investigation Unit is responsible for recovery of Fraud and CCAP is responsible for recovery of overpayments.**
- b. Check and describe all activities, including the results of such activity, that the Lead Agency uses to investigate and recover improper payments due to fraud. Consider in your response potential fraud committed by providers, clients, staff, vendors, and contractors. Include in the description how each activity assists in the investigation and recovery of improper payment due to fraud or intentional program violations. Activities can include, but are not limited to, the following:
 - i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency recovers payments over the amount of \$10. In the new 2024 Rules, the Child Care Affordability Program (CCAP) clarified language to show that CCAP collects on administrative errors as well as provider and parent errors. CCAP now has one**

staff member to solely focus on Program Integrity and Recovery. A CCAP Specialist reviews a report for any errors in payments, age changes, et cetera. If the specialist suspects fraud, the case is passed on to the specialist handling program integrity who will take a deep dive into the case. The Program and Integrity specialist sets up and collects on all payment plans. By looking for administrative errors over \$10, this activity assists the program in developing individual or group trainings and in resolving issues. Training may consist of 1 on 1 in supervisions, staff trainings, or emails. When nonintentional program violation occurs by provider error, CCAP has a Specialist available to meet with child care programs regarding billing questions/issues. The meetings can be in person or online. CCAP is in the process of coding all the over/underpayments, intentional, nonintentional program violations to align with the element numbers and reasons we will be using in our ACF 404 report. This report will help CCAP track any errors caught and fixed prior to any audit issues to help with identification of training needs.

- ii. **[x]** Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency coordinates with the Fraud Investigation Unit. Fraud looks to see if the provider or parent utilize other benefits services such as MaineCare or SNAP. The D.A. in the geographical location decides to peruse cases brought forth to them by the fraud investigation unit. In 2023, the Child Care Affordability Program (CCAP) sent 23 cases to fraud. None of the cases sent in 2023 have been prosecuted through the Fraud Investigation Unit to this date.**
- iii. **[x]** Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will initiate recruitment of payments and then if need refer to DHHS Fraud Investigation Unit. The Child Care Affordability Program (CCAP) created a standard operating procedure (SOP) for the Program Integrity and Recovery Process as an appendix to the SOP guide for all CCAP Specialist and an email address for Program Integrity issues. CCAP sends out receipts after each payment is made with a prepaid envelope for the next payment to be sent in. By including the prepaid envelope, CCAP found payments to be more consistent. If CCAP does not receive a payment, a 2nd notice will be sent. If the repayment plan is for a child care provider, CCAP will note, "online billing hold, program please contact CCAP." If the parent or provider defaults from the payment arrangements, they will be removed from the program. The SOP updates, letters, and faster communication with clients has streamlined the payment plan process.**
- iv. **[x]** Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis: **The lead agency will reduce payments through vendor ledger adjustments. The Child Care Affordability Program (CCAP) found vendor ledger adjustments in our current MACWIS system (child care management system) were harder to track and payments by check and money order have been preferred. If a child ended care, changed child care providers, or had a redetermination, the CCAP staff must remember to add back in the vendor adjustments. This resulted in a new vendor adjustment system built**

into Baxter, the new child care management system being implemented in 2025. The new vendor adjustments are tied to the specific provider file and not the child's file.

- v. ☒ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis: **The Leads Agency will refer case to the DHHS Fraud Investigation Unit pursuant to Title 22 M.R.S 13.**
- vi. ☒ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will accept payment by check and money order.**
- vii. ☒ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis: **When the Department makes a preliminary determination that a parent or child care provider may have committed a program violation, the case may be referred to the DHHS Fraud Investigation Unit pursuant to Title 22 M.R.S. §13 and the Department may pursue establishment of a program violation against the parent and/or child care provider administratively. The Unit is comprised of the Deputy Director Fraud Investigation and Recovery Unit and Fraud Investigators under said department. A final determination that a program violation was made shall be made only as the result of a decision made by an Administrative Hearing, a court, or waiver of the Administrative Hearing by the parent and/or child care provider. Failure to request an Administrative Hearing constitutes a waiver. Child care providers who previously had agreements with the Department and who were found to be engaged in fraud or a program violation in connection with the Child Care Subsidy program or have been sanctioned are not eligible to receive payments on behalf of parents receiving Subsidy. Improper payments are managed within CCAP and letters are sent requesting payment. Checks are processed within the CCAP unit or vendor adjustments are made in accordance with payment plans.**
- viii. ☐ Other. Describe the activities and the results of these activities:

c. Does the Lead Agency investigate and recover improper payments due to unintentional program violations?

☐ No.

☒ Yes.

If yes, check and describe below any activities that the Lead Agency will use to investigate and recover improper payments due to unintentional program violations. Include in the description how each activity assists in the investigation and recovery of improper payments due to unintentional program violations. Include a description of the results of such activity.

- i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency recovers a minimum dollar amount of an improper payment and identify the minimum dollar amount as over \$10.**

- ii. ☐ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis:
 - iii. ☒ Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will initiate recoupment of payments and then if needed, refer the case to the DHHS Fraud Investigation Unit. The Lead Agency has recouped \$615.15 for Provider nonintentional improper payments.**
 - iv. ☒ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will initiate recoupment of payments and then if needed, refer the case to the DHHS Fraud Investigation Unit. The lead agency has the option to create vendor with holdings for future months.**
 - v. ☒ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will refer cases to the DHHS Fraud Investigation Unit pursuant to Title 22 M.R.S. §13. The Lead Agency does not have access as of yet to the results of providers with sanctions of tax intercepts.**
 - vi. ☒ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will accept payments by check.**
 - vii. ☐ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis:
 - viii. ☐ Other. Describe the activities and the results of these activities:
- d. Does the Lead Agency investigate and recover improper payments due to agency errors?
- ☐ No.
- ☒ Yes.
- If yes, check and describe all activities that the Lead Agency will use to investigate and recover improper payments due to agency errors. Include in the description how each activity assists in the investigation and recovery of improper payments due to administrative errors. Include a description of the results of such activity.
- i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency recovers a minimum dollar amount of an improper payment and identify the minimum dollar amount as over \$10.**
 - ii. ☒ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency refers cases to the DHHS Fraud Investigation Unit if necessary.**
 - iii. ☒ Recover through repayment plans. Describe the activities and the results of

these activities based on the most recent analysis: **The Lead Agency will initiate recoupment of payments. In FY 23 the Lead Agency recovered \$1,509.32. In Agency error.**

- iv. ☒ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis: **The Lead Agency will initiate recoupment of payments. The Lead Agency has the option of creating a vendor with holding for future payments. will initiate recoupment of payments and then if needed, refer the case to the DHHS Fraud Investigation Unit.**
- v. ☐ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis:
- vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
- vii. ☐ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis:
- viii. ☐ Other. Describe the activities and the results of these activities:
- e. What type of sanction will the Lead Agency place on clients and providers to help reduce improper payments due to intentional program violations or fraud? Check and describe all that apply:
 - i. ☒ Disqualify the client. Describe this process, including a description of the appeal process for clients who are disqualified. Describe the activities and the results of these activities based on the most recent analysis: **The Client has ten (10) days to appeal the decision. If there was an intentional program violation by the Client, the Lead Agency shall impose a disqualification penalty for up to twelve (12) months. For the last quarter of SFY24, the Lead Agency had 1 identified CCAP parent requiring reimbursement to the provider prior to being able to submit a new application. The parents sent the recoupment of payment to the provider and submitted a new CCAP application.**
 - ii. ☒ Disqualify the provider. Describe this process, including a description of the appeal process for providers who are disqualified. Describe the activities and the results of these activities based on the most recent analysis: **The Provider has ten (10) days to appeal the decision. If there was an intentional program violation by the Provider, the Lead Agency shall impose a disqualification penalty for up to twelve (12) months.**
 - iii. ☒ Prosecute criminally. Describe the activities and the results of these activities based on the most recent analysis: **DHHS Fraud Investigation Unit works with law enforcement when a case has been referred to fraud. For FFY 23, there were no criminal charges.**
 - iv. ☐ Other. Describe the activities and the results of these activities based on the most recent analysis:

Appendix 1: Lead Agency Implementation Plan

The Appendix will be available for Lead Agencies to use in CARS after the Plan approval letter is issued.

For each non-compliance, Lead Agencies must describe the following:

- **Action Steps:** List the action steps needed to correct the finding (e.g., update policy manual, legislative approval, IT system changes, etc.). For each action step list the:
 - ***Responsible Entity:*** Indicate the entity (e.g., agency, team, etc.) responsible for completing the action step.
 - ***Expected Completion Date:*** List the expected completion date for the action step.
- **Overall Target Date for Compliance:** List date Lead Agency anticipates completing implementation, achieving full compliance with all aspects of the findings. (Note: Compliance will not be determined until the FFY 2025-2027 CCDF Plan is amended and approved).

Appendix 1: Form

[Plan question with non-compliance and associated provision will pre-populate based on preliminary notice of non-compliance]

A. Action Steps for Implementation	B. Responsible Entity(ies)	C. Expected Completion Date
Step 1:		
Step 2 (as necessary):		
[Additional steps added as necessary]		
Overall Target Date for Compliance:		